



PREVENTABLE

PRagmatic EValuation of evENTs And Benefits of Lipid-lowering in oldEr adults

Manual of Procedures V6.0

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CHAPTER 1. INTRODUCTION

The PREVENTABLE Manual of Procedures (MOP) is the operational plan to ensure the protocol is consistently implemented across sites. All study staff involved with PREVENTABLE should be familiar with the contents of this MOP prior to carrying out any study procedures.

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

1.1 STUDY OVERVIEW

PREVENTABLE (PRagmatic EVALuation of evENTs And Benefits of Lipid-lowering in oldEr adults) is a large study conducted in community-dwelling older adults without clinically evident cardiovascular disease or dementia at baseline. The primary objective of this study is to generate information about the role of statins in older adults to determine the risks and benefits for primary prevention of dementia, disability, or heart disease. The hypothesis is that this study will demonstrate the benefit of statins for reducing the primary composite outcome of death, dementia, and persistent disability and secondary composites including mild cognitive impairment and dementia and cardiovascular events.

1.2 STUDY ORGANIZATION

The National Institute on Aging (NIA), the Duke Clinical Research Institute (DCRI) Clinical Coordinating Center (CCC), and the Wake Forest University School of Medicine Data Coordinating Center (DCC) are working together to address current knowledge gaps about the role of statins in older adults. Other collaborators and sponsors in the design and conduct of PREVENTABLE include, but are not limited to:

- National Heart, Lung, and Blood Institute (NHLBI)
- National Patient-Centered Clinical Research Network (PCORnet)
- Veterans Affairs National Network Coordinating Center (VA CC)
- VA Cooperative Studies Program (CSP) Pharmacy Coordinating Center
- Mayo Clinic Biorepository
- Icon Laboratories
- Vanderbilt University

1.3 CENTRAL OPERATIONAL ROLES AND RESPONSIBILITIES

1.3.1 DUKE CLINICAL RESEARCH INSTITUTE CLINICAL COORDINATING CENTER

The Duke Clinical Research Institute (DCRI) serves as the CCC. The role of the CCC is to provide the study team with overall project management, site management for the non-VA sites, and administrative support. Duke selects, collaborates with, and monitors the non-VA clinical sites to carry out the protocol. The DCRI also provides guidance and oversight to the VA site management team. Additionally, the DCRI manages a centralized tele-site to allow remote enrollment and follow up of participants not affiliated with a site and to provide some centralized retention efforts.

1.3.2 WAKE FOREST SCHOOL OF MEDICINE DATA COORDINATING CENTER AND GERIATRIC OUTCOMES ASSESSMENT CORE

Wake Forest University School of Medicine serves as the DCC and the Geriatric Outcomes Assessment Core. Wake Forest designed and manages the Electronic Data Capture (EDC) and data management system, integrates and processes data from outside sources, performs data analyses, supports the Data Safety Monitoring Board (DSMB), oversees the Geriatric Assessment program, and provides centralized Call Center services.

1.3.3 VETERANS AFFAIRS NATIONAL NETWORK COORDINATING CENTER

The Veterans Affairs National Network Coordinating Center (VA CC) team acts as an intermediary between the PREVENTABLE CCC and DCC and the VA sites to provide site management support. The VA CC and the VA Central IRB (CIRB) submission teams manage and collect regulatory documents, support the VA sites and Duke contracting processes, work with the VA sites on enrollment issues, and answer day-to-day questions for VA sites.

1.3.4 VETERANS AFFAIRS COOPERATIVE STUDIES PHARMACY COORDINATING CENTER

The Veterans Affairs Cooperative Studies Program (VA CSP) serves as the Pharmacy Coordinating Center (PCC) and provides the atorvastatin procurement, placebo manufacturing, packaging, labeling, and direct-to-participant distribution of the study drug and placebo for PREVENTABLE. See **Chapter 10** for details.

1.3.5 VANDERBILT UNIVERSITY MEDICAL CENTER RECRUITMENT INNOVATION CENTER

The Vanderbilt University Medical Center Recruitment Innovation Center developed and manages REDCap eConsent processes for non-VA sites. See **Chapter 4** for details.

1.3.6 MAYO CLINIC BIOREPOSITORY

The Mayo Clinic Biorepository supports the collection, processing, and storage of biorepository samples from participants who have in-person baseline visits. See **Chapter 6** for details.

1.3.7 ICON LABORATORY SERVICES, INC.

ICON Laboratory Services supports the collection, processing, and storage of lipid panel samples for PREVENTABLE. ICON also supports the collection, processing, and storage of 3-month follow-up samples from approximately 2,000 participants randomized to the lipids sub-study group. See **Chapter 6** for details.

1.3.8 CHECKBOOK, INC.

Checkbook, Inc. provides direct-to-participant payments for PREVENTABLE primary and ancillary studies when participants reach specified milestones. See **Chapter 8** for details.

1.4 COMMITTEE OVERVIEW AND RESPONSIBILITIES

1.4.1 EXECUTIVE COMMITTEE

The Executive Committee is comprised of the National Institute on Aging (NIA) program officers and the lead study principal investigators (PIs) from the DCRI CCC and the Wake Forest DCC and Geriatrics Assessment Center and is supported by operational leadership from the CCC and DCC. This committee reviews study timelines and milestones to ensure the study is conducted to the highest standards while remaining on budget and on schedule. In addition, this committee is responsible for overseeing high-level day-to-day operational decisions and resolving issues.

1.4.2 STEERING COMMITTEE

The Steering Committee is comprised of the Executive Committee, representatives of PCORnet clinical research networks and VA sites, Biorepository and Ancillary Studies Core leaders, and Hub leaders. The Steering Committee is responsible for the clinical oversight of the study.

1.4.3 ANCILLARY STUDIES COMMITTEE

The Ancillary Studies Committee is led by 2 co-chairs with representatives from the NIA, CCC, DCC, and the Steering Committee. This committee works to protect the integrity of PREVENTABLE by reviewing, providing feedback on, and approving proposals for ancillary studies in collaboration with the Steering Committee. Individual ancillary studies that are approved by this committee provide their own funding and involve participants enrolled in the main study. All ancillary studies that are approved and operationalized are overseen by the Wake Forest IRB, with the exception of the Coronary Artery Calcium (CAC) ancillary study that is overseen by the Duke single IRB (sIRB). Details of the active ancillary studies can be found on the study website.

1.4.4 CLAIMS-BASED ASSESSMENT COMMITTEE

The Claims-Based Assessment Committee develops and advises on the algorithms to adjudicate cardiovascular outcomes, as well as other claims and data-driven event confirmation approaches.

1.4.5 BIOREPOSITORY COMMITTEE

The Biorepository Committee is led by 2 co-chairs with representatives from the NIA, CCC, DCC, and Steering Committee. This committee is responsible for reviewing ancillary study proposals, reviewing biospecimen proposals, and developing and maintaining biospecimen utilization policies.

1.4.6 GERIATRICS ASSESSMENT COMMITTEE

The Geriatrics Assessment Committee is comprised of experts with a background in geriatrics, neurology, and cognitive and physical function assessments in clinical studies. This committee reviews and advises on the study processes for accurate ascertainment of the dementia, mild cognitive impairment (MCI), and disability outcomes. This committee develops and carries out the adjudication process for the study.

1.4.7 MEASUREMENTS, PROCEDURES, AND QUALITY CONTROL SUBCOMMITTEE

The Measurements, Procedures, and Quality Control Subcommittee (MPQC) is charged with developing and implementing quality assurance and control mechanisms for all aspects of study conduct. The

MPQC monitors a variety of study data to identify study-wide and site-specific issues. Monitoring includes but is not limited to: (1) retention and adherence, (2) data validation, (3) clinical site responsiveness to study alerts, (4) visit completion (Call Center), (5) completion rates, quality, and results of 3-month lab assessments, and (6) timeliness of lab shipments. On a rotating quarterly basis, the Call Center, ICON Labs, and Mayo Biorepository staff attend the MPQC committee meetings. They may attend more frequently if needed to address study-wide concerns.

The MPQC generates and monitors the PREVENTABLE Action Card, a summary of the top 10–15 metrics most relevant during the current phase of the trial. Top metrics will change as the study shifts from recruitment to follow up to closeout.

The MPQC works closely with the Executive and Steering Committees to identify issues potentially affecting study integrity that may require retraining and/or modifications to the protocol or the MOP. For site-specific issues, the MPQC discusses possible actions with the appropriate site monitor, which may include follow-up phone calls or site visits.

1.4.8 HUBS

A Hub structure was implemented in Year 2 of the grant to better address recruitment and enrollment issues at sites. Six Hubs were created – 2 for VA sites and 4 for non-VA sites, each led by a Hub PI (or co-led by 2) and a Hub Coordinator appointed from successful sites and supported financially by the study. These teams work closely with their assigned sites to identify solutions to recruitment and retention barriers. Primary responsibilities of the Hubs are to:

- Establish calls with their sites (as a group and individually) to discuss strategies for recruiting and retaining participants
- Advise their sites on ongoing and iterative approaches to ensure robust processes for recruitment and retention
- Review overall and site-specific recruitment and retention progress
 - Site-specific metrics, including data and retention/adherence reports, are reviewed and discussed by Hub leadership
- Advise on approaches to mitigate under-recruitment and high dropout
- Provide Hub reports and share successful strategies with the Steering Committee and other Hubs
- Review, discuss, and advise on the results of recruitment and retention strategies applied by sites
- Advise on improving the inclusion of diverse participants
- Review and revise recruitment, enrollment, and site support materials as needed

1.4.9 PARTICIPANT ADVISORY GROUP (PREVENTERS)

Members of the Participant Advisory Group, nicknamed the PREVENTERS, help study clinicians and researchers better understand the viewpoint of participants who are eligible for or enrolled in the PREVENTABLE study. This diverse group consists of community members who are age 75 years or older, from urban and rural locations, and represent PCORnet and VA site networks. They review the protocol and study materials and advise on activities or processes that involve participants.

1.5 DATA SAFETY MONITORING BOARD

The DSMB reviews and advises on the scientific and operational progress of this study. The DSMB periodically reviews and evaluates data on recruitment, quality control, compliance, and safety events in accordance with the DSMB charter. The DSMB members may recommend corrective action, changes in the protocol, or other modifications to study conduct in their advisory role to the National Institutes of Health (NIH). The DSMB also reviews and advises on ancillary studies where direct participant contact is proposed and advises on proposed changes in the protocol originating from study leadership. As specified by the DSMB charter, the frequency of DSMB meetings and conference calls may vary depending on actual subject enrollment and safety event rates.

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CHAPTER 2. CLINICAL SITES

The roles and responsibilities of clinical site principal investigators and site staff include:

- Complying with the protocol, manual of procedures (MOP), central and local institutional review boards (IRBs; as applicable), and federal and state regulations
- Recruiting, screening, enrolling, and retaining participants
- Protecting participants' rights
- Placing orders for baseline prescriptions and renewals of the study drug
- Assisting the Call Center when a participant cannot be reached or additional contact information is needed
- Completing the Death Report Form for participants who pass away and providing information on the participant's death (if known)
- Facilitating the transfer of semi-annual follow-up data from the site's data team to the Wake Forest University School of Medicine (WFUSM) Data Coordinating Center (DCC)
- Resolving queries. See **Chapter 15** for details on how clinical sites assist with crosswalk information.
- Communicating questions, concerns, or observations to the Duke Clinical Research Institute Clinical Coordinating Center (CCC) and/or the DCC
- Maintaining the study regulatory file (electronic or paper) and retaining study records

NOTE: Throughout the MOP, unless otherwise noted, the "PREVENTABLE website" or the "study website" refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

2.1 SITE IDENTIFICATION AND ORGANIZATION

Most sites were identified through 2 site networks:

- The National Patient-Centered Clinical Research Network (PCORnet) made up of approximately 40 PCORnet health systems or health systems who use an unofficial data mart
- The Veterans Affairs (VA) National Network made up of approximately 60 VA hospitals

In addition to the PCORnet and VA sites, there are several independent sites that are not part of either group but are willing to provide electronic health record (EHR) data per the specifications of the PCORnet Common Data Model. Sites are referenced throughout the MOP as either non-VA sites or VA sites. Additional sites may be added as needed throughout the study.

2.1.1 NON-VA SITES

- Each site is encouraged to include multiple practices or clinics to offer this protocol to as many participants as possible.
- Each non-VA site is expected to enroll 375+ participants.
- Non-VA sites cede local review to the Duke single IRB (sIRB).
- Site management and monitoring is conducted by the DCRI CCC.

2.1.2 VA SITES

- Each VA site is expected to enroll 84+ participants.
- VA sites cede local review to the VA Central IRB (CIRB).
- Site management and monitoring is conducted by the Veterans Affairs National Network Coordinating Center (VA CC).

2.2 IRB APPROVAL PROCESS

The Duke IRB provides oversight for the overall conduct of the study at the DCRI CCC and the Coronary Artery Calcium (CAC) PREVENTABLE ancillary study. In addition, the Duke IRB serves as the sIRB for the non-VA sites and the CAC ancillary sites. A CIRB at the VA oversees the activities of the VA CC and the VA sites. The IRB of record for the Ancillary Studies program is the Wake Forest sIRB.

2.2.1 NON-VA SITES: DUKE UNIVERSITY SINGLE INSTITUTIONAL REVIEW BOARD (sIRB)

For the non-VA sites in this study, Duke University IRB provides sIRB review using the Smart IRB Master Common Reciprocal Institutional Review Board Authorization Agreement (SMART IRB Agreement) and the sIRB Letter of Indemnification (LOI) to establish reliance between the non-VA sites and the sIRB.

The sIRB is responsible for the review of the regulatory criteria for approval of human subjects research [45 CFR 46.111], HIPAA compliance, and authorization for the overall study. Additionally, the sIRB obtains the institutional/community profile information and study-specific local considerations from each site in order to adequately review and approve each site for participation in the research.

The Duke sIRB uses the IRB Reliance Exchange platform (IREx), a web-based platform that tracks reliance documentation, such as reliance agreement completion; agreement to cede oversight to the sIRB; study-specific local considerations from each site's human research protections (HRP) program and study team; and site IRB approvals. The study currently only uses IREx to notify sites of the annual continuing review approvals and to track reliance documentation prior to site activation.

2.2.2 VA SITES: VA CENTRAL INSTITUTIONAL REVIEW BOARD (CIRB)

The VA central IRB (CIRB) provides IRB review for the VA sites in this study. This process is directed and supported by the Veterans Affairs National Network Coordinating Center (VA CC).

2.2.3 HUMAN RESEARCH PROTECTIONS (HRP) PROGRAM LOCAL REVIEW

The HRP review is a study-specific review that must occur at each site. This review includes verification that site-specific information is appropriately incorporated in the protocol and consent documents. The site HRP program review also includes other responsibilities as outlined in the reliance agreement. This generally includes, but is not limited to, study-specific confirmation of conflicts of interest, training and qualifications of local research staff, ancillary reviews, and application of local laws and policies.

Non-VA sites document their HRP information in the IREx system. VA sites document their HRP information in the VA Innovation and Research Review System (VAIRRS), the VA's instance of IRBNet. VAIRRS is used by all VA medical centers with research programs and supports the management of research oversight committees.

2.3 SITE QUALIFICATIONS

Sites are identified and selected based on their ability to successfully conduct the study. VA sites are vetted by the VA CC with input from the DCRI CCC. Non-VA sites may be recommended by the PCORnet Coordinating Center, PREVENTABLE Steering Committee, or by self-referral. To be considered for selection, sites must demonstrate that they have:

- A sufficiently large population of potential participants, usually confirmed via a high-level data query of their local EHR system with thousands of possible participants that qualify at least by age and pertinent ICD codes;
- An engaged principal investigator (PI) prepared to support the study;
- A robust research infrastructure and experience conducting research studies with adequate staff to perform the study activities;
- No significant barriers to successful conduct of the study at the site (e.g., excessively high institutional indirect rate, inability to contact potential participants outside a single practice, etc.).

The study leadership, coordinating center informatics group, and site management teams should have in-depth discussions to identify sites that are best positioned for success.

2.4 TRAINING AND CERTIFICATIONS

Training is an ongoing process as changes to the study or protocol may occur throughout the study. Each person listed on the Site Staff Delegation and Signature Log (SDSL) must be qualified to perform the responsibilities listed at the time of delegation by the site PI and have documented protocol/study training. All completed training, no matter the format, should be documented on the Site Training Log and filed in the site's regulatory file (electronic or paper).

Types of study-specific training for site staff include, but are not limited to:

- Webinars (live and recorded)
- Teleconferences
- Face-to-face training sessions
- Manual of Procedures (MOP)

Site staff must complete study training prior to the site enrolling participants. Training slides and reference documents are available on the PREVENTABLE website under Login > Documents > Training. Other training materials can be found under Menu > Training Videos. Additional and refresher training will occur as determined by the DCRI CCC and the VA CC.

2.5 SITE ACTIVATION

The DCRI CCC (non-VA sites) or the VA CC (VA sites) coordinate site startup and ensure the:

- Site contract has been executed
- Regulatory documents have been completed and approved by the DCRI CCC or the VA CC (see **Section 2.5.1**)
- Protocol and study-specific trainings have been completed by the site principal investigator (PI) and primary study coordinator (PSC)

- Short Physical Performance Battery (SPPB) certification by at least one site staff member is recommended but not required for activation (see **Chapter 5**)
- Electronic case report form (eCRF) training has been completed by staff who require access
- Initial recruitment and enrollment supplies have been shipped and received
- Site initiation training has been completed

Sites are assigned a site number by the DCRI CCC or the VA CC. The site number is a 4-digit number beginning with 11–18 for non-VA sites and 20 for VA sites. This number is used to identify sites on all site-related materials and reports.

Site staff can access the eCRF, study documents, study announcements, and other materials on the PREVENTABLE website. Site staff on the Site Staff Delegation and Signature Log (SDSL) who require access to the password-protected part of the study website are granted access by the site management and monitoring (SMM) team or the Wake Forest DCC. Sites should review their list of users every 6 months to confirm all users still require access.

When a site staff member is no longer working on the PREVENTABLE study, the study coordinator will enter an end date on the SDSL and submit the updated log to the assigned clinical research associate (CRA). The CRA will notify the DCC to withdraw the former site staff member's access.

Upon review and approval of the above required documents and after completion of appropriate protocol and study-specific training, each site PI, study coordinator, and other appropriate site staff are notified by the DCRI CCC or VA CC of site activation and approval to initiate study enrollment. This notification should be filed in the site's regulatory file for documentation of the date of site activation. Upon this notification, the DCC will activate the site on the secure part of the study website and screening visits can begin. The DCC will notify site staff of activation on the study website.

2.5.1 DOCUMENTS REQUIRED BY EACH SITE PRIOR TO SITE ACTIVATION

Please refer to the regulatory document completion guidelines provided with the regulatory packet.

Email documents to PREVENTABLE@duke.edu (DCRI CCC) or VAPREVENTABLENETWORK@va.gov (VA CC) as indicated in the table below.

Document	Description
Site Staff Delegation and Signature Log (SDSL)	The SDSL is a living document and should be updated when there are any changes in study personnel. The original log is maintained at the site. Site staff must complete and submit this document to the SMM team prior to activation, with the site PI and primary study coordinator (PSC) listed. Following activation, all updates must be submitted to the site's CRA for review. Typically, the site PI, the PI's designees, and study coordinators are indicated as key personnel on the SDSL, but there may be others whom the PI deems appropriate to add to the log.
Investigator CV	The site PI must provide a current CV dated within 2 years. The site must submit this to the startup team prior to activation. Following activation, all updates must be submitted for review by the SMM team.

Medical License	The site PI's or designated medication prescriber's medical license must be current and available prior to activation. Site staff must maintain up-to-date applicable licensure and provide documentation to the DCRI CCC or VA CC.
Federalwide Assurance (FWA) Number	Site staff must submit their institution's FWA number to the SMM team prior to activation. Following activation, all FWA number renewals and other updates must be submitted to the DCRI CCC or VA CC for review by the study team. The VA CC confirms that the FWAs are current prior to annual submission to the CIRB.
IRB Approval of the Protocol and Informed Consent Form (ICF) Templates	The Duke sIRB and the VA CIRB have approved the protocol and ICF templates. The DCRI CCC or VA CC will review and approve edits made to ICF Part 2 prior to submission to the Duke sIRB or VA CIRB. Individual site IRBs may require a local review of the protocol, ICF, and amendments, and sites should ensure this step is completed as needed. If any site-level changes to the ICF template are requested by sites or their local IRBs, those changes must be submitted to and approved by the assigned CRA prior to submission to the Duke sIRB or the VA CIRB. The protocol and ICFs are submitted to the Duke sIRB by the DCRI CCC or to the VA CIRB by the VA CC at the time of annual review and any amendments.
IRB Approval of Participant-Facing Materials	The Duke sIRB or VA CIRB must approve all participant-facing materials. Minor site edits are allowed (e.g., site-specific updates to IRB-approved templates), but will need to be reviewed by the SMM team. Major edits to IRB-approved materials will require resubmission to the Duke sIRB or VA CIRB.
Documentation of Human Research Protection Training	Documentation of human research protection training is required for all key personnel listed on the SDSL. Sites should maintain up-to-date HRP training documentation for applicable site staff and be able to provide these as requested to the DCRI CCC or VA CC.
Executed Contract with Duke	Each site needs an executed contract with Duke prior to activation.
Good Clinical Practice (GCP)(Non-VA sites only)	Non-VA sites must provide a signed GCP Institutional Policy Statement Form, provided in the start-up packet, as documentation that their institution follows the NIH GCP policy. This form is collected every 3 years to confirm institutional policy is still in place. If there is no policy in place, a GCP Training Documentation Log will need to be completed prior to site activation and updated as expirations occur.

2.5.2 TRAININGS REQUIRED BY EACH SITE PRIOR TO SITE ACTIVATION

Training	Required For
Protocol Training	At minimum, the PI and PSC must complete protocol training prior to activation. Ongoing training of all current and new study staff must be documented on a training log and maintained throughout the lifecycle of the study.
Electronic Case Report Form (eCRF) and Randomization Training	PI and each site user requiring access
Study Reporting	PI and PSC (at minimum)
Biorepository and Lab Procedures	PI, PSC, and any site staff performing lab collection, labeling, processing, freezing, or shipping of biospecimens
Good Clinical Practice (GCP) and Investigator Responsibilities	PI (at minimum). PI designees and study coordinators are encouraged to complete the training.
Recruitment and Retention	PSC and the PI/designated medication prescriber
eConsent	Study coordinators and each site user requiring access to the eConsent system
Disability and Dementia Assessments	PSC and PI
Site Management and Monitoring	PI (at minimum). PI designees and study coordinators are encouraged to complete the training.
Study Drug Process VA Cooperative Studies Program (CSP) Overview (VA sites only)	PSC
Screening: VA CC Recruitment Lists (VA sites only)	PSC
NOTE: Recordings of webinars are available on the study website under Login > Quick Links > Training Videos or Documents > Training.	

A change in a site's PI requires collection of new or updated documents including the PI's CV, medical license, documentation of HRP training, updated ICF(s), IRB approval, Site Delegation and Signature Log (SDSL), GCP form, and contract amendment. Evidence of qualifications including licensure, credentials, and documented training for all site staff listed on the SDSL should be maintained in the study regulatory file (electronic or paper). Contact the assigned CRA for further instructions or questions about the documents.

2.6 SITE CONTRACTING AND PAYMENTS

A contract is required of all principal investigators (PIs) who accept an invitation for their institution to serve as a PREVENTABLE site. A contract that covers payment schedule information specific to PREVENTABLE must be executed with Duke University prior to activation of VA and non-VA sites. The language of this contract should be reviewed immediately upon receipt to identify any minor changes desired by the PI of the contracting institution. The contract with proposed changes should be returned promptly to the DCRI CCC contracts staff to avoid a delay in completion of a final contract for signature.

PREVENTABLE receives funding from the National Institute on Aging (NIA) and the National Heart, Lung, and Blood Institute (NHLBI), which are both part of the National Institutes of Health (NIH). Clinical site reimbursement processes are designed to promote overall study efficiency and permit flexibility to each site for use of funds. Payments are made based on each site meeting specific benchmarks for activation and participant recruitment and retention activities (e.g., enrollment, randomization, completion of annual phone assessments). The payment amounts are detailed in the site contract and these amounts are non-negotiable to maintain equity among sites. Site payments are triggered each month based on data available in the study database on the PREVENTABLE website at the time payments are processed. Checks are generated based on benchmarks met during a month and mailed to sites via USPS. Checks should arrive at sites approximately within the first 2 weeks of the month.

Payments that require submission of an invoice for reimbursement include orders of dry ice for biospecimen shipments, postage, community outreach events, and cohort assessment query (CAQ) fees. VA sites also have an invoiceable payment for annual pharmacy support. To submit an invoice for costs associated with the specified items, please email the invoice to Site-Payments@dm.duke.edu with “PREVENTABLE” and your site number in the subject line. Invoices must be submitted within 90 days of the end of the award year for invoiceable charges incurred within that same award year. “Award year” is defined as a given 12-month period from September 1st to August 31st of the following year.

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CHAPTER 3. RECRUITMENT

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

3.1 OVERVIEW

PREVENTABLE will randomize up to 20,000 participants age 75 and older across the United States, including large numbers of women, racial and ethnic minorities, and people with frailty, limited mobility, mild cognitive impairment, polypharmacy, and comorbidities. The study will determine the role of a moderate-intensity statin in preventing dementia and prolonging disability-free survival in participants without clinically evident cardiovascular disease or dementia at baseline.

Recruiting participants for clinical research studies is challenging and enrolling older adults can be particularly difficult. Unique barriers to recruiting and retaining older adults include a lack of perceived benefits and relevance of research studies, unfamiliarity with research, difficulty understanding and reading informed consent forms, refusal by family members who share in decision making, and a perceived burden on clinical site staff involved in recruitment. If not addressed, these barriers will result in a study that is underpowered, not generalizable, or longer and more expensive than anticipated. Clinicians’ enthusiasm for referring potential participants and endorsing primary prevention studies is also critical.

3.1.1 RECRUITMENT AND RETENTION CORE TEAM

A Recruitment and Retention (R&R) Core has been funded for the PREVENTABLE study. This group operates within the Clinical Coordinating Center (CCC) at the Duke Clinical Research Institute (DCRI) and functions under the leadership of Dr. Bryan Batch of the DCRI with periodic consultation from Dr. Consuelo Wilkins of Vanderbilt University. This group provides recruitment and engagement expertise and support with robust infrastructure, tools, resources, and strategies to proactively address and mitigate recruitment barriers across sites. This comprehensive approach involving all stages of the study facilitates the development of a high quality, feasible protocol, minimizes challenges, and optimizes recruitment and retention. R&R leadership works closely with the Hub Investigators and Hub Coordinators, expert teams that oversee groups of sites, provide operational strategies, and troubleshoot issues identified by their sites (see **Chapter 1, Section 1.4.8** for details). The PREVENTABLE Participant Advisory Group, called the PREVENTERS, reviews materials and processes for cultural sensitivity and plain language. A variety of recruitment and retention materials in English and Spanish are available on the PREVENTABLE website and storefront (see **Sections 3.4 and 3.5** for details).

To ensure efficient recruitment and maximum study retention, the R&R Core partnered with the National Center for Advancing Translational Sciences (NCATS)-funded Trial Innovation Network and leveraged the team’s experience with participant-centered approaches that accounted for the varying needs and preferences of older adults and the authentic engagement of participants, community groups, and clinicians. Retention efforts are described in **Chapter 11**.

3.1.2 CENTRAL RECRUITMENT SUPPORT

Version 5.0 of the study protocol expanded central recruitment and retention support provided by the PREVENTABLE Clinical Coordinating Center (CCC) through the DCRI Participant Research Operations (PRO) Team, also known as the Central Support Team. Central support is optional for sites to use as requested.

The PREVENTABLE CCC offers the following recruitment assistance options for sites:

- **Toll-free number for potential English- and Spanish-speaking participants to call for study information and pre-screening**
 - Sites may include the toll-free number (833-385-3899) in recruitment materials for potential English- and Spanish-speaking participants to call for study information and prescreening.
 - Interested and potentially eligible participants will be referred to an enrolling site near them, if applicable. If not, they will be referred to the PREVENTABLE Telesite, which can enroll English- and Spanish-speaking participants remotely across the US.
 - The hours of operation are 9 AM to 9 PM, Monday through Sunday.
- **Pre-stuffed bulk recruitment packets**
 - More information on where to locate and how to use the bulk recruitment packets can be found on the PREVENTABLE website under Login > Quick Links > Documents > Recruitment Resources > PREVENTABLE Recruitment Packets on the Storefront.
- **Outreach (direct-to-participant mailings and/or phone calls) to potential participants**
 - More information about this support can be found on the PREVENTABLE website under Login > Quick Links > Documents > Recruitment Resources > PREVENTABLE Central Recruitment Support Request.
 - Sites will need to confirm if their local site IRB/research administration allows the transfer of personally identifiable information (PII) for recruitment purposes. Although the site contract includes language about the exchange of PII, some institutions may request a Data Transfer Agreement (DTA) between Duke and their institution to support the transfer PII and to allow direct contact with their patients. Sites should contact their CRA/site manager so the CCC Operations Team can start the process of obtaining a DTA.

3.2 RECRUITMENT PLAN

An overall recruitment plan was developed for the study and the R&R team assists with developing site-specific plans tailored to their unique challenges. The purpose of the recruitment plan is to:

- Organize recruitment strategies for PREVENTABLE with an emphasis on evaluation and metrics to drive decision making and resource allocation.
- Provide data-based projections and milestones for recruitment between now and the proposed end of study.

- Identify potential threats or risks to recruitment progress and proactively provide mitigation strategies. These strategies, along with site updates on progress toward recruitment goals and successful stories, are discussed on regular calls with Hubs leadership and on monthly site calls with the monitor/Clinical Research Associate (CRA).
- Incorporate strategies for enrolling minorities. The Minority R&R team assisted sites with identifying target populations in their areas, possible ways to engage those populations, and approaches to encourage involvement in the study. Sites can request another consultation if needed.

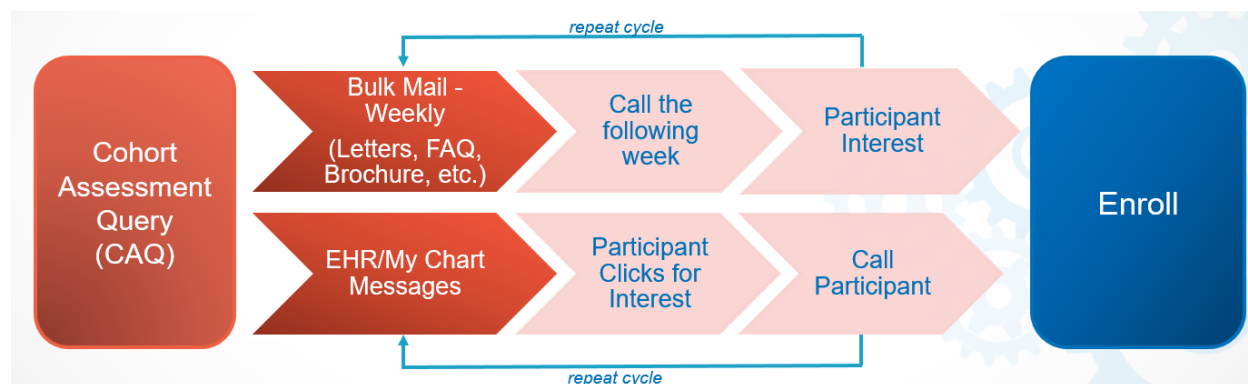
The overall study recruitment plan for PREVENTABLE addresses anticipated protocol-specific issues. Each enrolling site creates a companion site-specific recruitment plan to identify local stakeholders, define strategies to increase study awareness, plan communication, delegate personnel and resources, identify competing studies and prioritization plans, list potential recruitment and retention barriers, and identify risk mitigation actions. Based on the progress of recruitment, it is expected that the site principal investigator (PI) and study team periodically review the effectiveness of their recruitment plan and update their strategies as needed to meet their recruitment goals. Each non-VA site is expected to enroll at least 375 participants and each VA site is expected to enroll at least 84 participants.

Ideally, each participant should be connected with a study site's health system to support the collection of EHR data, but it is not essential. If a potential participant is not receiving care within a site's health system but they are interested in enrolling, the first step is to confirm their primary care physician (PCP). If the PCP is outside of the health system, then the site PI and the PCP may need to discuss if it is feasible to enroll the participant, if they are eligible to enroll, and to confirm the PCP's support for their patient's participation. It may be possible to request medical records from the PCP to allow the site to confirm the participant's eligibility. Alternatively, the participant could establish care at the study site with a PCP or specialist. This could be in addition to their usual PCP, or if they do not have a PCP, an opportunity to establish care. It is important to establish a clinical connection that is sufficient for the site PI to prescribe the study drug each year with adequate clinical information.

In addition to traditional recruitment at sites, the study offers several remote opportunities to engage potential participants and enroll them. The tele-site is available to remotely enroll and follow up with participants who are not near an existing site or who prefer not to enroll at their local site. The tele-site can also support sites that do not have Spanish-speaking staff by providing Spanish recruitment packets for sites to mail, speaking with potential participants, and enrolling them remotely. The tele-site will gather eligibility data, collect the baseline information to enroll participants, and reach out to participants each year to reorder the study drug as necessary. Centralized follow-up for all participants is performed by the Call Center at Wake Forest.

Initial recruitment data indicate that broad outreach is needed for sites to be successful with recruitment. Based on screening data, approximately 1–5% of people who are approached for the study eventually agree to participate, which means that a large pool of potential participants is required. Sites are sent either a list of potential participants that qualify based on general inclusion and exclusion criteria or sites are sent the Cohort Assessment Query (CAQ) code to deploy in their local EHR system (see **Chapter 4** for details). Modifications to the CAQ are often needed to better identify qualified participants in a site's EHR system. These modifications to the CAQ or a similar EHR-based query can be used to identify potential participants with an upcoming clinic visit so the provider can be contacted in advance, notify providers and site staff of eligible patients, and create reports to aid site staff in the conduct of the study.

The most successful sites use multiple “touches” to facilitate enrollment, including letters, postcards, emails, EHR portal messages, and face-to-face visits combined with follow-up phone calls to discuss the study with potential participants and their caregivers. Feedback from sites indicates that if a site sends a paper mailing to potential participants, they should follow up with a phone call in the next 1–2 weeks, followed by an electronic contact and another phone call (see diagram below). To track the status of potential participants as they move through the recruitment process, an example REDcap database is available on the PREVENTABLE website under Login > Quick Links > Documents > Recruitment Resources > CAQ, EHR and REDCap Tools.



3.3 EHR RECRUITMENT TOOLS

Instructions for Epic EHR modifications are available upon request. Sites that use Cerner or other EHR systems are encouraged to contact the CCC for general information about EHR modifications. Sites should contact their CRA to request access to a shared folder on Duke Box with EHR templates and tools. Available resources and instructions to recruit potential participants within Epic include:

- MyChart Research Recruitment Invitations
- MyChart Patient Messages
- Provider Schedule Column
- Workbench Reports
- SlicerDicer Tips

Additional tools and templates that can be deployed in Epic to increase protocol adherence, such as best practice alerts, are also available.

PREVENTABLE IT subject matter experts (SME) from both the Wake Forest University School of Medicine Data Coordinating Center (DCC) and the DCRI CCC are available upon request to facilitate local EHR programming efforts. Consultation with an IT SME has been helpful for institutions regardless of their EHR system as the SMEs have broad expertise.

3.4 RECRUITMENT MATERIALS

To support site recruitment efforts, a variety of IRB-approved, study-branded recruitment materials in English and Spanish are available on the PREVENTABLE website under Login > Quick Links > Documents > Recruitment Resources. Printed materials are available to order on the study storefront (see **Section 3.6**). New materials are added regularly, so please check the Documents section on the PREVENTABLE website for the most current items.

3.5 PREVENTABLE STOREFRONT

Bulk supplies of printed materials, biospecimen vials, and shipping supplies will be mailed to sites once they are activated. Additional supplies can be ordered through the PREVENTABLE storefront, similar to online shopping sites. Site staff must create a storefront account to order supplies and materials specific to their site's operational and recruitment needs. When a site places a resupply order, the CCC team will review the request if it exceeds maximum reorder quantities and, if appropriate, they will ask the storefront manager to ship the order.

NOTE: Items ordered before 1:00 p.m. ET, Monday–Friday will be shipped the same day. Items ordered after 1:00 p.m. ET or on weekends will be shipped the next business day.

To access the storefront, visit <https://eprint.universalprinting.com/Preventable>. A link to the storefront is also available on the study website under Login > Quick Links > Documents > Study Documents and Resources > Supplies.

Follow these steps to register for a PREVENTABLE storefront account:

1. Go to the storefront website: <https://eprint.universalprinting.com/Preventable>
2. Click on the red New User Registration button.

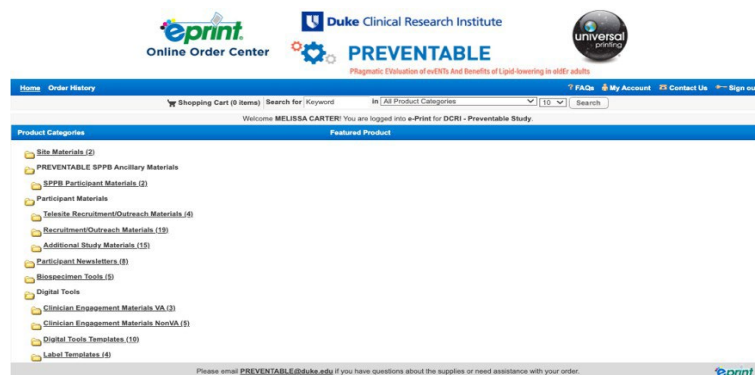
3. Select the Study Trial Sites option.

4. Enter all required information and click the **Submit** button.

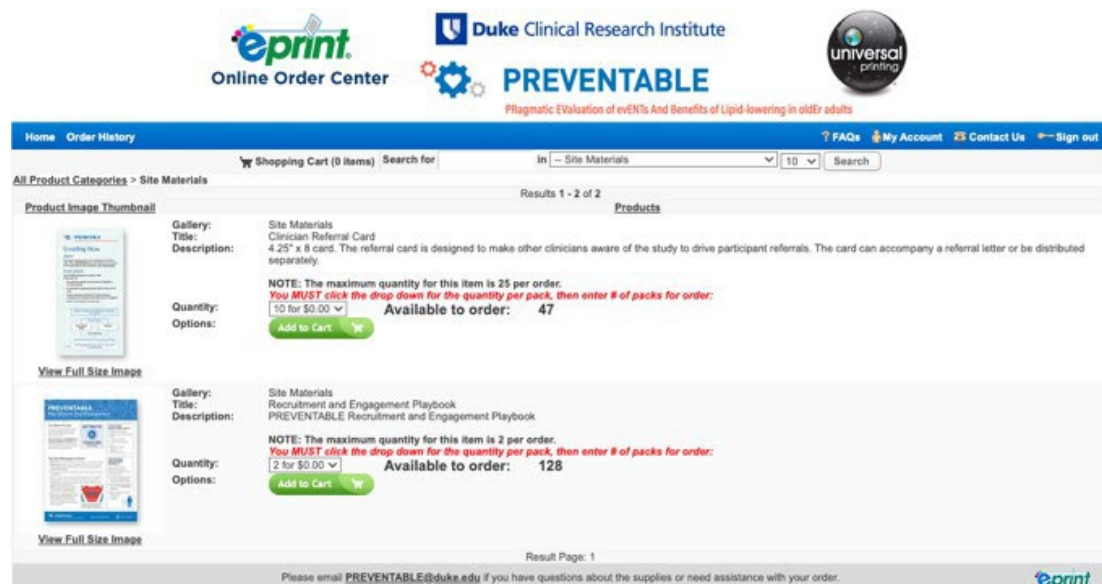
5. Verify your account through the email you will receive from eprint@universalprinting.com. You will not be able to access the storefront until you verify your account.

Follow these steps to order supplies from the PREVENTABLE storefront:

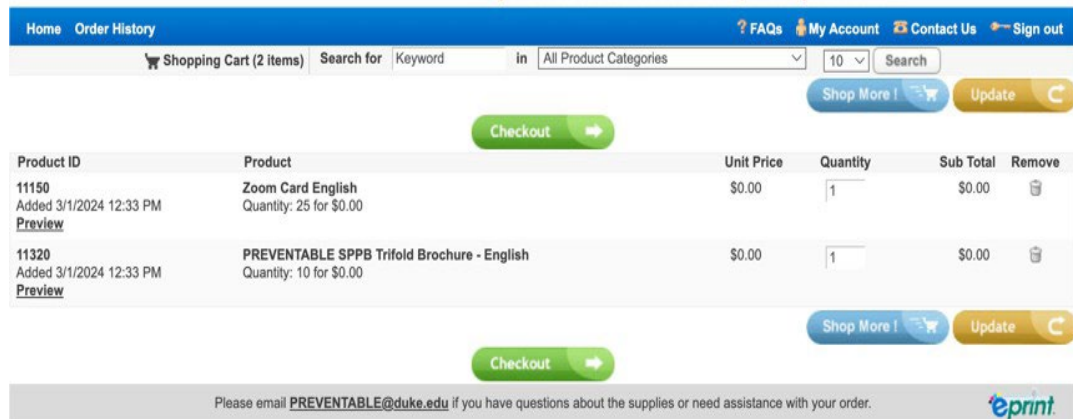
1. Log in to the storefront: <https://eprint.universalprinting.com/Preventable>. When you are logged in, you will see the storefront home page.
2. Click on a product category from the list on the left side of the home page to see details about the items in that category that are available to order.



3. Review the product names, descriptions, and maximum quantity of each item. Orders that exceed the maximum quantity will go through a review and approval process before shipping.



4. Select the quantity needed and click the green Add to Cart button. In the shopping cart, you can:
 - a. Preview – view an image of the product to verify you ordered the correct item
 - b. Quantity – change the number of products to be ordered
 - c. Remove – delete an item
 - d. Shop More – return to the home page to order other products
 - e. Update – save changes to the quantity of products
 - f. Checkout – complete your order



5. Click the green **Checkout** button when you are ready to submit your order.
6. On the Shipping Address and Shipping Options page, select your site number to populate the shipping address. Enter the Recipient and Location/Site Name.
 - To add a new permanent address to your account, click Create New Shipping Address in the Shipping Address drop-down menu, enter all required information, and click Save.
 - The default shipping method is FedEx Ground.
7. Click the blue **Next** button to review your order.

8. Double check the products and quantities and add comments in the Notes section if needed. To change your order, click on the Shopping Cart link under the blue menu bar.

The screenshot shows the 'Confirm Order' page of the PREVENTABLE online storefront. At the top, there's a navigation bar with links like Home, Order History, FAQs, My Account, Contact Us, and Sign out. Below this, a breadcrumb trail indicates the current step: Shopping Cart > Step 1: Shipping > Step 2: Confirm Order. A search bar is also present. The main content area displays user information for 'DCRI - Preventable Study' and 'CARTER, MELISSA'. Below this is a table listing the items in the cart:

Product ID	Product	Unit Price	Quantity	Shipping Method	Shipping Address	Sub Total
11150	Zoom Card English Quantity: 25 for \$0.00	\$0.00	1	FedEx Ground (no PO Boxes allowed)	MELISSA CARTER Durham Centre, Suite 800 9200 West Wisconsin Ave. Ste. 150P, 1P Pavilion Site 1203 Milwaukee, WI 53226 United States of America 414-384-2000	\$0.00
11320	PREVENTABLE SPPB Trifold Brochure - English Quantity: 10 for \$0.00	\$0.00	1	FedEx Ground (no PO Boxes allowed)	MELISSA CARTER Durham Centre, Suite 800 9200 West Wisconsin Ave. Ste. 150P, 1P Pavilion Site 1203 Milwaukee, WI 53226 United States of America 414-384-2000	\$0.00

Below the table is a 'Notes' section. At the bottom, there's a footer with contact information and the eprint logo.

9. Click the green **Submit Order** button when you are ready to finalize your order.
 - After submitting your order, you will receive an email confirming the order and your order will be shipped within the next 1–2 business days.
 - When the order ships, you will receive a shipment notification email that includes the FedEx tracking number.

Questions

Please email PREVENTABLE@duke.edu if you have questions about the supplies or materials. You may email dcricdistribution@dm.duke.edu if you need assistance with your storefront order.

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CHAPTER 4. SCREENING AND ENROLLMENT

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

4.1 SCREENING

The site principal investigators (PIs) or their designee will screen potential participants by reviewing participants’ medical records or by asking patients the prescreening questions and reviewing their current medications. Screening methods must be determined and agreed upon by the local site study team. The PI or their designee will approach potentially eligible participants about the study and provide them with study information in accordance with local site processes. If a potential participant meets the eligibility criteria, the PI or their designee will start the consent process.

Informed consent must be obtained prior to any study-specific procedures and a signed copy of the consent must be provided to the participant. Any protected health information (PHI) that is collected as part of the screening process is documented outside of the study website (e.g., a spreadsheet, tracking database, etc.) and is not entered into the study website until informed consent has been obtained. The only exception to this is the tele-site that enters minimal information from the participant directly into the study website after obtaining verbal consent from the participant.

4.1.1 PRESCREENING TOOL TO GENERATE LISTS OF POTENTIAL PARTICIPANTS

A Cohort Assessment Query (CAQ) that contains code for the basic inclusion and exclusion criteria can be deployed in a site’s electronic health record (EHR) system to generate a list of potential participants. The VA Data team at the VA Coordinating Center (VA CC) provides a list of potential participants to all VA sites. The Data Coordinating Center (DCC) provides a list of potential participants to some central Clinical Research Network (CRN) non-VA sites as identified by PCORnet. Other non-VA sites may run the CAQ with the assistance of their IT staff. The list of potential participants can be customized by adding items to the CAQ such as contact information, primary care provider, next scheduled appointment, etc.

PREVENTABLE sites have found it is more efficient to use the first list of potential participants for mass reach-out. Attempting to screen each person on the initial list often results in added effort for little return. Mass reach-out, such as recruitment letters or emails, followed within 1–2 weeks by a phone call to potential participants with reference to the recruitment letter provides the greatest yield. When possible, sites should review the medical record of each interested potential participant to confirm their eligibility prior to consenting the participant.

See **Appendix 4.2** for more information about how sites can use the CAQ. Sites may also use other screening or notification tools, including clinic workflow alerts via the local EHR, at strategic points of patient care. Potential participants may be contacted in person or via other recruitment methods such as advertising, mail, email, EHR portal messages, phone calls, or text messages.

4.1.2 RESCREENING

If a potential participant has previously been screened and found to be ineligible, they may be rescreened if, in the judgement of the PI or their designee, the participant may meet the eligibility criteria. Participants may be rescreened if:

- They are age 75 or older,
- Hospitalization for a primary diagnosis of heart failure was more than 12 months ago,
- It has been over a year since the participant used a statin, or
- Previous finding of an exclusion criterion was incorrect.

If a participant is rescreened, the same participant ID must be used. All forms should be reviewed and updated, including updating the date of the study visit. If it has been less than 45 days since the initial screening, only the relevant baseline forms that have changed need to be updated.

4.2 STUDY CRITERIA

The PI or their designee is responsible for confirming participant eligibility based on study criteria, potential drug interactions, and lab values from review of the medical record or participant self-report. It is highly recommended, but not required, that sites use the PREVENTABLE Eligibility Worksheet to confirm each participant's eligibility. The worksheet is available on the study website under Login > Quick Links > Documents > Study Documents and Resources > Study Logs and Forms. Inclusion/Exclusion Frequently Asked Questions (FAQs) are available under Quick Links > Documents > Study Documents and Resources > Quick References and FAQs.

It may not be possible to confirm all inclusion and exclusion criteria prior to a baseline visit or in a call with a potential participant. If a participant is not willing to complete cognitive testing over the phone or expresses major concerns about completing phone assessments during the study, they are ineligible for the study. The Call Center will mark their status as "refused" and the participant will not proceed to randomization.

An alert is sent to the site via the study website when a participant is marked as "refused." Site staff should follow up with the participant to confirm their reluctance to be randomized and either reschedule the baseline call or use the blue "Respond" button in the alert message and indicate that the participant refused.

Research records of participants who screen fail after consent should be maintained in the site's study record, along with documents for randomized participants, and documented per local site regulations.

4.2.1 INCLUSION CRITERIA

Per version 5.0 of the PREVENTABLE protocol:

- **Community-dwelling adults**
 - "Community-dwelling" means living independently or with minor assistance. Living in a group setting or in a continuous care retirement community (CCRC) is completely fine. However, if this living environment is needed because the participant is dependent in more than one ADL and not able to receive study drug by mail or take study drug themselves and answer the phone then participation is not possible.

- **Age ≥ 75 years**
- **Primary language is either English or Spanish**
 - *When considering enrollment of a participant, you should confirm the participant's primary language. Sites should specify if Spanish is the preferred language at the time of enrollment on the Contact Information form. Study drug labels are available in English and Spanish. The drug package insert is printed in both English and Spanish (front and back). There are Spanish versions of the cognitive assessments to use for Spanish speaking participants. There are also Spanish speaking staff at the call center. However, the call center cannot complete calls in languages other than English and Spanish. Therefore, if a participant confirms their native or primary/preferred language is not English or Spanish they cannot be enrolled.
- **Able to provide a Trusted Contact**

4.2.2 EXCLUSION CRITERIA

Per Version 5.0 of the PREVENTABLE protocol:

Clinically evident cardiovascular disease defined as prior myocardial infarction (MI), prior stroke, prior revascularization procedure, or a secondary prevention indication for a statin (clinician determined) -

- *Valvular heart disease is not an exclusion. The only cardiovascular exclusions are: (1) hospitalization for a primary diagnosis of heart failure in the prior 12 months (prior history of heart failure in the problem list is fine) and (2) clinically evident cardiovascular disease defined as prior myocardial infarction, stroke, revascularization procedure or a secondary prevention indication for a statin (clinician determined). The latter acknowledges that subclinical cardiovascular disease is frequently detected on imaging studies in this population. Yet, if not deemed to be an indication for a statin, then subclinical disease is not an exclusion.*
- The exclusion criteria for the study is “clinically evident cardiovascular disease”. Findings of vascular disease on CAC scan, ABI, or CT angiogram, prior stroke on Head CT, or possible MI on cardiac imaging are prevalent in this population. In addition, they may be artifacts or imaging ‘soft calls’. The key is a clinically evident event to correspond to an incidental finding. Participants with a new imaging finding but no clinical symptoms requiring treatment remain eligible **as long as the primary clinicians are aware of the finding and do not plan to start a statin**. *If this finding occurs after randomization and the participant will be started on a statin for long-term use based on this imaging finding, then the participant should stop study drug. The site should indicate study drug is permanently discontinued for a new indication for open label statin. Regardless, the participant stays in the study for follow up until study conclusion.*
- **Hospitalization for a primary diagnosis of heart failure in the prior 12 months**
 - *Note: History of heart failure in the absence of recent hospitalization or clinically evident cardiovascular disease is not an exclusion.*
- **Dementia**

Clinically evident or previously diagnosed. This could be indicated by taking any of the following prescription medications (donepezil, galantamine, rivastigmine, memantine). However, individuals without dementia may also be taking these medications and a prescription for any of these medications is not an exclusion. A diagnosis or clinical evidence

- of dementia should be confirmed by review of the medical records. Donepezil, Rivastigmine, Memantine, Galantamine, Suboxone medications were included in the cohort query to identify those with potential dementia, and are useful in the clinic since dementia itself may not be in the record. However, if they are used without a diagnosis of dementia, the individual is still eligible.*
- **Dependence in any Katz Basic Activities of Daily Living (ADL)** (with the exception of urinary or bowel continence)
 - *The Katz Index of Independence in Activities of Daily Living (ADL) is used to assess a participant's daily capabilities. This tool consists of six questions about a participant's ability to accomplish a specific task. The responses are scored from one (able to complete that task) or zero (unable to complete that task). The results for each question are totaled for an overall score of six (Independent) to zero (Very Dependent).*
 - **Severe hearing impairment** (preventing phone follow up)
 - **Unable to talk** (preventing phone follow up)
 - **Statin use in the past year or for longer than five years previously** (participant reported)
 - *Statin use is defined as regular, active use as an outpatient for longer than one month. Short-term use (inpatient or outpatient) or a prescription that was never filled or taken do not count. Clinician judgement on this criterion is acceptable.*
 - *Only current statin use is an exclusion. Other lipid lowering therapy such as Zetia is not an exclusion.*
 - **Ineligible to take atorvastatin 40 mg** (clinician determined)
 - **Documented intolerance to statins**
 - **Active liver disease** (clinician determined)

4.3 INFORMED CONSENT PROCESSES

Potential participants should be encouraged to discuss the study with their primary care provider prior to participating. Informed consent must be obtained prior to beginning any study-related activities that are not part of routine care and prior to entering any protected health information (PHI) into the study database (the exception is for telehealth participants who provide verbal consent for initial data collection). Inform participants that they can withdraw their consent at any time and for unstated reasons without any repercussions or impact on their usual health care.

All participants should receive a copy of the signed informed consent and contact numbers for the study team in case they have questions or they want to report any concerns related to the study. Both electronic consent (eConsent) and paper Informed Consent Forms (ICFs) are valid for PREVENTABLE. Both the Duke single IRB (sIRB) and VA Central IRB (CIRB) have approved sites to mail or email blank and signed ICFs to participants. **Sites should follow their local regulations for mailing or emailing ICFs to participants.**

Before any documents or materials are provided to potential or enrolled participants, they must be approved by the Duke single IRB (sIRB) or the Veterans Affairs (VA) Central IRB (CIRB) and any required local review board. Document the consent process per your site's local institutional policies.

Site staff are encouraged to follow the guidelines below to ensure the consent process is both voluntary and informed.

- **Environment:** The setting in which informed consent is obtained must be private so that participants can freely ask questions. To avoid making the participant feel pressured, it is best if only 1 person associated with the study is present when the participant reviews the informed consent.
- **Study Information:** Participants should be fully informed about the study and have adequate time to evaluate the advantages and disadvantages of participation. A participant brochure that describes the study and the possible benefits of participation is available in English and Spanish on the [public-facing PREVENTABLE website](#). Sites may order printed brochures on the [PREVENTABLE storefront](#). See **Chapter 3** for more information about the storefront.

Site staff should review the ICF with the participant and answer any questions. If needed, the ICF may be sent home with the participant so they have time to carefully review the ICF prior to signing. If local regulations allow, the ICF may be mailed or emailed to the potential participant prior to enrollment. If done in person or using the eConsent platform, the ICF should be signed by the participant and the person obtaining consent on the day of the baseline visit. If the informed consent process is conducted via mail, local site processes should be followed.

- **Study Discussion:** Potential participants should be encouraged to discuss the study with anyone they wish, particularly family and friends who might be affected and whose support will be essential for long-term retention and adherence. Family members or trusted contacts of the participant may raise important questions and considerations that the participant has overlooked. Furthermore, there is evidence to suggest that family support for studies of this kind increases the probability of participant cooperation over time.
- **Guardian Consent Not Permitted:** To be eligible for participation in PREVENTABLE, participants must have the capacity to provide their own informed consent. If a participant is incapable of understanding what is expected of them as a participant in the study, they should not be included. It is not permissible to obtain informed consent from a guardian or other legally authorized representative.
- **Obligations and Copies of the Informed Consent Form (ICF):** After the participant signs the ICF, the person who obtained the consent (site PI or their designee) will sign the form. The participant should be given a paper copy or emailed a copy of the ICF after it is signed and dated and should be encouraged to keep the copy for their reference throughout the study. Although participants are free to withdraw from the study at any time, the ICF describes the participant's obligations while they are enrolled and provides detailed information about the study.
- **Source Documentation:** Anyone who signs an ICF should personally date it. If informed consent is obtained the same day the participant's involvement in the study begins, it should be documented in the participant's study record that informed consent was obtained prior to participation in the research. A general statement for source documentation should be included. For example, "All the required elements of informed consent were presented to the participant. Voluntary consent was obtained and the participant's questions were answered prior to initiation."

4.3.1 NON-VA SITES: ECONSENT

The Duke sIRB has approved non-VA sites to mail or email blank and signed ICFs to participants. Participants at non-VA sites have the option of an electronic consent (eConsent) form whether they are

consented in person or remotely via telehealth. Depending on local site regulations, site staff can email the eConsent link directly to a potential participant. The recipient can either review and sign the consent independently or site staff can contact the potential participant to answer any questions and assist with the eConsent. After a participant signs the eConsent and returns it to the site, the person who obtained the consent (site PI or their designee) must sign the electronic attestation form. Depending on local site regulations, site staff may print and mail a paper copy, attach a copy to the participant's EHR, or email a copy to the participant.

The eConsent, provided via the REDCap platform developed at Vanderbilt University, has been 21 CFR Part 11 validated. Standard Operating Procedures (SOPs) and training slides for eConsent, provided by the Vanderbilt team that manages eConsent for PREVENTABLE, are available on the study website under Login > Quick Links > Documents > Training > eConsent. An eConsent demo is available under Login > Quick Links > Training Videos > Training > Consent and eConsent.

The PREVENTABLE eConsent form allows participants to review and sign the consent using a computer, tablet, or smartphone. If needed, iPads can be provided to non-VA sites to support eConsent processes. The eConsent includes the same wording as the paper version with additional features such as optimized font, participant-facing videos, text-to-speech functionality, and the option to read in Spanish. Upon completion, the system documents the 'consent transaction' and stores final consent PDFs in REDCap, allowing the research teams at sites or the coordinating centers to retrieve the consent type, status, and version at any time. Consent forms are also stored in a separate secure document system for redundancy and permanent storage.

NOTE: Local electronic consent platforms may be used instead of REDCap as long as those systems follow standard processes to ensure participants are consented properly. If a local consent system is used, a copy of the signed consent form must be uploaded to the Consent Information tab in participant's record on the study website before the study drug can be shipped.

Please follow your local site requirements for obtaining phone consent. If your local site allows phone consent, please check with the Clinical Coordinating Center (CCC) at the Duke Clinical Research Institute (DCRI) for sIRB requirements (i.e., approval of site-specific phone scripts).

If eConsent is used during a telehealth baseline visit, the site staff obtaining the consent must sign an attestation that an informed consent was obtained from the participant. An email will be sent to the site when a participant signs the eConsent to facilitate prompt documentation of the attestation. Some sites may have a local requirement that the consent is also signed by a licensed clinician. If a second signature is required per site regulations, there is a second signature line in the eConsent that can be forwarded to a study coordinator or to the PI.

Data transfers from the REDCap platform to the Data Coordinating Center (DCC) occur regularly and include the following variables: Participant ID, date of consent, yes/no responses to the layered portion of the informed consent (i.e., whether the participant agreed to allow their samples to be used for genetic research and whether they agreed to allow reporting of cognitive changes to their doctor). In addition, "a PDF of the signed informed consent and the data is automatically transferred to the PREVENTABLE website when the attestation is signed. This allows signed consent forms to be shared with the VA Cooperative Studies Program (CSP) Pharmacy Coordinating Center (PCC) which requires a signed consent prior to shipping the study drug. Data from the eConsent is displayed in the study website on the Consent Information tab for the participant.

It is critical to review the PDF of each signed consent form. If the signature cannot be read or if the participant signed with a name other than their full legal name (e.g., they signed with their middle name or a nickname), a combined PDF including the “Note to File” and the signed consent should be uploaded to the study website.

4.3.2 VA SITES: ECONSENT AND REMOTE CONSENT BY PHONE

Participants at VA sites may provide consent by DocuSign, Azure Rights Management System (RMS) encrypted email, or by phone after receiving mailed copies of the consent documents.

When a participant provides consent via any of these methods, the study team member who obtains consent must document the consent process as a “research visit note” in the participant’s EHR (each VA site may have a specific name for this note). The note should specify the date consent was obtained and whether consent was obtained via DocuSign, RMS email, or by mail after remote consent by phone.

The CIRB-approved documents mentioned in this section are available in the Lead Site’s project folder in IRBNet under Project Administration > Reviews > Board Documents.

NOTE: No research activities related to the study can begin until all steps of the informed consent process are complete.

4.3.2.1 VA SITES: DOCUSIGN

DocuSign is a VA-approved, secure method for electronically sending informed consent forms (ICFs) and obtaining wet or digital signatures from VA participants. VA sites should follow the DocuSign for Research SOP provided by the VA Office of Research and Development (ORD) and the DocuSign Email and Phone Script available in IRBNet.

Using DocuSign for eConsent

Sites will mail an IRB-approved Opt-In or Opt-Out Recruitment Letter to potential VA participants. The Recruitment Letter explains the study and provides a phone number to contact the study team (see “Rec Letter” in IRBNet). If the potential participant does not contact the site within 10 days of the date the recruitment materials were mailed, and the potential subject has not already contacted the PI and/or study team, a study team member will follow up with a phone call. During the phone call with a potential participant, the study team member obtaining consent should:

1. Introduce yourself and ask if the potential participant wants to learn more about the study. If so, provide a brief summary of the study and schedule a date and time with the potential participant to review the study documents (Informed Consent Form and HIPAA Authorization Form 10-0493) using DocuSign (see “Phone Call” script).
2. Send a DocuSign envelope (email with links to the study documents) to the potential participant (see DocuSign SOP for Sending Users). The email will include a reminder that the potential participant should not sign the documents before their appointment with the study team member (see “Email” template).
3. At the scheduled time, contact the potential participant via phone, VA Video Connect (VVC), or another approved secure communication method.

4. Guide the potential participant to open the DocuSign envelope (email) that includes links to the study documents. Open a copy of the study documents on your computer to reference during the call.
5. Review the study documents with the potential participant, ask questions to gauge their understanding, and address their questions or concerns.
6. If the potential participant agrees to participate in the study, guide them in completing and signing the study documents. After the participant signs the documents, they will click “Finish” to finalize the documents (see “E-consenting” script) and you will receive an email to log into DocuSign to complete the documents.
7. While still on the call with the participant, verify all fields are accurate and, if no corrections are needed, sign the documents.
8. Guide the participant to download a copy of the signed documents to their computer.
9. Thank the participant and end the call (see “E-consenting” script).
10. Save a copy of the signed documents for the study record in a folder on the VA network drive. Do not upload VA participant ICFs to the study website.

4.3.2.2 VA SITES: AZURE RIGHTS MANAGEMENT SYSTEM (RMS) ENCRYPTED EMAIL

VA sites may also use Azure RMS encrypted email to distribute CIRB-approved study documents and to obtain the electronic signatures of participants who would like to participate in the study but who are unable or unwilling to visit the site for study enrollment and follow-up.

The Azure RMS VA encrypted email and cloud-based protection service was approved for VA research by the VA research oversight teams (VA ORO, ORD, and OIT), to securely transmit IRB-approved informed consent forms to participants. Research participants can open and view RMS encrypted email from any compatible web browser or email account, such as Gmail, Yahoo, or Outlook.

If Azure RMS is used to recruit VA participants, site staff must confirm the correct email address for each participant, similar to determining the correct mailing address for physical letters. This is important to minimize the risk of sending sensitive information to unintended email recipients and to protect the privacy of potential participants.

Emails cannot be sent to potential participants unless they have already given verbal or written permission or opted in to receive emails about possible research opportunities. Sites must ensure that potential participants are not receiving unsolicited emails. VA sites should use the same considerations for emailing participants as they would when contacting potential participants by physical mail. Use the email template in IRBNet to ensure recruitment emails do not contain coercive or harassing language.

4.3.2.3 VA SITES: REMOTE CONSENT BY PHONE

VA participants also have the option to consent remotely by phone after receiving copies of the Informed Consent (ICF) and HIPAA Authorization (Form 10-0493) by mail. If a potential participant wants to enroll remotely by phone, the local site investigator (LSI), study coordinator (SC), or other qualified designee obtaining consent should:

1. Confirm the participant’s mailing address and mail 2 copies of the ICF and HIPAA Form to the participant –1 copy for the participant to sign and return to the site and 1 copy for the participant to keep for reference. Include a prepaid, self-addressed envelope for the participant to return the signed and dated forms.

- a. The participant must receive the ICF and HIPAA Form before the remote consent phone call. The remote consent cannot occur before the participant receives the ICF and HIPAA Form as they will need to refer to the documents during the phone appointment.
 - b. After the participant confirms they received the ICF and HIPAA Form, schedule a phone call or VA Video Connect (VVC) appointment at a convenient time for the participant and the study team member. Be sure to allow time to address the participant's questions or concerns.
2. At the beginning of the phone or video appointment, verify the participant's identity.
3. During the phone or video appointment, have the same consent discussion you would have with the participant during an in-person appointment.
4. After the consent discussion is complete and if the participant agrees to participate, confirm that the participant initialed and signed the ICF and HIPAA form appropriately.
5. Ask the participant to use the prepaid, self-addressed envelope they received with the ICF packet to return the signed and dated ICF and HIPAA Form to the site.
6. When a site receives a signed and dated ICF and HIPAA Form, the study team member who obtained consent must sign and date the ICF using the current date. Under the signature line, document that the consent was obtained over the phone, the date of the phone consent conversation, and the date the signed consent was received. For example, "Discussed with [participant name] via [phone] on [insert date] and received signed consent form on [insert date]." Include a brief reason for performing the informed consent discussion over the phone.
 - a. Per ORD regulations, the official date of the remote informed consent is the date the study team member who obtained consent receives the signed and dated ICF from the participant, not the date of the consent discussion with the participant.
7. Upload the final ICF and HIPAA Form (10-0493) and HIPAA Form (10-0493) to the ICF folder located in the Study Monitoring and Auditing section of your site's VA site-specific PREVENTABLE SharePoint within 5 business days of the document being signed.
8. Send a copy of the final ICF, signed by the participant and the study team member who obtained consent, and a copy of HIPAA Form signed by the participant, to the participant via mail or email depending on the participant's preference.

4.3.3 NON-VA AND VA SITES: PAPER CONSENT

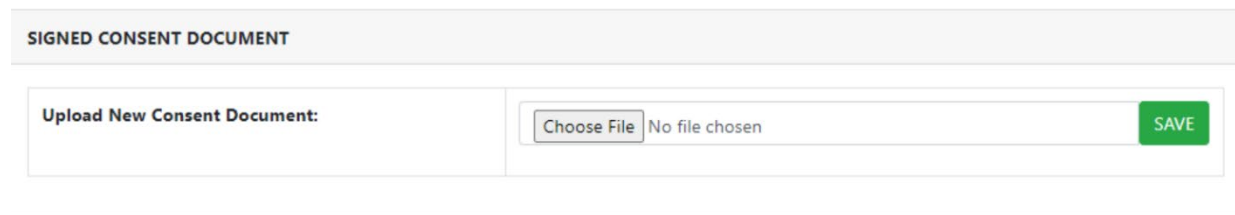
All sites have paper informed consent forms (ICFs) for participants. Sites are approved to mail or email blank and signed ICFs to participants.

Prior to using an ICF, site staff must confirm it is the most current ICF version for their site by checking the outcome letter provided with the sIRB-approved ICF. The ICF must be signed by the participant and uploaded before the study drug can be shipped to the participant.

For VA sites, the signed paper ICF must be scanned and uploaded to the Signed ICF folder in the Study Monitoring and Auditing section of their VA site-specific SharePoint folder within 5 business days of the document being signed. VA participant ICFs should not be uploaded to the study website.

For non-VA sites, the signed paper ICF must be scanned and uploaded to the study website. To upload a signed ICF to the study website, scan and save the signature page. At the top of the study website, click Login > Data > Data Entry, select the appropriate site, and type part of the participant's name (if the date of consent and Contact Information Form has been entered) or PID in the search bar. Under the Signed Consent Document tab, click "Choose File," select the ICF file saved on the computer, and click

“Save.” The study database will save the file with the participant ID and date automatically included in the file name. Review the file to make sure the correct document was saved in the participant’s record and upload a new file if needed.



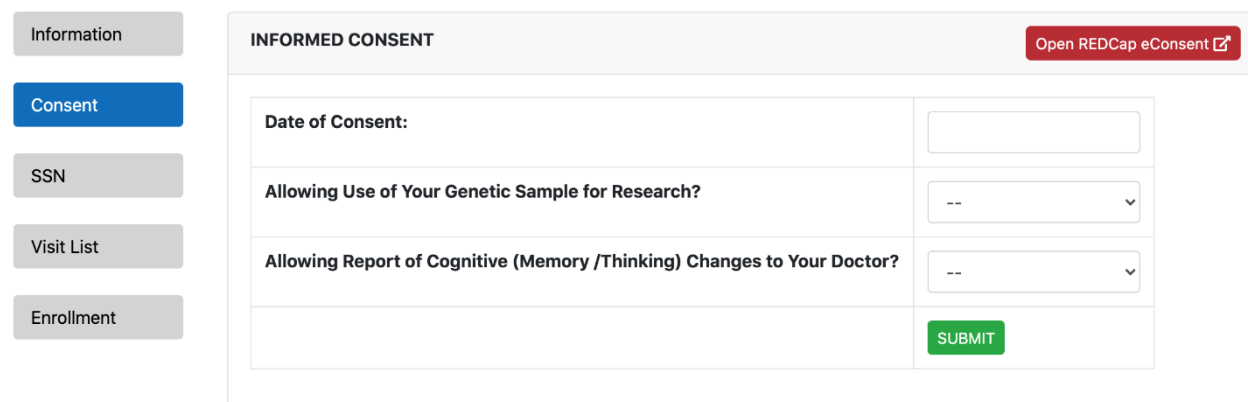
The screenshot shows a web interface titled "SIGNED CONSENT DOCUMENT". Below the title is a section labeled "Upload New Consent Document:". To the right of this label is a file selection area with a "Choose File" button and the text "No file chosen". To the far right of this section is a green "SAVE" button.

4.3.4 DOCUMENTING CONSENT IN THE STUDY WEBSITE

NOTE: The study website will not allow data to be entered for a participant until the date of consent is entered.

On the study website, site staff will need to enter the date of consent and yes/no responses to the layered portion of the ICF (i.e., whether the participant agreed to allow their samples to be used for genetic research and whether they agreed to allow cognitive changes to be reported to their doctor). To enter the ICF information, select Data Entry from the Data Management menu, select the appropriate site, and find the PID in the list or type all or part of the PID in the search field. On the Consent tab, enter the date of consent and the participant’s responses to the layered portions of the consent.

The layered portions of the consent can be updated if a participant wants to change their responses. The date of consent cannot be updated once entered. If the wrong date of consent is entered, log in to the study website and click “Submit a RAP” (Report a Problem) under Quick Links. Complete the RAP form and the Data Coordinating Center will update the date of consent.



The screenshot shows a web interface titled "INFORMED CONSENT". On the left is a sidebar with buttons: "Information", "Consent" (highlighted in blue), "SSN", "Visit List", and "Enrollment". On the right, the "INFORMED CONSENT" form is displayed. At the top right of the form is a red button labeled "Open REDCap eConsent" with an external link icon. The form contains three rows of input fields: "Date of Consent:" with a text box, "Allowing Use of Your Genetic Sample for Research?" with a dropdown menu showing "--", and "Allowing Report of Cognitive (Memory /Thinking) Changes to Your Doctor?" with a dropdown menu showing "--". At the bottom right of the form is a green "SUBMIT" button.

4.3.5 ANCILLARY STUDY CONSENT

If a participant is selected for an ancillary study and a separate consent is required, site staff will be able to document the date of consent on the Consent tab on the study website.

4.3.6 CONFIDENTIAL MASTER SUBJECT LOG

Per Good Clinical Practice (GCP) guidelines, sites will maintain a Master Subject Log of all participants who signed the ICF. The log is available on the study website under Login > Quick Links > Documents >

Study Documents and Resources > Study Logs and Forms. The log includes participant ID number (PID), initials, name, date of birth, medical record number, date/version of consent, and date of randomization.

NOTE: The Master Subject Log is confidential and must not be forwarded to the sponsor or coordinating center. It must be stored in a secure location and cannot be shared outside the site except to partners that have permission to receive PHI.

4.4 ENROLLMENT PROCESSES

Participants may be enrolled in person or via telehealth. If a participant is enrolled via a telehealth visit, the Short Physical Performance Battery (SPPB) and blood samples are not required per the protocol. However, sites should make every effort to obtain the SPPB and samples in other ways such as administering the SPPB during an in-person visit for routine care or scheduling a “lab and leave” visit. It is essential to collect as many SPPB results and blood samples as possible.

To allow time for SPPB results and blood samples to be collected prior to enrollment, sites can delay enrollment of an eligible participant by waiting to click the “Enroll Participant” button for up to 30 days from the date of consent. If a delay of more than 30 days is anticipated, the site should forgo the SPPB and blood samples and enroll the participant at the time of consent.

NOTE: Sites should not collect the baseline SPPB or blood samples after a participant is randomized.

4.4.1 PARTICIPANT ID (PID) NUMBERS

Each participant who signs a consent will be entered in the study website and assigned a “Participant ID (PID).” PIDs, generated by the DCC, are nine-digit numbers ranging from 100000118 – 999999869.

To initialize PIDs and print forms in the Eligibility and Baseline Visit Form packet, log in to the study website, click “Data” in the study website menu, select “Data Entry,” then click the gray “Initialize Participant(s)” button. There are 3 options to initialize a PID: print one full Baseline Packet, print at least one Eligibility Form, or Direct Data Enter New Participant. Once an option is selected, the date of consent must be entered in the study website before data can be entered in any forms.

PIDs are preprinted on the Eligibility and Baseline Visit Forms. When one of these forms is scanned, the PID is associated with the participant. The PID will follow the participant through screening, rescreening (if applicable), randomization, and follow-up visits, and will be preprinted on all study visit forms. PIDs are not linked to a study site and will not change during the study.

NOTE: If any data has been collected on a participant and a PID has been associated with them, the PID must *never* be used for another participant.

If a participant is rescreened at any time, keep the same PID and update the baseline visit at each rescreening visit, including the date of visit on all forms.

4.4.2 ELIGIBILITY AND BASELINE FORMS

Each form in the Eligibility and Baseline Forms packet will have the same PID in the header. **If it is determined that a participant is ineligible, DO NOT use the remaining uncompleted forms for another**

participant. Even if a participant has only completed the informed consent and does not proceed with any additional forms, the PID cannot be used for any other participant.

The Eligibility and Baseline Forms packet includes the:

- Eligibility Form
- Contact Information Form
- Demographics Form
- Medical History Form
- Blood Collection Form (for in-person visits)
- Short Physical Performance Battery (SPPB; for in-person visits)
 - The SPPB Form appears only if there is a staff member at the site who is certified to administer the SPPB.

Please refer to **Appendix 4.3** for instructions to complete these forms.

4.4.3 ELIGIBILITY WORKSHEET AND FORM

Sites are encouraged, but not required, to use the Eligibility Worksheet as they are reviewing participants' medical records to determine eligibility. The worksheet is available on the study website under Login > Quick Links > Documents > Study Documents and Resources > Study Logs and Forms. The Eligibility Worksheet may be used as documentation, in addition to the required Eligibility Form, that a participant met the inclusion/exclusion criteria prior to randomization.

Sites are required to complete the Eligibility Form in the study website for any participant who is tentatively considered eligible and consented. The Eligibility Form will indicate whether the participant is eligible to enroll or not.

If a printed Eligibility Form is used to prescreen and it is determined that a potential participant is not eligible, and thus not approached, the Eligibility Form does not need to be entered in the study website.

Site staff may review medical records to begin the process of completing the Eligibility Form. Medical record searches or reviews of existing databases can be done initially by using the Cohort Assessment Query (CAQ) or by setting up searches using variables that match the inclusion/exclusion criteria. The medical history must be verified with each potential participant to ensure that none of the exclusion criteria listed below are present.

In addition to completing the Eligibility Form, inclusion and exclusion criteria and eligibility should be carefully evaluated and clearly documented by the site PI or their designee in accordance with GCP guidelines. Examples of appropriate documentation of a participant's eligibility include:

- Completed Eligibility Worksheet, saved in the participant's study record
- A research note or other documentation of eligibility from the PI or their designee in the participant's medical record
- An email confirming eligibility, saved as a copy in the participant's study record and not in an email inbox
- Site-specific documentation: Sites should confirm the appropriateness of documentation with their CRA before using site-specific documentation to ensure GCP guidelines are followed.

Eligibility Form Questions

If any of the following are present by participant self-report, report by a participant's trusted contact, or review of the participant's medical history, the participant is NOT eligible for PREVENTABLE.

1. Does participant have clinically evident CVD (prior MI, prior stroke, prior revascularization procedure, or secondary prevention indication for a statin as determined by their clinician)?
2. Has the participant been hospitalized for a primary diagnosis of heart failure in the past 12 months? (History of heart failure without clinically evident cardiovascular disease is not an exclusion.)
 - The participant may be rescreened when more than 12 months have passed since the hospitalization.
3. Has the participant used a statin in the past 12 months?
 - The participant may be rescreened when more than 12 months have passed since the use of a statin.
4. Has the participant previously reported using a statin for more than 5 years (if unsure of duration or prior use, select [No])?
 - If possible, this should be verified in the participant's medical record and then confirmed by participant self-report or the participant's trusted contact.
5. Has the participant been previously diagnosed with or have clinically evident dementia? This could be indicated by taking any of the following prescription medications: donepezil, galantamine, rivastigmine, memantine), however individuals without dementia may also be taking these medications and a prescription for any of these medications is not an exclusion. A diagnosis or clinical evidence of dementia should be confirmed by review of the medical records.
 - **NOTE:** It is normal to have some memory changes as we age, particularly in our late 70s and 80s. This is normal aging and would not disqualify someone from participating in PREVENTABLE. Even if a potential participant has Mild Cognitive Impairment (MCI), they would still be able to participate. However, if they have a diagnosis of dementia from their doctor, they would not be eligible.
6. (Katz Basic ADL) Does the participant need help with bathing more than one part of the body, getting in or out of the tub or shower or requires total bathing?
7. (Katz Basic ADL) Does the participant need help with dressing self or need to be completely dressed?
8. (Katz Basic ADL) Does the participant need help transferring to the toilet, cleaning self or uses a bedpan or commode?
9. (Katz Basic ADL) Does the participant need help in moving from bed to chair or requires a complete transfer?
10. (Katz Basic ADL) Does the participant need partial or total help with feeding or require parenteral feeding?
11. Does the participant have a documented intolerance to statins?
 - This can be determined by self-report, medical records, or communication with the participant's clinician or staff familiar with the potential participant's history.
12. Does the participant have active liver disease?
 - Participants who have had a liver transplant are not eligible to enroll.

13. Is the participant ineligible to take Atorvastatin 40mg?
 - This should be determined by the PI, participant's primary care clinician, or medical physician familiar with the participant's history.
 - See **Chapter 10: Study Drug** for a list of questions to consider when determining if it is appropriate for the participant to take the study drug.
14. Does the participant have a severe hearing impairment that impacts their ability to complete telephone follow-up?
 - This should be determined by participant self-report or the participant's trusted contact.
15. Is the participant unable to talk?
16. Does the participant have a severe visual impairment that prevents their ability to complete cognitive testing?
17. Is the participant unwilling or unable to provide a social security number?
 - Inform the participant we must collect their Social Security number to complete follow up of health outcomes. The MBI number should also be collected, but is not required. See **Section 4.4.4** below for more details about the SSN and MBI.

NOTE: Site staff will ask the participant the Katz ADL questions during the Screening and Baseline visit. The Call Center does not ask the questions again during the baseline call, but does ask the questions during annual phone visits.

If the participant triggers for a disability at the baseline or an annual follow-up call, the Call Center will ask the trusted contact the Functional Activities Questionnaire (FAQ), Katz ADL, and the Patient Reported Outcomes Measurement Information System – Physical Function (PROMIS PF) questions. For the annual follow-up call to confirm dependence on the Katz, the Call Center will try to reach the same trusted contact that was initially interviewed. If unsuccessful after multiple attempts, the alternate trusted contact will be called and the entire Katz ADL will be administered.

If a participant indicates they are dependent in any of the Katz Basic Activities of Daily Living (ADL), except urinary or bowel incontinence, the participant should not be enrolled in the study. To have an accurate assessment of baseline ADL and to assess participants for new persistent disability throughout the study, a question about urinary or bowel incontinence is included on the Eligibility Form (Is the participant partially or totally incontinent of bowel or bladder?). This should be determined by participant self-report or the participant's trusted contact. If the participant has urinary or bowel incontinence, they are still eligible for the study.

The answer to these 5 questions must be "yes" for a potential participant to be eligible for PREVENTABLE:

1. Did the participant sign the Informed Consent Form?
2. Is the participant community-dwelling?
 - Community-dwelling may include individuals who live in an assisted living or continuing care community. However, if the individual is unable to manage their own medications, they are not eligible to enroll in the study.
3. Is the participant's primary language English or Spanish?
4. Is the participant 75 or older?

- The study website will use the participant's date of birth to calculate their age.
 - If the participant is not 75 or older, they must wait until they turn 75 to sign the ICF.
5. Is the participant able to provide a trusted contact?

4.4.4 SOCIAL SECURITY NUMBER (SSN) AND MEDICARE BENEFICIARY IDENTIFIER (MBI)

Participants are required to provide their Social Security Number (SSN) to participate in the study. Although not required, site staff should also obtain each participant's Medicare Beneficiary Identifier (MBI). The SSN and MBI allow the PREVENTABLE study team to collect important health information and outcomes via EHR systems, the National Death Index, and Medicare databases.

In this pragmatic trial, we are following health records related to the outcomes in the study (i.e., ICD codes that link to key study outcomes). This information is primarily collected from each participant's EHR and does not require the use of their SSN. If a participant is enrolled by the telesite or is enrolled by a site from which a participant does not receive health care (i.e., the participant is not included in their EHR system), their SSN will help the study team continue collecting the participant's health information through Medicare or the National Death Index. The SSN can also help link records across health systems if a participant moves.

The SSN and MBI are sensitive information and are stored separately from all other study data. These identifying numbers should NOT be written on study forms and must only be entered directly into the study website. Participants' SSNs and MBIs are not visible to site staff or the project team after they are entered in the study website. This information is only accessible to the Data Coordinating Center staff on a need-to-know basis for the purpose of linking health records for follow up.

4.4.5 DISCONTINUING A PARTICIPANT WHO DECLINES TO PROCEED TO ENROLLMENT

If a participant signs the consent and then declines to provide the required information or changes their mind about participating prior to enrollment, site staff can select "Discontinue Participant" in the Baseline Visit Form on the study website as long as the "Enroll Participant" button has not been clicked. The "Discontinue Participant" button is not available once the "Enroll Participant" button is clicked.

Selecting the "Discontinue Participant" option will mark the PID as discontinued/closed and should only be used if a participant chooses not to enroll in the study after they have consented. If a participant is found to be ineligible between when they consent and when they are enrolled, site staff should **NOT** select "Discontinue Participant" and the reason for ineligibility should be noted in the Eligibility Form.

The screenshot displays a web interface for participant management. On the left, a vertical sidebar contains five buttons: 'Information' (highlighted in blue), 'Consent', 'Visit List', 'Enrollment', and 'Comments'. The main content area on the right is divided into two sections. The top section, titled 'LIPID SAMPLE SUBSTUDY:', shows a value of 'UNKNOWN'. Below this is a section titled 'ENROLLMENT STATUS'. Within this section, 'Current Status:' is displayed as 'INCOMPLETE' in a yellow box. A prominent blue button labeled 'DISCONTINUE PARTICIPANT' is visible. At the bottom of the status section, 'Date of Enrollment:' is shown as '--'.

APPENDIX 4.1 COHORT ASSESSMENT QUERY

The PREVENTABLE Cohort Assessment Query (CAQ) is a prescreening tool non-VA sites can deploy in their local EHR system to generate lists of potential participants. Sites may contact Jennifer Hervey (jennifer.hervey@duke.edu) at the CCC for questions about the CAQ. VA sites do not need to use the CAQ as they are provided with lists of potentially eligible participants.

The CAQ is made up of 2 related, but distinct, pieces:

1. Technical specifications to assist with EHR query development
2. SAS program for assessment of PCORnet Common Data Model (CDM) data

CAQ Technical Specifications for Local EHR Data

The CAQ technical specifications are provided as an Excel workbook containing logic and code lists for inclusion criteria, exclusion criteria, and additional “soft inclusion and exclusion” concepts. This information, along with the slide deck that provides strategies for using the CAQ, can help streamline the recruitment process. CAQ materials are available on the study website under Quick Links > Documents > Recruitment Resources > CAQ, EHR and REDCap Tools.

Codes for the concepts are provided in the Excel workbook.

CAQ SAS Program for PCORnet CDM Data

The CAQ SAS program is provided to non-VA sites that have standardized their EHR data to the PCORnet Common Data Model (CDM). This program is executed locally at each site and provides 2 key items:

1. A descriptive summary of patients who meet the eligibility criteria and who can be assessed in the CDM data
2. A CSV file that provides the PCORnet CDM Patient ID and summary information for all patients who meet the eligibility criteria

PATID	ELIGAGE7	F_MD_BABS	F_MD_FIBRATES	F_MD_NIACIN	F_MD_EZETIMIBE	F_DX_S_ANGINA	F_DX_SCI	F_DX_HF	AGE	BMI
12345	1	0	0	0	0	0	0	0	87	
12346	1	0	0	0	0	0	0	0	91	17
12222	1	0	0	0	0	0	0	1	110	
45070	1	0	0	0	0	0	0	0	88	
56123	1	0	0	0	0	0	0	1	79	
27896	1	0	0	0	0	0	0	0	110	

To use this patient list for recruitment, the site data team will need to link the PCORnet CDM Patient ID to a local identifier, such as the medical record number, and send the linked file to the site study team. Site staff can then review the linked file (a data dictionary is provided with the SAS program) in conjunction with clinic data (i.e., upcoming appointments) to identify a list of patients to screen.

APPENDIX 4.2 ELIGIBILITY AND BASELINE VISIT FORMS

4.2a ELIGIBILITY FORM

After the consent date is saved in the study database, the Eligibility Form is usually the first form that is entered in the study database. To enter the data from a printed copy of the form, in the study website click on Data tab, select Data Entry, then scan the barcode on the Eligibility Form. After the barcode is scanned, the form associated with that PID will open in the study database.

To enter the data from the printed form, select the gray Visit List tab on the left side of the participant's dashboard, then select CL00 – Baseline Clinic and click on "Eligibility." On the Eligibility Form in the Exclusion Criteria section, click the button labeled "Set all to NO" to automatically enter all answers as "No," if appropriate. If a question needs to be updated to "Yes (Stop, Ineligible)," find the question in the electronic Eligibility Form and update the answer(s) as needed. Scroll to the bottom of the Eligibility Form and manually enter the answers for the sections titled "Katz ADL, Not Exclusion Criteria" and "Inclusion Criteria."

Eligibility Version 1

Participant ID:	100001270
Barcode ID:	
Visit Code:	CL00
Date of Visit (MM-DD-YYYY)	09-02-2020
Administered By:	1004

Exclusion Criteria

Set all to NO

Clear

1	Does the participant have clinically evident CVD (prior MI, prior stroke/TIA, prior revascularization procedure, or secondary prevention indication for a statin as determined by their clinician)?	
---	---	--

KATZ ADL, NOT EXCLUSION CRITERIA		
17	Is the participant partially or totally incontinent of bowel or bladder?	
INCLUSION CRITERIA		
18	Did the participant sign the Informed Consent?	
19	Is the participant community dwelling (including assisted living or CCRCs; manage their own medication) ?	
20	Date of birth	

[SUBMIT](#)

See **Chapter 9: Randomization** and **Chapter 12: Data Management and Security** for details.

4.2b CONTACT INFORMATION FORM

Phone follow up, drug shipment, and participant retention depend on having accurate contact information for each participant, including their name, address, phone number, and email.

It is **highly recommended** that the Contact Information Form be completed in the study database while the participant is at the study site. To ensure mailing addresses are valid, the study website will automatically validate the address entered with the United States Postal Service (USPS) database. If the address is not valid according to USPS, a suggested mailing address will appear. If the Contact Information Form is entered after the participant leaves the site and the address is not valid, site staff are responsible for obtaining a valid participant mailing address.

2. What is your mailing address?

✓ Verified

✓ Verified

✓ Verified ✓ Verified ✓ Verified

[Click to validate mailing address before continuing:](#) [VALIDATE ADDRESS](#) [CLEAR ADDRESS](#)

If the participant spends more than 1 month each year at a different address (e.g., a vacation home), this should be entered as an “Alt Address” in the Contact Information Form, along with the typical time frame the participant lives at the alternate address. The alternate address can be swapped with the mailing address if the study drug needs to be shipped to the alternate address.

Please note that the Contact Information Form has a section for Expected Time at the Alternate Address. However, this field does not automatically update the mailing address for the time frame provided. The

study coordinator must manually swap the mailing addresses. Only the address listed as the “mailing address” is provided to the VA CSP Pharmacy for study drug shipments.

NOTE: This information is used to create mailing address labels. Please only enter information as it should appear on the mailing address.
(Non-VA Sites only) Please note this address will be used for shipment of the study drug.

2. What is your mailing address?

3620 CARDINAL RIDGE DR

 Swap with Alt Address

NOTE: If a VA participant’s mailing address needs to be updated, it should be changed in both the PREVENTABLE Contact Information Form and in the VA Computerized Patient Record System (CPRS). For VA participants, the VA CSP Pharmacy will only ship the study drug to the mailing address in the CPRS.

If a participant’s mailing and physical addresses are the same, select the box to populate the physical address fields with information from the mailing address.

Ask the participant if they have a preferred time to be contacted. Morning, afternoon, and evening options are available for all days. All options preferred by the participant should be selected on the form. If they do not have a preference, “anytime” can be selected.

Accurate contact information is critical for continued follow up and shipment of the study drug. The Contact Information Form should be completed in its entirety. these questions are required for enrollment and randomization in the study:

- Question 1: Name
- Question 2: Mailing address
- Question 3 (and 3a, if applicable): Physical address
- Question 5: Preferred telephone number
- Question 13: Name of trusted contact 1
- Question 14: Preferred phone number of trusted contact 1

Trusted Contact Information

Participants are asked to provide at least 1 trusted contact who can provide health information about the participant in case of emergency or if the participant cannot be reached with the available contact information. For a participant to enroll in the study, at least 1 trusted contact must be entered in the Contact Information Form. A trusted contact should be someone who has regular contact with the participant and can be someone with whom the participant lives. Participants cannot list themselves as a trusted contact.

4.2c SSN AND MBI FORM

To enter the SSN and MBI, select Data Entry from the Data Management menu, select your site (if multiple sites are listed). Navigate to the list of randomized participants and type the participant’s name or PID in the search bar. Click on the participant’s name to navigate to their dashboard. Then click on the SNN & MBI tab on the left side. The SSN and MBI must be entered twice to ensure accuracy. If the numbers do not match, the system will prompt the user to enter them again. If the number of digits and

pattern are appropriate, the system will save the information and indicate “Entered” for the numbers. If the wrong SSN or MBI is entered, resubmit the form with the correct numbers.

Participants’ SSN and MBI are not visible to site staff or the project team after they are entered. This information is only accessible to the Data Coordinating Center (DCC) staff on a need-to-know basis for the purpose of linking health records to follow up on participants.

The screenshot shows a web interface with a left sidebar containing buttons for 'Information', 'Consent', 'SSN & MBI' (highlighted in blue), and 'Visit List'. The main content area is divided into two panels. The left panel is titled 'Social Security Number' with a status 'Not Yet Entered' in orange. It contains two input fields: 'SSN:' and 'Confirm SSN:', followed by a green 'Save' button. The right panel is titled 'Medicare Beneficiary Identifier' with a status 'Not Yet Entered' in orange and an information icon. It contains two input fields: 'MBI:' and 'Confirm MBI:', followed by a green 'Save' button.

4.2d ENROLLMENT CHECKLIST

The Enrollment Checklist on the enrollment tab of the participant dashboard indicates whether the participant is eligible based on the data entered. At the end of the Baseline Visit, the Enrollment Checklist will indicate “Eligible,” “Ineligible,” or “Incomplete” for each of the forms related to the Baseline Visit. If all items on the checklist indicate “Eligible,” site staff will see a green “Enroll Participant” button. Site staff must click the “Enroll Participant” button for the participant to be considered enrolled in the study.

The screenshot shows the 'Enrollment Checklist' section of a participant dashboard. On the left is a sidebar with buttons for 'Information', 'Consent', 'SSN', 'Visit List', and 'Enrollment' (highlighted in blue). The main area is titled 'Enrollment Checklist' and contains a list of items, each with a category button, an 'ENROLLMENT STATUS' (all are 'ELIGIBLE' in green), and a 'Rule Details' button. The items are: 'Eligibility', 'Contact Information', 'Demographics', 'Consent', and 'Social Security Number'. Below this list is a section titled 'Enrollment' which contains a large green button labeled 'ENROLL PARTICIPANT'. A red arrow points to this button.

The “Enroll Participant” button does not appear if 1 or more items on the Enrollment Checklist are identified as “Ineligible” or “Incomplete.” If the assessment of eligibility is incomplete or the participant is ineligible, the reason for ineligibility will be identified on the Enrollment Checklist.

The screenshot shows a sidebar on the left with buttons for 'Information', 'Consent', 'SSN', 'Visit List', and 'Enrollment'. The 'Enrollment' button is highlighted in blue. The main area is titled 'Enrollment Checklist' and contains three rows, each with a form button, a status, and a 'Rule Details' button.

Form	ENROLLMENT STATUS:	Action
Eligibility	INELIGIBLE	Rule Details
Contact Information	ELIGIBLE	Rule Details
Demographics	INCOMPLETE	Rule Details

For details on the reasons for eligibility, ineligibility, or incompleteness, click “Rule Details” on the Enrollment Checklist to expand and review the enrollment criteria associated with each form.

This screenshot shows the 'Eligibility' row expanded. A red arrow points to the 'Rule Details' button. The expanded content shows a red box for an ineligible reason and green boxes for eligible reasons.

Form	ENROLLMENT STATUS:	Action
Eligibility	INELIGIBLE	Rule Details

INELIGIBLE - 1. Does the participant have clinically evident CVD (prior MI, prior stroke/TIA, prior revascularization procedure, or secondary prevention indication for a statin as determined by their clinician)?

ELIGIBLE - 2. Has the participant been hospitalized for a primary diagnosis of heart failure in the past 12 months? (History of heart failure without clinically evident cardiovascular disease is not an exclusion.)

ELIGIBLE - 3. Has the participant used a statin in the past 12 months?

ELIGIBLE - 4. Has the participant previously reported using a statin for more than 5 years?

Clicking the “Enroll Participant” button will automatically send the participant’s information to the Call Center to perform the baseline call. While the participant is in the clinic or on the phone for the baseline visit, schedule the baseline phone call with the Call Center at a time that is convenient for the participant. Follow the steps in the video on the study website under Training Videos > Training > Baseline Clinic Visit Walkthrough > Enrolling a Participant and Scheduling the Baseline Call. See **Chapter 7** for more details about scheduling the baseline call.

After the baseline call is completed, the participant will be randomized in the system over night.

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CHAPTER 5. BASELINE VISIT

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

5.1 OVERVIEW

All PREVENTABLE study sites leverage their electronic health record (EHR) system to identify and screen eligible participants, although these processes vary across VA and non-VA sites. All participants are assessed for eligibility, provided study-specific information, and complete the informed consent process.

- Non-VA sites use a Cohort Assessment Query (CAQ) or other local query to identify and screen for eligible participants. The CAQ is run on the site’s (EHR) to generate a list of potentially eligible participants based on the protocol’s inclusion/exclusion criteria. Sites may apply additional criteria to the CAQ outputs or to local query programming text to further refine their search or add local specifications such as participant identifiers, contact information, upcoming appointment dates, primary care clinician, etc.
- VA sites use a site-specific recruitment list provided monthly by the VA Coordinating Center (CC) via SharePoint. The VA CC will run a query in the VA Informatics and Computing Infrastructure (VINCI) system to generate a list of potentially eligible participants based on the protocol’s inclusion/exclusion criteria. VA sites may use Excel to sort their lists by participant identifiers, contact information, upcoming appointment dates, primary care clinician, etc. to meet their recruitment needs.
- Potential participants for both VA and non-VA sites, received via self-referral, Zoom sessions, or other sources, can be screened by the central support team at the tele-site. Interested participants who pass the initial screening will be referred to the closest site for enrollment or they can be enrolled by the tele-site. Participants enrolled by the tele-site will be followed by the tele-site.
- Sites that do not have Spanish-speaking staff can direct Spanish-speaking potential participants to the tele-site for screening and enrollment.

5.2 DATA COLLECTION

After providing their informed consent, participants will:

- Be screened for eligibility (includes Katz Activities of Daily Living)
- Confirm their medical history, demographics, and contact information
- Provide their SSN (and MBI, if available)
- Have blood drawn for the biorepository and lipid panel analysis (required for in-person baseline visits)
- Perform the Short Physical Performance Battery (expected for in-person baseline visits, but not required)
- Be scheduled for the baseline phone call with the Call Center
- Be scheduled for a 3-month lab visit if randomized to the lipid sub-study group

5.2.1 MEDICAL HISTORY

Participant medical history and medications are primarily collected from the EHR if available for a participant. However, some questions are verified with the participant. The only non-prescription medications collected are aspirin and nonsteroidal anti-inflammatory drugs. If multiple lab values are in the EHR, the site should provide the most recent values in the prior 6 months.

5.2.2 DEMOGRAPHICS

Participant demographics are collected from the EHR and/or the participant. To facilitate data-driven follow up, participants are required to provide their Social Security number (SSN) and, if available, the Medicare Beneficiary Identifier (MBI). If the participant is unwilling to provide their SSN, they will not be enrolled and will become a screen failure.

5.2.3 PARTICIPANT CONTACT INFORMATION AND TRUSTED CONTACTS

Each participant is required to provide a valid phone number and address for shipping the study drug. It is highly recommended that the Contact Information Form be completed while the participant is at the study site. The study website will automatically validate the address entered on the form with the United States Postal Service (USPS) database. If the address is not valid according to USPS, the system will suggest a mailing address. If the contact information is entered after the participant leaves the clinic and the address is not valid, site staff are responsible for obtaining a valid address for the participant.

Each participant is also required to provide the name and contact information for at least 1, but preferably 2, trusted contacts. Just as participants might experience some health issues over the next 4 to 5 years, the participant's trusted contacts may also have some health issues or life changes that might prevent the Call Center from being able to reach them. If the Call Center is unable to reach either of the trusted contacts, they will notify the site that the trusted contact information needs review. If the participant does not provide the name and phone number for at least 1 trusted contact, they will not be enrolled and will become a screen failure.

A trusted contact should be someone who is able to answer questions about the participant with firsthand knowledge. Participants often list their spouse or partner as a trusted contact. Other family members, such as children, are also typically knowledgeable about the participant's abilities, even if they don't live nearby. Close friends, a minister, or in some cases, the participant's primary care physician or nurse, see the participant frequently and can provide the needed information. Participants cannot list themselves as a trusted contact. If a participant triggers the extended battery on the participant cognitive call or if a participant is unable to complete an annual call, the Call Center will call the trusted contact to ask questions about the participant's ability to manage everyday tasks such as shopping for groceries, playing games, preparing meals, walking short distances, or climbing flights of stairs. The information collected from the trusted contact helps to verify information collected from the participant or helps to gather information about the participant if they are unable or unwilling to complete the annual call that year.

It is important that participants inform their trusted contacts that they may be contacted by the PREVENTABLE Call Center at Wake Forest University throughout the study. A handout is available for participants to give to their trusted contacts to learn about the study and the role of trusted contacts. The Trusted Contact Handout is available in English and Spanish on the study website under Login > Quick Links > Documents > Enrolled Participants Resources > Welcome Packet (for Enrolled Study Participants).

5.3 LIPID PANEL AND BIOREPOSITORY SAMPLES

Per the protocol, the study blood draw is required for in-person baseline visits. Blood samples are not required for participants enrolled by telehealth. However, sites should make every effort to obtain the samples in other ways prior to randomization such as during an in-person visit for routine care or by scheduling a “lab and leave” visit. Drawing clinical lab lipid panels on PREVENTABLE participants is discouraged to prevent unmasking the participant. All other routine clinical lab testing is permitted.

If the baseline blood draw is scheduled at a later date, the samples should be collected before the baseline call with the Call Center.

Site staff should add a comment in the Comments tab of the participant’s record in the study website so the Call Center is aware they should postpone the baseline cognitive assessment call until after the blood draw. This will delay the participant’s randomization. Shipment of study drug to the participant is triggered after the baseline call and randomization.

The Call Center baseline call must be completed within 45 days of the Eligibility Form being entered in the study website. If the call is completed after 45 days, site staff will receive an alert (see **Chapter 9: Participant Randomization** for details). Site staff will need to review all responses on the Eligibility Form to reconfirm eligibility and then update the date of the visit on the Eligibility Form to the confirmation date. This is to ensure that the baseline blood sample is collected prior to the participant receiving and starting study drug.

Participants should not be billed for lab services. The study site should bill their fund code for research phlebotomy services. If a study coordinator is trained and qualified per institutional requirements, they can draw the sample. Because there are specific tube, processing, and storage requirements for the baseline blood samples, labs for PREVENTABLE cannot be drawn at an external lab. See **Chapter 6: Biorepository and Lab Procedures** for details.

5.4 ACTIVITIES OF DAILY LIVING AND SHORT PHYSICAL PERFORMANCE BATTERY

5.4.1 ACTIVITIES OF DAILY LIVING (ADL)

The Katz Index of Independence in Activities of Daily Living (ADL) is used to assess a participant’s ability to perform specific tasks for daily living and is incorporated into the Eligibility Form. The responses are scored based on ability to perform the task either independently or with supervision or assistance. To be eligible, a participant must be independent in all ADLs with the exception of urinary or bowel incontinence. See **Chapter 4: Screening and Enrollment** for details on study criteria and the Eligibility Form.

5.4.2 SHORT PHYSICAL PERFORMANCE BATTERY (SPPB)

The SPPB is expected to be performed for in-person baseline visits, but is not required. For participants enrolled by telehealth, the SPPB may be obtained separately in-person prior to enrollment, but is not required. The SPPB cannot be performed virtually.

The SPPB provides an objective assessment of functional limitation, defined as a restriction in basic physical actions. Details for collecting the SPPB are available in this chapter and on the study website

under Login > Quick Links > Documents > Training > SPPB Certification Materials. SPPB certification is required for site staff collecting SPPB data. The certification process is described below.

If site staff have not been certified to administer the SPPB, the SPPB Form will not appear in the database on the study website for that site. If a study team member who is not certified to administer the SPPB attempts to complete an SPPB Form, they will receive a warning message stating the identified user is not currently certified to administer the SPPB.

5.4.3 SPPB INITIAL CERTIFICATION

PREVENTABLE has established a standardized pragmatic approach for data collection, administration, and delivery of the SPPB. Site staff conducting the SPPB are required to complete training to become proficient in the tests and receive study certification for assessing and reporting participants' functional status. To become certified to administer the SPPB, site staff are required to:

- Review **Appendix 5.1** in this chapter of the MOP.
- Review the SPPB Initial Certification Checklist on the study website under Login > Quick Links > Documents > SPPB Certification Materials.
- Review the SPPB Form with script under Documents > Training > SPPB Certification Materials.
- Watch the instructional videos under Training Videos > Training > SPPB and Geriatric Assessments Trainings > SPPB Certification **Training** Videos (Initial and Recertification).
- Practice administering the SPPB with a volunteer to become proficient.
- View pre-recorded videos of mock administrations of each component of the SPPB under Training Videos > Training > SPPB and Geriatric Assessments Trainings > SPPB Certification **Timing** Videos (Initial and Recertification). Record the times for each video of the components on the SPPB Certification Checklist under Documents > Training > SPPB Certification Materials > SPPB Initial Certification Checklist: 3 balance stand tests, 3 four-meter walks, and 3 chair stand tests.
- Record a video (a phone recording is acceptable) of a single SPPB administration on a volunteer and complete the SPPB Form, or schedule a virtual meeting with the SPPB Certification Project Manager listed on the study website under Directory > Who To Contact to demonstrate the SPPB administration.
- Request access to the PREVENTABLE SharePoint site by clicking "Submit a RAP" under Quick Links in the study website menu. A link to the SharePoint site will be sent when access is granted.
- Upload 3 items to the PREVENTABLE SharePoint site for the Coordinating Center to review:
 - SPPB Initial Certification Checklist with recorded times
 - Video of the SPPB administration
 - Completed SPPB Form
- Certification requires (1) timing of the SPPB components in the videos to be within $\pm 5\%$ of the actual time, and (2) an acceptable video of the administration of the SPPB. See **Appendix 5.2** for the Coordinating Center Certification Review Checklist.

All required certification materials must be uploaded to the PREVENTABLE SharePoint site.

Site staff will be certified to administer the SPPB when the above proficiency is demonstrated.

5.4.4 SPPB ANNUAL RECERTIFICATION

It is recommended that site staff perform at least 3 SPPB tests a month. SPPB tests should be done on volunteers if needed to maintain proficiency. Recertification is conducted on an annual basis for quality control purposes. Site staff will receive an email when they are due for recertification. The recertification process is very similar to the initial certification process and requires site staff to:

- View the appropriate mock administrations of each component of the SPPB on the study website under Training Videos > Training > SPPB and Geriatric Assessments Trainings > SPPB Certification Timing Videos (Initial and Recertification) and record the times for each component on the SPPB Recertification Checklist under Documents > Training > SPPB Certification Materials. Timing of the SPPB components must be within $\pm 5\%$ of the actual time.
- Record a video of a single SPPB administration on a volunteer and complete the SPPB Form, or schedule a virtual meeting with the SPPB Certification Project Manager listed on the study website under Directory > Who To Contact to demonstrate the SPPB administration.
- Upload 3 items to the PREVENTABLE SharePoint site (the link will be provided in the recertification notification email) for the Coordinating Center to review:
 - SPPB Recertification Checklist with recorded times
 - Video of the SPPB administration
 - Completed SPPB Form

If the site staff member is adhering to the PREVENTABLE physical performance protocol based on satisfactory completion of above tasks, recertification will be awarded. If during the recertification process it is determined that the site staff member is not adhering to the protocol, the Coordinating Center will request a virtual retraining session.

5.5 ENROLLED PARTICIPANT MATERIALS

5.5.1 PARTICIPANT HANDOUTS

Each participant receives materials during the baseline clinic visit that describe what to expect as a PREVENTABLE participant. If the baseline visit is conducted via telehealth, site staff will mail or email (non-VA sites only) the Welcome Packet to the participant. These documents, available in English and Spanish, are available on the website under Documents > Enrolled Participants Resources > Welcome Packet (for Enrolled Study Participants). The materials include information on the next steps for enrollment and scheduled visits, the Call Center, the study drug, cognitive testing, and a letter for trusted contacts. Site staff should add the study team contact and phone number to the wallet card before giving it to a participant.

5.5.2 STUDY DRUG INFORMATION

A study drug information sheet is included in each shipment that describes how the study drug should be taken, common side effects, and what to do if the participant experiences side effects. The document is printed with English on one side and Spanish on the other side. Additionally, a phone number for the site is printed on the study drug bottle label. The site phone number is the “Contact Information for Enrolled Participants” number listed in on the study website under Directory > Site Contacts.

5.5.3 BROCHURE

Sites may want to provide a study brochure or other sIRB-approved materials to participants when they are considering consenting to the study or after they consent. The brochure includes basic information about the study background, scientific question, and participant procedures. This resource and others are available in English and Spanish as printed and digital documents.

The digital brochure is available on the study website under Documents > Recruitment Resources > Participant Materials > Community Resources. It is also available on the public-facing study website (<https://www.preventabletrial.org/home.cfm>). Sites may order printed brochures on the PREVENTABLE storefront: <https://eprint.universalprinting.com/Preventable> (see **Chapter 3** for details). A link to the storefront is also posted on the study website in the Directory under Who To Contact.

Study Coordinators should ensure that an approval letter is on site and filed in their official study record for all IRB-approved documents their site is using. VA sites should confirm cIRB approval as well.

5.6 BASELINE CALL TO ASSESS COGNITIVE AND PHYSICAL FUNCTION

Enrollment is complete after the participant has consented, baseline visit forms are completed, and a site staff member clicks the “Enroll Participant” button in the participant’s study record on the study website. After the “Enroll Participant” button is clicked, a “Schedule Baseline Call” button will appear for the site to schedule the call. Before a participant can be randomized, they must complete the baseline call with the Call Center. When scheduling the baseline call with the participant, confirm their phone is correct and working and let them know that when the Call Center calls, the caller ID will display 336-716-9997 or 336-721-7722, and, depending on their phone carrier, the caller ID may say “Winston-Salem, NC” or “Wake Forest.” See **Chapter 9: Randomization** for details.

Participants who cannot be reached by the Call Center or who do not complete the baseline cognitive assessment phone call will not be randomized. This includes situations where the Call Center determines that the cognitive assessments cannot be completed with a participant due to their inability to hear adequately on the phone or their inability to follow the questions asked in each assessment.

If the participant is unable or unwilling to complete the baseline call, site staff will receive an email and an alert in the study website notifying them of a Call Center Visit status of “Back to Clinic.” Site staff should contact the participant to troubleshoot the failed assessment phone call to check if it was a scheduling issue or technology problem that can be rectified. In all instances, sites are encouraged to call “Back to Clinic” participants to troubleshoot the issue, and if the problem cannot be resolved, thank them for their willingness to participate in the PREVENTABLE study.

APPENDIX 5.1 SHORT PHYSICAL PERFORMANCE BATTERY (SPPB)

5.1a SPPB OVERVIEW

The SPPB is expected to be performed for in-person baseline visits, but is not required. For participants enrolled by telehealth, the SPPB may be obtained separately in person prior to enrollment, but is not required.

The SPPB has 3 components: the balance tests, the gait speed test, and the chair stand test. The tests must always be performed in this order. To ensure reproducibility, it is imperative that all participants are given the same instructions and that the measurements associated with the tests are made in a uniform manner. The examiner should follow the script on the SPPB Form when giving instructions to the participant.

To complete the SPPB, the participant must be able to stand unassisted, walk (cane or walker acceptable if needed), and stand from a chair. If a specific movement is not attempted, the examiner should select one of the options on the SPPB Form to indicate why no attempt was made. The responses of “Not attempted, examiner felt it would be unsafe” and “Not attempted, participant felt unsafe” are primarily referring to balance issues that may make the participant a fall risk when conducting the tests.

It is critical, and sometimes very difficult, to distinguish between someone who is unable and therefore unwilling to try a test and a person who simply refuses to do the test because they are not interested in participating. This is a judgment that rests with the examiner. If a participant refuses to try any of these tests, mark “Participant refused” on the form unless it is clear they are refusing because they cannot do the test or are afraid to attempt it. Refusals are considered missing data. **Knowing a participant is unable to perform one of the tests is valuable information.**

5.1b SPPB EQUIPMENT

The following equipment is needed to administer the SPPB:

- SPPB Form with script on the study website under Documents > Trainings > SPPB Certification Materials
- Stopwatch
- Masking tape
- Measuring tape with 4 meters (157.48 inches or 13 feet and 1.48 inches) in length marked
- A straight-backed, armless chair with a hard seat. The chair seat should be between 17 inches and 18 inches high, and the depth of the seat should be between 18 inches and 19 inches. If this type of chair is not available, you may use a chair with a softer seat or a chair with arms. Do not use a folding chair, a soft chair, a deep chair, or a chair on wheels. The participant’s feet must rest comfortably on the ground while they are seated in the chair.

5.1c BALANCE TESTS

The first component of the SPPB is the balance testing. The balance tests assess the participant’s ability to hold 3 different standing positions with their eyes open. Only a stopwatch and the SPPB Form with the script are needed for the balance tests.

The 3 balance tests must be performed in this order:

1. Side-by-Side Stand
2. Semi-Tandem Stand
3. Tandem Stand

If you cannot steady the participant with support, **do not** try the balance test. Select “Not attempted” and mark “Not attempted, examiner felt it would be unsafe” for the reason. This applies to all 3 balance tests.

SIDE-BY-SIDE STAND



The first position tested is the side-by-side stand. The participant is asked to stand with their feet together in a side-by-side position for 10 seconds.

If the participant holds the side-by-side stand for 10 seconds, move on to the semi-tandem stand. If the participant holds the stand for less than 10 seconds, stop the balance testing, and move on to the gait speed test.

If the participant does not attempt this test, mark the reason on the SPPB Form, stop the balance testing, and move on to the gait speed test.

SEMI-TANDEM STAND



The second position tested is the semi-tandem stand. The participant is asked to stand with the side of the heel of 1 foot touching the big toe of the other foot for 10 seconds. Either foot can be placed in the forward position.

If the participant holds the semi-tandem stand for 10 seconds, move on to the tandem stand. If the participant holds the stand for less than 10 seconds, stop the balance testing, and move on to the gait speed test.

If the participant does not attempt this test, mark the reason on the SPPB Form, stop the balance testing, and move on to the gait speed test.

TANDEM STAND



The third and final position evaluated in the balance tests is the tandem position. The participant is asked to stand with the heel of 1 foot in front of and touching the toes of the other foot for 10 seconds. Either foot can be placed in the forward position.

If the participant does not attempt this test, mark the reason on the SPPB Form and move on to the gait speed test.

TO ADMINISTER THE BALANCE TESTS

1. Set the stopwatch to zero.
2. Follow the script on the SPPB Form and demonstrate the appropriate stand for the participant.

3. Place the participant close enough to a wall, counter, or table where they can reach out to it for support.
4. Ask the participant to put their feet in the correct position. If needed, lightly hold the participant's arm to help stabilize them while they put their feet in the correct position. Many people will need little or no support to help them into position, but others will need strong support until they are in position and steady.
5. When the participant is steady, ask "Are you ready?" When they say yes:
 - a. Release their arm, if providing support
 - b. Begin timing as you say, "Ready, begin"
6. Continue timing until the participant steps out of position; grabs your arm; touches the wall, counter, or table for support; or 10 seconds have passed.
7. Record the result on the SPPB Form. Record any time less than 10 seconds to the nearest hundredth of a second. The time does not need to be measured exactly if the participant holds the position for more than 10 seconds.

If the participant holds the stand for 10 seconds, move on to the next stand. If the participant holds the stand for less than 10 seconds, stop the balance testing and move on to the gait speed test.

BALANCE TEST TIPS

- Participants taking the balance tests **must be able to stand unassisted without using a cane or a walker**. Do not assume that a participant using a cane, walker, or wheelchair cannot stand unassisted. Ask the participant if they can stand without the device and if they are willing to try the balance tests. If they say yes, you can help them stand up to assume the correct position for testing.
- For each balance test, first describe and then demonstrate the appropriate position of the feet.
- Place the participant close enough to a wall, counter, or table where they can reach out to it if needed, especially for the harder balance tests (e.g., tandem stand).
- During each of the balance tests, the examiner should stand next to the participant as they get into position and then step back a half step as they perform the test. Do not stand so close to the participant that they have trouble using their arms to keep their balance. However, it is important that you stay close enough that the participant can grab your arm for support or you can quickly provide support if they begin to lose their balance. Based on prior experience, most participants simply step out of position if they lose their balance.
- Participants who cannot put their feet together due to lower body obesity or swelling should try to get their feet as close together as possible for the side-by-side stand. Similarly, they should try to get the heel of 1 foot as close as possible to the big toe of the other foot for the semi-tandem stand, and their feet as close as possible to a straight line with one foot in front of the other for the tandem stand.
- **Do not coach or provide encouragement during the balance tests**, e.g., "You only have 5 seconds to go."
- Do not continuously watch the stopwatch during the tests – watch the participant and be prepared to stop the stopwatch when the participant steps out of position, grabs your arm, or 10 seconds have passed. Glance at the stopwatch occasionally to check the time.

- It is not important to stop the stopwatch and the test at exactly 10 seconds. The time does not have to be measured exactly if the participant holds the position for more than 10 seconds.
- Record any time less than 10 seconds to the nearest hundredth of a second.
- If the participant cannot hold an easier stand (e.g., side-by-side or semi-tandem), skip the rest of the balance tests and move on to the gait speed test.
- If the participant does not attempt a balance test, mark the reason on the SPPB Form, stop the balance testing, and move on to the gait speed test. This applies to all 3 balance tests.

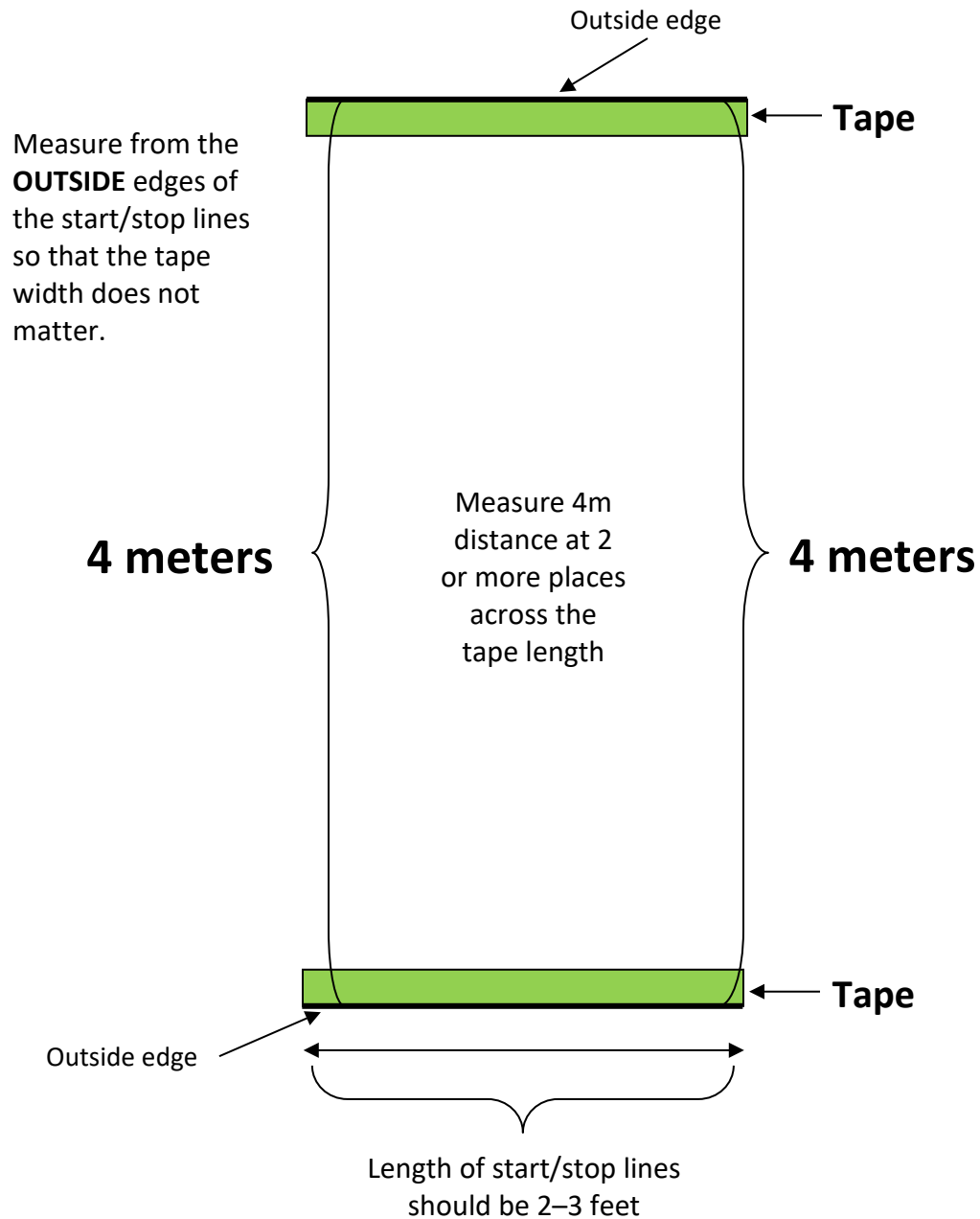
5.1d GAIT SPEED TEST

The second component of the SPPB is the gait speed test. This test assesses the participant's ability to walk 4 meters. A stopwatch, the SPPB Form with script, and a marked walking course are needed for this test. It is preferred that the course be laid out on a hard surface, but carpet is acceptable if it is the only surface available. Follow the instructions below for laying out the course.

To prepare the walking course for the gait speed test:

1. Use a tape measure and find the mark that indicates 4 meters (4 meters = 157.48 inches or 13 feet and 1.48 inches).
2. Make sure the walking course is unobstructed and includes at least an extra one-half meter (~18 inches) on each end.
3. Lay the tape measure down and tape one end to the floor. Tear off a 2- to 3-foot piece of masking tape and place it directly under the mark that indicates the starting line.
4. Place another piece of masking tape of similar length under the 4-meter mark. Measure the distance from the **outside edges** of the start/stop lines so the tape width does not matter (**see the Course Layout diagram on the next page**).
5. Check that the distance is 4 meters at multiple points across the length of the start/stop lines.
6. After the start/stop lines are in place, remove the tape measure and the course is ready for use.

COURSE LAYOUT FOR THE GAIT SPEED TEST



TO ADMINISTER THE GAIT SPEED TEST

1. Follow the script on the SPPB Form and demonstrate the walk for the participant.
2. Ask the participant to stand with their toes of both feet touching the starting line and, when instructed to begin, **walk at their usual speed**, as if they were walking down the street to go to the store.
3. Start timing on the stopwatch when the participant lifts their foot to begin walking (see **Figure 5.1. Start Gait Speed Test** – this is when their foot starts to move across the starting line). Do not start the stopwatch when you say “Ready, begin.” Wait until the participant actually begins to move before starting the stopwatch.
4. Stop timing when the heel of the first foot completely crosses the end line, not when the heel touches the ground (see **Figure 5.2. End Gait Speed Test** – notice that the foot is still in the air as it crosses the 4-meter mark). If the participant’s first foot lands on the end line but does not cross it, it is not the end of the test. You need to anticipate when the first foot will completely cross the 4-meter mark and be ready to stop timing as it completely crosses the line.
5. Record the time to the nearest hundredth of a second on the SPPB Form.



Figure 5.1. Start Gait Speed Test

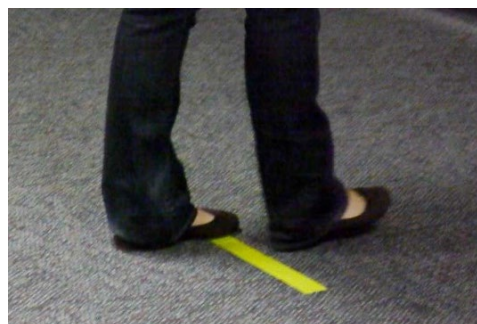


Figure 5.2. End Gait Speed Test

If the participant did not attempt or complete the first gait speed test, select the reason on the SPPB Form and move on to the chair stand test.

If you have trouble with the stopwatch or you think that the timing was not accurate, do not record the result and repeat the first gait speed test. A repeated gait speed test due to error in timing does not count as the second gait speed test.

If the participant completes the first gait speed test, ask them to complete a second gait speed test.

To administer the second gait speed test, reset the stopwatch and repeat steps 1–5 above.

If the participant does not attempt or complete the second gait speed test, select the reason on the SPPB Form and move on to the chair stand test.

If you have trouble with the stopwatch or you think that the timing was not accurate, do not record the result and repeat the second gait speed test.

GAIT SPEED TEST TIPS

- Participants who typically use a cane or walker may use the device during the gait speed test. Ask the participant if they ever walk at home without the device and if they think they can walk a short distance without the device for the test. If the participant can walk short distances without it, they should be encouraged to do so for gait speed test.

Doing the test without the device provides a much more accurate assessment of the functional limitations of the participant. Participants who normally use assistive devices should be watched very closely during the test to prevent falling. If an assistive device is used during the test, indicate that on the SPPB Form.

- Participants wearing high heels should change into another pair of shoes or go without shoes to perform the gait speed test, if possible.
- The best position for the examiner to maintain during the walk is to the side and slightly behind, outside of the participant's visual field. The position of the examiner is critical for the walk. If you are too close, you will set the pace. If you are too far behind, you will not be in a good position to provide support if the participant loses their balance. You also need to be in a good position to see the participant's foot crossing the finish line.
- **Timing begins when the participant lifts their foot to begin walking and ends when the heel of the first foot completely crosses the end line**, not when the heel touches the ground. If the participant's first foot lands on the end line but does not cross it, it is not the end of the test.
- Do not coach or encourage the participant during the gait speed tests.
- If you have trouble with the stopwatch or think the timing was not accurate, do not record the result and repeat the gait speed test. A repeated gait speed test due to error in timing does not count as the second gait speed test.

5.1e CHAIR STAND TEST

The third and final component of the SPPB is the chair stand test. This test assesses the participant's ability to stand up from a chair without using their arms. To perform this test, you will need a stopwatch, the SPPB Form with script, and a straight-backed, armless chair with a hard seat placed against a wall. If this type of chair is not available, a chair with a softer seat or a chair with arms may be used.

TO ADMINISTER THE SINGLE CHAIR STAND TEST

1. Follow the script on the SPPB Form and demonstrate the single chair stand for the participant.
2. Ask the participant to fold their arms across their chest and sit with their feet on the floor.
3. Ask them to try to stand up from the chair while keeping their arms folded across their chest. If the participant is unable to stand without using their arms, ask them to try to stand up using their arms and then end the test.
4. Record the result on the SPPB Form.

If the participant is able to stand up from the chair once without using their arms, they are then asked to **stand up and sit down 5 times as quickly as possible** for the repeated chair stand.

TO ADMINISTER THE REPEATED CHAIR STAND TEST

1. Follow the script on the SPPB Form and demonstrate the repeated chair stand quickly to emphasize this test should be done as quickly as possible.
2. Ask the participant to fold their arms across their chest and sit with their feet on the floor.
3. Ask the participant to stand up straight and sit back down as quickly as they can 5 times, without stopping in between, once you say "Ready, stand."

- If the participant does not stand up or sit down completely, stop the test and re-demonstrate.
- 4. Begin timing when you say, “Ready, stand.” This contrasts with the gait speed test where timing starts only when the participant begins to move.
- 5. Count out loud each time the participant stands up, up to 5 times. Count the stand number only after the participant has straightened up completely. Do not pace the test with your counting.
- 6. End the timing when the participant straightens their body completely on the fifth stand.
- 7. Record the result on the SPPB Form.

Watch the participant closely and stop the repeated chair test if the participant:

- Becomes tired or short of breath
- Uses their arms
- Has not completed all 5 chair stands after 1 minute

Stop the test at your discretion if, for any reason, there are concerns about the participant’s safety.

If the participant stops and appears to be tired before completing the 5 stands, ask if they can continue.

If the participant says “yes,” continue timing. If the participant says “no,” stop the test.

CHAIR STAND TIPS

- Although the gait speed test and single chair stand are done at usual speed, the repeated chair stand test should be done as quickly as possible.
- For efficiency, it is helpful to have 2 chairs available so that you can demonstrate the chair stand tests while the participant sits in the other chair and watches.
- To ensure safety, stand in front of the participant and be prepared to catch them if they fall forward. However, do not stand so close that the participant feels crowded and slows their pace during the chair stand test.
- **Timing begins as soon as you say, “Ready, stand” and ends when the participant straightens completely on the fifth stand.**
- Do not coach or encourage the participant during the test.
- Stop the test at your discretion if, for any reason, there are concerns about the participant’s safety.
- When training to administer the repeated chair stand test, it is good practice for 2 or more people to time the test so the times can be compared for precision.

APPENDIX 5.2 COORDINATING CENTER CERTIFICATION REVIEW CHECKLIST SPPB

PREVENTABLE Coordinating Center Certification Review Checklist

Short Physical Performance Battery (SPPB)

Name

Site

During Procedure:

1. ☐ Ensures participant's safety at all times.

Observes the Following Procedural Steps:

- 2. ☐ Assembles proper materials and equipment: Stopwatch, masking tape, tape measure, script, score sheet, and a straight-backed chair with a hard seat.
- 3. ☐ Properly sets up course layout for the gait speed test.
- 4. ☐ Greets participant and reviews procedures.
- 5. ☐ Ensures that participant is wearing proper footwear, i.e. not high heels.
- 6. ☐ Performs tests in their proper order.
- 7. ☐ Provides instructions exactly as they are written in the script.
- 8. ☐ Properly demonstrates each maneuver.
- 9. ☐ Ensures that participant understands the instructions.
- 10. ☐ Provides explanation when a specific maneuver is not attempted.
- 11. ☐ Positions participant correctly for each maneuver.
- 12. ☐ Positions himself/herself correctly for each maneuver.
- 13. ☐ For balance tests, stabilizes participant until feet are in correct position.
- 14. ☐ Times the balance tests correctly.
- 15. ☐ Properly determines whether cane or walker should be used for gait speed test.
- 16. ☐ Instructs participant to walk at their usual pace.
- 17. ☐ Times the gait speed test correctly (first and second trials).
- 18. ☐ Records whether walking aid was used.
- 19. ☐ Positions chair properly against wall.
- 20. ☐ Determines whether participant feels safe standing from chair.
- 21. ☐ Determines whether participant feels safe standing from chair with arms folded.
- 22. ☐ Records whether participant was able to stand from chair without the use of their arms.
- 23. ☐ Determines whether participant feels safe standing from chair five times without using their arms.
- 24. ☐ Instructs participant that five chair stands should be done as quickly as possible.
- 25. ☐ Times the repeated chair stand test correctly.
- 26. ☐ Stops the repeated chair stand test properly, i.e. participant has to use their arms, is unable to complete the test after one minute, or stops and is unable to continue.

Comments: _____

Reviewer: _____ Date reviewed: _____

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CHAPTER 6. BIOREPOSITORY AND LAB PROCEDURES

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

6.1 OVERVIEW

This chapter provides PREVENTABLE sites with standardized steps to carry out the collection, processing, storage, and shipment of participant blood samples for the Mayo Clinic Biorepository and the ICON central laboratory. Frequently Asked Questions (FAQs) for Biorepository and Lab Procedures are available on the study website under Login > Quick Links > Documents > Study Documents and Resources > Quick References and FAQs. ICON-specific FAQs are available in the iSite User Guide under Login > Quick Links > Documents > Vendors > ICON Lab.

For in-clinic baseline visits, participants will have blood drawn for lipid panel analysis and biorepository storage. Additionally, 2000 participants will be randomly selected for the lipid sub-study and asked to give a blood sample for lipid panel analysis at 3 months post randomization. Drawing clinical lab lipid panels on PREVENTABLE participants is discouraged to prevent unmasking the participant.

Site staff are required to complete the applicable training for processing biorepository and lipid samples. Completed training must be documented on the site’s training log. Training materials are available on the PREVENTABLE website:

1. **Slides:** Login > Quick Links > Documents > Training > Lab/Blood Collection and Shipping Procedures > Lab and Biorepository Training Slides May 2021
2. **Video:** Login > Quick Links > Training Videos > Training > Biorepository and Lab Procedures

Please refer to the PREVENTABLE Directory on the study website for who to contact regarding questions related to the Mayo Biorepository supplies, Mayo Biorepository process questions, and ICON Baseline and 3-month samples or supplies.

6.1.1 IMPORTANT REMINDERS

Collecting Samples

- Always use the correct ICON collection kit and Mayo Biorepository cryovials and caps for the specific visit and specimen type.
- Use only 1 collection kit per participant study visit.
- DO NOT use expired ICON kits. The expiration date is located on the collection kit label.
 - Each month, ICON emails a Kit Expiration Report with all kits expiring soon to all site staff with an iSite account. Sites should review these monthly reports and maintain their inventory so that kits with the earliest expiration dates are used first.
 - Destroy expired and unused ICON kits according to your site’s hazardous waste disposal policy and file documentation.

Shipping Samples

- Notify FedEx at least 1 day before the desired pickup date for frozen shipments.

- To avoid unnecessary queries, do not include the ICON requisition form in shipments of study samples. The requisition form is not used for the PREVENTABLE study.
- **Always include a printed copy of the shipping manifest in each box.**

6.1.2 FREQUENTLY USED WEBSITES

The following websites are needed to carry out the biorepository and lab procedures.

- **ICON Laboratory Services iSite Portal**: to order baseline and 3-month follow-up collection kits and other supplies for collecting and shipping samples. Please keep in mind the kits are costly and only order kits as needed.
- **Mayo Clinic Research Client Portal**: to order shipping boxes, the only biorepository supplies available to order from this portal.
- **PREVENTABLE Storefront**: to order supplies for the Mayo Biorepository. See **Chapter 3, Section 3.6** of the MOP for details about ordering supplies from the storefront.
 - 1.0 mL Thermo Scientific 2D barcoded cryovials are shipped in 96-count racks that sites must use to store the cryovials in their –70°C or –80°C freezer. Cryovial Racks cannot be ordered separately from cryovials.
 - Bulk screw-on tube caps available in natural, purple, and blue.
- **PREVENTABLE Website**: to access study information, including site locations, recruitment materials, and participant FAQs. Training materials, study forms, and the study database are available on the [password-protected part of the study website](#).

More information about these websites is provided in the related sections throughout this chapter.

6.1.3 ICON LAB MANUAL AND INVESTIGATOR SITE RESOURCE CENTER

Please review the **ICON Lab Manual** for a comprehensive guide to the central lab requirements for the PREVENTABLE protocol, including information and instructions on:

- Laboratory contacts
- Laboratory supplies
- Specimen collection procedures
- Specimen packaging
- Specimen shipping

The **ICON Lab Manual** is available on the PREVENTABLE website under Login > Quick Links > Documents > Vendors > ICON Lab.

The [ICON Investigator Site Resource Center webpage](#) provides additional helpful information such as:

- Printable copies of all laboratory licenses and certifications – Current certificates are sent to sites with initial shipments. Please check the website for updated certificates as needed.
- Frequently asked questions
- Training videos for common collection procedures
- Packaging instructions for ICON shipping boxes
- Holiday schedule for ICON Laboratory Services

6.1.4 GLOSSARY

Batch Shipping: Shipping a group of processed (frozen) samples from multiple participants collected over several weeks or months to the central lab (ICON) or the biorepository (Mayo Clinic).

White Blood Cells (WBCs): A specimen layer observed between the top plasma layer and bottom red blood cell layer in the lavender-top EDTA tube after centrifugation.

Red Blood Cells (RBCs): Also known as erythrocytes, these cells compose the bottom most layer in both the red-top serum and lavender-top EDTA tubes after centrifugation.

Buffy Coat: The buffy coat is the fraction of an anticoagulated blood sample that contains most of the white blood cells and platelets following density gradient centrifugation of the blood.

Temperature Definitions

- Ambient/Room Temperature: 19°C to 23.5°C
- Refrigerated: 2°C to 8°C
- Frozen: -65°C to -90°C

6.2 ORDERING ICON LAB SUPPLIES AND MAYO BIOREPOSITORY SUPPLIES

6.2.1 ICON LAB SUPPLIES

The first shipment of ICON kits will be initiated near site activation. Shipments of additional supplies are managed at the site level. Sites should monitor their inventory closely and re-order supplies when needed using the [iSite Portal](#).

*****If an urgent order is required, please [contact ICON Laboratory Services directly](#).*****

6.2.1.1 ISITE PORTAL

NOTE: All applicable site staff are required to register with iSite.

[iSite](#) is the ICON Laboratory Services secure web portal for clinical research sites. For security, only email addresses on file with ICON are able to register with iSite.

To request iSite access, send an email to the ICON Project Manager (as specified in the PREVENTABLE Directory on the study website) and the ICON site services team at LabSiteHelp@iconplc.com. In the email, include the name of the new user and their email address, the study name, and the site number.

For more details about the portal, see the iSite User Guide on the PREVENTABLE website under Login > Quick Links > Documents > Vendors > ICON Lab. A User Request Form to facilitate requests for new iSite accounts is also available on the study website.

Email LabSiteHelp@iconplc.com for any issues with iSite.

6.2.1.2 ICON COLLECTION KITS

Please see **the ICON Lab Manual, Section 4: Laboratory Supplies** for more information on ICON supplies and collection kits and **Section 6: Specimen Collection and Labeling Instructions** for a Specimen

Collection Quick Reference Guide. The **ICON Lab Manual** is available on the PREVENTABLE website under Login > Quick Links > Documents > Vendors > ICON Lab.

Collection Kit Name	Use For	Kit Contents	Initial Quantity
Baseline	Baseline visits only, as indicated on the kit label	- Barcode labels - Collection materials 1 K3 EDTA tube and 1 SST tube - Transport tube	50
3 Month FU	3-month follow-up visits only, as indicated on the kit label	- Barcode labels - Collection materials 1 SST tube - Transport tube	25
Unscheduled	- Retesting for a specific visit - As a substitute for a scheduled visit kit, only when a visit-specific kit is not available - Unscheduled kits should not be included in future shipments.	- Barcode labels - Collection materials 1 K3 EDTA tube and 1 SST tube - Transport tube	3
NOTE: Discard the requisition form included in collection kits; it is not used for the PREVENTABLE study. If the form is included in shipments, sites may receive unnecessary queries from ICON.			

Figure 6.1. Baseline Collection Kit Contents

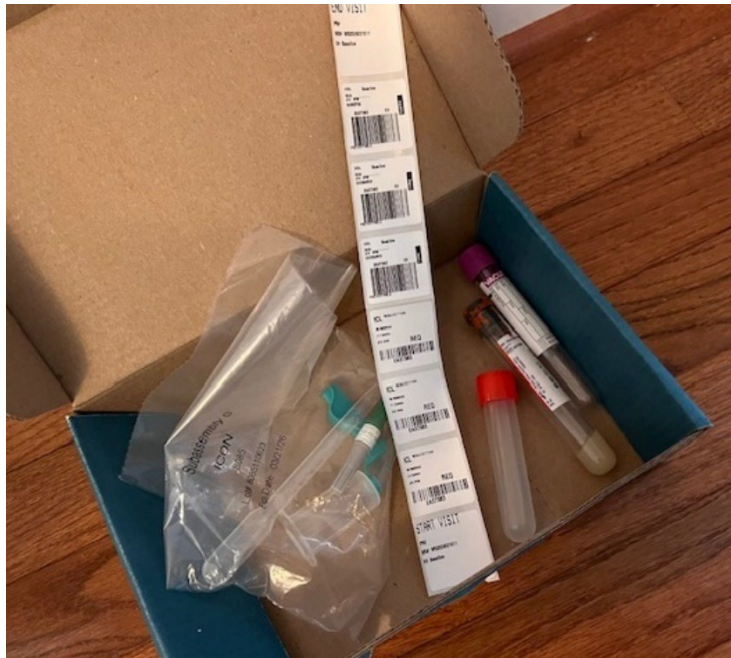
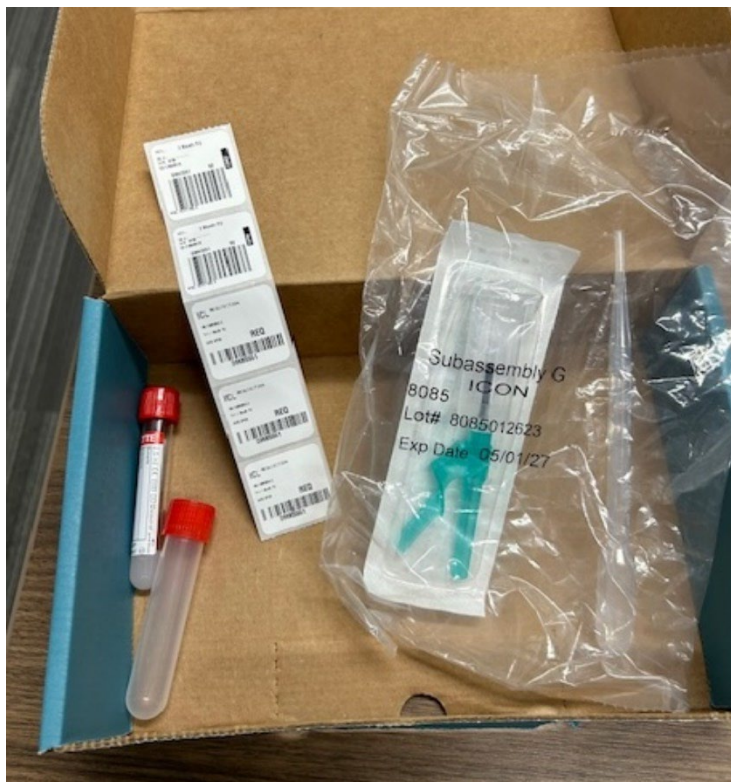


Figure 6.2. 3-month Collection Kit Contents



6.2.1.3 OTHER ICON SUPPLIES FOR COLLECTING AND SHIPPING SAMPLES

In addition to kits, ICON provides other supplies for collecting and shipping samples.

ICON Supplies	Initial Quantity
Shipping Frozen Boxes	5
Lab Certificate	1
4 Segmented Absorbent Pouches (not required)	20
Biohazard Bags	20
Box of 100 2 mL Transfer Pipettes	1
*49-insert (10 mL) Cryoboxes with Large Biohazard Bags	2

*Sites that are sent an 81-insert (5 mL) cryobox in their initial shipment should use the box and then order 49-insert (10 mL) cryoboxes when ordering additional supplies through the iSite portal.

6.2.2 MAYO BIOREPOSITORY SUPPLIES

The first shipment of Mayo Biorepository supplies, including cryovials and shipping materials, will be initiated near site activation. Shipments of additional supplies are managed at the site level. Sites should monitor their supply inventory closely and re-order supplies as needed using the [PREVENTABLE Storefront](#) and the [Mayo Clinic Research Client Portal](#).

6.2.2.1 PREVENTABLE STOREFRONT

Supplies for the biorepository, except shipping boxes, are available to order from the PREVENTABLE Storefront. See **Chapter 3, Section 3.6** of the MOP for instructions to order supplies from the storefront.

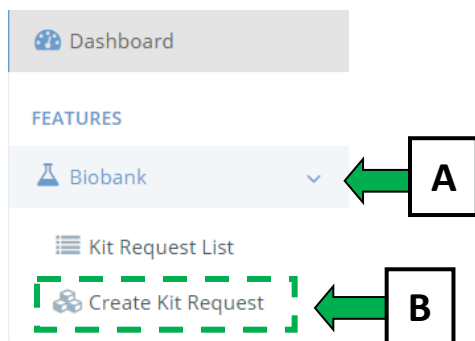
6.2.2.2 MAYO CLINIC RESEARCH CLIENT PORTAL

Biorepository shipping boxes are the only supplies available to order through the Mayo Clinic Research Client Portal. To order biorepository shipping boxes:

1. [Follow these steps to create an account on the Mayo Clinic Portal](#).
2. [Follow these steps to submit an access request](#) so Mayo can configure your account for the PREVENTABLE study. Please enter the correct study information, provided below, in the request:
 - Kit Location: Minnesota
 - IRB and/or Study Alias: NoIRB PREVENTABLE-External
 - Notes: EXTA00016; [name of your institution]

3. Once your access request is approved, follow these steps to submit a Kit Request:

- a. Click **Biobank** (A) under the **Features** section of the navigation menu.
- b. Click **Create Kit Request** (B) to start a new request.



- c. Enter the required information for step 1:
 - **Study Site:** NoIRB (PREVENTABLE-External)
 - **Mayo Location Fulfillment Center:** Minnesota
 - **Company/PAU/Activity Number:** Leave blank
 - **Client ID:** EXTA00016
 - **Requestor Telephone:** Provide a phone number if it is not automatically populated.
- d. Enter the kit information in the **Add Kits** section:
 - **Visit:** External
 - **Kit Type:** Select the appropriate shipping kit based on the number of boxes, or cryovial storage racks, being shipped. Do not overpack a mailer.

Mailer Type	Mailer Capacity
NoIRB (PREVENTABLE-External) Cicek Biorepository 10# Batch Shipping Kit	1 cryovial storage rack/box maximum
NoIRB (PREVENTABLE-External) Cicek Biorepository 20# Batch Shipping Kit	3 cryovial storage racks/boxes maximum

- **Number of Kits:** Type the number of kits in the Number of Kits field or use the + or – buttons (A) to enter the quantity of the Kit Type needed. Sites should not need to order more than 2 kits at a time.
- e. Click **Add New** (B) to add the kit(s) to your order. The items should appear on the right side of your screen (C) under the Kit(s) List Information section.

- f. Click **Continue** (D) to advance to the next page.

The screenshot shows the 'Add Kits' form. On the left, there are dropdown menus for 'Visit' and 'Kit Type', and a 'Number of Kits' field with a value of 1. A green arrow points from annotation A to the 'Number of Kits' field. To the right is a table titled 'Kit(s) List Information' with columns 'Visit', 'Kit Type', and 'Quantity'. It contains one row: 'Control Population External-R3005301', 'Mail Out - Blood (Control)', and '1'. A red 'X' icon is next to the quantity. A green dashed box surrounds the table, with annotation C pointing to it. Below the table is an 'Add New' button. A green arrow points from annotation B to this button. At the bottom right are 'Continue' and 'Save' buttons, with annotation D pointing to the 'Continue' button.

- g. Complete the required shipping information for step 2:

- **Delivery Method:** Ship External
- **Shipping Address:** In the drop-down menu (A), select the address where the shipping kit(s) should be delivered. To add an address to the drop-down menu, click **Create Adhoc Address** (B). Complete the required fields and click **Save Adhoc Address**.

The screenshot shows the 'KIT REQUEST SHIPPING INFORMATION' form. It has a 'Delivery Method' dropdown set to 'Ship External'. Below it is a 'Shipping Address' dropdown showing 'Marvin - 2915 Valley High Dr NW Rochester, MN ...'. A green arrow points from annotation A to this dropdown. To the right is a 'create Adhoc Address' button, with a green arrow pointing to it from annotation B.

- **Rush Request:** Select this option if the kit(s) need to be delivered before the first available date in the **Date Kits Needed** field.
- **Date Kits Needed:** Select the date the kits should be delivered from the pop-up calendar. If the kits are needed before the first available date, select **Rush Request** and return to the calendar to select the desired delivery date.
- **FedEx Ship Manager:** Click this button to enter additional shipping information.
 - **Service Type:** Select **R-FedEx Ground Service**. If "Rush Request" was selected, use an overnight or 2-day service option.
 - **Hold at Location:** Select **"No."** Only select "yes" if the recipient would rather drive to a FedEx location to pick up the package.
 - **Signature Options:** The standard practice is to select **"No Signature."** To require a signature for delivery, select the appropriate signature option.
 - **Ship Alert:** It is required to provide at least one email address to receive delivery notifications from FedEx. Follow these steps to enter ship alerts for up to 3 recipients:
 1. Click **Ship Alert**.
 2. Click **Add New**.
 3. Select **Recipient** for the "Alert Recipient Type."
 4. Enter a valid email address in the "Email ID" field.

5. Select the notifications you would like to receive from FedEx (A).
6. Click **Save Ship Alert** (B).

The screenshot shows a web form for setting up a ship alert. At the top, there is a dropdown menu for 'Alert Recipient Type' with 'Recipient' selected, and a text field for 'Email ID' with 'example@email.org'. Below these is a dashed green box containing five notification categories: 'Ship', 'Tendered', 'Exception', 'Estimated Delivery', and 'Delivery'. Each category has a 'Yes' or 'No' button. A green arrow labeled 'A' points to the 'Estimated Delivery' 'Yes' button. To the right of the dashed box is a 'Save Ship Alert' button with a printer icon, and a 'Cancel' button. A green arrow labeled 'B' points to the 'Save Ship Alert' button.

- h. Click **Continue** to advance to the last page. Review the details of the order carefully.
- i. After reviewing the request details and added attachments (if applicable), click **Submit**.

6.3 COLLECTING, PROCESSING, AND STORING SAMPLES (IN-CLINIC VISITS ONLY)

Sites will collect blood samples per the instructions in the **ICON Lab Manual** and their site's standard operating procedures (SOPs). It is important that samples are collected, processed, stored, and shipped by trained and qualified site staff.

For in-clinic baseline visits, participants will have blood drawn for lipid panel analysis at the ICON central lab and for storage in the Mayo Biorepository. A lipid sub-study sample will be collected at 3 months post randomization from 2,000 randomly selected participants.

****Participants do not need to be fasting for the baseline or 3-month blood draws.**

See **Appendix 6.1b** for instructions to complete the Blood Collection Form. For the baseline visit, the Blood Collection Form is included with the Baseline and Eligibility Form packet See MOP **Chapter 4: Screening and Enrollment** and **Chapter 5: Baseline Visit** for details.

See **Appendix 6.1c** for instructions to complete the 3-month Lipid Follow-up Visit Form. Both of these forms can be printed or the data can be entered directly into the study database.

The Blood Collection Form has a barcode that allows direct access to the specific form in the study database associated with the participant and the visit. Form barcodes may be scanned with the same barcode scanner used for scanning cryovial tube labels.

Complete the Blood Collection Form questions 1–4 at the time of collection and processing. Remaining form data will be obtained from the scanned lipid transport tube and Mayo Biorepository cryovial tubes or answered by clinical site staff, as appropriate.

6.3.1 COLLECTING SAMPLES

Below are recommendations for blood collection. Please follow your local site or lab SOPs when collecting participants' blood samples.

1. Ask the participant if they have an allergy to alcohol or latex.
 - If the participant is not allergic to alcohol, clean the venipuncture site with the 70% alcohol wipe and allow it to air-dry.
 - If the participant is allergic to alcohol, scrub the venipuncture site with soap and water and allow it to air-dry.
 - Based on the participant's response to a latex allergy, use the appropriate tourniquet and gloves.

Prior to collecting the blood sample, ask the participant the last time they ate or drank anything other than water, including candy, chewing gum, and black coffee. Note the participant's response for the Blood Collection Form.

****Participants do not need to be fasting for the blood draw.**

See **Appendix 6.1b** for instructions on entering the Baseline Blood Collection Form. See **Appendix 6.1c** for instructions on entering the 3-month Lipid Follow-Up Visit Blood Collection Form.

2. Apply the tourniquet, place the gauze roll in the participant's hand, and ask the participant to gently squeeze the gauze roll. If there is a delay in performing the venipuncture, do not keep the tourniquet on the participant's arm for more than 2 minutes. Follow the local site or lab guidelines for tourniquet duration if the delay is less than 2 minutes.

SAFETY NOTE: Immediately terminate the venipuncture if the participant becomes ill, experiences excessive pain, a hematoma develops, or if arterial perforation occurs (indicated by bright red blood or an arterial pulse).

3. Assemble the collection device and attach the first collection tube onto the collection assembly. If a winged butterfly needle is used, a waste tube should be attached first to fill the tubing prior to using the first collection tube.
4. Anchor the vein for added stability and alert the participant before inserting the needle.
5. Collect the blood samples in the standard phlebotomy order as shown in the table below and note the blood collection time:

Baseline Collection Order	Collection Specimen Tube	Expected Volume of Specimen to be Collected (mL)
1	Serum SST	10 mL
2	K3 EDTA	10 mL

6. Once collected, the EDTA tube must be quick-mixed by 8–10 gentle inversions. No mixing is required for the serum tube.
7. Gently release the tourniquet. Site staff must ensure that the tourniquet is completely off the participant's arm and that it does not remain on the participant's arm for more than 2 minutes at any point during the venipuncture procedure.
8. Lightly place gauze over the puncture site and remove the needle, activating the safety device in the process, if available.
9. Apply pressure on the gauze pad and wrap the puncture site with the gauze roll or apply a Band-Aid. Tell the participant that the gauze or Band-Aid can be removed after 15 minutes.
10. Dispose of all collection materials and biohazardous waste according to your local site or lab regulations.
11. Immediately following collection, mixing, and volume verification, prepare the tubes for processing. Proceed to the next section for blood processing.

6.3.2 PROCESSING SAMPLES

1. The serum tube must clot for a minimum of 30 minutes at room temperature in an upright position prior to centrifugation. After clotting, if the serum tube is not centrifuged immediately, it must be refrigerated or placed on wet ice until centrifuged. Do not place the serum sample in the freezer as freezing and thawing could lyse all the cells in the pellet and potentially contaminate the serum.

The EDTA tube can be centrifuged immediately; otherwise the tube must be refrigerated or placed on wet ice until centrifuged.

2. **Within 4 hours** of sample collection, centrifuge both tubes at **refrigerated temperature (4°C)** at a speed of **2000 x g** for **10 minutes**.

NOTE: Some centrifuge displays show revolutions per minute (RPMs). To convert from relative centrifugal force (RCF) (x g) to RPMs, refer to the Centrifugation Table on page 13 of the **ICON Lab Manual**.

3. Confirm both specimens fractionated and have clear and distinct layers. If a specimen does not have clearly defined fractionated layers, review the Biorepository and Lab Procedures FAQs on the study website under Login > Quick Links > Documents > Study Documents and Resources > Quick References and FAQs.
 - The serum tube will have 2 layers: clear serum at the top and red blood cells (RBCs) at the bottom.
 - The EDTA tube will have 3 layers: clear plasma on the top, a buffy layer (WBCs) in the middle, and red blood cells (RBCs) at the bottom.

4. Prepare to transfer the ICON lipid specimen by retrieving an ICON serum aliquot transport tube and an ICON cryobox. See **Figure 6.3. ICON Laboratory 49-insert Cryobox (lipid samples only)** below.

****Please ensure you have clearly labeled or written with a permanent marker on the outside of the ICON cryobox: study name (PREVENTABLE), site number, and Box 100, 101, 102, etc. Once cryoboxes are shipped to ICON, please continue the correct numbering sequence in subsequent shipments. Do not start over with Box 100.**

A participant identification number is required for proper specimen identification. Legibly print the participant identification (ID) number on the ICON serum aliquot transport tube barcode label provided in the ICON kit **BEFORE** transferring specimens. The baseline barcode should be affixed to the ICON serum aliquot transport tube as per the instructions in **Figure 6.4. ICON Laboratory and Mayo Clinic Biorepository Labeling** below.

5. Prepare to transfer the Mayo Biorepository specimens by retrieving the appropriate cryovial storage rack/box, 9 cryovial tubes, and 9 color-coded caps.

****Please aliquot biorepository samples into the cryovial tubes and place the correct color-coded caps on the cryovial tubes to distinguish the samples collected.**

- **Cryovial cap color coding for Mayo Biorepository samples:**
 - 4 natural caps (serum)
 - 4 purple caps (plasma)
 - 1 blue cap (buffy coat)

****Please ensure you have clearly labeled or written with a permanent marker on the outside of the Mayo Biorepository cryovial storage rack/box: study name (PREVENTABLE), site number, and Box 200, 201, 202, etc. Once cryovial storage rack/box are shipped to the Mayo Biorepository, please continue the correct numbering sequence in subsequent shipments. Do not start over with Box 200.**

6. Using the first pipette, transfer 0.5 mL (500 µL) of serum from the red-top tube into the ICON aliquot transport tube. See **Figure 6.5. Baseline Sample Collection and Processing Overview** below.
7. Using the same pipette, transfer 1 mL (1000 µL) of serum each into 3 of the cryovial tubes (**Figure 6.5**) to create the first 3 Mayo Biorepository serum aliquots.
8. Transfer the remaining serum to the fourth serum cryovial tube (**Figure 6.5**) for the Mayo Biorepository. The last serum aliquot should be approximately 0.5 mL (500 µL).
9. Cap all the cryovial tubes immediately. **It is critical that the caps are screwed securely on to the cryovial tubes.**
10. Discard the parent red-top tube according to your local site or lab SOPs.

11. Using a new pipette, transfer 1 mL (1000 µL) of plasma (top clear layer) from the lavender-top tube into the first Mayo Biorepository plasma cryovial tube (**Figure 6.5**). Repeat until the plasma is consumed or four 1 mL aliquots are created.

12. Using the same pipette, carefully transfer the entire buffy coat layer to 1 cryovial tube.
Using a pipette with a wider opening at the tip usually works best.

To learn more about buffy coat extraction, watch this Mayo Clinic video on [PREVENTABLE Buffy Coat Processing Tips and Best Practices](#). This video is also available on the study website under Login > Quick Links > Training Videos > Training > Biorepository and Lab Procedures.

NOTE: If during processing the buffy coat layer is difficult to detect, you inadvertently disturb the specimen layers, or accidentally mix plasma/RBCs with the buffy coat layer, review the Biorepository and Lab Procedures FAQs on the study website under Login > Quick Links > Documents > Study Documents and Resources > Quick References and FAQs.

13. Cap all cryovial tubes immediately. **It is critical that the correct color-coded caps are screwed securely on to the cryovial tubes.**

- **Cryovial cap color coding for Mayo Biorepository samples:**
 - 4 natural caps (serum)
 - 4 purple caps (plasma)
 - 1 blue cap (buffy coat)

14. Discard the parent EDTA tube according to your local site or lab SOPs.
15. Scan the lipid and biorepository tube barcodes and enter information for each sample in the study database. This information will be used to generate the shipping manifest. See **Appendix 6.1a** for detailed instructions on entering sample information in the database.
16. Place the 0.5 mL ICON serum sample in the ICON lipids storage cryobox (**Figure 6.3**), separate from the biorepository cryovial storage rack. The lipid samples will be shipped to ICON approximately every 3 months for analysis.

Fill every spot in the ICON lipids storage cryobox with samples, leaving no open spaces between participants. Samples should be put in the box in the order they are logged in the manifest to ensure accurate and timely processing once the shipment reaches the lab.

17. Place the Mayo Clinic Biorepository samples in the cryovial storage rack/box (**Figure 6.6**), separate from the lipid samples in ICON lipids storage cryobox. The biorepository samples will be shipped to the Mayo Clinic approximately every 3 months. Biorepository shipments may extend past the quarterly timeframe and can be batch shipped when the cryovial storage rack is full.

Fill every spot in the Mayo Biorepository cryovial storage rack/box with samples, leaving no open spaces between participants. Samples should be put in the storage rack/box in

the order they are logged in the manifest to ensure accurate and timely processing once the shipment reaches the biorepository.

18. Place the ICON cryobox and Mayo Biorepository cryovial storage rack/box in the -70°C or -80°C freezer. The lipid samples should be stored in ICON cryoboxes (**Figure 6.3**) separate from the Mayo Biorepository samples in the cryovial storage rack/box (**Figure 6.6**). Sites must use the Mayo Clinic Biorepository rack/box that the cryovials are delivered in when storing biorepository samples in the freezer.

Figure 6.3. ICON Laboratory 49-insert Cryobox (lipid samples only)



Figure 6.4. ICON Lab and Mayo Biorepository Labeling

For the baseline and 3-month lipid samples: Legibly print the participant ID (i.e., SUB:) on the label provided in the ICON kit and affix the label to the ICON 10 mL red-top transport tube. Labels noted with REQ are not required for the study but may be used internally at your site if needed.

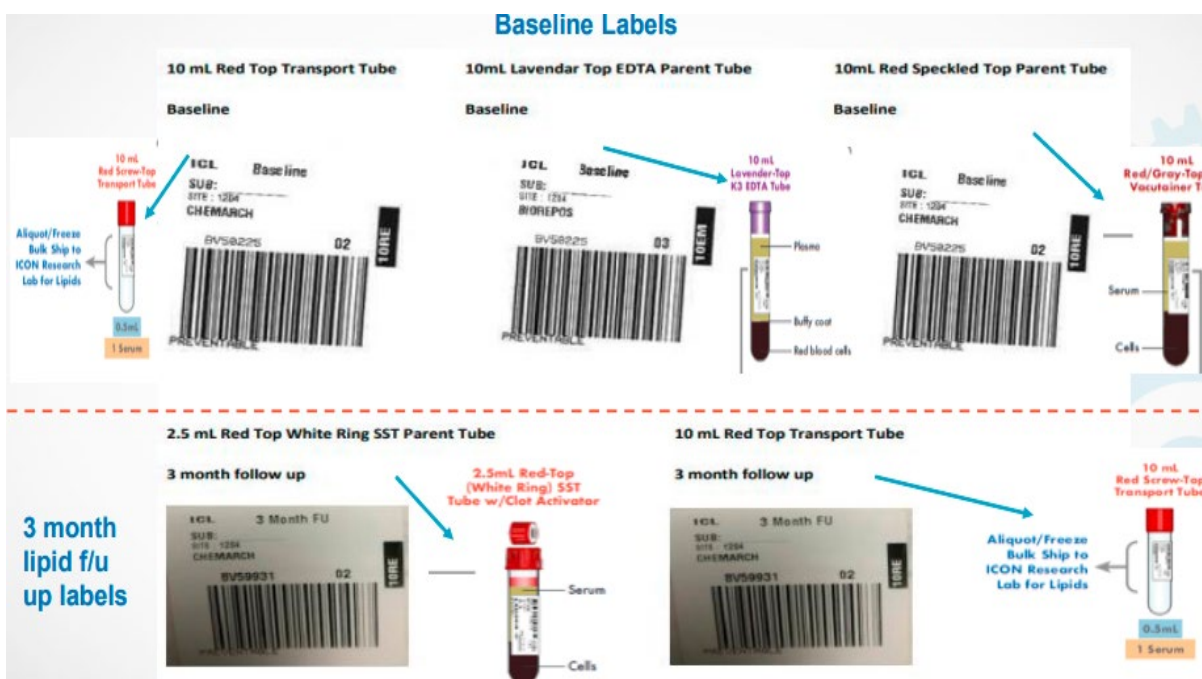


Figure 6.5. Baseline Sample Collection and Processing Overview

Baseline Sample Collection and Processing

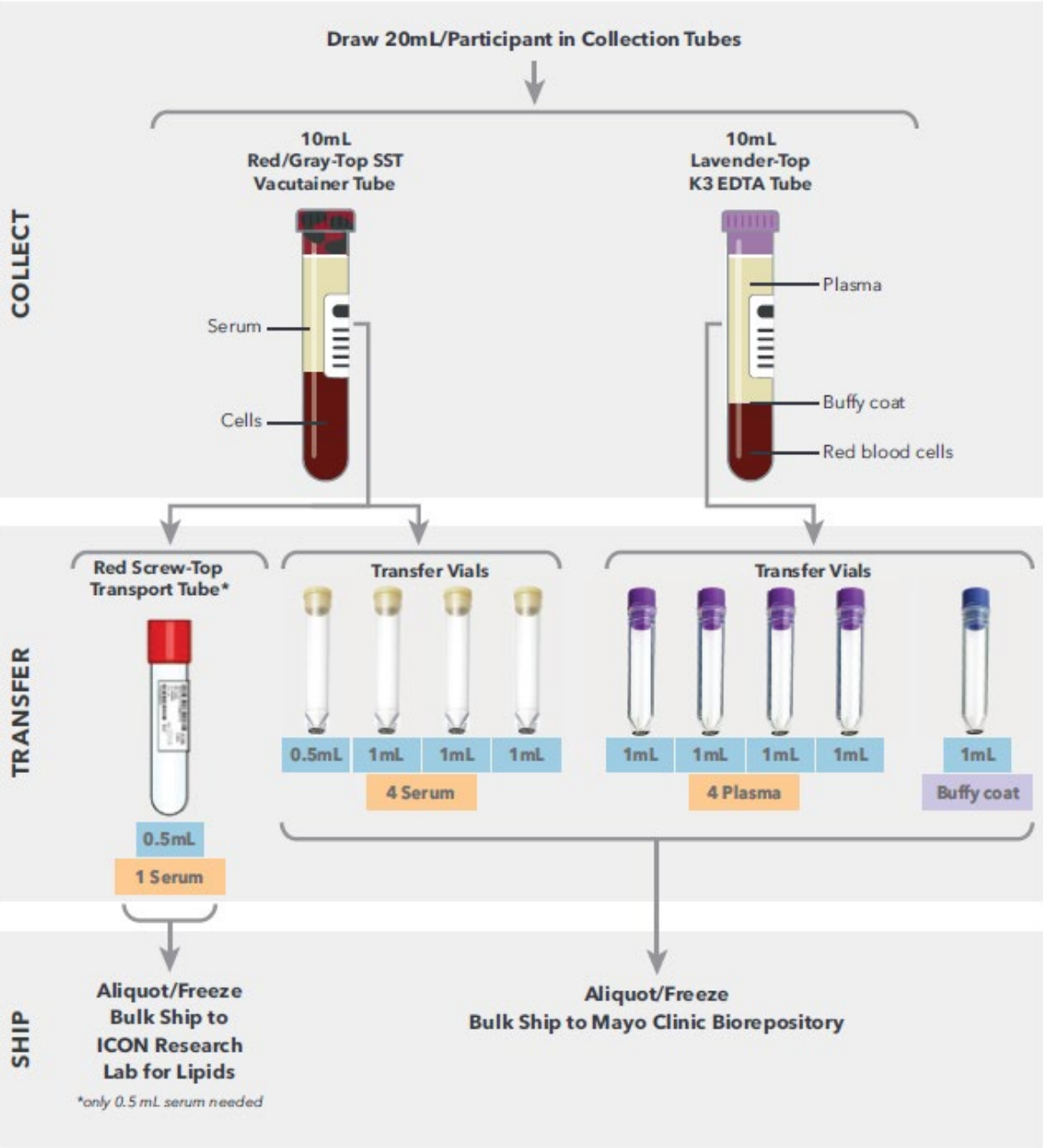


Figure 6.6. Mayo Biorepository Cryovial Storage Rack/Box with 96 Tubes (biorepository samples only)



6.3.3 COLLECTING 3-MONTH FOLLOW-UP SAMPLES (LIPID SUB-STUDY ONLY)

For the 3-month lab draw, the visit target is 90 days post randomization. The visit window is 60 to 180 days post randomization. See **Chapter 11** of the MOP for guidance on assessment windows and missed visits. **If baseline blood samples were not collected from a participant who was randomized to the lipid sample sub-study group, the 3-month lipid sample should not be collected.**

If a participant is randomly selected for the 3-month clinic visit (i.e., they have a Lipid Sample Sub-Study status of “Yes”) and the site entered “Yes” in response to the question “Were any samples obtained?” on the Baseline Blood Collection Form, then the data entry button for the 3-month visit will appear under the “Visit List” tab.

If a participant was randomly selected for the 3-month clinic visit but there was no baseline blood collected, the participant should not do the 3-month clinic visit and a data entry button for the 3-month visit will not appear.

Instructions for conducting a 3-month clinic visit for participants in the lipid sample sub-study group are provided below. Use only an ICON collection kit labeled for a 3-month visit; no biorepository supplies are needed at the 3-month visit.

1. Follow steps 1–4 in [Section 6.3.1: Collecting Samples](#) to collect the 3-month lipid panel sample.

3-Month Visit Collection Specimen Tube	Expected Volume of Specimen to be Collected (mL)
Red-Top Serum SST	2.5 mL

2. Collect only the red-top serum SST tube. See **Figure 6.7** below.
3. Terminate the venipuncture by following steps 7–10 in [Section 6.3.1: Collecting Samples](#).
4. Once the blood sample is collected, place the serum tube in an upright position at room temperature and allow it to clot for at least 30 minutes before processing. After clotting, if the serum tube is not centrifuged immediately, it must be refrigerated or placed on wet ice until

centrifuged. Do not place the serum sample in the freezer as freezing and thawing could lyse all the cells in the pellet and potentially contaminate the serum.

5. **Within 4 hours** of sample collection, centrifuge the red-top serum tube at **refrigerated temperature (4°C)** at a speed of **2000 x g** for **10 minutes**. After clotting, the serum tube must be refrigerated or placed on wet ice until centrifuged.

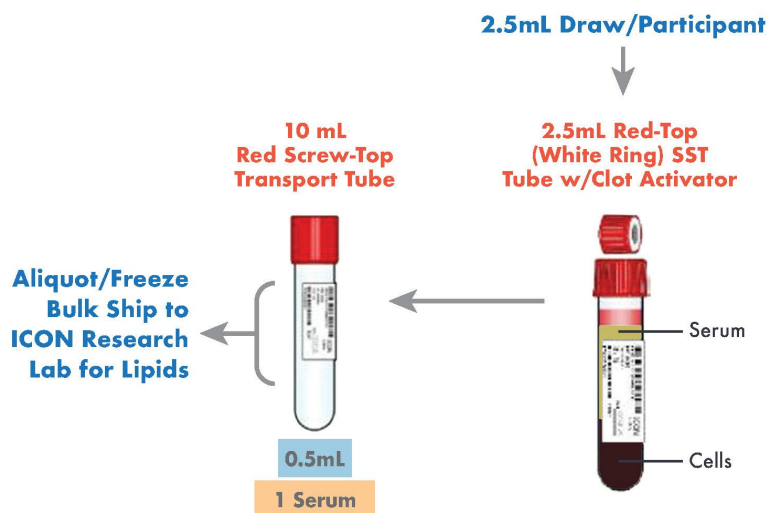
NOTE: Some centrifuge displays show revolutions per minute (RPMs). Refer to step 2 in **Section 6.3.2: Processing Samples** for the steps to convert the speed from relative centrifugal force (RCF) to RPMs.

6. Confirm the specimen fractionated and has clear and distinct layers. Serum will have 2 layers: clear serum at the top and red blood cells (RBCs) at the bottom. If a specimen does not have clearly defined fractionated layers, review the Biorepository and Lab Procedures FAQs on the study website under Login > Quick Links > Documents > Study Documents and Resources > Quick References and FAQs.
7. Prepare to transfer the specimen by retrieving an ICON serum aliquot transport tube and an appropriately labeled (i.e., study name and box number) ICON cryobox (**Figure 6.3**).
8. A participant identification number is required for proper specimen identification. Legibly print the participant identification number on the ICON serum aliquot transport tube barcode label **BEFORE** transferring specimens. Follow the instructions in **Figure 6.4** to affix the 3-month barcode label to the ICON serum aliquot transport tube.
9. Transfer 0.5 mL (500 µL) of serum from the red-top tube into the ICON aliquot tube.

NOTE: If during processing you inadvertently disturb the specimen layers or accidentally mix serum and red blood cells (RBCs), review the Biorepository and Lab Procedures FAQs on the study website under Login > Quick Links > Documents > Study Documents and Resources > Quick References and FAQs.
10. Cap the aliquot tube immediately. **It is critical that the cap is screwed securely on to the tube.** Discard the SST parent red-top tube according to your local site or lab SOPs.
11. Scan the aliquot tube barcode and enter the sample information in the study database, which will be used to generate the shipping manifest. See **Appendix 6.1a** for detailed instructions.
12. Transfer the 0.5 mL ICON serum sample to an ICON lipids storage cryobox. These samples will be shipped to ICON approximately every 3 months for analysis and may or may not be shipped in the same box as baseline samples, depending on a site's next shipment date.
13. Place the cryobox back in the -70°C or -80°C freezer.

Figure 6.7. 3-Month Follow-up Sample Collection and Processing Overview

3-Month Follow-up Sample Collection and Processing



6.3.4 DESTROYING SAMPLES

Please see **Appendix 6.2** and **Appendix 6.3** for instructions to remove a sample from the Mayo Biorepository or ICON Lab shipping manifests. All “removed” samples sent to the lab will be destroyed at the end of the study.

6.4 SHIPPING FROZEN SAMPLES

Site staff who are responsible for processing and shipping blood samples must complete the [Transporting Dangerous Goods training provided the Mayo Clinic](#) or a similar training provided by their institution. Course completion certificates do not need to be sent to the Coordinating Center, but should be saved in the site’s study record.

Sites may be reimbursed up to \$100 per year, by invoice only, for dry ice used for study sample shipments. Dry ice pellets, rather than slabs, are preferred. Please follow the instructions to submit invoices as outlined in the site agreement.

Do not include the ICON requisition form in any shipments to avoid unnecessary queries. The ICON requisition form is not used for the PREVENTABLE study.

6.4.1 DOCUMENTS NEEDED FOR SHIPPING

Refer to the **ICON Lab Manual, Section 11: Completing Courier Air Waybills and Related Shipping Documents** for instructions and examples of courier Air Waybills as well as other shipping documents that may be needed when shipping samples to the ICON lab.

The **ICON Lab Manual** is available on the PREVENTABLE website under Login > Quick Links > Documents > Vendors > ICON Lab.

6.4.2 SHIPPING FROZEN SAMPLES TO THE MAYO BIOREPOSITORY

1. Retrieve all Mayo Clinic Biorepository cryovial storage racks/boxes from the –70°C or –80°C freezer and immediately place them on dry ice to keep the samples frozen. ****Please ensure you have retrieved the storage rack(s) labeled with the correct box number(s).**
2. See **Appendix 6.2** for instructions to generate and print shipping manifests.
3. Confirm that the manifest matches the box number and the contents of the box being shipped to the Mayo Biorepository.
4. Print 2 copies of each shipping manifest. Place 1 of the 2 copies in a biohazard bag and seal it. Save the other copy in the site’s study record.
5. Obtain a biorepository shipping container. Shipping kits can be ordered by following the instructions in **Section 6.2.2.2: Mayo Clinic Research Client Portal**. Be sure to use the appropriate mailer type based on the number of cryovial storage racks/boxes being shipped. Do not overpack mailers.

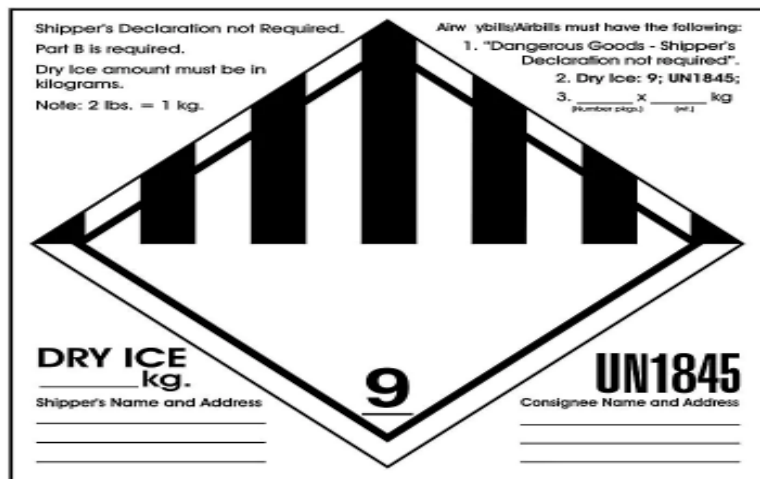
Mailer Type	Mailer Capacity
NoIRB (PREVENTABLE-External) Cicek Biorepository 10# Batch Shipping Kit	1 cryovial storage rack/box maximum
NoIRB (PREVENTABLE-External) Cicek Biorepository 20# Batch Shipping Kit	3 cryovial storage racks/boxes maximum

6. Open the cardboard shipping box, remove the lid of the insulated foam container located inside the shipping box, and place a 2-inch layer of dry ice at the bottom of the insulated container. Dry ice pellets, rather than slabs, are preferred.
7. Place up to 3 cryovial storage racks/boxes into biohazard bags containing an absorbent material and seal. Place the rack(s)/box(es) on top of the dry ice.
8. Fill the remaining space in the foam container with dry ice.
9. Place the lid securely on the insulated foam container and place the shipping manifest, sealed in a biohazard bag, on top of the foam lid. Close the flaps of the cardboard shipping box and tape it shut.
10. The shipping boxes are pre-labeled to fulfill all US Department of Transportation (DOT) and International Air Transport Association (IATA) requirements. Check all labels to make sure they are complete and not covered or damaged.

Locate the dry ice label (see **Figure 6.8** below) on the shipping box and complete the information.
11. Affix the completed Air Waybill to the shipping box.
12. Schedule a pickup time with FedEx at least 1 day before the desired pickup day.
 - a. Packages should be shipped Monday – Thursday only.

- b. When scheduling a shipment, be prepared to provide the account number and protocol number from the Air Waybill.
- c. Shipments may also be dropped off at a FedEx location that accepts UN3373 Biological Substance Category B hazardous materials. When [searching for a FedEx location](#), check the box for “Drop off” under “Find a location for.” Click on the “More Filters” link and select “Dangerous goods shipping” from the Service filters drop-down list.

Figure 6.8. Dry Ice Placard



6.4.3 SHIPPING FROZEN SAMPLES TO THE ICON LAB

Please refer to the **ICON Lab Manual, Section 9: Properly Packaging Specimens for Shipment** and **Section 10: Specimen Shipping Instructions** for detailed information on packaging and shipping lipid samples to the ICON lab. The **ICON Lab Manual** is available on the PREVENTABLE website under Login > Quick Links > Documents > Vendors > ICON Lab.

1. Retrieve all ICON lipids storage cryoboxes from the -70°C or -80°C freezer and immediately place them on dry ice to keep the samples frozen. ****Please ensure you have retrieved the cryobox(es) labeled with the correct box number(s).**
2. See **Appendix 6.3** for instructions to generate and print shipping manifests.
3. Confirm the manifest matches the box number and the contents of the box being shipped to the ICON lab.
4. Print 2 copies of each shipping manifest. Place 1 of the 2 copies in a biohazard bag and seal it. Save the other copy in the site's study record.
5. Obtain a shipping container. To order mailing boxes, follow the instructions in **Section 6.2.1: ICON Lab Supplies**.
6. Remove the lid of the insulated foam container located inside of the shipper box.
7. Place 2.5 lbs. (1.14 kg) of dry ice in the bottom of the insulated foam container.

NOTE: Dry ice pellets are recommended for optimal performance.

8. Place the cryobox and an absorbent sheet into the large (95kPa) biohazard bag. Remove the adhesive strip and seal the bag.
9. Place the sealed biohazard bag on top of the dry ice in the insulated foam container.
10. Place 4 lbs. (1.82 kg) of dry ice on top of the biohazard bag.
11. Place the lid securely on top of the insulated container and place the shipping manifest, sealed in a biohazard bag, on top of the foam lid.
Important Reminder: ICON requisition forms are not used for the PREVENTABLE study and should not be included in shipments to the lab to avoid unnecessary queries.
12. Close the flaps of the shipper box and seal the box with the enclosed sealing strip.
13. Enter the amount of dry ice used (in kg) in the space provided on the dry ice label of the shipper box (**Figure 6.8**).
14. Affix the completed Air Waybill to the shipper.
15. Schedule a pickup time with FedEx at least 1 day before the desired pickup day.
 - a. Packages should be shipped Monday – Thursday only.
 - b. When scheduling a shipment, be prepared to provide the account number and protocol number from the Air Waybill.
 - c. Shipments may also be dropped off at a FedEx location that accepts UN3373 Biological Substance Category B hazardous materials. When [searching for a FedEx location](#), check the box for “Drop off” under “Find a location for.” Click on the “More Filters” link and select “Dangerous goods shipping” from the Service filters drop-down list.

6.5 ICON QUERIES, ALERTS, AND MANIFEST DISCREPANCIES

ICON queries are emailed directly from ICON. ICON alerts are generated and managed within the study database on the PREVENTABLE website.

Manifest-related discrepancies are sent to sites from their Clinical Research Associate (CRA). These discrepancies are identified by the data management team when data entered in the study database does not correlate with the samples the ICON lab has received.

6.5.1 ICON ALERTS

6.5.1.1 80-DAY LAB TRACKING ALERT

Sites should ship ICON samples by 90 days. Lab Tracking alerts appear in the study database when the oldest sample in an ICON lipids storage cryobox reaches 80 days from its collection date and that box has not been shipped to the ICON lab. Click on the alert to go directly to the ICON Lab Tracking system.

Rows that are shaded in red indicate issues that need attention. The table displays the number of days since the oldest sample in the box was collected.

Box	Number of Samples	Oldest Collection Date	Number of Days	Ship Date	Tracking Number
100	1	03-05-2021	125	03-05-2021	34943543534
101	10	02-18-2021	140	05-18-2021	Testing12311234
111	1	08-05-2020	337	06-30-2021	124123123123
112	3	03-02-2021	128		

To resolve Lab Tracking alerts, click on the box number. On the next screen, enter the date shipped and the FedEx tracking number. Click “Submit” to save the shipping date and tracking number. After saving the shipping information, click the blue “Generate Manifest” button. Once the “Generate Manifest” button is clicked, an electronic copy of the manifest is immediately sent to the ICON lab so they know to expect the box for processing.

6.5.1.2 PARTICIPANT ISSUE ALERTS

3-month Visit Due for Lipid Sample Blood Collection Alert

This alert occurs when a participant randomized to the lipid sample sub-study group has a completed Baseline Blood Collection Form and the participant is past the 3-month target date for the collection of their lipid sample.

3-month Visit PAST Due for Lipid Sample Blood Collection Alert

This alert occurs when a participant randomized to the lipid sample sub-study group had baseline blood collected, they have passed the 180-day mark (the end of the 3-month visit window) since randomization, AND either no data has been entered in their 3-month Blood Collection Form OR a Reason Not Collected has not been entered.

See **Chapter 11** of the MOP for instructions to resolve Participant Issue alerts.

6.5.2 ICON QUERIES

Site staff can review and resolve queries directly in iSite, ICON's secure web portal. When new queries are available, ICON will send a notification to the email registered with ICON. Contact the Site Services department if further explanation of a query is needed.

For more information about resolving queries, please review the **iSite User Guide and ICON Lab Manual** on the PREVENTABLE website under Login > Quick Links > Documents > Vendors > ICON Lab:

- iSite User Guide, Section 6: Queries and Flagged Values
- ICON Lab Manual, Section 2: Contacting ICON Laboratory Services for Assistance, including details about ICON queries.

If you have difficulty resolving a query, please email the ICON Project Manager, listed in the PREVENTABLE Directory on the study website, for assistance.

6.5.3 MANIFEST DISCREPANCIES

The primary cause of manifest discrepancies is incorrect ICON cryobox numbers entered in the Blood Collection Form in the study database.

The purpose of the cryobox number is to generate the manifest to be included in each shipment. The boxes provided to store and ship samples to ICON are not pre-numbered when they arrive on site, so it is important for sites to establish a system to manage their process for numbering and tracking boxes. Below are tips to keep track of box numbers and to avoid manifest discrepancies and queries:

- Identify site staff to manage box numbering (e.g., study coordinator, lab tech, etc.).
- The first box is #100 and others follow sequentially (101, 102, etc.). Do not use a box number more than once.
- Identify a method to track box numbering so the site staff who are processing and shipping samples and updating Blood Collection Forms in the study database will know what box number was last used and what number should be next. Whatever method is chosen to track box

numbers, please ensure the correct box number is entered on the Blood Collection Form to avoid manifest discrepancies.

- Enter the shipped date and tracking number on the Blood Collection Form in the study database. The shipped date is the actual date the box was shipped to ICON, not the date planned to ship the box to ICON.
- Include a printed copy of the shipping manifest in each box and confirm the manifest matches the box number and the contents of the shipment. If the manifest is not included in each box of samples, ICON will generate queries for missing sample information. Save a copy of every shipping manifest in the site's study records.
- Do not include the requisition form from ICON collection kits in shipments of study samples. The requisition form is not used for the PREVENTABLE study and should be discarded. Sending the requisition form back to ICON may result in unnecessary queries.

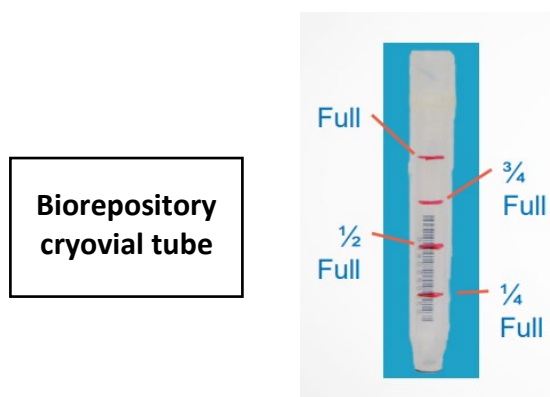
APPENDIX 6.1 SCANNING AND ENTERING SAMPLE INFORMATION IN THE STUDY DATABASE

6.1a SCANNING SAMPLE TUBE BARCODES INTO THE STUDY DATABASE

1. [Log in to the study website](#) and navigate to the participant list by clicking on Data > Data Entry.
2. Open the participant's Blood Collection Form for the specific visit in 1 of 2 ways:
 - Select your site and then select the participant from the list. Navigate to the Visit List tab on the left side of the screen and click on the corresponding form for the correct visit, OR
 - Scan the barcode on the printed Blood Collection Form to open the form in the study database for that specific participant and visit.
3. For the ICON sample, scan the barcode on the ICON serum aliquot transport tube to link the tube to the participant. Enter the lipids storage cryobox number (e.g., 100, 101) in the form.

Processing/Aliquot Creation for Banking				
ICON LIPID SAMPLE				
	Barcode	Type	Volume	Box
7		serum ▼		

4. For the biorepository samples, scan the cryovial barcode to link the tube to the participant. Select the type of sample based on the cryovial cap color coding for Mayo Biorepository sample and make sure it matches the cryovial cap color – serum (natural caps), plasma (purple caps), and buffy coat (blue cap). Select the approximate volume as shown in the image below (Full, 3/4 full, 1/2 full, 1/4 full). Enter the cryovial storage rack/box number (e.g., 200, 201) in the form.

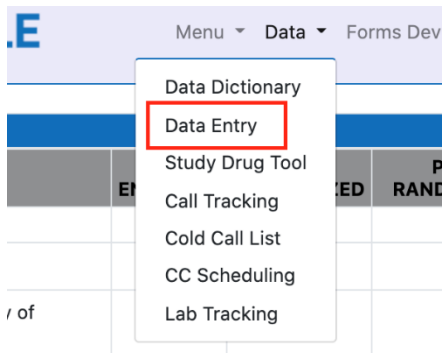


5. If you scan a barcode into the database, the additional fields in that row must also be entered (type, volume, box), otherwise it will be flagged as “missing” data. If scanners are unavailable, manually enter the barcode in the indicated field (Please note: Manual entry is discouraged due to potential for errors).

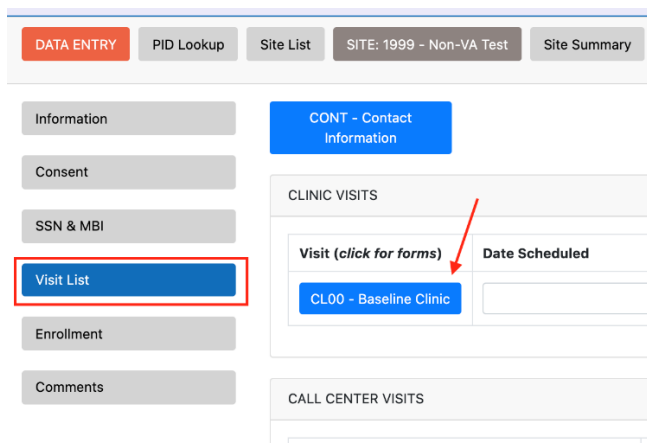
The information entered in the Blood Collection Form is used to create the shipping manifest, so please ensure all of the form data is accurate.

6.1b BASELINE BLOOD COLLECTION FORM

1. To access the Blood Collection Form, [log in to the study website](#) and navigate to the participant list by clicking on Data > Data Entry.



2. Select your site and then select the participant from the list. Navigate to the Visit List tab on the left side of the screen and click the Baseline Clinic button to access the baseline forms.



3. Use the printed Lab Blood Collection Form or enter the information directly into the study database. If the form button is blue, no data has been entered. If the form button is green, some or all the form data has already been entered.

VISIT: CL00 - Baseline Clinic		
PRINT	FORM	DATE ENTERED
--	Eligibility	08-31-2020
--	Demographics	08-31-2020
--	Medical History	02-01-2021
--	Lab Blood Collection	09-25-2020
☐	Short Physical Performance Battery	--
PRINT		

4. Complete the Lab Blood Collection Form questions 1–4 at the time of collection and processing.

Lab Blood Collection - Version 1		SUBMIT FORM
Participant ID:	100000118	
Barcode ID:	B100000010	
Visit Code:	CL00	
Date of Visit (MM-DD-YYYY)	09-23-2020	
Blood Collection		
1	Were any samples obtained?	
2	Blood Collection Date:	MM/DD/YYYY
3	Blood Collection Time: (24-hour clock)	HH:MM
4	To the nearest hour, how many hours since the participant last ate and/or drank anything other than water, including candy, chewing gum, and black coffee?	
5	Serum Tube (red) Barcode:	Barcode Format: AAnnnnn02(Alpha, number, 02) For Internal Use Only
6	EDTA Tube (purple) Barcode:	Barcode Format: AAnnnnn03(Alpha, number, 03) For Internal Use Only
		SUBMIT FORM

5. Form barcodes may be scanned with the same barcode scanner used to scan tube labels. If you scan a barcode into the database, the additional fields in that row must also be entered (type, volume, box), otherwise it will be flagged as “missing” data. If scanners are unavailable, manually enter the barcode in the indicated field following the format shown on the form (Please note: Manual entry is discouraged due to potential for errors).

The information entered in the Lab Blood Collection Form is used to create the shipping manifest, so please ensure all of the form data is accurate.

Processing/Aliquot Creation for Banking				
ICON LIPID SAMPLE				
	Barcode	Type	Volume	Box
7		serum		
MAYO REPOSITORY SAMPLES				
	Barcode	Type	Volume	Box
8	1231231231			
9				
10				
11				
12				

6.1c 3-MONTH LIPID FOLLOW-UP VISIT BLOOD COLLECTION FORM

1. To access the 3-month Lipid Follow-up Visit Form, [log in to the study website](#) and navigate to the participant list by clicking on Data > Data Entry.
2. Select your site and then select the participant from the list. Navigate to the Visit List tab on the left side of the screen and click the 3-month Clinic Visit button.

The screenshot shows a web interface with a top navigation bar containing buttons for 'DATA ENTRY' (highlighted in orange), 'Site List', 'SITE: 1999 - Non-VA Test', and 'Site Summary'. Below this is a sidebar with buttons for 'Information', 'Consent', 'SSN & MBI', 'Visit List' (highlighted with a red box), 'Enrollment', and 'Comments'. A red arrow points from the 'Visit List' button to a 'CLINIC VISITS' section on the right. This section contains a table with columns 'Visit (click for forms)' and 'Date'. The table lists three visits: 'CL00 - Baseline Clinic', 'CL03 - 3 Month Clinic Visit' (highlighted in blue), and 'DETH - Death Clinic Visit'.

3. Use the printed Lab Blood Collection Form or enter the information directly into the study database.
4. Complete the Lab Blood Collection Form questions 1–4 at the time of collection and processing.

The screenshot shows the 'Lab Blood Collection 3 Month - Version 1' form. It has a blue header with a 'SUBMIT FORM' button. The form is divided into several sections. The first section contains fields for 'Participant ID' (800282094), 'Barcode ID', 'Visit Code' (CL03), and 'Date of Visit (MM-DD-YYYY)'. The second section, 'Blood Collection 3 Month', contains questions 1 through 5. Question 1 is a dropdown menu. Question 2 is a date field (MM/DD/YYYY). Question 3 is a time field (HH:MM). Question 4 is a text field. Question 5 is a text field for 'Serum Tube (red) Barcode' with a 'Barcode Format: AAAnnnn02(Alpha, number, 02)' and a 'For Internal Use Only' checkbox. The third section, 'Processing/Aliquot Creation for Banking', contains a table for 'ICON LIPID SAMPLE' with columns 'Barcode', 'Type', and 'Box'. The table has one row for 'ICON Sample barcode' with a 'serum' type and an empty box. The form ends with a blue footer containing a 'SUBMIT FORM' button.

5. Form barcodes may be scanned with the same barcode scanner used to scan tube labels. If you scan a barcode into the database, the additional fields in that row must also be entered (type, volume, box), otherwise it will be flagged as “missing” data. If scanners are unavailable, please manually enter the barcode in the indicated field following the format shown on the form (Please note: Manual entry is discouraged due to potential for errors). **The information entered in the Lab Blood Collection Form is used to create the shipping manifest, so please ensure all of the form data is accurate.**

APPENDIX 6.2 SHIPPING FROZEN SAMPLES TO THE MAYO BIOREPOSITORY

Site staff will need to generate a shipping manifest for each cryovial storage rack/box included in shipments to the biorepository, e.g., if you are shipping three 96-count storage racks/boxes, you will need to generate 3 manifests. To generate shipping manifests:

1. [Log in to the study website](#) and navigate to Data > Lab Tracking. Select your site and choose “Mayo” as the lab.

The screenshot shows a navigation bar with buttons: LAB TRACKING (highlighted in red), Site List, SITE: 1999 - Non-VA Test, and Lab List. Below this is a section titled 'Choose the Lab:' with two buttons: ICON and MAYO (highlighted in blue).

2. Select the box(es) containing the samples you are shipping. The term “box” on the study website refers to the cryovial storage rack/box. The study database will display a manifest for each Mayo Biorepository cryovial storage rack/box. Carefully review the information on each manifest to make sure the box number is correct and the number of samples on the manifest matches the number of samples in the cryovial storage rack/box. If any information is incorrect or missing, the information must be corrected by updating the Blood Collection Form for that participant.

The screenshot shows the 'LAB TRACKING' interface with the 'LAB: MAYO' button selected. Below the navigation bar, a message states: 'If any of the samples listed in the table below are incorrect, this information must be corrected by updating the Blood Collection Form for that participant.' The 'Box Information' section includes a 'Date Shipped' field (04-29-2021) and a 'Tracking Number' field (123124135), with a 'SUBMIT' button. Below this is a 'Generate Manifest' button. The 'Sample Count' is 3. A table displays the sample information:

Sample #	Blood Collection Form Question #	PID	Barcode	Sample Type	Volume	
1	8	100058035	123456789	Serum	500 mL	REMOVE
2	9	100058035	987654321	Plasma	750 mL	REMOVE
3	10	100058035	111111111	WBC	500 mL	REMOVE

3. If a sample has been destroyed or a sample needs to be removed from the shipping manifest, click the “Remove” button and enter the reason for removal. Removed samples will turn red. If a sample is removed by mistake, click on the “Add Back” button to add it back to the manifest.

Remove sample from Manifest

Accession:

123456789

Reason for removal:

SUBMIT

LAB TRACKING

Site List

SITE: 1999 - Non-VA Test

Lab List

LAB: MAYO

Box List

BOX: 210

If any of the samples listed in the table below are incorrect, this information must be corrected by updating the Blood Collection Form for that participant.

Box Information

You must enter this information before you can generate the manifest.

Date Shipped:

04-29-2021

Tracking Number:

TEST_1

SUBMIT

Generate Manifest

Sample Count: 3

Sample #	Blood Collection Form Question #	PID	Barcode	Sample Type	Volume	
1	8	100058035	123456789	Serum	500 mL	ADD BACK
2	9	100058035	987654321	Plasma	750 mL	REMOVE
3	10	100058035	111111111	WBC	500 mL	REMOVE

- Enter the date shipped and the FedEx tracking number in the Box Information section and click the green “Submit” button to save the shipping information.
- After saving the shipping information, click the blue “Generate Manifest” button and print 2 copies. Place a copy of the manifest in the box to be shipped to Mayo and save the other copy in the site’s study records. Copies of all shipping manifest should be saved in the site’s study records.

APPENDIX 6.3 SHIPPING FROZEN SAMPLES TO THE ICON LAB

Site staff will need to generate a shipping manifest for each lipids storage cryobox included in shipments to ICON, e.g., if you are shipping three 49-count cryoboxes, you will need to generate 3 manifests. To generate shipping manifests:

1. [Log in to the study website](#) and navigate to Data Management > Lab Tracking. Select your site and choose “ICON” as the lab.

LAB TRACKING Site List SITE: 1999 - Non-VA Test Lab List

Choose the Lab:

ICON MAYO

2. Select the cryobox(es) containing the specimens you are shipping. The study database will display a manifest for each ICON cryobox. Carefully review the information on each manifest to make sure the box number is correct and the number of samples on the manifest matches the number of samples in the box. If any information is incorrect or missing, the information must be corrected by updating the Blood Collection Form for that participant.

LAB TRACKING Site List SITE: 1999 - Non-VA Test Lab List LAB: ICON Box List BOX: 199

If any of the samples listed in the table below are incorrect, this information must be corrected by updating the Blood Collection Form for that participant.

Box Information

You must enter this information before you can generate the manifest.

Date Shipped: 04-21-2021 Tracking Number: DFGHDKD123421 SUBMIT

Generate Manifest

Accession	Site	Subject	Year of Birth	Gender	Visit Name	Container Name	Collection Date	Collection Time	
BY8797202	1999	100001270	1927	F	CL00	CHEMARCH	06-Apr-2021	14:35	REMOVE
HE1234902	1999	100000460	1940	M	CL00	CHEMARCH	13-Apr-2021	12:34	REMOVE
HY7654502	1999	100001785	1925	M	CL00	CHEMARCH	13-Apr-2021	12:34	REMOVE

- If a sample has been destroyed or a sample needs to be removed from the shipping manifest, click the “Remove” button and enter the reason for removal. Removed samples will turn red. If a sample is removed by mistake, click on the “Add Back” button to add it back to the manifest.

Remove sample from Manifest

Accession:

HE1234902

Reason for removal:

SUBMIT

LAB TRACKING

Site List

SITE: 1999 - Non-VA Test

Lab List

LAB: ICON

Box List

BOX: 199

If any of the samples listed in the table below are incorrect, this information must be corrected by updating the Blood Collection Form for that participant.

Box Information

You must enter this information before you can generate the manifest.

Date Shipped:

04-21-2021

Tracking Number:

DFGHDKD123421

SUBMIT

Generate Manifest

Accession	Site	Subject	Year of Birth	Gender	Visit Name	Container Name	Collection Date	Collection Time	
BY8797202	1999	100001270	1927	F	CL00	CHEMARCH	06-Apr-2021	14:35	ADD BACK
HE1234902	1999	100000460	1940	M	CL00	CHEMARCH	13-Apr-2021	12:34	REMOVE
HY7654502	1999	100001785	1925	M	CL00	CHEMARCH	13-Apr-2021	12:34	REMOVE

- Enter the date shipped and the FedEx tracking number in the Box Information section and click the green “Submit” button to save the information.

LAB TRACKING

Site List

SITE: 1999 - Non-VA Test

Lab List

LAB: ICON

Box List

BOX: 112

If any of the samples listed in the table below are incorrect, this information must be corrected by updating the Blood Collection Form for that participant.

Box Information

You must enter this information before you can generate the manifest.

Date Shipped:

06-08-2021

Tracking Number:

124123123123

SUBMIT

Generate Manifest

5. After saving the shipping information, click the blue “Generate Manifest” button. Clicking the “Generate Manifest” button will immediately send an electronic copy of the manifest to the ICON lab so they know to expect the box for processing.
6. Once the shipping manifest has been generated, print 2 copies. Place a copy in the box to be shipped to ICON and save the other copy in the site’s study records. Copies of all shipping manifest should be saved in the site’s study records.

Do not include the ICON requisition form in any shipments to avoid unnecessary queries. The ICON requisition form is not used for the PREVENTABLE study.

APPENDIX 6.4 REQUESTING AND PAIRING BARCODE SCANNERS

Sites that do not have a barcode scanner may request one by emailing PREVENTABLE@duke.edu and copying their assigned CRA on the email.

To pair the barcode scanner with a computer

1. Turn on the scanner by pressing the main button.
2. Insert the USB dongle in a USB port on a computer.
3. Wait 30 seconds for the devices to pair automatically.

To pair the barcode scanner with an Android or iOS device

1. Press the main button for 8 seconds to enable Bluetooth pairing.
2. In your device's Bluetooth connection settings, search for the device "ScanAvenger."
3. Click "pair" in your device's Bluetooth settings to connect your device to the scanner.

To test the scanner, scan a barcode that is not a configuration barcode in the scanner's user guide.

To exit setup mode, double tap the main button on the scanner.

Barcode Scanner Indicators	
<u>LED Light</u>	<u>Description</u>
Blue LED Light	Scanner is on
Red LED Light	Scanner is charging
<u>Beep</u>	<u>Description</u>
1 long beep	Power on/off
1 short beep (low frequency)	Common barcode scanned or scanner paired successfully
5 short beeps (low frequency and stops scanning)	Scanner is out of power and needs to be charged
2 short beeps (low frequency)	Scanner disconnected from device

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CHAPTER 7. CALL CENTER BASELINE AND ANNUAL TELEPHONE VISITS

Most of the information in this chapter is specific to Call Center staff. However, some content is applicable to both site staff and Call Center staff. Sections that are applicable to both groups are identified with “[Sites and Call Center]” in the section heading.

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

7.1 OVERVIEW [SITES AND CALL CENTER]

The PREVENTABLE primary outcome of dementia is determined by the annual administration of a standardized cognitive test battery to all PREVENTABLE participants. To determine the primary study outcome of persistent disability, physical function questionnaires are administered during annual follow-up calls.

Following the in-person or telehealth baseline visit conducted by site staff, each participant receives a baseline phone call from the PREVENTABLE Call Center, based at Wake Forest University School of Medicine. The Call Center is staffed by trained professionals who have experience conducting phone interviews, especially cognitive testing with older adults. The components of the baseline and annual follow-up phone assessments are outlined in this chapter.

The Call Center will complete the baseline call within 21 days of the date of enrollment. Annual calls take place within a 12-week window based on the participant’s randomization date (6 weeks prior/6 weeks following). Participants will be called annually until the study ends, consent is fully withdrawn or a modified retention status is agreed upon, or the participant is adjudicated as probable dementia. See **Chapter 16** for details on the processes for participants adjudicated as probable dementia.

NOTE: Call scripts and assessment questions are provided in this chapter only for training purposes. When conducting baseline and follow-up calls with participants or their trusted contacts, Call Center staff should use the call scripts and test administration forms in the Call Tracking system that are approved by the central IRB.

Call Center staff will print the forms packet before each call to record the participant’s or trusted contact’s responses during the call. To print the forms packet, login to the PREVENTABLE website and navigate to Documents > Call Center and select the appropriate call type to print the forms packet. The Call Center staff member will follow the instructions on the form or in the Call Tracking System to score and enter the responses into the relevant call screen within the Call Tracking System. Callers will enter the data via either the Data Entry tab or through the guided data entry in the Call Tracking System.

7.2 SCHEDULING THE BASELINE PHONE CALL [SITES AND CALL CENTER]

During the baseline visit, site staff will provide a Welcome Packet to participants and inform them of the baseline call, the next step in the PREVENTABLE study. Participants must complete the baseline phone assessment to be randomized into the study. See **Chapter 9** for details.

The Welcome Packet documents, available in English and Spanish, are provided on the study website under Quick Links > Documents > Enrolled Participants Resources. The Welcome Packet includes the Enrolled Participant Handout, Trusted Contact Handout, Participant Cognitive Testing Information Sheet, Meet the Call Center Flyer, Study Drug Handout, and the Wallet Card. If the baseline visit is conducted via telehealth, site staff will mail or email the Welcome Packet to the participant.

While the participant is in the clinic or on the phone for the baseline visit, site staff will schedule the baseline phone call with the Call Center at a time that is convenient for the participant. Follow the steps in the video on the study website under Quick Links > Training Videos > Training > Baseline Clinic Visit Walkthrough > 5. Enrolling a Participant and Scheduling the Baseline Call. The Call Center operates during the evenings and some weekend hours to accommodate all continental US time zones. Site staff should write the day and time of the baseline call on the Enrolled Participant Handout and review it with the participant.

If the baseline call is not scheduled during the baseline visit, site staff will attempt to contact the participant to schedule the baseline call. If the baseline call is not scheduled within 3 days of the baseline visit, the Call Center will attempt to contact the participant to schedule the baseline call. If the Call Center is unable to contact the participant after 3 attempts, site staff will be notified to follow up with the participant via a “Back to Clinic” alert on the study website. See **Chapter 9** for details on alerts for the baseline call.

Participants who have provided permission to be contacted via text message or email will receive automatic reminder messages 3 days and 1 day prior to their Call Center appointment.

7.3 BASELINE PHONE VISIT OVERVIEW

To complete the baseline call, participants will need to be:

- **Available to complete the full call:** The call takes between 20–40 minutes.
- **In a quiet location:** Participants will need to be able to clearly hear the Call Center staff. They will be asked to minimize distractions and turn off radios, TVs, and cell phones (if using a land line).
- **Alone in the room:** Participants will be asked to separate themselves from other people during the call. It is important that participants answer questions without any assistance.

The baseline call includes the following assessments:

- Telephone Interview for Cognitive Status-modified (TICS-M)
- Patient Reported Outcomes Measurement Information System – Physical Function (PROMIS PF)

If the participant’s TICS-M score is less than 31, the Extended Cognitive Assessment Battery tests will be administered in the following order:

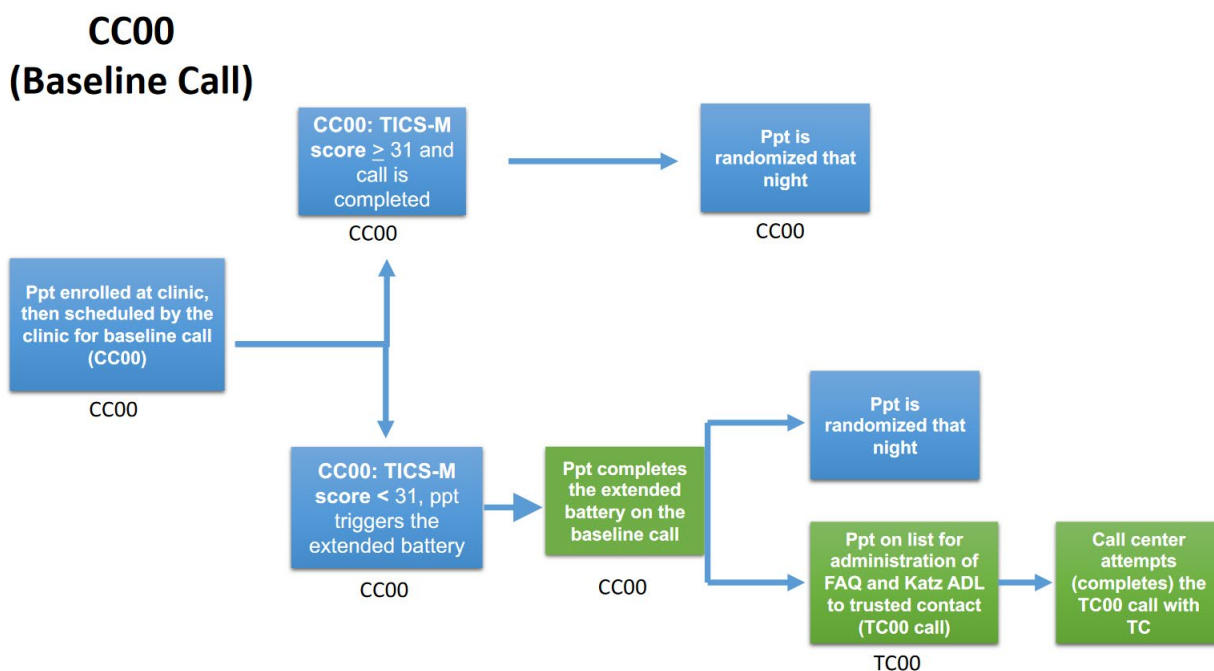
- Craft Story – Immediate Recall
- Number Span
- Category Fluency – Animals and Vegetables
- Phonemic Fluency – Letter F and Letter L

- Oral Trail Making Test
- Craft Story – Long Delay Recall
- PHQ-8

Additionally, after a baseline call with a participant who scores less than 31, the Call Center will reach out to a trusted contact to collect additional information about the participant via the:

- Functional Activities Questionnaire (FAQ)
- Katz Activities of Daily Living (Katz ADL)
- Patient Reported Outcomes Measurement Information System – Physical Function (PROMIS PF)

Figure 7.1. Baseline Call Workflow



7.4 ANNUAL FOLLOW-UP PHONE VISITS OVERVIEW

On an annual basis, the Call Center will contact participants who have a retention status of Active or Modified Follow-up (allowing annual phone calls) to schedule and complete the annual cognitive battery. Participants who opted in to receive texts or emails on the Contact Information Form will receive an automatic text or email notification that the Call Center will contact them within the next 2 weeks to schedule their annual call.

The annual follow-up calls can take between 20–40 minutes to complete. In addition to the assessments completed at baseline, the Katz Index of Independence in Activities of Daily Living (Katz ADL) and questions about study drug adherence will be administered during the annual calls. In annual follow-up calls, the Katz ADL will be administered before the PROMIS PF, and prior to the Extended Cognitive Battery, if the extended battery is required.

7.4.1 COVID QUESTIONS

During annual follow-up visits, ask the participant about their COVID-19 status.

Say, “Have you tested positive for COVID in the past year?” If yes, ask, “Was this a clinic/lab or home-based (self) test?”

Enter the response(s) in the participant’s dashboard on the study website.

7.4.2 ADHERENCE QUESTIONS

Additionally, during annual follow-up visits, ask the participant about their study drug adherence.

Say, “I see that we shipped your last shipment of study medication on (populated) date. Did you receive the study medication?”

If the participant reports not receiving the study drug, verify the mailing address on the Contact Information Form with the participant. The site staff will receive a “Study Drug Not Received” alert on the study website and will need to follow up with the participant or the study pharmacy to resolve the alert.

Say, “Think about the past month. Over the past month, in a 7-day week, on average how many days would you estimate that you take the study tablets?”

If the participant reports taking the study drug for 4 days or less, notify the participant that the site staff will contact them to follow up. Site staff will receive a “Participant has been Non-Adherent” alert on the study website to contact the participant to review the study procedures and discuss the importance of remaining adherent to the study drug. If a participant is non-adherent, site staff are responsible for updating the study drug status on the participant’s Information tab.

7.4.3 KATZ ACTIVITIES OF DAILY LIVING (ADL) – PARTICIPANT

The Katz ADL assesses functional status as a measurement of the participant’s ability to perform activities of daily living independently.

The Katz ADL ranks performance in 5 ADLs and can be administered by observation or interview, and in some cases, by interviewing the trusted contact.

The Call Center staff will ask the participant about their abilities in bathing, moving in and out of bed or chairs, using the bathroom, getting dressed, and feeding themselves.

The participant should indicate independence (no supervision, direction, or personal assistance) = 1 point or dependence (with supervision, direction, personal assistance, or total care) = 0 points.

Examples for each category are provided and may be stated to the participant. Based on the participant’s responses, place a 1 (independent) or 0 (dependent) on the line provided.

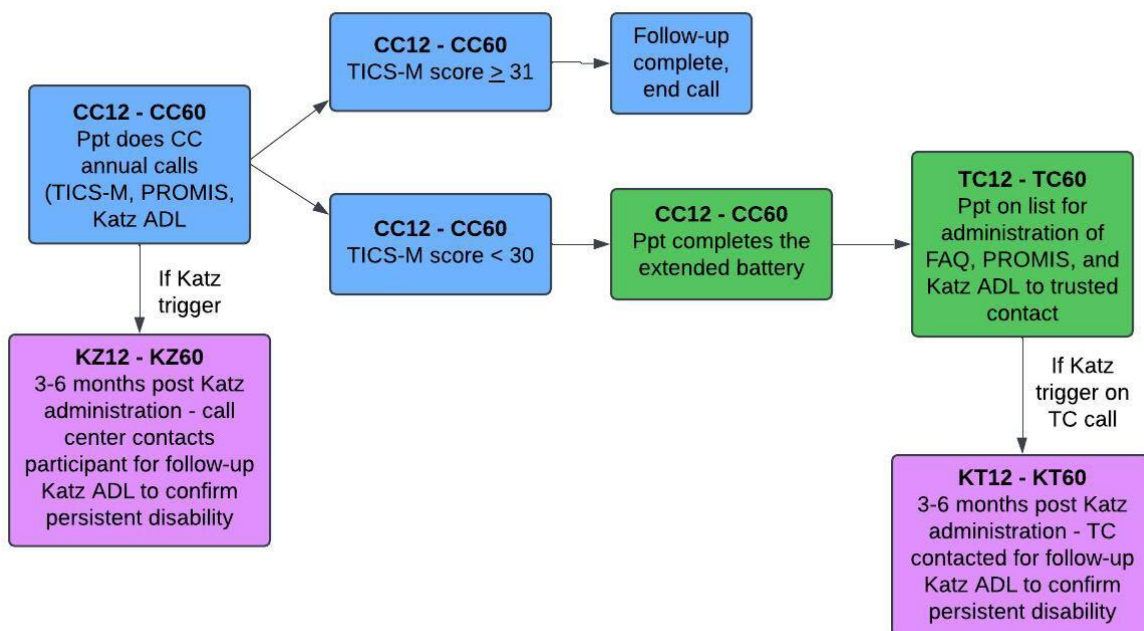
If the participant indicates a dependence in any of the categories, the Call Center will conduct a follow-up call 3–6 months later to determine the study outcome of persistent disability. The entire Katz ADL will be re-administered. Any confirmation of continued dependence (same or different ADL) will be

considered an “outcome event” and the participant will not be administered the Katz ADL or PROMIS PF during subsequent calls.

If a participant triggered the Extended Cognitive Battery and either the participant or the trusted contact indicates a dependence in any of the categories, the Call Center will contact the participant and the trusted contact 3–6 months later to follow up and determine the study outcome of persistent disability. Any confirmation of continued dependence (same or different ADL) by the participant or the trusted contact will be considered an “outcome event” and the participant and trusted contact will not be administered the Katz ADL or PROMIS PF during subsequent calls.

For the trusted contact follow-up call to confirm dependence on the Katz ADL, the Call Center will make every effort reach the same trusted contact that was initially interviewed. If unsuccessful after multiple attempts, the alternate trusted contact will be called and the entire Katz ADL will be administered. Any confirmation of continued dependence (same or different ADL) by the alternate trusted contact will be considered an “outcome event” and the participant and trusted contact will not be administered the Katz ADL or PROMIS PF during subsequent calls.

Figure 7.2. Annual Follow-Up Call Workflow



7.5 CALL CENTER TIPS FOR CONDUCTING PHONE VISITS

Optimizing Phone Connections

- Use a phone head set with built-in headphones and a microphone to improve voice quality and allow for hands-free typing or writing.
- At the very beginning of the call, explain to the participant what you will do if the call is interrupted. Typically, the Call Center will immediately call back the participant.

- Work with the participant to determine the best way to use their phone during testing. Options include holding it, using the speakerphone, or using headphones/ear buds with a built-in microphone. If the participant is hands-free during the call, the risk of inadvertent disconnection by accidentally pressing phone buttons is reduced.
- Ask the participant to always speak directly into the microphone. If you cannot hear them clearly, you may need to remind them to speak into the microphone throughout the call.

Improving Voice Clarity

- Speak a bit slower than you would face to face and say each word clearly and completely, similar to the pace newscasters use.
- Enunciate each word. Some sounds are not heard as clearly or are difficult to distinguish from other sounds over the phone, e.g., the 'p' in 'whip' is often heard as a 't' like in 'wit.' This can be a problem on word list recall tasks. You may need to exaggerate certain letters or syllables.
- If you speak with an accent, you may need to adjust your speech for some words. You can practice with a person who does not speak your native language until you are ready.
- If the participant indicates Spanish is their preferred language on the Contact Information Form, a fluent Spanish-speaking Call Center staff member will conduct the call. Otherwise, calls will be completed in English.

Managing Data Integrity

- Follow the IRB-approved scripts and the test administration forms in the Call Tracking System when conducting baseline and follow-up calls with participants or their trusted contacts. Call scripts and assessment questions are provided in this chapter only for training purposes.
- The audio of each phone visit is recorded. Recordings can be used to check administration errors and scoring accuracy and to facilitate discussions among team members about specific responses.
- Call Center staff are encouraged to discuss challenging situations with each other and, as a team, propose administration or scoring rule adjustments if needed. For example, if a particular word on a word list recall task is repeatedly misunderstood, the team may propose to accept it if it is consistently reported incorrectly by the participant.
- Audio recordings of phone visits will be used for periodic quality assurance review and recertification of Call Center staff.
- **REMEMBER:** Each participant must be given the cognitive tests in the same language every time. If the cognitive battery was administered in Spanish during the baseline call, it must be administered in Spanish during annual follow-up calls.

7.6 BASELINE CALL

Call Center staff will see their upcoming calls on the main Call Tracking System page. To start a call, click on the participant's name and follow the IRB-approved script in the Call Tracking system. The script below is provided for training purposes only.

Introduction to the Baseline Call

"Hello this is _____ (insert caller name) and I'm calling today on behalf of the PREVENTABLE study. Is this _____ (insert participant preferred name) or is _____ (insert participant preferred name) available?"

If the participant was not the person who answered the phone, please re-introduce yourself and PREVENTABLE once the participant is on the line.

"We appreciate your participation in PREVENTABLE. By taking part, you are playing an important role in this major research study that will help doctors learn whether regular use of a statin will help prevent memory impairment or disability in older adults."

"I know that you completed a visit with _____ clinic and now I'd like to ask you some questions over the phone. This could take up to 20–40 minutes. Do you have time to speak with me now?"

Schedule an alternative time if the participant is unable to talk.

"I see that you have previously indicated that you prefer to be addressed as _____. Is that how you would like me to address you today?"

"Just as a reminder, I will be recording this interview to allow us to accurately capture your answers. Recordings are only used for quality control to make sure that I am conducting the interview in the correct way. If for any reason we get disconnected, I will call you back right away at this same number."

Next say, **"Before we get started, I want to make sure that there are no distractions around you while we complete this next set of questions."**

Ask the questions below to ensure that the participant will not be distracted or use any aids such as paper, pencils, calendars, newspapers, or a watch with a date on it during the call. If the participant says they are in a room with other people, pets, or electronics (TV, radio, computer, cell phone [if using landline]), please explain the importance of minimizing distractions so they can concentrate during the tests. If the participant needs to move to another location or remove any items that might aid responses, wait for them to move to a new location, place any aids out of sight, and get comfortable.

"Are you in a quiet environment where you will not be disturbed for the next 20–40 minutes?"

Yes/No

If No, say, **"Please go to a quiet place where you will not be disturbed for the next 20–40 minutes."**

"Do you have pets that need to be taken care of so that you can stay focused during this interview?"

Yes/No

If Yes, say, **"Please go ahead and take care of your pet, so we can continue without interruption."**

“Do you have any pencils, pens, paper or any calendars, newspapers, or a watch with a date on it in front of you? Everything I ask you to do today must be done in your head. It’s very important that you don’t write anything down.”

Yes/No

If Yes, say **“Please put them away. OK?”**

“Do you have a TV, radio, or computer turned on in front of you?” Yes/No

If Yes, say, **“Please turn everything off so that you are not distracted during our interview.”**

“Is there anyone nearby, in the same room?” Yes/No

If Yes, say, **“Please separate yourself from them by going into a different room, or by asking them to move into another room so that you are not disturbed during our interview.”**

“Do you need to use the bathroom or get a drink of water?” Yes/No

If Yes, say, **“Please go ahead and take care of this so we can continue without interruption.”**

Tell the participant that once the testing starts, it is preferable not to stop before completing the entire call.

Ask the following questions to determine if the participant can hear well enough to conduct the phone interview. Place an X in the appropriate box on the participant’s call screen.

“I have a few questions about your hearing to make sure you can hear me well enough over the telephone.”

“Do you usually have trouble hearing when one person speaks to you?” Yes/No

“Do you usually have trouble hearing over the phone?” Yes/No

“Can you hear me well enough?” Yes/No

“Do you use a hearing aid?” Yes/No

If Yes, **“Are you wearing it now?”** Yes/No

If the participant is not wearing their hearing aid and cannot hear you, ask if they can put on the hearing aid, or if that is not possible, ask to reschedule the call.

If No, **“Would you please put it on?”** Yes/No

Wait for the participant to confirm that the hearing aid is in place.

“Would you please repeat the following statement, ‘I have a cat so all I need is a dog.’”

Did the participant correctly repeat the statement? Yes/No

If No, then repeat the statement, **“I have a cat so all I need is a dog.”**

Did the participant correctly repeat the statement? Yes/No

If Yes, proceed with testing.

If No, but you think, based on the interaction up to this point, that their poor hearing can be compensated for, proceed with testing and speak slowly, loudly, and as clearly as possible.

NOTE: For participants who have difficulty hearing, it is not necessarily helpful merely to increase the volume of your voice. A change in pitch tends to be effective, with a lower-pitched voice more audible than a loud, high-pitched voice.

If No, and you think the participant’s difficulty hearing will severely bias the cognitive test results, **DO NOT CONTINUE** with testing. Inform the participant that it is not possible to continue with the testing today. Here is some suggested text for that situation:

“Thank you very much _____ (participant preferred name) but you may not be able to hear me well enough to complete the questionnaires over the telephone so I will not continue at this time. On behalf of the PREVENTABLE study team, thank you for taking time to speak with me today.”

NOTE: If the baseline call cannot be completed, site staff will receive a “Call status is Back to Clinic – Baseline” alert on the study website and the participant will not be randomized.

7.6.1 INTRODUCTION TO TESTING

The script provided will help the participant understand what you can and cannot do during the specific tests.

“For this next part, I have a script that I must stick to. It is very important that I provide the same instructions to all potential participants so that my interviews are all the same. I may sound a bit ‘robotic’ but that’s why. Does that make sense?”

“Some of the questions I am going to ask will be easy to answer, and others will be more difficult to answer. You are not expected to get all questions right, so don’t worry about how well you are doing; just try to give each question your full attention and effort.”

“I will provide instructions before each task. For most tasks, however, once we start I cannot repeat the instructions, so please listen carefully when I tell you what to do. I will try to speak very clearly. I also cannot tell you whether your answers are right or wrong, or offer any help. All that matters is that you do your best. Do you understand?”

“Finally, once we begin this next portion, it’s important that we continue without stopping if at all possible. Do you need to take care of anything before we begin? If so, I’ll wait for you.”

“Do you have any questions?”

Allow time for the participant to ask questions to ensure they understand the process. Once all questions have been answered, administer the TICS-M and then the PROMIS PF.

7.6.2 TELEPHONE INTERVIEW FOR COGNITIVE STATUS-MODIFIED (TICS-M)

The TICS-M (Brandt, 1988) assesses global cognitive functioning similar to the Modified Mini-Mental State Examination (MMSE) but can be administered over the phone. It assesses several cognitive abilities and can be completed in about 10 minutes.

Materials Needed

- Test administration form
- Pen, if using a paper form

Instructions for Administering the TICS-M

- Use reasonable judgment in allowing sufficient time for the participant to answer a question.
- If the participant changes an incorrect response to a correct response at any time during the TICS-M, credit should be given for the correct response.
- Likewise, if the participant changes a correct response to an incorrect response, the new response should be recorded and credit should not be given.
- Participant responses should be recorded on the provided lines or entered directly into the TICS-M Form for that participant's call within the call tracking system.
- Score each item. Enter the score for each item into the TICS-M Form in the participant's call screen in the Call Tracking system and the score will be automatically calculated at the bottom of the screen. **The maximum score for the TICS-M is 50.**

Question 1

Orientation to Name:

- The participant should provide their first and last name.
- 1 point for each correct first and last name.

Orientation to Time and Place:

- **Year:** Two- or four-digit year is acceptable.
- **Season:** If near seasonal transitions, accept either season as correct (e.g., late August – accept summer or fall, early March – accept winter or spring). Some regions have no clear seasonal change. Other common local references such as “dry season,” “rainy season,” etc. are also acceptable.
- **Month:** The participant should provide the current month.
- **Today's date:** The participant should provide the current numerical date.
- **Day of the week:** The participant should name the specific day, i.e., Monday, Tuesday, etc.
- **Age:** The participant should give their correct age (the participant's age is displayed at the top of the participant's call screen in the Call Tracking System on the study website).
- **Phone number:** The participant should give their phone number. This should match the phone number used to contact them.

Scoring: 1 point for each correct response; 0 for incorrect response. The maximum score is 9 for questions 1a to 1i.

Question 2

Sustained Attention and Concentration:

- Ask the participant to count backwards from 20 to 1.
- If the participant did not correctly count backwards on Trial 1, then administer Trial 2.

Scoring: 2 points if the participant correctly completed Trial 1; 1 point if they correctly completed Trial 2; 0 points if they did not complete the task.

Question 3

Word Recall:

- Slowly read the list of 10 words and ask the participant to recall as many of the words as possible.
- Mark each word the participant can recall.
- If the participant states they did not hear or understand the word, you may repeat the word.
- If the participant misunderstands a word, you may accept the following substitutions:
 - Pipe: Hype, Type, Kite, Height, Pike
 - Silk: Quilt, Milk
 - Whip: With, Hip, Quip, Wit
 - Pillow: Willow, Hello, Jell-O, Yellow
 - Chest: Test
- If you give credit for the substitution word on the first recall, you cannot give credit for the correct word on the second recall. For example, if the participant says “hype” on the first recall, credit may be given, but then says “pipe” on the delayed recall, that would be considered incorrect and no credit would be given.

Scoring:

- 1 point for each correct response
- No penalty for repetitions or intrusions, but do not count toward total
- Plurals are considered correct
- Maximum Score = 10

Question 4

Attention and Mental Calculation – Serial 7s:

- Ask the participant to subtract 7 from 100 and continue subtracting until you ask them to stop.
- Do not coach the participant by saying “and 7 from ____ is?” Instead, provide a general prompt of “keep going.”

- Write the participant's response in the box below each number. Do not place checkmarks in the boxes.

Scoring:

- **NOTE:** Be careful when scoring this item. An answer is considered correct if it is **exactly 7 from the previous answer**, regardless of whether the previous answer was correct.
- Give 1 point for each correct subtraction, even if the subtraction was from a previously incorrect response.
- Maximum Score = 5

Questions 5–8

Naming:

- Q5: Score = 1 point for a response of “scissors” or “shears”; 0 for any other response.
- Q6: Score = 1 point for a response of “12”; 0 for any other response.
- Q7: Score = 1 point for a response of “cactus” only; 0 for any other response.
- Q8: Score = 1 point for a response of “sheep” or “lamb” only; 0 for any other response.

Questions 9–10

Repetition:

- Q9: ‘No ifs, ands or buts’ – Articulate the words so all plural “s” endings are audible. The participant must repeat the phrase exactly as it is stated for 1 point.
- Q10: ‘Methodist Episcopal’ – The participant must repeat the phrase exactly as it is stated for 1 point.

Scoring: The score is based only on the participant's **first attempt** to repeat the phrase.

Questions 11–12

Recent Memory:

- The participant must give both the first and last name to receive 2 points.
- Prompt the participant for the first name if only the last name is given.

Scoring:

- 1 point for correct first name
- 1 point for correct last name
- 0 if not known

Question 13

Motor Speed and Dexterity:

- Ask the participant to tap 5 times on their phone's receiver.
- If the participant is using a cell phone, they may be instructed to tap on the back of the phone or clap their hands so the call is not disconnected.

Scoring:

- 2 points if 5 taps are heard
- 1 point if the participant taps more or less than 5 times
- 0 points if no taps are heard

Questions 14–15

Opposites:

- Q14: Opposite of west
 - You may spell “west” if the participant does not hear the word correctly, e.g., they hear wet and responds with dry.
 - **Scoring** = 1 point for “east”; 0 for any other response.
- Q15: Opposite of generous
 - Acceptable responses include selfish, greedy, stingy, tight, cheap, mean, meager, skimpy, or other appropriate antonyms.
 - **Scoring** = 1 point for a correct response; 0 for an incorrect response.

Question 16

Short-Term Memory:

- Ask the participant to recall as many of the words as possible from the list of 10 words previously read.
- Place a check for each word the participant can recall.

Scoring:

- 1 point for each correct response
- No penalty for repetitions or intrusions
- Plurals are considered correct
- **Maximum Score = 10**

TICS-M Total Score

The TICS-M must be scored and the data entered while the participant remains on the phone. Score each TICS-M question on the form and enter the scores in the TICS-M Form for the participant's call in the Call Tracking system. The total score will be automatically calculated. If the participant's total score

is less than 31, administer the Extended Cognitive Battery following the administration of the PROMIS PF.

The Extended Battery data entry screen button will automatically appear after the PROMIS PF Form is submitted. Click “Submit and Continue with Extended Battery.”

7.6.3 PROMIS PHYSICAL FUNCTION (PROMIS PF)

The PROMIS Physical Function instrument measures self-reported **current capability** rather than actual performance of physical activities. This includes the functioning of upper extremities (dexterity), lower extremities (walking or mobility), and central regions (neck, back), as well as important activities of daily living, such as running errands.

A single Physical Function capability score is obtained from the 20-item short form.

Materials Needed

- Test administration form
- Pen, if using a paper form

Instructions for Administering the PROMIS PF

PROMIS instruments are scored using item-level calibrations. The score will be automatically calculated when the responses are entered in the participant-specific call screen in the Call Tracking System on the study website.

The PROMIS PF 20-item short form is divided into 2 sections. Each section has 5 response options ranging in value from 1 to 5. Provide the response options to the participant as often as necessary.

Questions 1–14

Say, “Now I’m going to ask you some questions about activities you do and how you are able to function. I want you to tell me if you can do this

- Without any difficulty
- With a little difficulty
- With some difficulty
- With much difficulty
- Unable to do”

“I can repeat those choices after each question if you need me to. I want you to choose the response that **best describes** your ability to do each task. Do you have any questions?”

Read questions 1–14 and place an X in the box that indicates the participant’s response about their current ability to do the stated task. Repeat a question if the participant did not hear or understand. If the participant is unable to decide on a response, ask them to select the one that describes their ability most of the time.

Questions 15–20

Say, “Now I’m going to ask you a few more questions about your health and I want you to select the response that best describes your ability to do each task. I want you to tell me if your health limits you from doing these things:

- Not at all
- Very little
- Somewhat
- Quite a lot
- Cannot do”

Read questions 15–20 and place an X in the box that indicates the participant’s response. You may repeat a question if the participant did not hear or understand. If the participant is unable to decide on a response, ask them to select the one that describes their ability most of the time.

Scoring: The PROMIS PF score will be automatically calculated from the responses entered into the PROMIS Form in the participant’s call screen in the call tracking system. Ending the Baseline Call after the PROMIS PF

If the participant’s TICS-M score is 31 or higher, the call will end following the administration of the PROMIS PF.

Say, “That ends the questions I have for you today. Do you have any questions about PREVENTABLE and what will happen next?”

“Please refer to the Participant Handout for additional instructions:

- Study meds will be mailed and should be expected within the next week.
- Begin taking the study meds as instructed.
- Phone numbers are listed if there are any questions following this phone call.
- Call center will call in 1 year to follow up and ask the same type of questions.”

“Thank you for being part of this important study. Your contribution will provide valuable information to doctors whether regular statin use can help prevent memory impairment and disability in older adults.”

If the participant lives with another PREVENTABLE participant

Say something like, “Now that you have completed the phone interview, since (Name of 2nd Participant) is also participating in the PREVENTABLE study and is in the same household, I ask that you please do not discuss any of the questions that I have asked you with (Name of 2nd Participant). We will ask that you go to another room in the house when we are completing the phone interview with (Name of 2nd Participant). We need to ensure that the phone interviews are conducted with each participant in the same manner, and having prior knowledge of the questions that may be asked could have an impact on the valuable information that we are gathering as part of the PREVENTABLE study. We very much appreciate your understanding and cooperation while participating in the PREVENTABLE study.”

“Thank you for being part of this important study. Your contribution will provide valuable information to doctors whether regular statin use can help prevent memory impairment and disability in older adults.”

Validity of the Participant’s Responses

After the phone call, record your impression of the validity of the participant’s responses. If there is an indication that the responses do not accurately reflect the participant’s cognitive abilities, enter the reason for your assessment. This information is extremely helpful in adjudication and data analysis. For example, it can be challenging to judge whether the participant’s difficulty hearing significantly influenced test results, but knowing it might have been a factor is helpful.

7.7 EXTENDED COGNITIVE BATTERY

Participants that score less than 31 on the TICS-M will continue with additional cognitive tests. **Do not relay this information to the participant** but make a seamless transition to the Extended Cognitive Battery tests following the administration of the PROMIS PF (and the Katz ADL at follow-up visits).

The order of tests for the Extended Cognitive Battery is:

- Craft Story – Immediate Recall
- Number Span
- Category Fluency – Animals and Vegetables
- Phonemic Fluency – Letter F and Letter L
- Oral Trail Making Test
- Craft Story – Long Delay Recall
- PHQ-8

7.7.1 CRAFT STORY – IMMEDIATE RECALL

This test assesses the participant’s ability to recall a short story. Read the story to the participant in a clear voice. Immediately after you finish the story, ask the participant to retell the story from memory.

Materials Needed

- Test administration form
- Pen, if using a paper form

Instructions for Administering the Craft Story – Immediate Recall

- Read the story with adequate volume and clarity for the participant to understand.
- No repetitions are permitted.
- It is important to get a sense of the participant’s hearing acuity and adjust your voice accordingly.

Make sure you have the participant’s attention and read the below instructions to them. Then read the story slowly, articulating clearly and with normal inflection.

Say, **"I am going to read you a story. Listen carefully, and when I am through, I want you to tell me everything you can remember. Try to use the same words I use but you may also use your own words. The story is..."**

As soon as the story is over, give the recall instruction.

Say, **"Now tell me the story I just told you. Try to remember as much as you can."**

Record the participant's recall, writing between the lines of the story on the test form. For rapid and accurate transcribing of the story, use abbreviations as needed and omit filler or irrelevant words. If absolutely necessary, say **"A little slower, please,"** rather than miss story elements. Make sure your transcription of the story is legible before proceeding.

Write legibly and leave room between each line for scoring. If it is difficult to write the participant's response, you may review the audio recording and transcribe the results after the session.

After the participant tells the story, you **must** give the delayed recall cue.

Say, **"Later on, I will ask you to tell me this story again, so try not to forget it."** Note the time so you can later calculate the time that elapsed between the Craft Story – Immediate Recall test and the Craft Story – Long Delay Recall test.

Scoring

- Scoring can be deferred until after the entire cognitive battery is administered but should be scored close to the completion time.
- Give the participant a point for a paraphrased response that captures the elements of the story although not necessarily with the exact words (see table below).
- Enter the total score on the test form.

Craft Story Scoring Guidelines

Item	General Rule: Acceptable 1-point responses	Alternative 1-point responses	0-point responses	Points
Maria's	"Maria" or a variant of the name	Mary, Marie	Martha, Anna	
child	"child" or a phrase denoting it was a young person	son, kid, boy, teenager, young man	guy, children, daughter	
Ricky	"Ricky" or a variant of the name	Rick, Rich, Richie, Richard, Ricardo	Randy, Rusty, Robert	

played	"played" is required	to play, plays	did (soccer)	
soccer	"soccer" is required	futbol	baseball, volleyball, other sport	
every Monday	"Monday" or an indication that it occurred on a weekday		every day, another day of the week	
at 3:30	an indication that the activity took place in the afternoon	after school, every afternoon	after dinner, at night, in the morning	
He liked going to the field	an indication that he went to an outdoor area	went outside, went to the yard, going to the backyard	went to the school, gym	
behind their house	"house" or word denoting a house	home, residence, where they lived	any other building	
and joining	an indication that he participated in a game	played w/ other kids, became part of the team, playing w/ the team	watching, played in the park	
the game.	"game" in any context			
One day	"One day" is required			
he kicked	indication that he performed the activity with his foot	booted, punted	threw, hit (with no mention of the foot)	
the ball	"Ball" is required	football, soccer ball	baseball, volleyball	
so hard	an indication that force was used	so much force, so strongly, (kicked it) so far		
that it went over	"Over" is required			

the neighbor's	an indication that the person lived in the same area	nearby resident	friend's	
fence	"fence" or a word denoting a fence of some kind	garden wall, wall	property line, street	
where three	"Three" is required	three (boys)		
large dogs lived.	an indication that there were dogs present	hounds, doggies	puppies, cats, kittens, other animals	
The dogs' owner	an indication that the person was responsible for the dogs	neighbor (if owner implied and "neighbor" mentioned twice)	a bystander, the police	
heard loud barking	an indication that the dogs were making noise	yelping, baying, yapping, heard the dogs	saw the dogs running around	
came out	a word or phrase indicating that the owner was present	(owner) saw the ball	his mother came out, the dogs came out	
and helped them	a word or phrase indicating that help was provided	assisted, aided, had to help		
retrieve the ball.	an indication that they got the ball back	gave him the ball, return the ball, (helped him) get the ball		

7.7.2 NUMBER SPAN TEST – FORWARD AND BACKWARD

This is a test of 2 different working memory constructs. The first, Forward Number Span, measures the participant's capacity for holding information very briefly for the purpose of repeating it exactly. The second, Backward Number Span, measures the participant's ability not only to hold the information, but also to manipulate the numbers and reverse the sequence. The number span test is also referred to as Digit Span.

Numbers for both forward and backward span tests are presented as sequences ranging from 2 to 9 numbers. Two trials are administered at each sequence length. There are 2 scores:

- The total number of correct trials
- The longest span length repeated correctly prior to failing 2 consecutive trials of the same length

Materials Needed

- Test administration form
- Pen, if using a paper form

Instructions for Administering the Forward Number Span

Say, **“I am going to ask you to repeat some numbers for me. Wait until I finish saying the numbers and then repeat them in the same order. For example, if I say 1-8-7, you would say 1-8-7. If I say 2-9-8, what would you say?”** If the participant gives the wrong answer, say, **“Actually you would say 2-9-8.”**

Say, **“Repeat only the numbers I say each time,”** then administer the items in the order indicated on the test form. Before each item, say, **“Ready?”**

Write the participant’s response on the test form. Continue testing until the participant fails 2 number spans of the same length.

Scoring – Forward Number Span

On the test form, record the total number of forward trials correct and the forward longest number span repeated correctly prior to the 2 failed trials. If the test was not completed, write the reason in the comments box of the (Baseline or Annual) Call Center Cover Page.

Instructions for Administering the Backward Number Span

Say, **“I am now going to ask you to repeat some numbers for me but to reverse them from the way I say them. Wait until I finish saying the numbers and then repeat them in reverse order, or backward. For example, if I say 3-7-4, you would say 4-7-3. If I say 7-3-6, what would you say?”** If the participant gives the wrong answer, say, **“Actually, you would say 6-3-7.”**

Say, **“Repeat only the numbers I say each time, backward, in reverse order.”** Then administer the items in the order indicated on the test form. Before each item, say, **“Ready?”**

If the participant repeats the numbers in the forward order on either span of the first 2 trials, you may remind them to say the numbers in reverse order by saying, **“Remember, after I say the number sequence, repeat the sequence backwards. Ready?”** This unrequested reminder of the instructions can only be given once during the first number span. Do not give them the number sequence again.

Write the participant’s response on the test form. Score the response as correct if the participant gives the correct reverse order after the prompt. For all future test items, if the participant repeats the numbers in forward order, score as incorrect.

Continue testing until the participant fails 2 number spans of the same length.

Scoring – Backward Number Span

On the test form, record the total number of backward trials correct and the backward longest number span repeated correctly prior to the 2 failed trials. If the test was not completed, write the reason in the comments box of the (Annual or Baseline) Call Center Cover Page.

7.7.3 CATEGORY FLUENCY – ANIMALS AND VEGETABLES

This is a widely used measure of semantic memory (verbal fluency, language). The participant is asked to name different examples of a given semantic category and the number of unique examples named is scored.

Materials Needed

- Test administration form
- Pen, if using a paper form
- Stopwatch

Instructions for Administering the Category Fluency Tests

Say, “I am going to give you a category and I want you to name, as fast as you can, all of the things that belong in that category. For example, if I say ‘articles of clothing’, you could say ‘shirt’, ‘tie’, or ‘hat’. Can you think of other articles of clothing?”

Allow up to 20 seconds for the participant to give 2 responses. Examples of responses and follow-up instructions are provided in the table below.

Response	Instruction
No response	“You could have said ‘shoes’ or ‘coat’ since they are articles of clothing.”
One or more incorrect responses, no correct response	“No, ____ is (are) not an article(s) of clothing. You could have said ‘shoes’ or ‘coat’ since they are articles of clothing.”
One or more correct response, no incorrect responses	“That’s right. You also could have said ‘shoes’ or ‘coat’.”
One or more correct responses, one or more incorrect responses	“____ is (are) correct, but ____ is (are) not an article of clothing. You also could have said ‘shoes’ or ‘coat’.”
Two or more correct responses	“That’s right.”

Read the instructions for the Animals category:

Say, “Now I want you to name things that belong to another category: Animals. You will have 1 minute. I want you to tell me all the animals you can think of in 1 minute. Ready? Begin.”

Start the timer as you say “Begin.” Write the participant’s actual responses as legibly as possible on the Worksheet for Category Fluency – Animals. Stop the test at 1 minute (60 seconds). One prompt (“Tell

me all the animals you can think of.”) is permitted if the participant does not respond for 15 seconds or expresses incapacity (e.g., “I can’t think of anymore.”). It is also permissible to repeat the instructions or category if the participant specifically requests it. Do not cue the participant about including animals other than mammals. However, if the participant asks if they can include animals other than mammals before responding or they ask during the test, you are permitted to say “yes.”

Next, read the instructions for the Vegetables category:

Say, **“Now I want you to name things that belong to another category: Vegetables. You will have 1 minute. I want you to tell me all the vegetables you can think of in 1 minute. Ready? Begin.”**

Start the timer as you say **“Begin.”** Write the participant’s actual responses as legibly as possible on the Worksheet for Category Fluency – Vegetables. Stop the test at 1 minute (60 seconds). One prompt (**“Tell me all the vegetables you can think of.”**) is permitted if the participant does not respond for 15 seconds or expresses incapacity (e.g., “I can’t think of anymore.”). It is also permissible to repeat the instructions or category if the participant specifically requests it.

Scoring

Wait to score the Category Fluency tests until after all Extended Cognitive Battery tests are complete. The tests should be scored close to the completion time. The Category Fluency – Animals total score is the number of correct, unique animal names the participant provided within the 1-minute time limit.

- **Give credit for:**
 - Breeds (e.g., Yorkshire Terrier)
 - Male, female, and infant names of a species (e.g., bull, cow, calf)
 - Both superordinate and subordinate examples of a species (e.g., both dog and terrier are credited)
 - Birds, fish, reptiles, and insects
- **Do NOT give credit for:** Repetitions or mythical animals

Write the total number of correct, unique animals on the test form and enter the score in the Category Fluency- Animals field in the Extended Battery Form in the call tracking system. If the test was not completed, write the reason on the Extended Cognitive Battery cover sheet.

The Category Fluency – Vegetables total score is the total number of correct, unique names of vegetables the participant provided within the 1-minute time limit.

- **Give credit for:**
 - Both superordinate and subordinate examples (e.g., both peppers and jalapenos are credited)
 - Less specific names (e.g., greens)
 - Nuts, (e.g., peanuts, acorns)
 - Names of vegetables found in other cultures but perhaps unfamiliar to you (e.g., jicama) are acceptable only if they can be verified in the dictionary. After finishing the

Vegetables task, ask the participant to spell the word if you are unsure of the correct spelling.

- Grains (e.g., corn, rice, wheat, oats, etc.), gourds, sugarcane, herbs, and seaweed
- Tomato, avocado, and pumpkin
- Legumes
- **Do NOT give credit for:**
 - Repetitions
 - Prepared vegetable products, e.g., pickles, tomato sauce, ketchup, etc.
 - Spices

Write the Vegetables total score on the test form and enter the score in the Category Fluency-Vegetables field in the Extended Battery Form in the call tracking system. If the test was not completed, write the reason on the Extended Cognitive Battery cover sheet.

7.7.4 PHONEMIC (VERBAL) FLUENCY TEST

This is a widely used measure of word generation that may be sensitive to dysfunction in the dominant frontal lobe. In this version, the participant is asked to say as many words as possible that begin with the letter “F” in 1 minute (60 seconds), and then say as many words as possible that begin with the letter “L” in 1 minute (60 seconds).

Materials Needed

- Test administration form
- Pen, if using a paper form
- Stopwatch

Instructions for Administering the Phonemic (Verbal) Fluency – Letter F Test (Use the letter P for Spanish speakers.)

Say, “I’m going to say a letter of the alphabet. When I ask you to start, tell me as many words as you can that begin with that letter. You will have 1 minute before I tell you to stop. None of the words can be numbers or names of people or places.”

“For example, if I gave you the letter B, you could say brown, bottle, or bake, but you wouldn’t say Barbara, Boston, or billion. Also, please try not to give me the same word with different endings, so if you said bake, you wouldn’t also say baked or bakes, and if you said big, you wouldn’t also say bigger and biggest.”

“Let’s begin. Tell me all the words you can, as quickly as you can, that begin with the letter ‘F’. Ready? Begin.”

Start the timer as you say “Begin.” Write the participant’s actual responses as legibly as possible on the test form. Stop the test at 1 minute (60 seconds).

If the participant pauses for 15 seconds say,

- “Keep going.”
- “What other words beginning with “F” can you think of?”

If the participant gives 3 consecutive words that do not start with the designated letter say, **“We are now using the letter F.”** This prompt can be provided only once.

Record all responses on the test form, including repeated words and words starting with letters other than F.

Instructions for Administering the Phonemic (Verbal) Fluency – Letter L Test (Use the letter M for Spanish speakers.)

Say, **“Now I want you to do the same for another letter. The next letter is L. Ready? Begin.”**

Start the timer as you say **“Begin.”** Write the participant’s actual responses as legibly as possible on the test form. Stop the test at 1 minute (60 seconds).

If the participant pauses for 15 seconds say,

- **“Keep going.”**
- **“What other words beginning with “L” can you think of?”**

If the participant gives 3 consecutive words that do not start with the designated letter say, **“We are now using the letter L.”** This prompt can be provided only once.

Record all responses on the test form, including repeated words and words starting with letters other than L.

Scoring

Give the participant credit for any word that begins with the specified letter, can be found in a dictionary, is not a proper noun or a number, and is not a repetition within that trial.

Although scoring most responses is straightforward, some responses are ambiguous. For example, “frank” can refer to a man’s name, a food item, or an adjective. The approach with scoring ambiguous responses is to give the benefit of the doubt and score the item as correct for the first instance of the response in a trial.

In some instances, the context in which the response is given provides clues to the participant’s intended meaning. For example, the word “for” is ambiguous and could be a preposition (for), golf term (fore), or number (4). If the word is given along with other numbers (e.g., “4, 5”), the response can be interpreted as the number 4 and be scored as a rule violation. If the response is at all ambiguous, give the benefit of the doubt.

Other types of responses that should be scored as correct include:

- Contractions
- Compound words or conjoined words that convey a single meaning (e.g., Ferris wheel)
- Slang words if they can be found in a dictionary
- Proper nouns that are not the names of people or places (e.g., days of the week, months of the year, brand names)

Write the number of correct F-words provided in 1 minute (60 seconds) on the test form and enter the score in the Verbal Fluency F-words field in the participant's Extended Cognitive Battery data entry screen within the Call Tracking System. If the test was not completed, write the reason on the (Baseline or Annual) Call Center Cover Page.

Write the number of correct L-words provided in 1 minute (60 seconds) on the test form in the Verbal Fluency L-words field in the participant's Extended Cognitive Battery data entry screen within the Call Tracking System. If the test was not completed, write the reason on the (Baseline or Annual) Call Center Cover Page.

7.7.5 ORAL TRAIL MAKING TEST

This test is a brief measure to assess mental sequencing and switching.

Materials Needed

- Test administration form
- Pen, if using a paper form
- Stopwatch

Instructions for Administering the Oral Trail Making Test

- Read the instructions verbatim to the participant.
- Accurate timing is essential to scoring. Start timing as soon as you say **"Begin."**
- Testing time is a maximum of 5:00 minutes (300 seconds) for each part of the test – Part A and Part B.
- Record the time to complete the test as minutes:seconds, rounding to the nearest second.
- Record the number of errors. After 5 errors, discontinue the test and record the time as 5:00 minutes.
- **Discontinue Rule for Part A and Part B:** If the participant makes 5 errors or reaches 5 minutes (300 seconds), discontinue the test. Check the "Discontinued" box in the data entry screen and enter the time as 5:00 minutes (300 seconds) and the score as 5 errors.

Instructions for Administering Part A

Say, **"I'd like you to count from 1 to 25 as quickly as you can. 1, 2, 3, 4, and so on. Ready?"** Wait for a response and then say, **"Begin."** Start timing as soon as you say **"Begin."**

Mark the participant's responses on the Oral Trail Making Test – Part A: Scoring Worksheet. If the participant makes a mistake, score as an error by writing 'E' beneath the incorrect number. Stop the participant and say, **"You were correct up to __, continue from __"** and ask them to continue with the series from the last correct number. Do not stop timing during corrections.

If the participant stops for 5 seconds or more, you may prompt them with **"Please keep going."** If the participant does not recall where they are in the series, provide the last correct response by saying, **"You were correct up to __, continue from __."** and score as an error by writing 'E' under the number where the error occurred.

After a further delay of 15 seconds or more, discontinue the test, by saying, **“That's fine, let's go on to the next task.”** Check the “Discontinued” box on the data entry screen and enter the time as 5:00 minutes (300 seconds) and the score as 5 errors.

Scoring Part A

On the test form, record the total number of errors and the time required to complete the task, including the time to offer corrections, in minutes:seconds, rounding to the nearest second.

Remember: Discontinue the test if a participant makes 5 errors or if they do not complete the task in 5 minutes. Check the “Discontinued” box in the data entry screen and enter the score as 5 errors and the time as 5:00 minutes (300 seconds).

If a participant does not attempt the test, check the “Discontinued” box, enter the score as 5 errors and the time as 5:00 minutes (300 seconds). Make a note on the (Baseline or Annual) Call Center Cover Page to record the reason the test wasn't attempted, e.g., the participant refused.

Instructions for Administering Part B

Introduce the task by saying, **“Now I'd like you to switch between numbers and letters when you count. So you would say the number 1, and then say the letter A, then number 2, then letter B and so on. Let's do a practice trial first. Count to the number 4, switching between numbers and letters. Ready?”** Wait for a response and say, **“Begin.”** Mark the participant's responses on the Oral Trail Making Test – Part B: Scoring Worksheet.

If the participant makes a mistake, slowly say **“No, that was incorrect, it should be 1, A, 2, B, 3, C, 4.”** Allow the participant to practice up to **3 times**. You may repeat the instructions with guidance twice. If the participant still does not understand after you give the instructions a total of 3 times or they cannot do the practice correctly, discontinue Part B and go on to the next task.

Check the “Discontinued” box and enter the time as 5:00 minutes (300 seconds) and the score as 5 errors. Make a note on the (Baseline or Annual) Call Center Cover Page that the participant did not attempt Part B due to failure to successfully complete the practice rounds.

If the participant completes the practice correctly say, **“Now I want you to switch between numbers and letters when you count like 1, A, 2, B, 3, C, and so on until you reach the number 13. Ready?”** Wait for a response and say, **“Begin.”** Start timing as soon as you say **“Begin.”**

Mark the participant's responses on the Scoring Worksheet. If the participant makes a mistake, score it as an error and write 'E' beneath the incorrect number or letter. Stop the participant and provide the last 2 correct responses by saying, **“You were correct up to __ and __, continue from __.”** Do not stop timing during corrections.

If the participant stops for 5 seconds or more, you may prompt with "Please keep going." If the participant does not recall where they are in the series, provide the last 2 correct responses by saying, **“You were correct up to __ and __, continue from __.”** and score as an error by writing 'E' under where the error occurred. After a further delay of 15 seconds or more, discontinue.

Scoring Part B

On the test form, record the total number of errors and the time to complete the series, including the time to offer corrections, in minutes:seconds, rounding to the nearest second.

Remember: Discontinue the test if a participant makes 5 errors or if they do not complete the task in 5 minutes. Check the “Discontinued” box in the data entry screen and enter the score as 5 errors and the time as 5:00 minutes (300 seconds).

If a participant does not attempt the test, check the “Discontinued” box, enter the score as 5 errors and the time as 5:00 minutes (300 seconds), and note on the (Baseline or Annual) Call Center Cover Page the reason the test wasn’t attempted, e.g., the participant refused or failed the practice.

7.7.6 CRAFT STORY – LONG DELAY RECALL

This is a test of delayed recall (episodic memory) of the story read to the participant during the Craft Story – Immediate Recall test. **It is important that you do not read the story again to the participant.**

Instructions for Administering the Craft Story – Long Delay Recall

Administer this test **about 20 minutes** after the Craft Story – Immediate Recall. If 20 minutes have not elapsed, administer the Craft Story – Long Delay Recall and record the actual time elapsed.

Say, **“I read you a story a few minutes ago. Can you tell me what you remember about that story now?”**

If the participant does not recall the story or having been told a story, say, **“It was a story about a boy. Can you tell it to me now?”**

If the participant asks a question about the story or for repetition of some or all of it, say, **“Please tell me as much as you remember about the story.” Do not read the story again to the participant.**

Record the participant’s response on the test form, writing between the lines of the story. For rapid and accurate transcribing of the story, use abbreviations as needed and omit filler or irrelevant words. If absolutely necessary, say **“A little slower, please,”** rather than miss story elements. Make sure your transcription of the story is legible before proceeding.

Write legibly and leave room between each line for scoring. If it is difficult to write the participant’s response, you may review the audio recording and transcribe the results after completing the call.

Scoring

Score the Craft Story – Long Delay Recall according to the instructions for Craft Story – Immediate Recall. Indicate whether the story cue was provided by checking the appropriate box next to “Cue needed.”

7.7.7 PATIENT HEALTH QUESTIONNAIRE-8 (PHQ-8) [SITES AND CALL CENTER]

This 8-item questionnaire assesses depression.

Materials Needed

- Test administration form
- Pen, if using a paper form

Instructions for Administering the PHQ-8

Read the instructions verbatim to the participant.

Read each question and the response options to the participant and record their response.

Say, “I just have a few more questions. Over the last 2 weeks, how often have you been bothered by any of the following problems.” Read the items to the participant as they are listed on the form.

Scoring

- Each question can have a score of 0 to 3 points.
 - 0 = Not at all
 - 1 = Several days
 - 2 = More than half the days
 - 3 = Nearly every day
- The score will be automatically calculated when the answers are entered in the participant’s dashboard on the study website. Record the score on the test form.
- **Maximum score = 24**

PHQ-8 Alert to the Clinic

Study coordinators will receive an immediate alert on the study website if a participant at their site scores above 14 on the PHQ-8. The alert instructs the site staff to confirm if the participant has given their permission for the site to report any changes in cognitive status to their primary care provider (PCP).

- If so, the PI should send the High PHQ-8 Score Letter to the participant’s PCP. The letter template is available on the study website under Quick Links > Documents > Enrolled Participant Resources.
- If not, the PI should contact the participant to advise on seeking medical follow-up.

7.7.8 ENDING THE BASELINE CALL AFTER THE EXTENDED COGNITIVE BATTERY

Say, “That ends the questions I have for you today. Do you have any questions about PREVENTABLE and what will happen next?”

“If you have agreed, you will be paid \$75 for completing this baseline call. You should receive this check by mail within 14 days and the return address will be “PREVENTABLE Study” in Durham, North Carolina. If for some reason you have not received your check in that time frame, please reach out to your site coordinator at the clinic.”

“Please refer to the Participant Handout for additional instructions:

- Study meds will be mailed and should be expected within the next week.
- Begin taking the study meds as instructed.
- Phone numbers are listed if there are any questions following this phone call.
- Call center will call in 1 year to follow up and ask the same type of questions.”

“Many of our PREVENTABLE participants are selected to have additional questionnaires completed by the person you listed as a trusted contact. If these additional questionnaires are needed, a call will be made to the person you listed as the trusted contact. I see that you listed _____ (trusted contact 1) and _____ (trusted contact 2, if applicable) as your trusted contacts. Do these people know that you are a participant in PREVENTABLE? And do they know you well enough to answer some questions about your daily activities? The phone number you have listed for _____ (trusted contact 1) is _____, is that still correct? The phone number you have listed for _____ (trusted contact 2) is _____, is that still correct? It would be very helpful if you would let them know that we might be calling.”

“Thank you for being part of this important study. Your contribution will provide valuable information to doctors whether regular statin use can help prevent memory impairment and disability in older adults.”

If the participant lives with another PREVENTABLE participant

Say “Now that you have completed the phone interview, since (Name of 2nd Participant) is also participating in the PREVENTABLE study and is in the same household, I ask that you please do not discuss any of the questions that I have asked you with (Name of 2nd Participant). We will ask that you go to another room in the house when we are completing the phone interview with (Name of 2nd Participant). We need to ensure that the phone interviews are conducted with each participant in the same manner, and having prior knowledge of the questions that may be asked could have an impact on the valuable information that we are gathering as part of the PREVENTABLE study. We very much appreciate your understanding and cooperation while participating in the PREVENTABLE study.”

“Thank you for being part of this important study. Your contribution will provide valuable information to doctors whether regular statin use can help prevent memory impairment and disability in older adults.”

Validity of the Participant's Responses

After the phone call, record your impression of the validity of the participant's responses. If there is an indication that the responses do not accurately reflect the participant's cognitive abilities, enter the reason for your assessment. This information is extremely helpful in adjudication and data analysis. For example, it can be difficult at times to judge whether hearing significantly influenced test results, but knowing it might have been a factor is helpful.

7.8 TRUSTED CONTACT FOLLOW-UP CALL

When a participant triggers the Extended Cognitive Battery, the Call Center will interview a trusted contact. At the end of the baseline call with the participant, the Call Center staff will:

- Inform the participant that many PREVENTABLE participants are selected to have additional questionnaires completed by a trusted contact.
- Remind the participant the Call Center will contact their trusted contact(s).
- Ask the participant to confirm the name and phone number of their trusted contact(s).
- Ask the participant to confirm that their trusted contact(s) is/are
 - Aware the participant is enrolled in the PREVENTABLE study, and
 - Aware that the participant provided the trusted contact's name and phone number to the study.
 - If not, ask the participant to let their trusted contact(s) know they may receive calls from the study team or Call Center.
- Confirm that the participant's trusted contact(s) is/are someone who is in close contact with the participant and who can answer questions about the participant's daily activities.

The Call Center should attempt to reach the trusted contact at least 3 times. The attempts should be made over a period of 2 weeks on different days and at different times. If you are unable to reach that trusted contact after 3 attempts, choose another trusted contact if one is listed on the Contact Information Form and follow the same process. If you are still unable to reach a trusted contact for the participant, enter in the call outcome as "Review Trusted Contacts." The site's study coordinator will receive an automatic email and a "Participant Issue" alert on the study website notifying them to "Review Trusted Contact Info." See **Chapter 9** for steps to resolve this alert for the baseline call.

The questionnaires administered during the trusted contact call are:

- Functional Activities Questionnaire (FAQ)
- Katz Activities of Daily Living (Katz ADL)
- Patient Reported Outcomes Measurement Information System – Physical Function (PROMIS PF)

7.8.1 FUNCTIONAL ACTIVITIES QUESTIONNAIRE (FAQ)

The FAQ is a brief, 10-item questionnaire that has been validated for determining the impact of cognition on important daily functions. The FAQ is essential to PREVENTABLE adjudicators' ability to determine the cognitive outcome. Results from the FAQ, in combination with the cognitive tests, are used to classify the cognitive status of participants.

Materials Needed

- Test administration form
- Pen, if using a paper form
- Name, phone number, and relationship of trusted contact

Instructions for Administering the FAQ

The FAQ will be administered to a trusted contact for any participant that scores below 31 on the TICS-M.

The FAQ is administered to a person listed as a trusted contact of the participant – someone who is in close contact with the participant and is familiar with the participant’s daily functioning. Ask the contact to respond to each question based on the participant’s ability during **the past 4 weeks**.

"Hello this is _____ (caller name) and I'm calling today on behalf of the PREVENTABLE study. Mr./Mrs. _____ (participant first and last name) has volunteered for the PREVENTABLE study and identified _____ (trusted contact name) as a trusted contact. Is _____ (trusted contact name) available?"

Pause and wait for the trusted contact to verify they are on the line. **"How are you today?"**

"Mr./Mrs. _____ (participant first and last name) gave us permission to contact you if we needed to collect some additional information about how their health might be affecting their ability to do certain things. This won't take more than 10 minutes and we really appreciate your willingness to do this. Do you have time to speak with me now?"

Schedule an alternative time if the trusted contact is unable to talk.

"I would like to ask you some questions about how Mr./Mrs. _____ (participant last name) handles some common daily functions.

Using the following responses, please choose the most accurate representation of his/her level of ability to perform each activity in the past 4 weeks:

- Normal
- Has difficulty, but does by self
- Requires assistance
- Dependent
- Not Applicable (e.g., never did)

In the past 4 weeks, did Mr./Mrs. _____ (participant last name) have any difficulty or need help with..."

Unlike cognitive tests where the script must be strictly followed, the FAQ is a semi-structured interview where you have flexibility to ask for additional information or clarification to rate the trusted contact’s responses correctly.

Examples

- Information about the trusted contact should be collected at the beginning of the assessment and entered into the FAQ Form to help adjudicators determine the trusted contact's reporting reliability. Callers will ask the trusted contact for their name, age, relationship to the participant, how long they have known the participant, how often they have contact with them, and what type of contact (in person, phone, email, etc.).
- The questions should be asked as written on the form, e.g., "In the past 4 weeks, has Mr. X had any difficulty with or needed help in writing checks, paying bills, or balancing a checkbook?" However, you may ask follow-up questions and provide clarification to the trusted contact when necessary in order to assign a score based on the contact's response. This process allows for some judgment on your part.
- If necessary, you should continue to ask questions to make sure the trusted contact's knowledge of the participant's status is accurate. For example, if the trusted contact reports "dependent" and you believe they misunderstood the term, you should seek clarification, e.g., "So you're saying, Mr. X cannot perform this activity on his own."
- If after asking questions for clarification, the response "Not Applicable" is selected, provide a reason for that response in the comment box on the form to aid the adjudicators.
- Any information that might be helpful to the adjudicators in their deliberation should be included in the comments box on the form. This could include information such as the participant was in the room so the trusted contact was not able to speak freely, the participant recently had a medical event which limits their activities, etc. If in doubt, include a note in the comments box.

Record the responses on the FAQ Form and enter the responses in the FAQ data entry screen on the appropriate call screen in the Call Tracking System.

7.8.2 KATZ ACTIVITIES OF DAILY LIVING (ADL) QUESTIONNAIRE – TRUSTED CONTACT

The Katz ADL assesses functional status as a measurement of the participant's ability to perform activities of daily living independently.

The Katz ADL ranks performance in 5 ADLs and can be administered by observation or interview, and, as in this case, by interviewing the trusted contact.

Call Center staff will ask the trusted contact about the participant's ability in bathing, moving in and out of bed or chairs, using the bathroom, getting dressed, and feeding themselves.

The trusted contact should indicate independence (No supervision, direction, or personal assistance needed) = 1 point, or dependence (With supervision, direction personal assistance, or total care needed) = 0 points. If the trusted contact reports that the participant refuses to perform the task, even though they may be able to do the task, it should be scored as dependent.

Examples for each category are provided and may be stated to the trusted contact. Based on the trusted contact's responses, write a 1 or 0 on the line provided on the form.

7.8.3 PROMIS PHYSICAL FUNCTION – TRUSTED CONTACT

The PROMIS Physical Function (PF) instrument administered to a trusted contact measures the participant's **current capability** rather than actual performance of physical activities. This includes the functioning of upper extremities (dexterity), lower extremities (walking or mobility), and central regions (neck, back), as well as important activities of daily living, such as running errands.

A single Physical Function capability score is obtained from the 20-item short form.

Materials Needed

- Test administration form
- Pen, if using a paper form
- Name, phone number, and relationship of trusted contact

Instructions for Administering the PROMIS PF – Trusted Contact

PROMIS instruments are scored using item-level calibrations. The score will be automatically calculated upon data entry.

The PROMIS PF short form is divided into 2 sections. Each section has 5 response options ranging in value from 1 to 5. Provide the response options to the trusted contact as often as necessary.

Questions 1–14

Say, “Now I’m going to ask you some questions about activities that _____ (insert participant’s preferred name) does and how he/she is able to function. I want you to tell me if he/she can do this

- Without any difficulty
- With a little difficulty
- With some difficulty
- With much difficulty
- Unable to do

I can repeat those choices after each question if you need me to. I want you to choose the response that best describes _____ (insert participant’s preferred name) ability to do each task. Do you have any questions?”

Read questions 1–14 and place an X in the box that indicates the response about the participant’s current ability to do the task. Repeat a question if the trusted contact did not hear or understand. If they are unable to decide on a response, ask them to select the one that describes the participant’s ability most of the time.

Questions 15–20

Say, “Now I’m going to ask you a few more questions about _____ (insert participant’s preferred name) health and I want you to select the response that best describes his/her ability to do each task. I want you to tell me if _____ (insert participant’s preferred name) health limits him/her from doing these things:

- **Not at all**
- **Very little**
- **Somewhat**
- **Quite a lot**
- **Cannot do"**

Read questions 15–20 and place an X in the box that indicates the trusted contact's response. You may repeat a question if they did not hear or understand. If the trusted contact is unable to decide on a response, ask them to select the one that describes the participant's ability most of the time.

Scoring

A PROMIS PF score will be automatically calculated from the responses entered into the Trusted Contact PROMIS Form in the Call Tracking system.

7.8.4 ENDING THE TRUSTED CONTACT CALL

Say, **"Those are all the questions I have for you. On behalf of the PREVENTABLE Study, I would like to extend our appreciation to you for taking time to speak with me."**

7.9 DEMENTIA QUESTIONNAIRE (DQ)

For participants who cannot be assessed by phone and those that are lost to follow-up or have died, the Call Center will administer the Dementia Questionnaire (DQ) to a trusted contact. The DQ is used to assess the participant's cognitive and behavioral changes and to determine how cognitive problems affect the participant's everyday functions.

Materials Needed

- Questionnaire form
- Pen, if using a paper form
- Name, phone number, and relationship of trusted contact

Instructions for Administering the DQ

- It is important to inform the trusted contact that this type of interview is standard procedure in research studies and that the participant has given permission for it. If a trusted contact refuses to complete the DQ:
 - Request that the trusted contact speak with the participant to confirm that the participant gave their permission for the interview, OR
 - Request that the participant speak with their trusted contact to confirm that permission has been given for the interview.
- Conduct the interview without interruptions, if possible. It may be necessary to call the trusted contact and schedule a time for the interview. **If possible, you should not conduct the interview if the study participant is present.**
- Make sure you have the Questionnaire Form prior to calling the trusted contact.

- If the study staff cannot reach the trusted contact after three (3) call attempts, the site staff are responsible for sending a letter to the trusted contact stating the Call Center would like to schedule a call.
- On the form, some of the answer choices are abbreviated:
 - DK/NR = Don't know OR No Response
 - NA = Not applicable
- For questions requiring a 'year' response, 'year' refers to the approximate year of onset. Try to confirm the year if possible. If the trusted contact does not know the year, code as "8888."

Say, "Hello _____ (trusted contact name), my name is _____ (insert caller name) and I'm calling from the PREVENTABLE study. Mr./Mrs. _____ (participant first and last name) is a participant in the study and he/she received assessments of his/her cognitive functioning. Mr./Mrs. _____ (participant last name) has given us permission to speak with you about him/her. I would like to ask you a series of questions about Mr./Mrs. _____ (participant last name)'s memory and other cognitive abilities. By answering these questions, we will get a better idea about his/her cognitive ability and health status. The interview will take approximately 20 minutes. Is this a good time for us to talk?"

Schedule an alternative time if the trusted contact is unable to talk.

Dementia Questionnaire

Informant's Information: Questions 1–4

1. Informant's name and age, if informant seems reliable, informant's relationship to participant
2. How long have you known him/her?
3. How often did/do you have contact with him/her?
4. Most frequent type of contact?

I. Memory/Cognition: Questions 5–14

Ask all questions. If yes, obtain year of onset.

Did (does) (s)he have any problems with:

5. Memory?
6. Remembering people's names?
7. Recognizing familiar faces?
8. Finding way about indoors?
9. Finding way on familiar streets? (Tip: Ensure that the contact answers according to what is familiar to the participant.)
10. Remembering a short list of items? (grocery list, to do list, etc.)
11. Trouble finding the right word or expressing self?
12. Trouble grasping situations or explanations?

- 13. Talking become less over time?
- 14. Tendency to dwell in the past?

II. Onset and Course: Questions 15–19 *(Ask questions in this section ONLY if subject has/had cognitive problems. Code everything as NA otherwise.)*

"You mentioned (s)he has/had problems with (refer to Questions 5-14)"

- 15. Did these problems begin suddenly or slowly? (Choose only one answer.)
- 16. Have these problems stayed the same, been steadily getting worse, or have there been abrupt declines? (Choose only one answer. Make any necessary notes explaining your answer.)
- 17. Is a doctor aware of these problems? (If Yes, go to question 18. If No or DK/NR, go to question 20.)
- 18. What does the doctor believe is causing the problems? (Choose only one answer.)
- 19. Did (s)he receive medications for memory problems?

III. Recognition of Problem: Questions 20–21 *(Ask questions in this section ONLY if subject has/had cognitive problems, i.e. said 'YES' to any of Q's 5-14. Otherwise, code Q 20a-c as 99 and Q21 as 9999.)*

- 20. What was first noticed?
- 21. When was the last time (s)he seemed to be really well or his/her old self?

IV. Caregiving: Questions 22–25 *(Ask as open-ended question, interviewer puts the story together)*

- 22. Is/was an in-home paid caregiver ever utilized (full time or part-time)? If Yes:
 - a. How long as caregiver utilized (in months)?
 - b. Why was a caregiver utilized?
- 23. Did (s)he ever move in with relatives? If Yes:
 - a. How long did (s)he live with relatives (in months)?
 - b. Why did (s)he move in with relatives?
- 24. Did (s)he ever reside in assisted living and/or board & care home? If Yes:
 - a. How long did (s)he reside in assisted living/board & care (in months)?
 - b. Why did (s)he move to assisted living and/or board & care home?
- 25. Did (s)he ever reside in a nursing home? If Yes:
 - a. How long did (s)he reside in a nursing home (in months)?
 - b. Why did (s)he move to a nursing home?

V. Daily Functioning: Questions 26–33

Use the following interpretation for the responses when indicated:

- Yes, some trouble: independent but more difficult or problems noticed (Q26-30, 32 - Independent but more difficult or problems noticed, Q31 - rare accidents)
- Yes, needs help: need prompting, supervision, assistance, or dependent (Q26-30, 32 - Needs prompting, supervision, assistance, or dependent, Q31 - frequent accidents, needs reminding, wears pads)
- No: no problems or never did but could

Does/did (s)he ever have

26. Trouble with household tasks (meal preparation, cleaning, gardening, outdoor projects)?
27. Trouble with handling money (paying bills, making change, writing checks, balancing checkbook, taxes, investments)?
28. Trouble dressing self (choosing or changing clothes, tying shoes, using fasteners)?
29. Trouble feeding self (using cups or utensils, cutting meat, buttering bread)?
30. Trouble bathing (getting in and out of a shower or tub, washing independently)?
31. Trouble controlling bladder or bowels?
32. Trouble remembering to take medications?
33. Did (s)he ever drive? (If Yes, go to question 33a. Otherwise, go to 34.)
 - a. Did (s)he ever stop driving? (If Yes, obtain year and go to question 33b. Otherwise, go to Question 33c.)
 - b. Why did (s)he stop driving?
 - c. Is (did) (s)he having(have) any problems? (If Yes, go to question 33d. Otherwise, go to 34.)
 - d. What type of problems?

VI. Medical Problems: Questions 34–48

Did (s)he ever have

34. High blood pressure?
35. Stroke (symptoms unresolved or > 24 hours)? If Yes, obtain year and go to part IX.
36. TIA (symptoms unresolved or < 24 hours)? If Yes, obtain year and go to part IX.
37. Is one side of the body weaker than the other side? If Yes, obtain year and reason why.
38. Parkinson's disease (resting tremor, shuffling gait, limb rigidity)? If Yes, obtain year of onset.
39. Epileptic seizures or fits? If Yes, obtain year of onset.
40. Drinking problem? (If Yes, go to question 40a. Otherwise, go to 41).

- a. Does/did (s)he have memory/thinking problems that could be due to drinking?
- 41. Ever depressed or sad for two weeks or more? (If Yes, obtain year and go to question 41a. Otherwise, go to 42.)
 - a. Ever seek treatment?
- 42. Ever the opposite of depressed, high, euphoric, on top of the world, or too good? (If Yes, go to question 42a. Otherwise, go to 43.)
 - a. Ever seek treatment?
- 43. Hallucinations (see or hear things/people not there)? If Yes, obtain year.
- 44. Delusions (false beliefs, people stealing, wanting to harm, cheating)? If yes, obtain year.
- 45. Agitation and nervousness? If yes, obtain year.
- 46. Ever seek psychiatric help for any reason (If yes, go to question 46a. Otherwise, go to 47.)
 - a. Ever hospitalized for psychiatric illness? If yes, obtain year.
- 47. Did they ever have a neurological or psychiatric exam? If yes, obtain year.
- 48. Did (s)he ever have a CT scan or MRI of the head? If yes, obtain year.

IX. Stroke/TIA Information

Note: When coding only TIAs, if there are multiple TIAs with no clear dates, code the first and the last year when the TIAs were known to have occurred.

- a. Record whether this information applies to a stroke or a TIA
- b. Was (s)he admitted to a hospital?
- c. Did (s)he experience problems with any part of her/his body? If yes, go to d. Otherwise, go to g.
- d. Which part?
- e. What was the problem?
- f. How long did it last?
- g. Did (s)he experience any speech problem (slurring, etc.)? If yes, go to h. Otherwise, go to i.
- h. How long did it last?
- i. Did (s)he have problems with her/his memory BEFORE the stroke/TIA?
- j. Did (s)he have problems with her/his memory AFTER the stroke/TIA?
- k. Did her/his memory get better or worse AFTER the stroke/TIA?
- l. Was (s)he given any medications? If yes, what?

- m. More than one stroke/TIA? If yes, obtain year and go to aa. Otherwise, go back to Part VI, question 37.

IX. Stroke/TIA Information (Continued)

- aa. Record whether this information applies to a stroke or a TIA
- bb. Was (s)he admitted to a hospital?
- cc. Did (s)he experience problems with any part of her/his body? If yes, go to dd. Otherwise, go to gg.
- dd. Which part?
- ee. What was the problem?
- ff. How long did it last?
- gg. Did (s)he experience any speech problem (slurring, etc.)? If yes, go to hh. Otherwise, go to ii.
- hh. How long did it last?
- ii. Did (s)he have problems with her/his memory BEFORE the stroke/TIA?
- jj. Did (s)he have problems with her/his memory AFTER the stroke/TIA?
- kk. Did her/his memory get better or worse AFTER the stroke/TIA?
- ll. Was (s)he given any medications? If Yes, what?

RETURN TO PART VI, QUESTION 37

Record any additional comments at the bottom of the form.

Say, **“Those are all the questions I have for you Mr/Ms _____. On behalf of the PREVENTABLE study, I would like to extend our appreciation to you for taking time to speak with me.”**

7.10 CERTIFICATION PROCESS FOR CALL CENTER STAFF

To ensure the reliability of cognitive data, Call Center staff must be trained and certified prior to collecting any data.

- Call Center staff will attend an in-person/web-based training session that will cover the participant cognitive battery and the trusted contact questionnaires. Training sessions will include an overview of the tests, video demonstrations, live practice, and time for questions.
- Following in-person training, Call Center staff will be required to practice the tests and questionnaires (“batteries”) with volunteers, and when confident in their skills, administer the tests over the phone to the trainer. Following the administration, each Call Center staff member will score the entire battery and submit it for review. When an acceptable level of proficiency is achieved, they will be certified to administer the tests to PREVENTABLE participants.

- The PREVENTABLE Geriatrics Outcomes Assessment Center will periodically evaluate forms and recordings from certified Call Center staff to ensure the tests are being administered correctly. Each review will include feedback on the administration, scoring, and adherence to protocol. If adjustments are necessary, the Call Center staff member will review training materials and have a consultation with the PREVENTABLE neuropsychologist for additional training.

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CHAPTER 8.. PARTICIPANT PAYMENTS

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

8.1 OVERVIEW

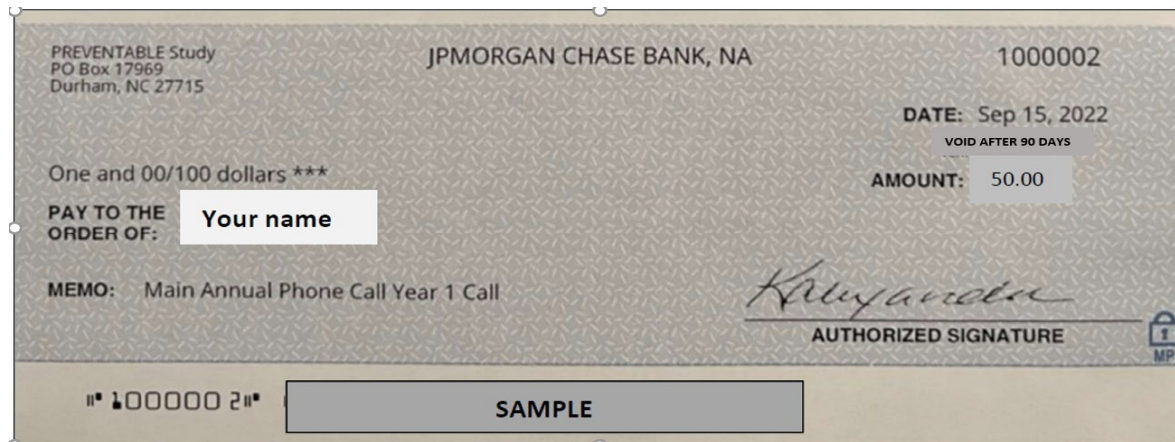
In September 2022, the Duke Clinical Coordinating Center (CCC) implemented centralized direct-to-participant payments via Checkbook, Inc. For each payment, Checkbook mails a check via USPS to the address on the participant’s Contact Information Form. Payments are sent when the participant reaches the milestones described in Section 8.2 Participant Payment Schedule. Site staff can monitor the progress of checks on the study website in the Payments tab of the participant dashboard.

Sites may supplement study payments with additional funds (e.g., payment for parking, meals, travel, etc.), but those decisions must be made at the local level, included in the site-specific ICF template, and approved by the Duke sIRB or VA CIRB. No additional site funding for these payments will be provided by the CCC.

Figure 8.1. Sample Check Image



Figure 8.2. Sample Check Image



8.2 PARTICIPANT PAYMENT SCHEDULE

Participants are paid when they complete the study activities listed in the table below.

Study Activity	Amount	Trigger
Randomization	\$75	Upon randomization
3-month visit if selected for the lipid substudy group	\$50	The Blood Collection 3-month Visit Form indicates completion of blood collection
Annual phone calls	\$50 each	TICS-M or Katz ADL Form entered
(Ancillary Study) CAC scan	\$50	CAC Incidental Findings Form completed by CAC core lab (approximately 6 weeks after the scan is submitted)
(Ancillary Study) SPPB annual visits	\$50 each	SPPB Form for Year 1, Year 2, Year 3 entered
(Ancillary Study - subset of SPPB) SPPB and Frailty annual visits	\$25 each*	Frailty Form for Year 1, Year 2, Year 3 entered
(Ancillary Study) ActivPAL activity monitor	\$30 each (baseline, 1 year, 3 year)	Documentation of receipt of ActivPAL entered by the University of Utah

*Participants in **both** the SPPB and Frailty ancillary studies will receive a total of \$75 each year, paid in 2 separate checks: \$50 for SPPB and \$25 for Frailty. A participant must be in the SPPB ancillary study to also participate in the Frailty ancillary study.

8.3 OPTING OUT OF PAYMENTS

Participants can choose not to receive payments. The participant's payment preference applies to both the primary PREVENTABLE study and any ancillary studies, i.e., a participant cannot choose to receive payments for the primary study, but not the CAC study.

The default response for "Can we pay you?" in the Payments tab of the participant dashboard is "Yes." If a participant does not want to receive payments, change the answer to "No" and click submit. If a participant opts out of payments and later changes their mind, click on "Submit a RAP" in Quick Links on the home page of the study website for payments to be sent to the participant.

The screenshot shows the participant dashboard interface. At the top, there are navigation tabs: DATA ENTRY (highlighted in red), PID Lookup, Site List, SITE: 1990 - Non-VA Turned On, Site Summary, PID: 900031603, and NAME: Bruce Banner. On the left, a sidebar lists various sections: Information, Consent, SSN & MBI, Visit List, Enrollment, Comments, and Payments (which is highlighted in blue). The main content area is divided into two sections. The left section, titled 'Payment Preferences', contains a form with the question 'Can we pay you?' followed by a dropdown menu currently set to 'YES' and a green 'SUBMIT' button. This entire section is circled in red. The right section, titled 'Current Contact Information', has an 'EDIT Contact Info' button and two text input fields: 'Address' (containing '1921 NEW FALMOUTH DR GREENSBORO NC 27410') and 'Preferred Phone Number' (containing '3366878878 - Text ONLY'). At the bottom, there is a 'Payments Status' section.

8.4 TRACKING PAYMENTS

Site staff can track the status of payments on the study website. The status of each payment is displayed in the Payments tab of the participant dashboard.

Payment Status Descriptions:

- **In Process:** The Wake Forest Data Coordinating Center (DCC) sent the information to Checkbook for the check to be created.
- **Mailed:** The check has been printed and is in the mail from Checkbook to the participant.
- **Paid:** The check has been cashed.
- **Returned:** The check has been returned as undeliverable to the CCC or DCC.
- **Reissue:** A reissue has been requested by the site and approved by the Payment Manager.
- **Stop-Payment:** The DCC requested that Checkbook put a stop payment on this check, based on feedback from the clinic and/or Payment Manager.
- **Void:** The stop payment request has been completed by Checkbook.
- **Expired:** The check has reached its expiration date (90 days).

Payments Status

STUDY	PAYMENT TYPE	CHECK NUMBER	AMOUNT	STATUS	DATE	ACTIONS
Main	Main 3 Month Visit	1000006	\$1	VOID	09-21-2022 11:09:14	
Main	Main 3 Month Visit	1000006	\$1	IN_PROCESS	09-21-2022 11:09:27	
Main	Main Randomization	1000001	\$1	PAID	09-22-2022 08:09:30	
Main	Main Randomization	1000001	\$1	MAILED	09-15-2022 16:09:51	
Main	Main Randomization	1000001	\$1	IN_PROCESS	09-15-2022 14:09:09	

8.5 RESOLVING ALERTS FOR RETURNED AND UNCASHED CHECKS

Alerts for returned or uncashed checks can be resolved by requesting a reissue or stop payment and entering a comment.

NOTE: If a stop payment is requested, a re-issue request cannot be submitted for the same payment.

8.5.1 RETURNED CHECKS

The return address for all main PREVENTABLE study and CAC study checks is the Duke Clinical Coordinating Center (CCC). The return address for all other ancillary study checks is the Wake Forest Data Coordinating Center (DCC).

If the CCC or DCC receives undeliverable mail without “deceased” written on it, the Payment Manager will update the payment status to indicate that the check was returned. This will generate a “Payments – Returned Check” alert for site staff to resolve.

To resolve this alert, contact the participant to confirm their mailing address, update their Contact Information Form if needed, and submit a reissue request. If the participant cannot be reached after multiple attempts, request a stop payment to resolve this alert. If the participant has passed away, enter the Death Form and request a stop payment.

DATA ENTRY

PID Lookup

Site List

SITE: 1990 - Non-VA Turned On

Site Summary

Alert List

Initialize Participant(s)

ENTER BARCODE

PAYMENTS - Returned Check

ACTION NEEDED:

- Click on participant from list below to see the current payment status and contact information.
- Contact the participant to verify contact information and, if needed, make any updates to the Contact Information Form.
- Click on the participant, if the check needs to be re-issued, click the Request Re-Issue button. If the check needs to be cancelled, click the Stop Payment button.
- Enter a comment in, confirming whether you updated/confirmed the contact information or were unable to reach the participant after multiple attempts and thus are requesting a stop payment.
PLEASE NOTE: Adding a comment can help to avoid additional requests for clarification from the Payment Manager.
- Clicking either “Request Re-Issue” or “Request Stop Payment” and entering a comment will resolve the alert.

PID	Name	Check Number	Payment Type	Payment Amount	Status	Status Date	Comments
900031603	BRUCE BANNER	2150067	Main Annual Phone Call Year 1 Call	\$50	RETURNED	04/02/2024	1 - Comments

Sites will receive a “Payments – Returned Check Deceased” alert if the CCC or DCC receives undeliverable mail with “deceased” written on it. Site staff should review the EHR or local obituaries or reach out to the trusted contact(s) to verify the participant’s death. If the participant’s death is confirmed, enter the Death Form and request a stop payment. If the information from USPS is incorrect and the participant has not passed away, request a reissue or stop payment depending on if the participant confirms they want to receive or decline the payment. Requesting a reissue or stop payment and entering a comment will resolve this alert.

DATA ENTRY

PID Lookup

Site List

SITE: 1990 - Non-VA Turned On

Site Summary

Alert List

Initialize Participant(s)

ENTER BARCODE

PAYMENTS - Returned Check Deceased

ACTION NEEDED:

- Click on participant from list below to see the current payment status and contact information.
- Review the EHR, local obituaries and/or reach out to the Trusted Contact(s) to verify death or contact information and make any updates.
If you receive confirmation of participant's death, please enter the Death Form.
If you receive a new address, please update the Contact Information Form.
- Click on the participant. If the check needs to be re-issued, click the Request Re-Issue button. If the participant is deceased, click the Stop Payment button.
- Enter a comment in, confirming whether you updated the contact information and/or were able to confirm the participant's death and thus are requesting a stop payment.
PLEASE NOTE: Adding a comment can help to avoid additional requests for clarification from the Payment Manager.
- Clicking either “Request Re-Issue” or “Request Stop Payment” and entering a comment will resolve the alert.

PID	Name	Check Number	Payment Type	Payment Amount	Status	Status Date	Comments
900031603	BRUCE BANNER	3100034	STAND Ancillary Study ActivePAL Returned Study Start	\$30	RETURNED	04/02/2024	1 - Comments

8.5.2 UNCASHED CHECKS

If a check has not been cashed in 45 days, an alert for “Payments – Check mailed more than 45 days ago and not yet cashed” will be generated for the site to follow up with the participant. If the participant received the check but has not cashed it, remind them to cash the check before it expires (90 days). The check expiration date is printed on the check. If the participant did not receive the check, confirm their current mailing address, update their Contact Information Form if needed, and submit a reissue request.

If a check has not been cashed in 75 days, a new alert for “Check mailed more than 75 days ago and not yet cashed” will be generated for the site to follow up with the participant. If necessary, a reissue request can be submitted at this time.

If the alert for “Check mailed more than 45 days ago and not yet cashed” is not resolved before day 75, the alert will be automatically resolved and a new alert for “Check mailed more than 75 days ago and not yet cashed” will appear.

NOTE: Uncashed checks are void after 90 days. The expiration date is printed on the check. These payment alerts are intended to ensure participants are receiving and cashing their checks before the checks expire. If a participant deposits an expired check, they may be charged fees by their bank for processing an invalid check.

PAYMENTS - Check mailed more than 45 days ago and not yet cashed.

ACTION NEEDED:

1. Click on the participant from the list below to see the current payment status and contact information.
2. Contact the participant to ensure the check was received.
3. If the participant did not receive the check:
 - a. Click on the participant. If the check needs to be re-issued, click the “Request Re-Issue” button.
 - b. Check the box confirming you either updated or verified the contact information.
Enter a comment explaining the reason for requesting a re-issue.
PLEASE NOTE: Adding a comment can help to avoid additional requests for clarification from the Payment Manager.
 - c. This will resolve the alert.
4. If you find out the participant has died or you need to cancel the check:
 - a. Click on the participant. Click “Request Stop Payment”
 - b. Enter a comment explaining the reason for cancelling the check.
PLEASE NOTE: Adding a comment can help to avoid additional requests for clarification from the Payment Manager.
 - c. This will resolve the alert.
5. If the participant did receive the check:
 - a. Please remind the participant that the check will expire after 90 days and encourage them to cash the check.
 - b. Click “Resolve” to resolve the alert.

PID	Name	Check Number	Payment Type	Payment Amount	Status	Status Date	Comments	Click to Resolve
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8.6 SUBMITTING REQUESTS TO REISSUE CHECKS OR STOP PAYMENTS

On the Payments tab in the participant dashboard, site staff can request a reissue if a participant did not receive a payment. The reissue button will be available on the participant’s Payments tab 14 days after the check is mailed to allow time for USPS delivery.

If a participant has passed away or if a participant decides they no longer want to receive payments, submit a stop payment request.

All reissue and stop payment requests are reviewed by the CCC. Once a request is submitted, the Payment Status will show “Request Pending” in the Actions column for that payment. The CCC Payment Manager will approve or decline the request and enter a comment with the reason for their decision. If a reissue is approved, a new check number will be associated with the payment type for that participant.

Payments Status						
STUDY	PAYMENT TYPE	CHECK NUMBER	AMOUNT	STATUS	DATE	ACTIONS
STAND	STAND Ancillary Study ActivePAL Returned Study Start	3100034	\$30	RETURNED	04/02/2024 17:04:07	REQUEST RE-ISSUE REQUEST STOP PAYMENT
STAND	STAND Ancillary Study ActivePAL Returned Study Start	3100034	\$30	MAILED	03/21/2024 14:03:40	

To submit a reissue request, confirm the address on the Contact Information Form is correct, enter a comment with the reason for the request, and click the green “Yes” button.

CONFIRM REQUEST RE-ISSUE FOR THIS PAYMENT

STUDY	PAYMENT TYPE	CHECK NUMBER	AMOUNT	STATUS	DATE
STAND	STAND Ancillary Study ActivePAL Returned Study Start	3100034	\$30	RETURNED	04/02/2024 17:04:07

Please confirm that the address in the Contact Information Form is correct before requesting a reissue.

required
☐ Has the address on the Contact Information Form been updated or verified?

required
 Enter Comment:

Are you sure you want to request a re-issue for this payment? YES NO

To submit a stop payment request, enter a comment with the reason for the request and click the green “Yes” button.

NOTE: If a stop payment is requested, a re-issue request cannot be submitted for the same payment.

CONFIRM REQUEST STOP PAYMENT FOR THIS PAYMENT

STUDY	PAYMENT TYPE	CHECK NUMBER	AMOUNT	STATUS	DATE
STAND	STAND Ancillary Study ActivePAL Returned Study Start	3100034	\$30	RETURNED	04/02/2024 17:04:07

Enter Comment: * required

Are you sure you want to request this payment be stopped? YES NO

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CHAPTER 9. PARTICIPANT RANDOMIZATION

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

9.1 OVERVIEW

Site staff are responsible for verifying eligibility, consenting, and enrolling participants. Enrollment is complete at the end of the baseline visit. Randomization occurs after the Call Center completes the baseline cognitive assessment call with the participant. To be eligible for randomization, each participant must:

1. Sign the consent,
2. Complete the baseline visit,
3. Be enrolled in the study by site staff clicking the “Enroll Participant” button on the participant’s Enrollment Checklist, and
4. Complete the entire baseline call with the Call Center.

Randomization is an automated process completed by the Data Coordinating Center (DCC) after the Call Center completes their baseline cognitive assessment of the participant. The study drug is not shipped to the participant until the randomization process is complete.

Once the participant is randomized, the Participant Information tab, as well as dynamic web reports, are updated to indicate that the participant is randomized. Site staff are responsible for updating the participant’s electronic health record (EHR) to indicate participation in PREVENTABLE.

The screenshot displays the participant information page for Mary Poppins (PID: 100000975). The top navigation bar includes tabs for DATA ENTRY, PID Lookup, Site List, SITE: 1999 - Non-VA Test, Site Summary, PID: 100000975, and NAME: Mary Poppins. The left sidebar lists various information tabs: Information, Consent, SSN & MBI, Visit List, Enrollment, Comments, and Payments. The main content area shows the following details:

- Enrollment Status:** ELIGIBLE (green box)
- Date of Enrollment:** 08/31/2020
- Randomization Status:** ELIGIBLE (green box, circled in red)
- Date of Randomization:** 09/27/2020
- LIPID SAMPLE SUBSTUDY:** NO

A message states: "The following **Study Drug** and **Retention Status** Histories information is used in statistical analysis. If there is an error in either of these tables, please submit a detailed RAP so that it can be corrected." Below this, there is a section for **STUDY DRUG STATUS** with a button to **ENTER NEW STUDY DRUG STATUS**.

9.2 DATA COLLECTION REQUIREMENTS FOR RANDOMIZATION

These forms must be entered in the study website prior to randomization:

9.2.1 IN-CLINIC OR TELEHEALTH BASELINE VISIT

- Eligibility Form
- Contact Information Form
- Demographics Form
- Informed Consent Form
- Social Security Number (SSN)

9.2.2 CALL CENTER BASELINE COGNITIVE ASSESSMENT

- PROMIS PF
- TICS-M
- If a participant's TICS-M score is less than 31, the Call Center will administer the Extended Cognitive Battery tests in this order:
 - Craft Story – Immediate Recall (verbal memory)
 - Number Span (attention/concentration)
 - Category Fluency – Animals and Vegetables (semantic verbal fluency)
 - Phonemic Fluency – Letter F and Letter L (verbal fluency)
 - Oral Trail Making Test (executive function)
 - Craft Story – Long Delay Recall
 - PHQ-8 (depression)

The date of consent and responses to all layers of the informed consent must be entered in the Consent tab in the participant's dashboard.

- **Non-VA sites:** For participants who sign the eConsent, the signed ICF will be transferred from REDCap to the participant's dashboard by the DCC. For participants who sign a paper ICF, site staff must scan and upload the signed ICF to the participant's Consent tab.
 - **NOTE:** The signed ICF does not have to be uploaded to the website prior to enrollment, but it must be uploaded for the participant to be randomized after the baseline call.
- **VA sites:** VA sites should **not** upload signed ICFs to the study website. VA sites must upload signed ICFs to their site-specific SharePoint folder within 5 business days of the ICF being signed.

See **Chapter 4: Screening and Enrollment** for details about processing ICFs for VA and non-VA sites.

Participants must meet all eligibility criteria on the Eligibility Form (see **Chapter 4**). The baseline call should be completed as soon as possible after the participant is enrolled. If 45 days have passed since the participant signed the ICF (or the participant was enrolled?), before the participant can be randomized:

1. The participant will need to reconfirm their consent (Can they confirm verbally or will they need to sign a new ICF?).
2. Site staff must review all responses on the Eligibility Form, reconfirm eligibility, and update the date of visit.

Accurate contact information is critical for continued follow up and shipment of the study drug. The Contact Information Form should be completed in its entirety, but these questions are required for enrollment and randomization in the study:

- **Question 1:** Name
- **Question 2:** Mailing address (must be validated)
- **Question 3 (and 3a, if applicable):** Physical address
- **Question 5:** Preferred telephone number
- **Question 13:** Name of trusted contact 1
- **Question 14:** Preferred phone number of trusted contact 1

The Demographics Form must be completed in its entirety.

Participants are required to provide their SSN to enroll in the study. The SSN must be entered in the SSN & MBI tab in the participant's dashboard. If available, the participant's Medicare Beneficiary Identifier (MBI) should also be entered but is not required. Participants' SSNs and MBIs are not visible to site staff or the project team after the numbers are entered. This information is only accessible to DCC staff on a need-to-know basis for the purpose of linking health records to follow up on participants.

The Medical History Form should be completed in its entirety, but is not required to be entered prior to randomization. The Short Physical Performance Battery (SPPB) and the Blood Collection Form should also be completed, but are not required for randomization.

9.3 RANDOMIZATION PROCESS

After clicking the "Enroll Participant" button on the Enrollment Checklist, a "Schedule Baseline Call" button will appear. While the participant is in the clinic or on the phone for the baseline visit, site staff should schedule the baseline call with the Call Center at a time that is convenient for the participant. Follow the steps in the video on the study website under Login > Quick Links > Training Videos > Training > Baseline Clinic Visit Walkthrough > Enrolling a Participant and Scheduling the Baseline Call.

The Call Center will attempt to complete the baseline call with the participant at the scheduled date and time. On the baseline call, if the participant completes the TICS-M and scores less than 31, the Call Center will administer the Extended Cognitive Battery tests and enter the data in the participant's dashboard. Once all baseline call forms are complete, the participant will automatically be randomized overnight by the DCC. However, if the signed ICF has not been uploaded to the study website (for non-VA sites) or SharePoint (VA sites) or the Eligibility Form was completed more than 45 days before the baseline call was completed, the participant will not be randomized.

Site staff can monitor each participant's progress by reviewing the Enrollment, Randomization, Retention, and Study Drug statuses on the Information tab in the participant's dashboard. Once the participant is randomized, an automatic email is sent to site staff and the DCC will notify the VA Pharmacy Coordinating Center that the study drug can be shipped to the participant. Site staff can check the delivery status in the Study Drug System under the Data tab on the study website.

9.4 RANDOMIZATION KEY REMINDERS

- Once a participant is randomized, it is not possible to 'un-randomize' them. Any data in the database up to the point they withdraw consent will be included in the study analysis.
- Once a participant has been randomized, they cannot switch to the other study arm.
- The automated system is set up to minimize mistakes, but site staff must be vigilant when completing the enrollment process.

9.5 ENROLLMENT AND RANDOMIZATION ERRORS

If eligibility criteria are entered incorrectly or other errors occur during enrollment or randomization:

- DO NOT remove the participant from the study.
- Submit a Report-a-Problem (RAP) through the study website to notify the DCC.

- Notify your clinical research associate (CRA) immediately and provide the participant ID number (PID).
- If appropriate and directed by your CRA, submit a Protocol Deviation Form. This form is available on the study website under Quick Links > Site Management. See **Chapter 13: Quality Control and Monitoring** for details about protocol deviations.

If a participant is found to be ineligible or withdraws their consent and they have been enrolled but not randomized, the participant's status can be updated by going to the Visit List and selecting "Respond" > "Participant Ineligible" or "Participant Refused" as appropriate. This action will remove the person from the list of participants to be contacted by the Call Center; site staff will not need to notify the Call Center. The participant will remain enrolled but will not be randomized.

If a participant is found to be ineligible for the study after they are randomized, the participant will remain in their assigned to the treatment arm. Continue study procedures per the protocol, submit a RAP, and notify your CRA. The DCC and your CRA will work with your site to take the appropriate next steps. If there are safety concerns, the participant's study drug status should be marked as "inactive-permanent" and the study drug will not be shipped to the participant. However, the participant will still be contacted for follow up per the protocol. If there are not safety concerns, the study drug can still be shipped to the participant and follow-up visits will continue.

9.6 ALERTS THAT PREVENT RANDOMIZATION

The alerts described in this section are categorized as "Participant Issues" that will prevent a participant from being randomized. These alerts can be accessed by clicking on the red "Alert" button on the main page of the study website. Alerts also appear under the Alerts header on the Site Summary page.

To resolve an issue causing an alert, follow the "Action Needed" instructions above the participant list for each type of alert as shown in the screenshots below. More details about resolving alerts that prevent randomization are also provided in this section.

9.6.1 BACK TO CLINIC

If the Call Center has made 3 unsuccessful attempts to reach the participant to complete the baseline call, the site will receive a "Call status is Back to Clinic – Baseline" alert.

Call status is Back to Clinic - Baseline

ACTION NEEDED:

The PREVENTABLE Call Center has been unsuccessful in completing the participant's baseline call.

Refer to the Comments tab on the participant's dashboard for any Call Center comments, then reach out to the participant and confirm if they are either refusing to participate in the study or to reschedule their call center appointment.

Update the participant's status by clicking on their name in the list below, then submitting a change from either the blue Respond or Reschedule option in the Call Center Visits section.

PID	Name	Date of Enrollment	Date Randomized	Date of Current Status	Comments
900001291	Oscar Blair	08/11/2020	08/10/2020	06/29/2021	1 - Comments
900000562	John Wick, III	08/07/2020	08/10/2020	06/29/2021	Enter Comment

To resolve this alert, click on the participant in the alert list to access their Call Center Visits.

On the Call Center Visits list, select “Reschedule” if you are able to reach the participant and confirm a new date and time that the participant agrees to complete the call.

CALL CENTER VISITS			
Visit	Call Status	Date	Action
CC00 - Baseline Call Center	Back to Clinic	09/10/2020	RESPOND / RESCHEDULE CALL

If the participant is not willing or available to reschedule the baseline call, select “Respond” and choose the appropriate participant status from these options:

- **Participant Ineligible:** should be selected if the Call Center finds out that the participant is ineligible for some reason, e.g., they started a statin between the time that the baseline clinic visit was completed and when the Call Center attempted the baseline call.
- **Participant On Hold:** should be selected if the participant is enrolled and has all the baseline clinic forms completed, but would like to wait a short period of time to complete the baseline call. A participant could go “On Hold” if they want to talk to their family or primary care provider about the study before they are randomized, if they are unexpectedly hospitalized between the time they were enrolled and when the baseline call is scheduled, or other unforeseen circumstances.
- **Participant Refused:** should be selected if site staff confirms that the participant changed their mind between being enrolled and completing the baseline call. If the participant refuses to complete the baseline call or says that they changed their mind and no longer want to participate in the study, the Call Center will note this in the Comments section.
- **Unable to Reach Participant:** should be selected if site staff unsuccessfully attempts to contact the participant multiple times to either reschedule the baseline call or confirm the issue that the Call Center noted in the Comments section. This response should also be used if site staff learns that the participant has passed away. In these circumstances, the Death Form must be completed.

It’s very important to include notes about the participant’s status or outcome in the Comments section. Click “Send” to save the response. Once a “Respond” or “Reschedule Call” answer is submitted, the Back to Clinic alert will resolve at the top of the following hour.

NOTE: If any of the above responses are selected and submitted at the Baseline Call Center Visit (CC00), the participant will not be randomized. The participant must complete the baseline call to be randomized.

9.6.2 CONTACT INFORMATION INCORRECT

To resolve an alert for “Call status is Contact Information Incorrect,” review the participant’s electronic health record (EHR) and/or contact the participant to update their contact information.

[DATA ENTRY](#) [PID Lookup](#) [Site List](#) [SITE: 1705 - Wake Forest - Cardiology](#) [Site Summary](#) [Alert List](#) [Initialize Participant\(s\)](#)

Call status is Contact Information Incorrect

ACTION NEEDED:
The PREVENTABLE call center has attempted to call the participants identified below and the contact information for each is either incorrect or missing.
Please review the contact information and make any required updates.
Update the participant's status by submitting a change under either the Respond or Reschedule option in the Call Center Visits section under the participant's Visit List tab.

PID	Name	Date of Enrollment	Date Randomized	Date of Current Status
900006250	Judy Jetson	08/20/2020	N/A	10/12/2022

Once the participant’s contact information is confirmed, select either “Reschedule Call” or “Respond” if the call is not able to be rescheduled. For the “Respond” option, choose the appropriate participant status as described in **Section 9.6.1: Back to Clinic**, add a comment about the outcome, and click “Send” to save the response.

CALL CENTER VISITS			
Visit	Call Status	Date	Action
CC00 - Baseline Call Center	Contact Info Incorrect/Missing	09/10/2020	RESPOND / RESCHEDULE CALL

9.6.3 MAX CALLS MADE

If the baseline call is rescheduled and the Call Center is not able to reach the participant after 3 additional attempts, for a total of 6 separate failed attempts, the site will receive an alert for “Call Status is Max Calls Made.”

Call status is Max Calls Made - Baseline

ACTION NEEDED:
The PREVENTABLE Call Center has attempted at least six distinct times to call the participant(s) identified below and have been unsuccessful.
To resolve the alert, please update the participant's status by submitting a change under either the Respond or Reschedule option in the Call Center Visits section under the participant's Visit List tab.

To resolve this alert, site staff will need to contact the participant to determine the issue and then select either “Reschedule Call” or “Respond” if the call is not able to be rescheduled. Choose the appropriate participant status option as described in the Back to Clinic alert section, add a comment about the outcome, and click “Send” to save the response.

CALL CENTER VISITS

Visit	Call Status	Date		Action
CC00 - Baseline Call Center	Max Calls Made	06/29/2021		RESPOND / RESCHEDULE CALL

If the participant is not able or willing to complete the baseline call, choose “Respond” and then select either “Participant Refused” or “Unable to Reach Participant.”

If the participant confirms they will answer the phone to complete the baseline call, site staff should select “Reschedule Call.” If the baseline call is rescheduled, the Call Center will only attempt **one more time** to call the participant at the scheduled time. If the call is unsuccessful, the Call Center will not attempt to contact the participant again and the participant will not be randomized.

If the participant later contacts the Call Center and data are collected, the Call Center will enter in that information at that time and the participant will proceed to randomization if they still meet all the other eligibility criteria.

9.6.4 CC00 [BASELINE CALL] UNABLE TO COMPLETE DUE TO MEDICAL CONDITION

At the beginning of the baseline call, the Call Center administers a simple hearing test to ensure the participant can hear well enough on the phone to complete the call. If the participant fails the hearing test or there is another medical reason the participant is unable to complete the call, then the Call Center will not administer the cognitive tests and the site will receive an alert for “CC00 Unable to Complete Due to Medical Condition.” Sites may also receive this alert if:

- The participant cannot hear well enough to complete the call without using the talk-to-text function on their phone.
- The participant reports at the baseline call that they were prescribed a statin by their doctor after they completed the baseline clinic visit.
- The participant had a stroke or other medical issue after the baseline clinic visit and is not able to complete the baseline cognitive call.

To resolve the alert, first review the comments from the Call Center on the participant’s Comments tab or by clicking on the comment icon in the row for the CC00 – Baseline Call Center under the “Call Center Visits” section.

CC00 Unable To Complete Due To Medical Condition

ACTION NEEDED:

The Call Center was unable to complete the participant's baseline call. This could be due to the participant not hearing well enough during the first set of questions to fairly complete the phone assessment. Or, it could be due to another medical reason that the call center has documented in the comments.

Please review the comments for the below participant(s) to find out why the call was unable to be completed.

- If you are able to help the participant resolve the issue (i.e. they couldn't find their hearing aids that they normally wear in time for the call but they found them and they are working) then please work with the participant to reschedule the call using the blue “Reschedule” button on the participant's visit list tab.
- Use the “Respond” button to resolve the alert if the action requires confirming any other piece of information with the participant (i.e. use “Participant On Hold” response if the participant is recovering from surgery and unable to do the call within the next week or two, use “Participant Ineligible” if they reported to the Call Center that they started taking a statin between when they completed the baseline clinic visit but before doing the baseline call, etc.”

The alert will resolve once you submit EITHER a “Respond” or “Reschedule” on the participant's visit list tab next to the CC00 call type.

Site staff should determine if they can help the participant resolve the issue so the participant could attempt the cognitive call again. For example, if the participant couldn't find their hearing aids that they normally wear in time for the call, but they found their hearing aids and they are working, select "Reschedule" to reschedule the call. Include a comment on the reschedule screen with any relevant information for the Call Center.

If the participant is unable to attempt the call again because the site learns the participant is ineligible after the baseline visit but before the baseline call, the participant had a stroke and will be unable to recover enough to participate, etc., then select "Respond" and select the appropriate participant status to inform the Call Center.

9.6.5 ELIGIBILITY FORM IS MORE THAN 45 DAYS OLD

If an Eligibility Form was completed more than 45 days ago and the participant has not been randomized, the site will receive an alert on the study website for "Eligibility Form is more than 45 days old." This alert typically occurs when the baseline call cannot be completed within 45 days of enrollment.

To resolve this alert, click on the participant's name in the alerts list and reconfirm all eligibility criteria on their Eligibility Form. Once the form has been reviewed and/or updated, change the Date of Visit at the top of the form to the date the Eligibility Form was reviewed and click submit.

9.6.6 CONSENT FORM NEEDS TO BE UPLOADED

This alert is only generated for non-VA sites as VA sites are not required to upload the ICF to the study website. If the ICF has not been uploaded to a non-VA participant's dashboard on the study website and the baseline call has been completed, site staff will receive an alert that on the study website for "Consent Form needs to be uploaded."

To resolve this alert, click on the participant's name in the alert listing and upload a PDF of their signed ICF. This alert will resolve overnight during the randomization process.

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CHAPTER 10. STUDY DRUG

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

10.1 OVERVIEW

The PREVENTABLE study drug is a moderate-intensity statin (atorvastatin 40 mg) or matched placebo taken once daily. The VA Cooperative Studies Program (CSP) Clinical Research Pharmacy Coordinating Center (PCC) dispenses masked atorvastatin 40 mg tablets and placebo tablets directly to study participants at VA and non-VA sites.

The benefits of taking statins for older adults without heart disease are not fully understood. This moderate-intensity statin, atorvastatin 40 mg, was selected to ensure therapeutic effect and LDL-C lowering to provide a definitive study result. The goal of the study is to learn if taking a statin could help older adults live well for longer by preventing dementia, disability, or heart disease.

The 40 mg dose of atorvastatin is endorsed by NIH leadership and the Data and Safety Monitoring Board (DSMB). Atorvastatin 40 mg is also currently being studied in the Australian STAREE (Statins in Reducing Events in the Elderly) trial and is well tolerated in over 90% of study participants.

PREVENTABLE allows for cutting the study drug in half if intolerance is thought to be due to dosing level. If study drug intolerance persists, discontinuation of the study drug is appropriate. In this case, the participant will continue in PREVENTABLE with study visits, and the discontinuation of the study drug will be reported in the study website. See **Chapter 11: Retention and Follow-Up** for details.

Neither the active study drug nor the placebo contains **peanuts or peanut derivatives**. Furthermore, both tablets are processed in pharmaceutical facilities which do not handle any food products.

Neither the active study drug nor the placebo contains **gluten** as an inactive ingredient, thus they do not pose concern for participants with Celiac’s disease.

The PCC pharmacist will prepare and ship the masked study drug directly to each participant based on the participant’s randomization assignment provided by the Data Coordinating Center (DCC). The participant will receive a 3-month supply of the study drug labeled with a unique bottle number. The PCC will typically ship the study drug via U.S. Postal Service (USPS) priority mail, a 2-day delivery for most locations. Delivery of the study drug is tracked by the DCC using a free USPS web tool. Site staff can check the delivery status of study drug orders in the Study Drug System on the study website.

Study drug renewals are handled differently for VA and non-VA sites. VA sites must reorder the study drug for participants every 3 months using their Computerized Patient Record System (CPRS), or VA EHR. Non-VA sites must reorder the study drug for participants annually using the Study Drug System on the PREVENTABLE website. The Study Drug System identifies participants who need an initial order or refill of the study drug, and also serves as a tracking tool for study drug orders and prescriptions.

NOTE: The PCC can ship only to addresses in states, districts, and territories that are part of the US.

10.1.1 DOSING GUIDELINES

The study drug tablet is taken orally once daily. Atorvastatin 40 mg, a moderate-intensity statin, or placebo is assigned to each participant. Atorvastatin 40 mg use is covered under a labeled indication. See the product insert for additional information on the active drug.

Medication Interactions

The following medicines may alter pharmacokinetics or pharmacodynamics of atorvastatin. While they may not cause interactions that are clinically relevant, site prescribers should consider these interactions when confirming if it is appropriate for the participant to take the study drug:

- a) **Verapamil:** Atorvastatin may increase the serum concentration of verapamil and vice versa due to inhibition of the CYP3A4-mediated metabolism. Consider avoiding verapamil in this population.
- b) **Diltiazem:** Diltiazem may increase the concentration of atorvastatin and vice versa due to inhibition of the CYP3A4-mediated metabolism. Consider reducing the dose of diltiazem to 240 mg or less.
- c) **Transporter Inhibitors:** Inhibitors of the OATP1B1 (e.g., cyclosporine) can increase the plasma bioavailability of atorvastatin. Concomitant use of cyclosporine was associated with higher plasma concentrations of atorvastatin.
- d) **Erythromycin/Clarithromycin:** Co-administration of atorvastatin (10 mg once daily) and erythromycin (500 mg four times a day) or clarithromycin (500 mg twice daily), known inhibitors of cytochrome P450 3A4, was associated with higher plasma concentrations of atorvastatin.
- e) **Protease Inhibitors:** Co-administration of atorvastatin and protease inhibitors (e.g., ritanovir), known inhibitors of cytochrome P450 3A4, was associated with higher plasma concentrations of atorvastatin.

The following medicines have their pharmacokinetics or pharmacodynamics altered by atorvastatin:

- a) **Digoxin:** When multiple doses of digoxin (0.25 mg once daily) and 10 mg atorvastatin were co-administered, steady-state plasma digoxin concentrations were unaffected. However, steady-state plasma digoxin concentrations increased by approximately 20% following administration of digoxin with 80 mg atorvastatin daily. Patients taking digoxin should be monitored appropriately.

Gemfibrozil or fenofibrate are each acceptable to prescribe together with atorvastatin. However, when both gemfibrozil and fenofibrate are prescribed together, these medications can increase the risk of drug-drug interactions and side effects from atorvastatin. For participants taking either of these medications, site investigators should discuss the possible drug interactions with the participant and their primary care provider, if applicable, before the participant enrolls and before the participant is approved to continue the study drug. See **Section 10.3** of this chapter for more information about gemfibrozil and fenofibrate.

10.1.2 PLACEBO

The placebo pill contains approximately 400 mg (or 0.015 ounces) of microcrystalline cellulose fiber. This fiber cannot be digested or absorbed by humans and should not cause any noticeable health effects. Because the fiber is not absorbed, it should not cause any significant change in blood sugar levels nor is there any caloric content.

The placebo tablet is similar in composition to the atorvastatin tablet, which also contains the same cellulose fiber. The other components of the placebo tablet are colloidal silicon dioxide, sodium starch glycolate, and sodium stearyl fumarate. These ingredients are present in amounts even smaller than the cellulose fiber.

10.1.3 TIMING OF TAKING THE STUDY DRUG

Participants do not have to take the study drug at the same time each day. The effect should not vary if not taken at the same time each day. It is sometimes recommended that statins be taken at nighttime, as that is when the body makes the most cholesterol, but the difference in effect is very small. It is more important for the participant to take the medication every day at whatever time is easiest for them to remember. Please refer to the Study Drug Handout in **Appendix 10.4** for details.

10.2 STUDY DRUG SHIPMENT PROCESS

Study drug ordering procedures are different for VA and non-VA sites. This section includes information about initial study drug orders, study drug prescriptions renewals, and unscheduled study drug orders.

The study drug is centrally dispensed directly to participants at both VA and non-VA sites by the VA Cooperative Studies Program (CSP) Clinical Research Pharmacy Coordinating Center (PCC) in compliance with the applicable provisions of the VHA Handbook 1108.04, Investigational Drugs and Supplies. The PCC is accredited by The Joint Commission thus the following considerations should be noted:

- The Joint Commission's (TJC) standards stipulate that any individual who provides care, treatment, or services can report concerns about safety or the quality of care to TJC without retaliatory action from the organization. Accordingly, any study team member, including the PI or the local site investigator (LSI) and their designee, site coordinators, and research assistants, can report study-related safety or quality concerns anonymously and confidentially directly to TJC. For details about filing a concern, visit:
https://www.jointcommission.org/report_a_complaint.aspx
- Site staff and study participants may report study-related concerns directly to TJC by
 - Submitting an online report at:
<https://apps.jointcommission.org/QMSInternet/IncidentEntry.aspx>
 - Mailing a report to:
Office of Quality and Patient Safety
The Joint Commission
One Renaissance Boulevard
Oakbrook Terrace, Illinois 60181
 - TJC does not accept faxed or emailed submissions.

10.2.1 STUDY DRUG ACCOUNTABILITY

- No study drug is stored in local research pharmacies. Therefore, local research pharmacists will neither be responsible for physically dispensing the study drug or for maintaining a study drug inventory record.
- The PCC will maintain study drug records for PREVENTABLE, including receipts from vendors, study drug dispensed to participants, and study drug disposal.
- Participants will receive a Study Drug Handout (see **Appendix 10.4**) with each shipment that includes information about disposing their old study drug bottle upon receiving a new supply.

10.2.2 DESCRIPTION OF STUDY DRUG SHIPMENTS

- Generic atorvastatin 40 mg tablets or a matching placebo will be dispensed to participants. The PCC may dispense different generic products for atorvastatin depending on available supplies. Accordingly, the PCC may dispense different matching placebos during the trial. The placebo will be manufactured under cGMP conditions at the PCC.
- Each 200 cc study drug bottle will contain 100 tablets of either active atorvastatin 40 mg tablets or matching placebo. Every supply will include additional tablets to ensure participants do not run out of the study drug if their shipment is delayed.
- The PCC will monitor the stability/expiration dates of the study drug on a schedule consistent with ICH guidelines to ensure the drug will be stable from time of receipt at the PCC to the time participants take the study drug. Per 21 CFR §211.137 (g), provision of lot numbers and expiration dates is not required for investigational drugs that are placed on a stability testing program. Therefore, the PCC will not dispense study drug bottles with a pre-specified expiration date. The “Use by” date on each study drug bottle is unrelated to expiration date.
- The PCC will attach a light purple label to each bottle of the study drug. The labels will be printed in English or Spanish according to the participant’s preferred language indicated on their Contact Information Form. Each study drug label will include the:
 - Name of the study
 - Participant’s first and last name
 - Prescription number, study drug name, dosing instructions, and quantity
 - Date the prescription was filled and the “use by” date
 - Study doctor’s name and the site’s phone number and address

The PREVENTABLE Trial

LName, FName
Rx #123456789 – 1234

TAKE ONE TABLET BY MOUTH ONCE DAILY.

ATORVASTATIN 40 mg or PLACEBO TABS

QTY: 100 Tablets Filled:00/00/0000
Dr. First Initial & Last Name Use By:00/00/0000
Study Phone Number:(XXX)XXX – XXXX

123456789
VA CSPCRPCC, 2401 Centre Ave S.E., Albuquerque, NM 87106 (505)248 – 3203
LDP525010Z

CAUTION: New Drug – Limited by Federal Law to Investigational Use.

The PREVENTABLE Trial

LName, FName
Rx #123456789 – 1234

TOME UN COMPRIMIDO POR VÍA ORAL UNA VEZ AL DÍA.

COMPRIMIDOS DE ATORVASTATINA 40 mg o PLACEBO

CANT: 100 comprimidos Surtida:00/00/0000
Dr. First Initial & Last Name Usar hasta el:00/00/0000
Número de teléfono del Estudio:(XXX)XXX – XXXX

123456789
VA CSPCRPCC, 2401 Centre Ave S.E., Albuquerque, NM 87106 (505)248 – 3203
LDP525020Z

ADVERTENCIA: fármaco nuevo, limitado por las leyes federales al uso en investigación.

10.2.3 CONTACT INFORMATION FOR PARTICIPANTS AT NON-VA SITES

For non-VA sites, shipment of the study drug relies on the information entered in the study website. Before dispensing the drug, the PCC confirms the information entered in the Contact Information Form matches the information on the Informed Consent Form. If the information does not match, the PCC will cancel the order and notify the site of the cancellation reason via an alert in the Study Drug System to allow for correction and reordering, if appropriate. The PCC reserves the right to cancel study drug orders for any reason.

The mailing address entered in the Contact Information Form on the study website is used to create mailing address labels for study drug shipments to non-VA participants. The form includes a “Swap with Alt Address” button for site staff to easily change mailing addresses for participants that have more than one listed address. The form also includes a field for site staff to enter a date in the future to receive an alert to change the participant’s address.

NOTE: The participant’s addresses are not automatically switched on the date entered on the Contact Information Form. Site staff must manually change the addresses by clicking the “Swap with Alt Address” button. See **Appendix 10.1a and 10.1b** for details.

As a reminder, the PCC can ship only to addresses in states, districts, and territories that are part of the US.

10.2.4 PLACING INITIAL STUDY DRUG ORDERS

Each participant must be randomized before they can receive the study drug. Once the participant is randomized, an automatic email is sent to site staff and the DCC will notify the PCC that the study drug can be shipped to the participant. Site staff can check the delivery status in the Study Drug System under the Data tab on the study website.

Every shipment of the study drug includes the Study Drug Handout (see **Appendix 10.4**), available in English and Spanish, to provide participants with key information about the study drug. The Study Drug Handout is also available on the study website under Documents > Enrolled Participants Resources > Welcome Packet.

Each study drug shipment contains an easy-open cap that the participant may use instead of the child-resistant cap. A PREVENTABLE pillbox is provided in the initial shipment and the first shipment in Year 3. Additional pill boxes can be requested from the Pharmacy if needed in the interim.

10.2.4.1 FOR NON-VA SITES

Once a participant is enrolled, their study drug prescription is valid for 1 year starting from the date of enrollment, not randomization. However, the initial order for a participant’s study drug is not placed until the participant is randomized. After a participant is randomized, the initial order for the study drug will be automatically sent from the DCC to the PCC (i.e., Atorvastatin 40 mg or matching placebo, take one tablet by mouth once daily, 100 tablets, PRN refills). The participant should receive their first bottle of the study drug in the mail within 5 business days.

10.2.4.2 FOR VA SITES

Once a participant is randomized, the DCC will send the masked randomization assignment to the PCC. The site's medications prescriber must go into the VA EHR and follow the steps in **Appendix 10.2** to initiate an order for the study drug. The Study Drug System will also send an "Initial Order Expected" alert (see **Appendix 10.1h**) to remind site staff to place an order in their internal system, the Consolidated Mail Outpatient Pharmacy (CMOP). The order will be sent to the PCC by the VA's EHR system once the site's local research pharmacist verifies the prescription and routes the order for fulfillment by the PCC. The participant should receive their first bottle of study drug in the mail within 5 business days.

NOTE: Before each site places the initial order for their first participant, the site's pharmacy Automated Data Processing Applications Coordinator (ADPAC) must follow the steps in **Appendix 10.2** to create an entry for the study drug in the local drug file.

10.2.4.3 FOR VA AND NON-VA SITES

Prescription orders should be entered as "40 mg of atorvastatin or placebo administered as one tablet taken orally once daily" even if a participant is taking a modified study drug dose. The pharmacy can only fill this specific order. See **Section 10.5** for more information on modified doses of the study drug.

NOTE (VA sites): The text for the dosing instructions on the study drug bottle is determined by what is entered in the VA EHR when the study drug is ordered.

10.2.5 RENEWING STUDY DRUG PRESCRIPTIONS

Study drug prescriptions must be renewed every 12 months for participants at non-VA sites and must be renewed every 3 months for participants at VA sites.

See **Section 10.3** for more information.

10.2.5.1 FOR NON-VA SITES (EVERY 12 MONTHS)

The Prescriptions filters in the Study Drug System allow non-VA site staff to monitor current study drug prescriptions, including a specific filter for participants whose prescription is close to expiration, to assist in the prescription renewal process. When a participant is due for a prescription renewal, an alert will appear on the study website dashboard under Study Drug System alerts. See **Appendix 10.1h** for instructions to resolve prescription renewal alerts for non-VA sites.

10.2.5.2 FOR VA SITES (EVERY 3 MONTHS)

The Order filters allow VA site staff to identify participants whose last orders were 80 days ago. If a study drug order has not been placed for a participant within the past 90 days, an alert will appear on the study website dashboard under Study Drug System alerts. See **Appendix 10.1h** instructions to resolve prescription renewal alerts for VA sites.

10.2.6 INITIATING UNSCHEDULED STUDY DRUG ORDERS

Occasionally, participants may require additional shipments of the study drug outside of the regular 3-month shipment schedule. Unscheduled shipments may be needed if:

- A regularly scheduled shipment was returned to the PCC as undeliverable.
 - The Study Drug System will send an alert to site staff indicating the reason why a study drug shipment was undeliverable, including instructions on what actions to take.
- A participant did not receive a shipment, although delivery was confirmed by USPS.
- A shipment arrived with a damaged bottle.
- A participant lost their study drug bottle.
- A participant moves or travels and forgot to take their study drug with them.
- The participant discontinued the study drug, but is ready for re-challenge.

See **Appendix 10.1e** for instructions to initiate an unscheduled shipment of the study drug.

10.3 CONSIDERATIONS FOR STUDY DRUG ORDERS AND RENEWALS

Each participant's eligibility to receive the study drug is confirmed through the enrollment and randomization processes. A participant's eligibility to continue receiving the study drug is assessed by the site staff. If the participant is deceased or the site has indicated they are "inactive-permanent" or "inactive-temporary" the study drug alerts will not fire. If the data entered indicates that a participant is deceased or the study drug status indicates that they are "inactive-permanent" or "inactive-temporary," the study drug alerts will not fire.

Delegated site staff are responsible for verifying each participant's eligibility to continue receiving the study drug prior to confirming study drug renewal. In all instances, site staff may use their discretion to determine if it is appropriate to renew the study drug for a participant. The following questions must be considered when renewing the study drug:

1. If yes to any, study drug cannot be renewed at this time. Update the Study Drug Status (can reconsider as needed):
 - a. Has this participant died?
 - b. Is the participant currently known to be taking an open-label statin?
2. If any of the following are present, verify with site PI or medications prescriber that the participant is still eligible for study drug renewal. If needed, update the Study Drug Status:
 - a. Is there currently unexplained persistent transaminase elevations?
 - b. Is there current active liver disease?
 - c. Has the participant experienced markedly elevated creatine phosphokinase (CPK) levels or myopathy while taking study drug?
 - d. Has the participant experienced any other adverse effect while taking study drug that might prevent the participant from being able to take atorvastatin 40 mg once daily in a reasonably safe manner?
 - e. Is there an active order in the participant's medical record for an open-label statin medication?
 - f. Is the participant now chronically using any of the medications listed in the table below?

Clarithromycin (Biaxin®)	Fosamprenavir (Lexiva®)	Niacin [Dose $\geq$ 1 g/day] (Niaspan®)
Colchicine (Colcrys®, Gloperba®, Mitigare®)	Gemfibrozil (Lopid®)	Pibrentasvir (Mavyret®)
Cyclosporine [Systemic] (Gengraf®, Neoral®, SandIMMUNE®)	Glecaprevir (Mayvret®)	Ritonavir (Norvir®, Viekira®, Kaletra®, Technivie®)
Darunavir (Prezcobix®, Prezista®, Symtuza®)	Grazoprevir (Zepatier®)	Saquinavir (Fortovase®, Invirase®)
Elbasvir (Zepatier®)	Itraconazole (Onmel®, Sporanox®, Tolsura®)	Simeprevir (Olysio®)
Erythromycin (E.E.S.®, E.E.S.-400®, Ery-Tab®, Erythrocin™)	Lopinavir (Kaletra®)	Tipranavir (Aptivus®)
Fenofibrate (Tricor®, Triglide®, Lipofen®, Fenoglide®, Trilipix®, Antara®)	Nelfinavir (Viracept®)	

The medications in the table above have the potential for drug-drug interactions with atorvastatin. If a participant is taking one of these medications, the study PI or site prescriber should review the potential for the interaction to pose unreasonable risk to this participant at the time of study drug renewal. While these medications may be taken in combination with atorvastatin, participants who are using one of these medications with the study drug should be monitored to ensure the participant is tolerating both without side effects.

Some of these medications may be prescribed for short periods of time to treat an acute illness, e.g., a short course of colchicine to treat a gout flare or a short course of erythromycin or clarithromycin to treat an infection. In these cases, the study drug can be paused temporarily.

There are no known interactions between the study drug and Tylenol (**acetaminophen**) or between the study drug and any **vitamins or minerals**.

A PREVENTABLE participant who is newly prescribed **gemfibrozil** or **fenofibrate** does not need to stop participating in the study. Gemfibrozil or fenofibrate may be prescribed with atorvastatin as long as the participant is tolerating both medications. If the participant is not tolerating both medications and is already enrolled in PREVENTABLE, the site investigator should assess if the study drug should be continued and discuss the best course of action with the participant, including pausing or discontinuing the study drug. In all cases, participant retention in study activities including annual calls, or minimally, collection of their EHR data, should be discussed.

Participants may temporarily stop the study drug while taking **Paxlovid** (nirmatrelvir tablets; ritonavir tablets) for COVID-19. The typical course for Paxlovid is 5 days. Participants taking Paxlovid should stop taking the study drug for approximately 1-2 weeks while taking Paxlovid.

Colchicine and Study Drug Interaction

Of the medications listed in the table above, **colchicine** is the most used in this study's age group. Most use is short term which can be managed by pausing the study drug while the participant is taking colchicine. Colchicine is sometimes prescribed for long-term use for a chronic condition. "Long-term use" is defined as taking the medication daily without any plans to discontinue use at some point in the future. In these cases, the ability to safely take study drug with colchicine should be evaluated. If it is determined that study drug needs to be paused while the colchicine use is ongoing, the site should monitor the participant to see if they stop taking colchicine at a future date, at which point study drug should be restarted.

10.4 ASSESSING MUSCLE SYMPTOMS

Muscle symptoms may include aches, cramps, heaviness, weakness, pain, tenderness, or stiffness.

Participants should be advised to promptly tell their primary care doctor or study doctor if they have unexplained muscle pain, tenderness, or weakness, particularly if accompanied by malaise or fever. The study drug may need to be discontinued if markedly elevated CPK levels occur or myopathy is diagnosed or suspected. The study drug should be temporarily paused or discontinued in any participant with an acute, serious condition suggestive of a myopathy or who has a risk factor for developing renal failure secondary to rhabdomyolysis (e.g., severe acute infection, hypotension, major surgery, trauma, severe metabolic, endocrine, or electrolyte disorders, or uncontrolled seizures).

Potential participants may be hesitant to take a statin because of their reputation for causing muscle aches. Despite the extraordinary benefits of statins for cardiovascular health, many people who begin statins abandon them because of muscle pain they attribute to the statin, known as statin-associated muscle symptoms (SAMS). The SAMSON trial, a published study, suggests the incidence of SAMS is unlikely to be as high as perceived. The study compared participants on a statin, placebo, and no pill and found that around 90% of symptoms reported by participants taking the statin were also reported by participants taking the placebo. This is due to the "nocebo" effect, which occurs when patients' negative beliefs about a drug cause them to experience side effects. Around 10% of SAMSON participants did not tolerate a statin, but the trial results should reassure those already taking a statin or considering taking a statin that most people do not have statin-related side effects, even if they have some muscle aches.

Participants may have muscle symptoms unrelated to the study drug. For this reason, it is important for the site staff to discuss symptoms with the participant and pause and re-challenge if possible. When

evaluating a participant for statin-associated muscle symptoms it is important to consider other causes of muscle symptoms, including:

- Recent physical exertion
- Changes in exercise patterns
- Drug interaction with a statin
- Underlying muscle disease
- Hypothyroidism
- Concurrent illness

The Statin-Associated Muscle Symptoms Clinical Index (SAMS-CI) may be helpful in determining the likelihood that a participant's muscle symptoms are due to statin use. The authors of the SAMS-CI tool have provided permission for the tool to be used in the PREVENTABLE trial. The SAMS-CI uses a standardized set of questions, along with a scoring algorithm, to support detection of SAMS in clinical practice and recommends treatment options. See **Appendix 10.3** for a copy of the SAMS-CI. The American College of Cardiology app called "Statin Intolerance," available on the Apple and Android app stores is another recommended tool.

Consuming large amounts of grapefruit or grapefruit juice can increase the levels of certain statins in the blood. This is because proteins in grapefruit can affect how the liver processes the statin. The risk of grapefruit causing a problem is very low because statins are very safe. A moderate amount of grapefruit is not likely to cause any adverse effects. If a participant who consumes grapefruit notices muscle aches from the statin, they can first try eliminating grapefruit from their diet. If muscle aches persist, they should discuss study drug intolerance with their study doctor. We are not aware of any interactions between statins and grape juice. See the Study Drug Handout in **Appendix 10.4** for more details.

Small studies suggest Coenzyme Q10 can be helpful for muscle aches, but the evidence is low quality. However, participants may take Coenzyme Q10 if they find it beneficial.

NOTE: There is no need for annual checks on liver function enzymes in the absence of symptoms when levels are normal at baseline. Potential participants with active liver disease at baseline are excluded, and the annual drug renewal process ensures regular review for new liver disease. All participants should inform their health care providers of the atorvastatin and placebo study drug in PREVENTABLE to ensure clinical support if new symptoms occur.

10.5 STUDY DRUG DISCONTINUATION AND RE-CHALLENGE PROCEDURES

If a participant discontinues the study drug due to possible SAMS, it may be appropriate to re-challenge the participant if (1) there is no improvement in symptoms following study drug discontinuation or (2) it becomes clear from other events that the symptoms are unlikely to be related to study drug. Sites have discretion to determine the re-challenge regimen for their participants.

One possible re-challenge regimen is: After stopping treatment for 14 days, restart at 1 tablet every other day for 7 days, then increase to 1 full tablet daily. If similar muscle symptoms recur or are exacerbated with the re-challenge, the participant should discuss their symptoms with their site investigator to decide if they should permanently discontinue the study drug. The participant will still be followed for the remainder of the study.

If a participant experiences muscle symptoms or any other side effects, they should tell their site investigator. Some symptoms attributed to the study drug may be unrelated. If the participant and the site investigator agree that the side effects may be related to the study drug, then the participant should take a 2-week break from taking the study drug. After this, the participant should resume taking the study drug at half dose by cutting the tablet in half or taking the pill every other day. With the guidance of the site investigator, the participant can increase their dose over time back up to 1 full tablet daily.

If the participant wants to discontinue the study drug after these steps, they should speak with their site investigator. The site investigator would indicate discontinuation of study drug in the participant's record on the study website. Even if the participant discontinues the study drug, they will remain in the study.

10.5.1 RE-CHALLENGE OF THE STUDY DRUG AND STUDY DRUG STATUS

The Study Drug Status on a participant's Information tab in the study website automatically defaults to "Active-full" following randomization. "Active-full" status describes a participant that is taking the study drug per the protocol. Any time a participant is to be re-challenged with the study drug, update their Study Drug Status to "Active-full" or "Active-modified" on the study website.

If a participant tells site staff they are taking the study drug for 4 days or less per week, or they report not taking the full dose each day, the participant's Study Drug Status needs to be changed to "Active-modified" with the date that the participant reports starting that change in adherence. Site staff should review the study procedures with the participant and discuss the importance of following study procedures, including taking the study drug as prescribed. See **Appendix 10.1f** for instructions to update the Study Drug Status.

NOTE: A participant can remain in the study even if they are not taking the study drug as prescribed. See **Chapter 11** for details about retention status.

When determining if a modified dose is appropriate, please note that the study drug tablet is a smaller-sized tablet (oblong, 15mm x 8mm). If a participant has difficulty swallowing or develops a swallowing issue after they are randomized, they may cut the pill in half or crush it and take it with applesauce or pudding. Grinding or crushing the study drug should be recommend as a last resort as there may be issues with palatability.

NOTE: The Study Drug System will not automatically update the frequency of study drug shipments for participants with a modified Study Drug Status. The system will continue to handle shipments and alerts as if the participant's status were "Active-full."

10.5.2 STUDY DRUG DISCONTINUATION

The use or discontinuation of the study drug is at the discretion of the site investigator. Site investigators and site staff should use the criteria outlined in **Sections 10.3 and 10.4** when determining if a participant should discontinue the study drug.

Any time the study drug is discontinued for more than 14 days by the participant or site investigator, it should be documented on the participant's Information tab on the study website. See **Appendix 10.1g** for instructions to update the Study Drug Status to "Inactive-temporary" or "Inactive-permanent."

NOTE: A participant can remain in the study after discontinuing the study drug. See **Chapter 11** for details about retention status.

10.5.3 STUDY DRUG DISPOSAL

Participants that receive a new bottle of the study drug and still have study drug tablets in their old bottle should be encouraged to stop taking the study drug from the old bottle and immediately start taking the study drug from the new bottle.

Participants that need to dispose of the study drug should mix the unused study drug tablets with dirt, cat litter, or used coffee grounds in a plastic bag and to dispose of it in the trash. See the Study Drug Handout in **Appendix 10.4**.

Participants on a modified study drug status will continue to receive study drug shipments as if on a fully active study drug status. These participants should be encouraged to safely dispose the extra study drug and gradually return to “Active-full” status, taking the study drug once a day as prescribed, under the guidance of the site investigator.

10.5.4 MISSED DOSES OF THE STUDY DRUG NOT DUE TO INTOLERANCE

Participants may miss multiple doses of the study drug due to hospitalization, illness, travel, or other circumstances. In these cases, the participant should restart the study drug when they are able to, as long as there are no new reasons the participant should not continue taking the study drug. If the site is aware that the study drug will be paused for more than 14 days, the site should change the participant’s study drug status to “Inactive-temporary” on the study website.

10.6 STUDY DRUG SYSTEM ALERTS

Sites can access their Study Drug System alerts on the study website. See **Appendix 10.1h** for details on these alerts and how to resolve them.

10.7 UNMASKING

Efforts to ensure that neither participants nor clinicians are aware of treatment assignment are vitally important to maintain study masking. Several study procedures are in place to prevent unmasking:

- Only the Data Coordinating Center (DCC) and the study pharmacy have access to participants’ treatment assignments.
- The study drug is mailed directly to participants in packaging that does not indicate if the contents are the active treatment or placebo.
- The appearance of the atorvastatin tablets matches the placebo tablets. However, the atorvastatin tablets may taste bitter if crushed and a bitter taste is not notable with the placebo.
- Clinicians at participating sites are asked not to order lipid panels while study participants are taking the study drug. However, if the study drug is permanently discontinued, lipid panels can be ordered as needed to direct clinical care.
- Sites use a variety of methods to communicate about the study with outside clinicians, including automated notifications in the EHR where possible.

At the baseline visit, participants receive a PREVENTABLE wallet card with the enrolling site's contact information. Clinicians may contact either the enrolling site or the PREVENTABLE Helpline to request information about study participation.

10.7.1 EMERGENCY UNMASKING AND HELPLINE

Due to extensive knowledge of and experience with atorvastatin and the inert nature of the placebo, there is no need for emergency unmasking in this study. However, any participant with a suspected study drug-related event should be treated as though they are taking the active study drug (atorvastatin). In the rare event that unmasking is needed to inform clinical care, site investigators or clinicians outside the study may call the Helpline at **1-833-385-1824** to discuss unmasking. Helpline staff or the assigned CRA or Site Manager can request permission from study leadership to provide the participant's study drug assignment.

Site staff may use the Helpline for clinical questions, but it is preferred that site staff contact the site's assigned CRA or Site Manager, or the study clinical PI, Dr. Karen Alexander (karen.alexander@duke.edu). The CRA or Site Manager can facilitate clinical advice from study leadership.

NOTE: The Helpline is not to be used for medical emergencies as the number is monitored only during business hours, Monday-Friday, 8 a.m. – 5 p.m. ET. Please leave a message and your call will be returned within 48 hours.

The following are some potential scenarios and recommended steps:

- If a participant ingests a large quantity of study drug tablets (i.e., more than 10 tablets), treat them as if they took the active study drug (atorvastatin 40 mg).
- If a participant ingests a small quantity of study drug tablets (i.e., fewer than 5), pause taking the study drug for the corresponding number of days and resume taking the study drug after the pause to ensure the participant has an adequate supply.
- If a participant has an allergic reaction possibly related to the study drug, discontinue the study drug and call the Coordinating Center to discuss unmasking.

10.7.2 UNINTENTIONAL UNMASKING OF PARTICIPANT OR SITE STAFF

Drawing clinical lab lipid panels on PREVENTABLE participants is actively discouraged to prevent unmasking participants. All other routine clinical laboratory testing is permitted. If lipid panels are drawn for a participant during routine clinical care, the site should notify the Site Management and Monitoring team at the Duke Clinical Research Institute (DCRI) or the VA of unintentional unmasking as soon as possible. The site PI and the Clinical Coordinating Center (CCC) PI should discuss options for continued follow-up. The participant can continue taking the study drug if appropriate.

If the decision is made to start an open-label statin, the participant should stop taking the study drug and site staff should change the participant's Study Drug Status on the study website (see **Appendix 10.1g**). The participant will continue to be followed for the remainder of the study.

Unintentional unmasking is not an sIRB-reportable protocol deviation but should be documented in the site's source notes and in the comments section of the participant's record on the study website. The site should remind the clinicians involved in the unmasking to avoid checking lipid panels for study participants.

At the end of the study, all participants will receive information about the trial results and their study drug assignment. They can share this information with their healthcare providers to decide what to do after the study drug is no longer provided.

APPENDIX 10.1 STUDY WEBSITE AND STUDY DRUG SYSTEM PROCEDURES

10.1a CONTACT INFORMATION FORM

For participants at non-VA sites, the mailing address entered in the Contact Information Form on the study website is used to create mailing address labels for study drug shipments. The form includes a “Swap with Alt Address” button. This feature allows site staff to easily switch the mailing address with the alternate address when a “Snowbird” participant informs the site that they need the study drug shipped to their other address.

NOTE: This information is used to create mailing address labels. Please only enter information as it should appear on the mailing address.
(Non-VA Sites only) Please note this address will be used for shipment of the study drug.

2. What is your mailing address?

Swap with Alt Address

145 BICKERSTAFF RD

As a reminder, the PCC can ship only to addresses in states, districts, and territories that are part of the US.

Participants with more than one address listed in their Contact Information Form are easily identified by a snowflake indicator shown under the Participant List view. The Snowbird field can be filtered to identify participants that the site may need to check on seasonally to confirm address changes. There is also a filter on the Site Summary page to create a list of snowbird participants.

DATA ENTRY

PID Lookup

Site List

SITE: 1999 - Non-VA Test

ENTER BARCODE

Site Summary

Participant List

Initialize Participant(s)

Search

All Participants: 63

PID	First Name	Last Name	Snowbird	Lipid Study	Enrollment Status	Date
100000975	Mary	Poppins	*	NO	ELIGIBLE	08-3
100001785	Peter	Pan	*	YES	ELIGIBLE	10-2
100014682	Flynn	Rider	*	NO	ELIGIBLE	11-2

10.1b CONTACT INFORMATION SNOWBIRD DATES ALERT

The Contact Information Form includes a field to enter a date in the future for site staff to receive an alert to swap addresses for a snowbird participant. The mailing address does not automatically switch on the date entered. The site staff must manually switch the address by clicking on the “Swap with Alt Address” button on the Contact Information Form.

3.c Date Clinic wants to be alerted to review Contact Information and swap addresses if appropriate:

01/07/2024

On the date entered, the alert will appear as a “Contact Information Snowbird Dates” alert under the Participant Issues alert category.

1 Contact Information Snowbird Dates

Click on the number next to the alert and follow the instructions in the blue box to resolve the alert.

Contact Information Snowbird Dates

ACTION NEEDED:

The following participant(s) are Snowbirds, and the clinic has specified a date in the Contact Information form (question 3.C) to be alerted for reviewing alternate mailing addresses and swapping them accordingly.

Non-VA Sites: The VACSP Pharmacy will ship study drug to the address listed in the Mailing Address (Question 2 on Contact Info) at the time the order is placed.

VA Sites: In addition to swapping addresses in the Contact Info form, you must add or confirm that the alternate address is in CPRS/Cerner to ensure that the pharmacy ships to the updated mailing address.

To resolve:

1. Review the Contact Information form for details on the Alternate Address information (question 3).
2. If needed, contact the participant to confirm the alternate address or traveling dates.
3. Click the blue button “Swap with Alt address” to update the participant’s mailing address (question 2 in the Contact Information form), and save the form.
4. If an order is required now, place a Study Drug Order immediately. If not, the Study Drug will be shipped to the address on question 2 as scheduled.
5. Click “Resolve” to clear the alert and remove it from the alerts list.
6. Update the date in 3.C on the Contact Information form for the next time you would like to be notified that the participant is switching addresses again and save the contact information form.

Note: Use the Swap feature “Swap with mailing address” when the participant returns to their main address, and utilize the alert function for such cases.

PID	Name	Date of Enrollment	Date Randomized	Date To Be Alerted About Snowbird Address	Click to Resolve
900006331	Harry Potter		08/10/2020	01/07/2024	RESOLVE

10.1c STUDY DRUG SYSTEM INTERFACE

Site staff can use the alerts and filters in the Study Drug System to manage study drug prescriptions and orders. The filters can be used to create a list of participants with important information including each participant’s prescription details, days until their prescription ends, date of birth, date of randomization, and current study drug status.

- Filters
 - Study Drug Status: Filter data based on participants’ study drug status.

- Prescriptions (non-VA sites only): View information about all prescriptions, including current prescriptions and prescriptions expiring in 30 days for active participants.
- Orders: View all orders placed and current orders. For VA sites only, the Orders filter shows participants whose last order was 80 days prior.
- Alerts: View important events including canceled orders, pending study drug prescription renewals, expired study drug prescriptions, and undeliverable orders.

I. STUDY DRUG SYSTEM SITE SUMMARY FOR NON-VA SITES

The Non-VA Site Summary in the Study Drug System offers various filters to help users refine data visibility. These filters allow tracking of all randomized participants and their study drug status. The Prescriptions filters help monitor current prescriptions, those close to expiration, and overall prescription activity. The Orders filter provides details and the history of all orders, highlighting current active orders. The Study Drug System has important customized alerts. These alerts are essential for managing study logistics, identifying issues like canceled orders, pending renewals, or potential disruptions in study drug delivery.

Study Drug System	Site List	SITE: 1705 - Wake Forest - Cardiology	Site Summary
FILTERS		ALERTS	
25	All Randomized Participants	1	Order Has Been Cancelled (Non-VA Sites)
Participant Study Drug Status		1	Order is Pending Rx Renewal
7	Active-full	5	Study Drug Prescription Expired or Expiring within 10 Days
0	Active-modified	1	Study Drug Undeliverable
1	Inactive-temporary		
17	Inactive-permanent		
Prescriptions			
21	All Prescriptions		
2	Current Prescriptions		
1	Prescriptions Expiring in 30 days - Active Participants		
Orders			
24	All Orders		
9	Current Orders		

II. STUDY DRUG SYSTEM SITE SUMMARY FOR VA SITES

The Study Drug System Site Summary page for VA sites is customized for study drug management purposes. Data can be filtered to view study drug information for all randomized participants, distinguishing between active and inactive study drug statuses. The Orders filter provides both current and historical order details, including the last order placed, and highlights individuals whose last order was placed 80 days prior. The system also includes alerts for various aspects of study drug management like canceled orders and VA participants for whom a study drug order has not been placed in more than 90 days.

Study Drug System	Site List	SITE: 2021 - Long Beach VAMC	Site Summary
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FILTERS		ALERTS	
7	All Randomized Participants	1	Order Has Been Cancelled (VA Sites)
Participant Study Drug Status		3	VA - No Drug Order in 90 Days
2	Active-full		
0	Active-modified		
1	Inactive-temporary		
4	Inactive-permanent		
Orders			
9	All Orders		
7	Current Orders		
2	Last Order 80 Days Ago		

III. INFORMATION PAGE

After navigating to a participant within the Study Drug System, site staff can view the participant's Information page, which includes their participant ID (PID), name, date of birth, retention status, study drug status, and their primary and alternate contact information. Click on the "Contact Information Form" button to update participant's contact information.

Study Drug System	Site List	SITE: 1705 - Wake Forest - Cardiology	Site Summary	PID: 900000058 - Harry Potter
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Information	Participant Information		Primary Contact Information	
Prescriptions	PID:	900000058	Mailing Address:	3028 CAMBRIDGE RD WINSTON SALEM, NC 27104
Drug Orders	Name:	Harry Potter	Phone Number:	336-111-2222
	Date of Birth:	01/02/1925	Alternate Contact Information	
	Retention Status:	UPDATE 02/24/2023 - Active, complete all procedures	Mailing Address:	4631 BRIDLE PATH WINSTON SALEM, NC 27103
	Study Drug Status:	UPDATE 09/19/2023 - Active-full	Phone Number:	336-999-8888

Contact Information Form - this will open in a new tab

IV. PRESCRIPTIONS PAGE (NON-VA SITES ONLY)

The participant's Prescriptions page in the Study Drug System provides detailed prescription information including the RX number, creation date, confirmation status, and start and end dates. On the Prescriptions page, site staff can also view the participant's medications prescriber and confirm study drug prescription renewals.

PREVENTABLE Manual of Procedures

Chapter 10: Study Drug

Study Drug System

Site List

SITE: 1504 - U Pittsburgh

Site Summary

PID:

Information

Prescriptions

Drug Orders

Current Medications Prescriber:

Confirm Study Drug Prescription Renewal 

Prescriptions

Rx Number	Create Date	Confirmed By	Confirmed Date	Start Date	End Date
12403	01/16/2023 03:15 AM		06/15/2023	02/28/2023	02/28/2024
4313	02/02/2022 09:33 PM		05/23/2022	02/27/2022	02/27/2023
541	02/26/2021 09:17 AM		02/26/2021	02/26/2021	02/26/2022

V. DRUG ORDERS PAGE

The Drug Orders page in the Study Drug System provides detailed information about the participant's study drug orders including current prescription information, RX number, refill number, and start and end dates. The Drug Orders page also includes order details such as status, order date, ship date, and delivery date. Tracking information is regularly updated through USPS, providing current updates on order status, including whether it is canceled, delivered, in transit, and more. Drug order alerts are triggered based on tracking information updates.

Non-VA site staff can place unscheduled drug orders from the Drug Orders page if there is an active prescription.

Study Drug System

Site List

SITE: 1102 - Rush

Site Summary

PID:

Information

Prescriptions

Drug Orders

Current Prescription Info:

Rx Number:

19633

Start Date:

10/18/2023

End Date:

10/17/2024

Place Unscheduled Drug Order

Drug Orders

Rx Number	Refill Number	Status	Order Date	Ship Date	Delivery Date	Tracking Info
19633	1	delivered	01/21/2024 03:15 AM	01/23/2024 12:32 PM	01/25/2024 12:27 PM	 Delivered
8643	3	cancelled	07/09/2023 03:15 AM	07/11/2023 02:07 PM	07/13/2023 12:00 AM	 In Transit
8643	0	cancelled	12/25/2022 03:15 AM	12/27/2022 11:41 AM	12/30/2022 12:00 AM	 Alert

VI. CONFIRM STUDY DRUG PRESCRIPTION RENEWAL PAGE (NON-VA SITES ONLY)

The Confirm Study Drug Prescription Renewal page prompts site staff to consider several factors before renewing the study drug for a participant (see **Section 10.3**). This page outlines specific conditions that may affect the participant's eligibility for renewal. Site staff are asked to confirm or deny certain criteria, such as vital status, ongoing medical conditions (e.g., liver disease), adverse effects experienced during the study, and medication use. If any of these conditions are present, site staff should verify the participant's eligibility for renewal with the site PI or medications prescriber before proceeding with the renewal. This ensures participants receive the study drug safely and in accordance with the study criteria.

Confirm Study Drug Prescription Renewal
Confirm Renewal

Consider the following to Confirm Renewal of Study Drug for [REDACTED]

1. If yes to any, study drug cannot be renewed at this time. Update the Study Drug Status (can reconsider as needed).
 - a. Has the participant died?
 - b. Is the participant currently known to be taking an open-label statin?
2. If any of the following are present, verify with site PI or prescriber that this participant is still eligible for study drug renewal at this time.
 - a. Is there currently unexplained persistent transaminase elevations?
 - b. Is there current active liver disease?
 - c. Has the participant experienced markedly elevated creatine phosphokinase (CPK) levels or myopathy while taking study drug?
 - d. Has the participant experienced any other adverse effects while taking study drug that might prevent the participant from being able to take Atorvastatin 40 mg once daily in a reasonably safe manner?
 - e. Is there an active order in the participant's medical record for an open-label statin medication?
 - f. Is the participant now chronically using any of the medications listed in the table below?

Clarithromycin (Biaxin®)	Gemfibrozil (Lopid®)	Piorentasvir (Mavyret®)
Colchicine (Colcrys®, Glorperba®, Mitigare®)	Glecaprevir (Mavyret®)	Ritonavir (Norvir®, Viekira®, Kaletra®, Technivie®)
Cyclosporine (Systemic) (Gengraf®, Neoral®, Sandimmune®)	Grazoprevir (Zepatier®)	Saquinavir (Fortovase®, Invirase®)
Darunavir (Prezcobix®, Prezista®, Symtuza®)	Itraconazole (Orinell®, Sporanox®, Tolsura®)	Simeprevir (Olysio®)
Ebastin (Zepatier®)	Lopinavir (Kaletra®)	Tegasev (Aptivus®)
Fenofibrate (Tricor®, Triglide®, Lipofen®, Fenoglide®, Trilipix®, Antara®)	Nelfinavir (Viracept®)	
Fosamprenavir (Lexiva®)	Niacin (Dose ≥ 1 g/day) (Niaspan®)	

Confirm Renewal

10.1d SCHEDULED PRESCRIPTION RENEWALS

NOTE: Only site staff who have been delegated the responsibility of submitting orders for the study drug will be able to attest to continued appropriateness and complete this step.

FOR NON-VA SITES (every 12 months)

Prescription renewal alerts for non-VA sites will appear on the study website dashboard under Study Drug System alerts, with a number that indicates how many Study Drug System alerts need action. Clicking on the number will open the Study Drug System Site Summary page to review a list of participants whose prescriptions are expiring within 30 days. The alerts section will highlight, by participant, any Study Drug Prescription Expired or Expiring within 10 Days. For each participant requiring a study drug prescription renewal, see **Sections 10.3 and 10.4** to review the criteria to determine if it is appropriate for a participant to continue taking the study drug.

- a. If it is appropriate for the participant to continue taking the study drug:
 - i. On the participant's Prescription tab, click on "Confirm Study Drug Prescription Renewal." The list of questions in **Section 10.3** will be displayed.
 - ii. Attest to the participant's continued eligibility for the study drug by clicking "Confirm Renewal." This will automatically transmit the renewed order for study drug (i.e., Atorvastatin 40 mg or Matching Placebo, Take one tablet by mouth once daily, 100 tablets, PRN Refills) from the DCC to the PCC. The participant should receive their study drug in the mail within 5 business days.
 - iii. The Drug Orders page will show the new record and a status of "Ordered," and the participant will be removed from the Study Drug System alerts list.
- b. If it is *not* appropriate for the participant to continue taking the study drug:
 - i. Navigate to the participant's Information tab under Data Entry > Study Drug Status and click "Enter New Study Drug Status" to update the Study Drug Status for the participant. Enter the reason the study drug is to be stopped, the effective date, conversation details, and which site investigator agrees and signed off on the decision. See **Appendix 10.1g**.
 - ii. To document a participant's death, see **Chapter 11**.
 - iii. Once the reason for discontinuation is documented and saved, the participant will be removed from the Study Drug System alerts list.

The Site Summary page for non-VA sites includes a filter for "Prescriptions Expiring in 30 days - Active Participants," that displays the number of days until the current prescription expires or the next prescription renewal is needed, "Days until Rx Ends." Sites should use this to plan for reviewing the Confirmation Renewal questions and the participant's EHR, and to allow time to confirm the participant's eligibility to continue taking the study drug. Failing to confirm a prescription renewal will generate a "Study Drug Prescription Expired or Expiring within 10 days" alert. See **Appendix 10.1h** for instructions to address Study Drug System alerts.

FOR VA SITES (every 3 months)

Prescription renewal alerts for VA sites will appear on the study website dashboard under Study Drug System alerts. Clicking on the number of alerts will open the Study Drug System Site Summary page. This includes information such as the number of participants in different study drug statuses and orders, including a list of participants whose last orders were 80 days ago. Additionally, the alerts section will highlight any Initial Orders Expected or participants with No Drug Order in 90 Days. For each participant, review the criteria to determine if it is appropriate for the participant to continue taking the study drug (see **Sections 10.3 and 10.4**).

- a. If it is appropriate for the participant to continue taking the study drug:
 - i. Follow the steps in **Appendix 10.2** to place an order for the study drug in the VA EHR.

- ii. The participant will be removed from the Study Drug System alerts list once the PCC notifies the DCC that the study drug was shipped to the participant.
- b. If it is *not* appropriate for the participant to continue taking the study drug:
 - i. Navigate to the participant's Information tab under Data Entry > Study Drug Status and click "Enter New Study Drug Status" to update the Study Drug Status for the participant. Enter the reason the study drug is to be stopped, the effective date, conversation details, and the site investigator that approved the decision. See **Appendix 10.1g**.
 - ii. To document a new death, see **Chapter 11**.
 - iii. Once the reason for discontinuation is documented and saved, the participant will be removed from the Study Drug System alerts list.

The Site Summary page for VA sites includes a filter for "Last Order 80 days Ago," that displays the number of days since the last order. Sites should use this list to plan time to review the Confirmation Renewal questions and each participant's EHR to confirm the participant's eligibility to continue taking the study drug before placing a study drug order. If the participant's prescription is not renewed, an alert will be generated for "VA – No Drug Order in 90 Days."

10.1e INITIATING UNSCHEDULED STUDY DRUG ORDERS

FOR NON-VA SITES

On the study website, navigate to Data > Study Drug System and click the "All Randomized Participants" filter. Search the participant by name or PID, click on their name, click on the Drug Orders tab, and click the "Place Unscheduled Drug Order" button. Complete the steps to attest to the participant's eligibility for the study drug and the order will be automatically placed. A valid prescription and assigned medications prescriber is required for the system to place orders.

FOR VA SITES

Follow the steps in **Appendix 10.2** to place an order for the study drug in the VA EHR. The Study Drug System will show a new order for the participant once the PCC notifies the DCC that the study drug was shipped to the participant.

10.1f RE-CHALLENGE OF THE STUDY DRUG AND STUDY DRUG STATUS

Any time a participant is re-challenged with the study drug, update their Study Drug Status to "Active - Full" or "Active – Modified" on the study website:

1. In the study website menu, click on Data Entry > Study Drug System.
2. Type in the participant's name or PID or select the participant from the list.
3. Click on the participant's Information tab.
4. Click on the "Enter New Study Drug Status" button and change it to "Active – full" or "Active – modified."

5. Enter the reason and effective date. The new effective date cannot be prior to the current effective date.
6. Enter detailed information in the Conversation Details section.
7. Select the site investigator that approved the decision.
8. Click the green “Enter” button to save the information.

For non-VA sites: If sufficient time has passed since the last study drug shipment and an active prescription exists for a participant, the Study Drug System will automatically place an order the night after the site staff updates the Study Drug Status. This can be confirmed by finding the new order in the Study Drug System. Alternatively, the site’s medications prescriber can order the study drug for the participant using the procedures for unscheduled drug shipments described in **Appendix 10.1e**.

The following **Study Drug and Retention Status** Histories information is used in statistical analysis.
If there is an error in either of these tables, please submit a detailed RAP so that it can be corrected.

STUDY DRUG STATUS
ENTER NEW STUDY DRUG STATUS


STATUS:	REASON:	INVESTIGATOR:	DATE ENTERED:	EFFECTIVE DATE:	INACTIVE-PERMANENT APPROVAL:
CURRENT STUDY DRUG STATUS					
Active-full			01/28/2024	01/28/2024	--
STUDY DRUG STATUS HISTORY					
Active-full			01/28/2024	01/28/2024	--
Active-full			01/28/2024	01/28/2024	--

If the participant wishes to modify their study drug status, please document this. If you find out after the fact that a participant stopped taking the study for a period of time, and has since resumed taking the study drug, this only needs to be documented if they stopped taking study drug for 14 days or longer. Enter a status of temporarily inactive, effective date the date they stopped taking study drug. Then immediately enter a status of active-full (or active-modified, as appropriate), effective date the date they resumed taking the study modification. If a participant stopped the study drug for 13 days or less, this does not need to be entered into the system.

Please note that changes to study drug DO NOT impact retention. A person can still be followed even if they are on a modified dose or no longer taking study drug. A detailed comment must be entered documenting conversations with the participant.

CURRENT STUDY DRUG STATUS:	REASON:	INVESTIGATOR:	DATE ENTERED:	EFFECTIVE DATE:
Active-full			01/28/2024	01/28/2024

ENTER NEW STUDY DRUG STATUS

Study Drug Status:

Reason:

Effective Date: ***New Effective Date cannot be prior to current Effective Date***

Conversation Details:

Select which investigator signed off and agrees with this decision:

ENTER

10.1g STUDY DRUG DISCONTINUATION

Any time study drug is discontinued for more than 14 days by the participant or clinician, it should be documented in the study website:

1. In the study website menu, click on Data Entry > Study Drug System.
2. Type in the participant's name or PID or select the participant from the list.
3. Click on the participant Information tab.
4. Click on the "Enter New Study Drug Status" button and change it to "Inactive-permanent" or "Inactive-temporary."
5. Enter a reason from the drop-down menu. Discontinuation options include:
 - Myopathy, myalgia, joint pain, or other muscle-related symptoms
 - Liver enzyme elevation
 - Open-label statin prescribed
 - Other intolerances (i.e., brain fog, difficulty concentrating, fatigue, lack of energy, sleep disturbances)
 - Full withdrawal
 - GI symptoms including diarrhea, stool softening, or nausea
 - Provider choice unrelated to symptoms
 - Participant choice unrelated to symptoms
6. Enter the effective date. The new effective date cannot be prior to the current effective date.
7. Enter detailed information in the Conversation Details section.
8. Select the site investigator that approved the decision.
9. Click the green "Enter" button to save the information.
10. Shipments of the study drug to the participant will automatically stop.

The following **Study Drug** and **Retention Status** Histories information is used in statistical analysis.
If there is an error in either of these tables, please submit a detailed RAP so that it can be corrected.

STUDY DRUG STATUS		ENTER NEW STUDY DRUG STATUS 			
STATUS:	REASON:	INVESTIGATOR:	DATE ENTERED:	EFFECTIVE DATE:	INACTIVE-PERMANENT APPROVAL:
CURRENT STUDY DRUG STATUS					
Active-full			01/28/2024	01/28/2024	--
STUDY DRUG STATUS HISTORY					
Active-full			01/28/2024	01/28/2024	--
Active-full			01/28/2024	01/28/2024	--

If the participant wishes to modify their study drug status, please document this. If you find out after the fact that a participant stopped taking the study for a period of time, and has since resumed taking the study drug, this only needs to be documented if they stopped taking study drug for 14 days or longer. Enter a status of temporarily inactive, effective date the date they stopped taking study drug. Then immediately enter a status of active-full (or active-modified, as appropriate), effective date the date they resumed taking the study modification. If a participant stopped the study drug for 13 days or less, this does not need to be entered into the system.

Please note that changes to study drug DO NOT impact retention. A person can still be followed even if they are on a modified dose or no longer taking study drug. A detailed comment must be entered documenting conversations with the participant.

CURRENT STUDY DRUG STATUS:	REASON:	INVESTIGATOR:	DATE ENTERED:	EFFECTIVE DATE:
Active-full			01/28/2024	01/28/2024

ENTER NEW STUDY DRUG STATUS

Study Drug Status:

Reason:

Effective Date: ***New Effective Date cannot be prior to current Effective Date***

Conversation Details:

Select which investigator signed off and agrees with this decision:

ENTER

ENTER NEW STUDY DRUG STATUS

Study Drug Status:

Inactive-permanent

Reason:

- ✓ --
- Myopathy, myalgia, joint pain, or other muscle related symptoms
- Liver enzyme elevation
- Open-label statin prescribed
- Other intolerances (i.e. brain fog, difficulty concentrating, fatigue, lack of energy, sleep disturbances)
- Full withdrawal
- GI symptoms including diarrhea, stool softening or nausea
- Provider choice unrelated to symptoms
- Participant choice unrelated to symptoms

10.1h STUDY DRUG SYSTEM ALERTS

The Study Drug System provides important customized alerts to help sites manage study logistics. See below for details on alerts and how to resolve them.

I. OUTSTANDING ORDER ALERT

An “Outstanding Order” alert indicates an order has been in the same status for more than 7 days. The order status in the Study Drug System is based on the shipment’s status in the USPS system.

- a. Instructions – To resolve this alert, contact the participant to determine if they received the shipment and then take the appropriate action(s) below.
- b. Actions
 - i. **Confirm Delivery:** If the participant confirms they received the study drug, click "Confirm Delivery in the alert."
 - ii. **Place Order:** If the participant confirms they did not receive the study drug or tracking information indicates the study drug may not have been delivered:
 1. **Non-VA sites:** click on "Place Order" to place a new study drug order.
 2. **VA sites:** place a new study drug order in the VA EHR system.
 - iii. **Update Study Drug Status:** If the participant should no longer receive the study drug, click the “Update Study Drug Status” button to go to the participant’s Information tab and update their Study Drug Status.
 - iv. **Submit a RAP:** If none of the other actions seem appropriate, click “Submit a RAP” (Report a Problem) in the study website main menu or quick links.

The alert will resolve once “Confirm Delivery” is clicked, a new order is placed (non-VA sites), the PCC notifies the DCC the order has been shipped (VA sites), or the participant’s Study Drug Status is changed.

II. STUDY DRUG HAS BEEN CANCELED (NON-VA SITES)

An alert for “Study Drug Has Been Canceled” indicates the order was canceled by the pharmacy and the participant will not receive the shipment.

- a. Instructions – To resolve this alert, review the cancellation reason provided, confirm information with the participant if necessary, and then take the appropriate action(s) below. Please enter comments to document follow-up.
- b. Actions
 - i. **Update Consent:** Click the “Update Consent” button to go to the participant’s Information tab to update their consent document. Once updated, click on “Place Order” to place a new study drug order.
 - ii. **Update Contact Information Form:** Click the “Update Contact Information Form” button to go to the participant’s Information tab, click on the Contact Information Form, and update the participant’s name, mailing address, or any other contact information that needs to be updated. Once updated, click on “Place Order” to place a new study drug order.
 - iii. **Update Study Drug Status:** If the participant should no longer receive the study drug, click the “Update Study Drug Status” button to go to the participant’s Information page and update their Study Drug Status.
 - iv. **Place Order:** If you have confirmed and/or corrected the reason for cancellation, click on “Place Order” to submit a replacement study drug order.
 - v. **Duplicate Order:** If the order was canceled because a duplicate order was accidentally placed, click “Confirm Duplicate Order.”
 - vi. **Submit a RAP:** If none of the other actions seem appropriate, click “Submit a RAP” (Report a Problem) in the study website main menu or quick links.

The alert will resolve once a new study drug order is placed, the participant’s Study Drug Status is changed, or “Confirm Duplicate Order” is clicked.

III. STUDY DRUG HAS BEEN CANCELED (VA SITES)

An alert for “Study Drug Has Been Canceled” indicates the order was cancelled by the pharmacy and the participant will not receive the shipment.

- a. Instructions – To resolve this alert, review the cancellation reason provided, confirm information with the participant if necessary, and then take the appropriate action(s) below. Please enter comments to document follow-up.
- b. Actions
 - i. **Update Contact Information Form:**
 - a. Click the “Update Contact Information” button to go to the participant’s Information tab, click on the Contact Information Form, and update the

participant's name, mailing address, and any other contact information if needed.

- b. Ask the participant to update their contact information in the VA EHR system. The VA participant's contact information must be updated in both the study website and the VA EHR system because the PCC must use the participant's shipping address in the VA EHR provided by CMOP. Only participants can update their own address in the VA EHR. To update their own address, the participant must:
 - i. Contact or go to the Admissions and Eligibility Department, or
 - ii. Call an Enrollment Coordinator at 1-877-222-8387.

Once the contact information is updated, place a new order in the VA EHR System.

- ii. **Update Study Drug Status:** If the participant should no longer receive the study drug, click the "Update Study Drug Status" button to go to the participant's Information tab and update their Study Drug Status.
- iii. **Duplicate Order:** If the order was canceled because a duplicate order was accidentally placed, click "Confirm Duplicate Order."
- iv. **Submit a RAP:** If none of the other actions seem appropriate, click "Submit a RAP" (Report a Problem) in the study website main menu or quick links.

The alert will resolve once the PCC notifies the DCC the new study drug order has been shipped, the participant's Study Drug Status is changed, or "Confirm Duplicate Order" is clicked.

IV. STUDY DRUG UNDELIVERABLE ALERT

A "Study Drug Undeliverable" alert indicates that USPS updated the shipment's status to "Delivery Failed" or "Returned to Sender."

- a. Instructions – To resolve this alert, click the "Alert" button to go to the participant's Drug Order screen and review the shipment's tracking information. Contact the participant to confirm their mailing address and then take the appropriate action(s) below.
- b. Actions
 - i. **Confirm Delivery:** If the participant confirms they received the study drug, click "Confirm Delivery."
 - ii. **Update Contact Information Form:** If the participant's contact information has changed, click "Update Contact Information Form" to go the participant's Information tab, click on the Contact Information Form, and update the participant's mailing address and other contact information if needed.

VA sites only: ask the participant to update their contact information in the VA EHR system. The VA participant's contact information must be updated in both the study website and the VA EHR because the PCC must use the participant's shipping address in the VA EHR provided by CMOP. Only participants can update

their own address in the VA EHR. To update their own address, the participant must:

- Contact or go to the Admissions and Eligibility Department, or
- Call an Enrollment Coordinator at 1-877-222-8387.

Once the participant's contact information is updated, place a new order for the study drug.

- Place Order:** If the participant confirms they did not receive the study drug or tracking information indicates the study drug may not have been delivered:
 1. **Non-VA sites:** click on "Place Order" to place a new study drug order.
 2. **VA sites:** place a new study drug order in the VA EHR system.
- Update Study Drug Status:** If the participant should no longer receive the study drug, click "Update Study Drug Status" to go to the participant's Information tab and update their Study Drug Status.
- Submit a RAP:** If none of the other actions seem appropriate, click "Submit a RAP" (Report a Problem) in the study website main menu or quick links.

The alert will resolve once "Confirm Delivery" is clicked, a new order is placed (non-VA sites), the PCC notifies the DCC the new order has been shipped (VA sites), or the participant's Study Drug Status is changed.

V. STUDY DRUG PRESCRIPTION EXPIRED OR EXPIRING WITHIN 10 DAYS (NON-VA SITES ONLY)

A "Study Drug Prescription Expired or Expiring within 10 Days" alert indicates the participant's study drug prescription will expire in the next 10 days. New study drug orders and automatic shipments cannot be processed until the prescription renewal is authorized by the medications prescriber and the renewal is confirmed.

- Instructions – To resolve this alert, review the participant's study record and EHR and ensure the participant is eligible to continue taking the study drug.
- Actions
 - Review the Participant's Medical Information:** Begin by reviewing the participant's medical information to verify their eligibility to continue receiving the study drug. See Sections 10.3 and 10.4 of this chapter for eligibility criteria.
 - Confirm Renewal:** If the participant is eligible to continue receiving the study drug, click "Confirm Study Drug Renewal."
 - Update Study Drug Status:** If the participant should no longer receive the study drug, click "Update Study Drug Status" to go to the participant's Information tab and update their Study Drug Status.

The alert will resolve once the renewal is confirmed or the participant's Study Drug Status is changed. Once the renewal is authorized by the medications prescriber and is confirmed, the prescription will be renewed for another year and automatic shipments will continue.

VI. ORDER IS PENDING RX RENEWAL (NON-VA SITES ONLY)

An “Order is Pending Rx Renewal” alert indicates the participant’s study drug prescription needs to be renewed because it has expired and the last order was over 90 days ago.

- a. Instructions – To resolve this alert, review the participant’s study record and EHR and ensure the participant is eligible to continue taking the study drug.
- b. Actions
 - i. **Review the Participant's Medical Information:** Begin by reviewing the participant's medical information to verify their eligibility to continue taking the study drug. See **Sections 10.3 and 10.4** of this chapter for eligibility criteria.
 - ii. **Confirm Renewal:** If the participant is eligible to continue receiving the study drug, click "Confirm Study Drug Renewal."
 - iii. **Update Study Drug Status:** If the participant should no longer receive the study drug, click “Update Study Drug Status” to go to the participant’s Information tab and update their Study Drug Status.

The alert will resolve once the renewal is confirmed or the participant’s Study Drug Status is changed. Once the renewal is authorized by the medications prescriber and is confirmed, the prescription will be renewed for another year and automatic shipments will continue.

VII. INITIAL ORDER EXPECTED ALERT (VA SITES ONLY)

An “Initial Order Expected” alert indicates an order needs to be placed in the VA EHR system.

To resolve this alert, place a study drug order in the VA EHR system. Once the PCC notifies the DCC the order was shipped, the alert will be resolved.

VIII. NO ORDER IN 90 DAYS (VA SITES ONLY)

A “No order in 90 days” alert indicates an order needs to be placed in the VA EHR system.

To resolve this alert, place a study drug order in the VA EHR system. Once the PCC notifies the DCC the order was shipped, the alert will be resolved.

APPENDIX 10.2 VA SITES: ADDITIONAL INFORMATION ABOUT THE STUDY DRUG

10.2a CENTRALIZED DISPENSING OF THE STUDY DRUG

The process for dispensing the study drug to participants at VA sites differs from the process at non-VA sites. This appendix focuses on the information needed for study staff and local research pharmacists at VA sites to dispense the study drug to VA participants.

Following randomization, the site's medications prescriber will enter each participant's study drug prescription in the VA's electronic health record (EHR) system. After the local research pharmacist verifies the prescription, the EHR system will send the order to the VA Cooperative Studies Program (CSP) Clinical Research Pharmacy Coordinating Center (PCC). The PCC will ship the study drug directly to participants in compliance with the applicable provisions of the VHA Handbook 1108.04. See Section 10.2 for information about reporting study-related concerns to The Joint Commission, the accrediting organization for the PCC.

Because atorvastatin is an FDA-approved drug product, a VA Form 10-9012 (Investigational Drug Information Record) is not nationally required and will generally not be provided to VA sites (refer to VHA Handbook 1108.04). However, this form can be provided if it is required by local site policy.

10.2b ENTERING THE STUDY DRUG IN THE LOCAL DRUG FILE (LOCAL RESEARCH PHARMACISTS)

With assistance from the local pharmacy Automated Data Processing Applications Coordinator (ADPAC), the VA local research pharmacist at each site will need to enter PREVENTABLE's study drug profile in the local drug file and link the entry to the National Drug File (NDF) entry. Linking the entry in the local drug file to the entry in the NDF is required for study drug orders to be sent from the site to the PCC.

- A specific NDF entry has been created for the PREVENTABLE study drug. Local research pharmacists will need to work with their local pharmacy ADPAC to **ensure that the local study drug entry in the pharmacy drug file is linked to the study drug's NDF entry.**
 - The recommended local VA Print Name is **"INV-CIRB#20-18 ATORVASTATIN 40MG/PBO TAB"**. To ensure the name of PREVENTABLE's study drug remains uniquely associated with this study, the study CIRB number assigned to PREVENTABLE during VA CIRB's review will be used in the drug name.
 - The study drug's VA Product Identifier listed in the NDF is **I0686**, and the NDC number in the NDF entry is **99700-3333-01**. (Note: This is not a real NDC number, but is needed to find the profile for the PREVENTABLE study drug in the VA system).
 - After linking the local entry to the NDF, add **"IW"** to the DEA, SPECIAL HDLG field.
 - Adding "I" is required because it identifies the product as investigational. Adding "W" will make the order non-renewable and reduces the risk of non-study providers inadvertently renewing the study drug.
 - The study drug entry in the local drug file also needs to be set up such that the product can be fulfilled by the PCC.

- For informational purposes, the full NDF entry for PREVENTABLE's study drug is:
 - **VA Product Name:** INV-CIRB#20-18 (PREVENTABLE) ATORVASTATIN 40MG/PBO TAB
 - **VA Generic Name:** INV-CIRB#20-18 (PREVENTABLE) ATORVASTATIN 40MG/PBO TAB
 - **VA Print Name:** INV-CIRB#20-18 ATORVASTATIN 40MG/PBO TAB
 - **National Formulary Name:** INV-CIRB#20-18 (PREVENTABLE) ATORVASTATIN 40MG/PLACEBO TAB
 - **VA Product Identifier:** I0686
 - **Dosage Form:** TAB
 - **Dispense Unit:** TAB
 - **VA Class:** CV350
 - **Active Ingredient:** ATORVASTATIN
 - **NDC Number:** 99700-3333-01
 - **Package Size:** 100 TAB
 - **Package Type:** Bottle
 - **Manufacturer:** Apotex Inc.
- VA Pharmacy Benefit Management has indicated that certain drug-drug interactions and therapeutic duplication checks will automatically be performed at the time of prescription verification. This is because the blinded study drug is linked to commercially available atorvastatin in First Databank.

10.2c ORDERING THE STUDY DRUG IN THE VA CPRS (LOCAL SITE INVESTIGATORS)

Before ordering the study drug, ensure the following requirements have been satisfied:

- The study drug has been entered in the local drug file.
- The current version of the delegation log, including any updates, has been provided to the local research pharmacist to verify the name(s) of the site investigator(s) authorized to order the study drug.
- This chapter of the MOP (**Chapter 10**) and this appendix (**Appendix 10.2**) have been provided to the local research pharmacist.
- A copy of each participant's most recent signed and dated Informed Consent Form (ICF) is provided to the local research pharmacist. The VHA Handbook 1108.04 requires that the local research pharmacist be provided with a copy of the most recent ICF for each participant for whom the study drug is to be dispensed. The signed and dated ICF must be received by the local research pharmacist when the initial study prescription is ordered. Without a copy of the signed and dated ICF, research pharmacists cannot review and process initial study prescriptions.

If the ICF is provided to the local research pharmacist electronically rather than in hard copy, the site investigator must provide written assurance that the signed consent is available for viewing and printing in the electronic medical record (VHA Handbook 1108.04) at the time the pharmacist reviews the initial study prescription.

To place the initial or a subsequent order for the study drug in the VA CPRS (screenshot below):

1. Open the Outpatient Medications Order menu.
2. Select the drug product to order: **"INV-CIRB#20-18 ATORVASTATIN 40MG/PBO TAB"**. Order checks may screen for drug-drug interactions or duplication of therapy.
3. Select the route of administration (ORAL) and directions for use under the "Schedule" drop-down menu. Ensure that directions for use are identical or substantively equivalent to: "Take one tablet by mouth once daily." Directions cannot exceed 100 characters and spaces.
4. Ensure that the "Days Supply" field is **90 days** and that the "Refills" field is 0 (zero).
5. Select a tablet quantity of **100**.
 - a. **NOTE:** The tablet quantity may auto-populate based on the directions for use. In the example described under #4 above, the tablet quantity based on directions for use would be 90. However, because each study drug bottle should always contain 100 tablets, the quantity field should be populated with 100 tablets. The additional tablets ensure that participants do not run out of the study drug due to shipping delays.
6. Select "Mail" for the Pick Up option and "Routine" for the Priority option.
7. Accept and then sign the study drug order.

The screenshot shows the "Outpatient Medications" window. At the top, the drug name is "INV-CIRB#20-18 ATORVASTATIN 40MG/PBO TAB". Below this, there are tabs for "Dosage" (40 MG) and "Complex". To the right, there are fields for "Route" (ORAL) and "Schedule" (DAILY). A "PRN" checkbox is also present. Below these fields is a "Comments" section. Further down, there are fields for "Days Supply" (90), "Qty (TAB)" (100), and "Refills" (0). There are also radio buttons for "Pick Up" options: "Clinic", "Mail" (selected), and "Window". A callout box with an arrow points to the "Mail" option, containing the text: "Specify Day Supply, Qty, refills (0), and pickup (Mail). Quantity will always be 100". To the right of the "Pick Up" options is a "Priority" dropdown menu set to "ROUTINE". At the bottom of the form, there is a summary section showing the drug name, directions "TAKE ONE TABLET BY MOUTH DAILY", and "Quantity: 100 Refills:0". There are "Accept Order" and "Quit" buttons at the bottom right.

If, after submitting an order, you realize that an error was made in the order, contact both your local research pharmacist and the PCC (505-248-3203). The error can be reported by phone or email using the participant ID number and the date the prescription was issued in the EHR.

The PCC must use the participant's shipping address in the VA EHR system as provided by the CMOP. If a participant moves, their address must be updated in both the study website and in the VA EHR. Only participants can update their own address in the VA EHR. To update their own address, the participant must:

- Contact or go to the Admissions and Eligibility Department, or
- Call an Enrollment Coordinator at 1-877-222-8387.

10.2d VERIFYING AN ORDER FOR THE STUDY DRUG (LOCAL RESEARCH PHARMACISTS)

When verifying each initial order or renewal of the study drug, local research pharmacists must:

1. Verify the order is accompanied by a signed informed consent from the participant, as stated in the VHA Handbook 1108.04.
2. Verify the appropriateness of the study drug order generated by authorized local site investigators, ensuring that:
 - a. All prescription orders are written for the correct study participant.
 - b. Prescription orders are consistent with the study protocol requirements, the appropriateness criteria described in **Section 10.3**, and the prescribing information for atorvastatin.
 - c. The participant's address is current and the participant's mail status is set to "Regular Mail" under prescription delivery.
 - d. The directions for use are clearly understandable, consistent with the study protocol, and do not exceed 100 characters and spaces. Directions for use do not include abbreviations or other confusing language. For this study, the directions should be identical or substantively equivalent to: "Take one tablet by mouth once daily."
 - i. Do not add the participant's study identification number or any additional caution statement to the directions.
 - ii. If your site requires an indication for use in the directions, that indication can only reference use for the trial and must not reference using the product for a clinical diagnosis (e.g., Indicating that the study drug is taken "for heart disease" is inaccurate and will result in the order being canceled, while an indication that the study drug is being taken "for the PREVENTABLE trial" is acceptable.).
 - e. The prescription is ordered by a site investigator or site staff locally authorized to prescribe the PREVENTABLE study drug (review current study delegation log).
 - f. The prescription is not refillable (i.e., refill quantity is 0).
 - g. The tablet quantity is 100.
3. Route all verified study drug prescriptions to the CMOP. This will allow the PCC to fill and mail verified study drug orders to study participants.

10.2e PRESCRIPTION CANCELLATIONS (SITE STAFF)

The PCC reserves the right to cancel study prescriptions that may show deficiencies including, but not limited to:

- The quantity of tablets requested is not 100.
- The CMOP and study website entries for the social security number of a participant for whom the study drug prescription is written do not match.
- The number of days in the "Days Supply" field of the prescription is not 90.
- There are abbreviations or misspellings in the directions for use or comment box.
- Directions for use or comments are confusing or inappropriate given the use of a study drug.
- Directions for use are too long to fit on the PCC pharmacy label.

- The study prescription is flagged as being associated with an invalid address.
- The study prescription is tied to an invalid or missing Informed Consent Form.
- The study drug supply stored at the PCC has been recalled by its manufacturer.
- The study prescription is a duplicate.

If a prescription is cancelled, the PCC will provide a standard CMOP numerical code with a brief text description of the reason for cancellation.

- The local research pharmacist will receive a notice from CMOP and will notify the prescriber that the prescription was canceled for the reason specified by the code.
- The study website will indicate that a prescription is still needed for the participant.

NOTE: It is crucial that the reason(s) for a canceled prescription are promptly addressed by the end of each business day by the site staff and the local research pharmacist. Otherwise, the study participant could run out of the study drug.

APPENDIX 10.3 STATIN-ASSOCIATED MUSCLE SYMPTOM CLINICAL INDEX (SAMS-CI)

The Statin-Associated Muscle Symptom Clinical Index (SAMS-CI): Revision for Clinical Use, Content Validation and Inter-rater Reliability. Rosenson RS, et.al. Cardiovascular Drugs and Therapy (2017) 31:179-186.

Cardiovasc Drugs Ther (2017) 31:179–186

Statin-Associated Muscle Symptom Clinical Index (SAMS-CI)

Instructions:

- Use with patients who have had muscle symptoms that were **new or increased** after starting a statin regimen.
- A **statin regimen** includes any statin at any dose or frequency, including a statin the patient has used previously, at the same or a different dose.
- **Muscle symptoms** may include aches, cramps, heaviness, discomfort, weakness, or stiffness.
- Interpret overall score in light of **other possible causes** of the muscle symptoms, such as:

Recent physical exertion	Hypothyroidism	Concurrent illness
Changes in exercise patterns	Drug interaction with statin	Underlying muscle disease
- **See reverse** for Frequently Asked Questions

How many statin regimens has the patient had that involved new or increased muscle symptoms?

One
Complete the questions on the left side of this page.

Two or more
Complete the questions on the right side of this page.

Regarding this statin regimen:

A. Location and pattern of muscle symptoms

(If more than one category applies, record the highest number.)

Symmetric, hip flexors or thighs	3	Enter score:
Symmetric, calves	2	
Symmetric, proximal upper extremity	2	
Asymmetric, intermittent, or not specific to any area	1	

B. Timing of muscle symptom onset in relation to starting statin regimen

<4 weeks	3	
4–12 weeks	2	
>12 weeks	1	

C. Timing of muscle symptom improvement after withdrawal of statin (If patient is still taking statin, stop regimen and monitor symptoms.)

<2 weeks	2	
2–4 weeks	1	
No improvement after 4 weeks	0	

Rechallenge the patient with a statin regimen, (even if same statin compound or regimen as above) then complete final question:

D. Timing of recurrence of similar muscle symptoms in relation to starting second regimen

<4 weeks	3	
4–12 weeks	1	
>12 weeks or similar symptoms did not reoccur	0	

Total:
All four scores above must be entered before totaling

Regarding the statin regimen before the most recent regimen:

A. Location and pattern of muscle symptoms

(If more than one category applies, record the highest number.)

Symmetric, hip flexors or thighs	3	Enter score:
Symmetric, calves	2	
Symmetric, proximal upper extremity	2	
Asymmetric, intermittent, or not specific to any area	1	

B. Timing of muscle symptom onset in relation to starting statin regimen

<4 weeks	3	
4–12 weeks	2	
>12 weeks	1	

C. Timing of muscle symptom improvement after withdrawal of statin

<2 weeks	2	
2–4 weeks	1	
No improvement after 4 weeks	0	

Regarding the most recent statin regimen: (even if same statin compound as above)

D. Timing of recurrence of similar muscle symptoms in relation to starting regimen

<4 weeks	3	
4–12 weeks	1	
>12 weeks or similar symptoms did not reoccur	0	

Total:
All four scores above must be entered before totaling

Total score:		2–6	7–8	9–11
Interpretation	Likelihood that the patient's muscle symptoms are due to statin use:	Unlikely	Possible	Probable

10 Oct 2016. Based on Rosenson et al. An assessment by the Statin Muscle Safety Task Force: 2014 update. J Clin Lipidol. 2014 May-Jun;8(3 Suppl):S58–71.

APPENDIX 10.4 STUDY DRUG HANDOUT

The Study Drug Handout is available in English and Spanish on the study website under Documents > Enrolled Participants Resources > Welcome Packet.

Information About Your Study Drug

Thank you for participating in the PREVENTABLE study. You might be taking a drug called **atorvastatin** (brand name Lipitor®) or you might be taking a placebo. A placebo is a pill that does not have medicine in it but looks like the atorvastatin pill.

Atorvastatin is a type of statin. A statin is a commonly used drug to lower cholesterol. Atorvastatin is well known and has been prescribed to older adults for many years.

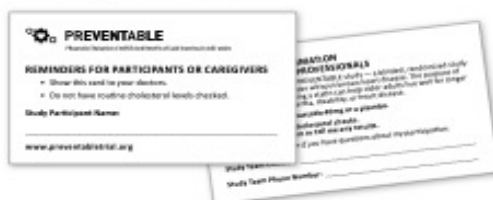
You will receive a shipment of study drug every three months to last you 90 days with a few extra days' supply.

DISPOSING OF UNUSED STUDY DRUG

- If you get a new bottle of study drug and you still have some study drug in your old bottle, stop taking the drug from your old bottle and immediately start taking the drug from your new bottle.
- Properly dispose of any unused study drug. Mix your unused study drug with dirt, cat litter, or used coffee grounds in a plastic bag and dispose of it in your trash.

WHO SHOULD KNOW THAT I MAY BE TAKING ATORVASTATIN?

- Add PREVENTABLE study drug (atorvastatin 40mg or placebo) to your medications list.
- Keep your study identification card with you to show your doctors.



 **PREVENTABLE**
Pillaging Evaluation of eNtIs And Benefits of Lipid-lowering in older adults
JULY 14, 2020

TAKING YOUR STUDY DRUG

- Take your study drug at about the same time each day. 
- Study drug can be taken with or without food.
- If you miss a dose of study drug, take it as soon as you remember. Do not take your study drug if it has been more than 12 hours since you missed your last dose. Wait and take the next dose at your regular time.
- Avoid or limit alcohol intake (wine, beer, and liquor) to less than three (3) drinks per day.
- Don't drink grapefruit juice or eat grapefruit more than once a week.
- Some prescription or over-the-counter drugs may increase your risk of side effects.
- Store your study drug in a cool, dry place. Protect it from light and do not store in the bathroom.
- The appearance of your study drug pill may change (color, size, shape, or markings) because the suppliers of the drug may change.

WHAT ARE THE MOST COMMON SIDE EFFECTS WITH ATORVASTATIN?

Muscle pain or weakness, or joint pain are the most common side effects. Please refer to your study consent form for more information about side effects. Talk to your primary care doctor, study doctor, or study team if you are experiencing any symptoms. It is also important for you to maintain your regular visits with your primary doctor while participating in this study.

www.preventabletrial.org

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CHAPTER 11. RETENTION AND FOLLOW-UP

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

11.1 OVERVIEW

Retention in care (retention) is the **process of ongoing participation in PREVENTABLE**, while study drug status (adherence) relates to the continuation of the study drug. Retention is addressed separately from study drug adherence. A participant may remain in the study if they stop taking the study drug.

Retention is crucial to evaluating the effects of interventions in research studies. PREVENTABLE uses several strategies to maximize retention including:

- Assessing protocol-related barriers to retention prior to enrollment.
- Carefully screening and recruiting participants capable of completing study visits and phone calls.
- Systematically applying specific strategies to address problems that occur during the study (i.e., sharing best practices, creating tools, amending the protocol, etc.).
- Adequately training site staff on study procedures, including strategies for managing different levels of retention (e.g., modified follow-up vs full withdrawal of consent).

Retention reports that summarize the retention status of every participant are available on the PREVENTABLE website under Quick Links > Reports. Real-time reports are available to track retention at the overall study-, hub-, site-, and individual participant-level.

11.1.1 RECRUITMENT AND RETENTION (R&R) CORE TEAM

A Recruitment and Retention (R&R) Core team made up of investigators, study coordinators, Hub representatives, Clinical Coordinating Center (CCC) staff, and Data Coordinating Center (DCC) staff provide their unique perspectives on recruitment and retention. The R&R Core team reviews reports, identifies challenges, and recommends best practices and retention strategies. The team meets once a month virtually to monitor retention performance, discuss effective retention strategies, and provide feedback to sites.

11.1.2 CENTRAL RETENTION SUPPORT

Version 5.0 of the study protocol expanded central recruitment and retention support provided by the PREVENTABLE Clinical Coordinating Center (CCC) through the DCRI Participant Research Operations (PRO) Team, also known as the Central Support Team. Central support is optional for sites to use as requested.

The Central Support Team is available to assist sites with retention efforts for enrolled participants, such as direct-to-participant mailings, phone calls, and other communications, to address study drug adherence and/or retention needs. Initially, these efforts will be limited to centrally originated messages to all participants (e.g., holiday cards, study drug letters, etc.). Individual site requests may be available at a later date.

11.2 PARTICIPANT RETENTION ACTIVITIES

Retention is enhanced when participants feel they are important and valued by the study staff and when they feel they are part of a community. Pleasant, kind, helpful, and attentive staff facilitate bonding and retention in PREVENTABLE. Suggestions to enhance retention include:

- Try to connect with each participant by asking about their family, friends, and upcoming events.
- Develop a calendar and reminder system to send out greeting cards or emails for birthdays and holiday celebrations.
- If a participant passes away, use a blank study card to send a note of sympathy to their family. Blank study cards can also be used for communication with participants for other reasons.
- Introduce participants to all staff with whom they will interact.
- Provide participants with an estimate of how long a visit will take, whether in person or remote, and let them know what to expect at the baseline visit, 3-month lab visit (if applicable), and annual phone assessments.
- Confirm contact information for participants and their trusted contacts during every contact with the participant.
- Encourage participants to visit the PREVENTABLE website (www.preventabletrial.org or preventablecommunity.org) to view videos and read news stories about the study.
- Ask for feedback and encourage participants to call with any concerns or questions throughout their participation in PREVENTABLE. Sites are responsible for mailing retention messages to participants who have not opted in to receive automated emails or text messages (email or text for non-VA sites; email only for VA sites).

11.2.1 WELCOME PACKET

It is important to provide key information to participants when they join the study to welcome them to the PREVENTABLE community. A Welcome Packet is available on the study website under Quick Links > Documents > Enrolled Participants Resources > Welcome Packet. The Welcome Packet includes several documents in English and Spanish: enrolled participant handout, wallet card, study drug flyer, trusted contact handout, and information about the Call Center staff and the cognitive testing administered by them. Most of these documents can be printed on site as needed and are available in English and Spanish on the PREVENTABLE Website under Login > Quick Links > Documents > Enrolled Participant Resources. The wallet card and call center flyers can be ordered on the [PREVENTABLE Storefront](#).

11.2.2 FOLLOW-UP MESSAGES

11.2.2.1 3-MONTH LIPID SUB-STUDY PHONE AND EMAIL REMINDERS

For participants randomized to the 3-month lipid sub-study group, site staff should remind participants by phone and/or (e)mail of their upcoming 3-month lipid study visit approximately one week ahead of the scheduled visit. Because participants are randomly assigned to the lipid sub-study after the baseline call and randomization, site staff will not know if a participant is assigned to the 3-month lipid blood draw at the baseline visit. To check if a participant is randomized to the 3-month lipid sub-study, navigate to the Info tab in the participant's dashboard on the study website. Once they are randomized, either "YES" or "NO" will be displayed to indicate if the participant needs the 3-month blood draw. "UNKNOWN" is the default status for the lipid sub-study until after a participant is randomized.

11.2.2.2 3-MONTH RETENTION EMAILS AND TEXT MESSAGES

The DCC will send automated retention messages about 3 months after randomization to participants who opted into receiving emails and/or texts. For VA participants, the DCC will only send automated emails.

For participants who did not opt into receiving emails and/or texts, sites should send a 3-month greeting card with a retention message. The greeting cards, available in English and Spanish, are available to order on the Storefront.

11.2.2.3 ANNUAL PHONE VISIT REMINDERS

Sites are encouraged to use the PREVENTABLE Participant Yearly Follow-up Letter template to send a letter to participants approaching their annual phone call assessment. This template is available on the study website under Quick Links > Documents > Enrolled Participants Resources.

About 6 weeks before a participant's annual visit window, the DCC will send an automated message to participants who opted into receiving emails and/or text messages. The message reminds participants that the Call Center will call them to schedule their upcoming annual phone assessment. For VA participants, the DCC only sends automated email messages. If a participant did not opt into receiving emails and/or texts, the reminder message will not be sent to them. However, the Call Center will still call them to schedule their annual phone assessment.

11.2.3 NEWSLETTERS

The CCC develops and distributes a semi-annual participant newsletter to provide health information and to reinforce the importance of each participant's contributions to the study. Newsletters include study progress updates, staff profiles, and highlights from study participant who volunteer to share their experience in the study. Newsletters must be IRB-approved before they are distributed to participants. Newsletters are included in the study drug shipment boxes to all participants taking the study drug. The Clinical Coordinating Center (CCC) will centrally distribute newsletters to participants with a current study drug status of "Inactive-Temporary" or "Inactive-Permanent," as these participants do not receive study drug shipments.

11.3 ASSESSMENT WINDOWS AND MISSED VISITS

11.3.1 ASSESSMENT WINDOWS

Each PREVENTABLE assessment should be completed as close as possible to the visit target window. Visit targets and windows are calculated from the date of randomization and are pre-populated in the Visit List on participants' dashboard. Assessment targets and windows for follow-up Katz Activities of Daily Living (ADL) calls to trusted contacts are based on the date of the baseline or annual call.

Visit or Phone Call	Target Date	Window
3-month Lipid Sub-study Lab Draw Visit	90 days post randomization	60-180 days post randomization

Annual Visit	1 year post randomization and annually through the end of the study	+/- 6 weeks
Trusted Contact Phone Call	Within 30 days of the trigger	Opens immediately after the trigger and extends for 180 days
Katz ADL to Confirm a Persistent Disability	90 days after the visit when the participant or trusted contact reported the initial dependence	Second (follow-up) Katz ADL is administered between 90 and 180 days after the participant or trusted contact reported the initial dependence

An example of a participant, who was selected to be part of the 3-month lipid sample sub-study and triggered both the Katz ADL follow-up (based on participant's responses about daily activities) and trusted contact follow-up (based on the participant's TICSm score) is provided below. In this example, the participant was randomized on September 1, 2020, had their 3-month lipid blood draw on November 28, 2020, and the first annual phone call on September 3, 2021.

	Actual Date of Visit	Window Opens	Target Date	Window Closes
Randomization	9/1/2020			
3-month Lipid Sub-study Lab Draw Visit (if applicable)	11/28/2020	10/31/2020	11/30/2020	2/28/2021
1st Annual Visit (Phone Call)	9/3/2021	7/20/2021	9/1/2021	10/13/2021
Trusted Contact Phone Call (if needed)		9/3/2021	10/3/2021	3/2/2022
3-Month Katz ADL Assessment (if needed)		12/2/2021	12/2/2021	3/2/2022
2nd Annual Visit (Phone Call)		7/20/2022	9/1/2022	10/13/2022

11.3.2 MISSED VISITS

If a participant does not complete a visit and the visit window has passed, document the visit as missed in the participant's dashboard on the study website. To document a visit as missed, on the Participant Status page, click on the Visit List tab, select the appropriate visit, select "Reason Not Collected," and choose the reason the visit was missed from the drop-down menu. If the visit/call takes place outside the target visit window, site staff should still collect the visit data and enter it in the participant's dashboard on the study website as soon as possible.

11.4 3-MONTH LAB VISIT

Participants who are randomly selected for the lipid sub-study group will have a 3-month follow-up lab visit. These participants should be notified they were selected for the 3-month follow-up and they are expected to return to the site or central lab for the lab visit. If a participant is selected for a 3-month lab visit but a baseline blood sample was not collected, they should not return for a 3-month lab visit.

Participants are not required to fast for the 3-month lab visit. However, fasting status at the time of collection should be recorded in the study website, as this status is of interest in some analyses of metabolic markers.

A reminder call or appointment card may be helpful to ensure the participant remembers to return for their 3-month lab visit. Refer to **Section 11.3.1** for information on the visit window for the 3-month lab collection. See **Appendix 11.1f** for information about alerts and the Blood Collection Form for the 3-month lipid sample visit.

NOTE: It is important to remind all participants that they should not have clinical lipid panels checked during their participant in the study to prevent unmasking the participant and their clinicians. All other routine clinical laboratory testing is permitted.

11.5 PARTICIPANT FOLLOW UP: SITE RESPONSIBILITIES

Site contact with participants, in person and via phone or mail, promotes retention and is strongly encouraged to facilitate the feeling of community among participants. After the initial baseline visit, follow up of participants is done by both the central Call Center and the local site. See **Appendix 11.1a** for details about using the central comments feature on the study website that the Call Center and sites often use to document and share information about a participant.

Annual phone visits are completed by the Call Center. However, if the Call Center makes 3 or more unsuccessful attempts to reach the participant and/or their trusted contacts, the visit status will automatically update to “Back to Clinic.” The site will receive a “Back to Clinic” alert and site staff will need to assist the Call Center with reaching the participant. See **Appendix 11.1c** for details on the “Back to Clinic” alert for annual visits and how to resolve it.

11.5.1 PRESCRIPTION RENEWALS

Sites will receive an alert on the study website when a participant’s prescription needs to be reviewed. Annually (for non-VA sites) or every 3 months (for VA sites), the site principal investigator (PI)/ medication prescriber is required to review each participant’s eligibility to continue taking the study drug. If it is appropriate for the participant to continue the study drug, the PI/medication prescriber must enter a prescription renewal in the study website for non-VA participants or in the Computerized Patient Record System (CPRS) for VA participants.

See **Chapter 10: Study Drug** for more information about prescription renewals.

NOTE: If a participant develops dementia during the study as indicated by Call Center testing, an alert on the study website will notify site staff of probable dementia adjudication from the Coordinating Center. This may be months after the site becomes aware of a participant’s memory impairment.

The site PI should use their clinical judgement regarding safe continuation of the study drug in participants with dementia. The continuation of the study drug is a priority unless the site investigator and clinician determine it to be inappropriate. Primary cause of death and secondary outcomes may still be influenced by continuation of the study drug. Participants adjudicated with a probable dementia classification should continue in the study, if possible, so they can continue to be followed for cardiovascular disease (CVD) endpoints and mortality through the end of study follow up. See **Appendix 11.1j** and **Chapter 16** for next steps after a participant receives a “Cognitive Outcome” adjudication classification.

11.5.2 STUDY DRUG AND RETENTION STATUSES

Maintaining up-to-date study drug and retention statuses for participants is critical to ensure Call Center follow-up is only completed on participants who are still retained in the study and that the study drug is only shipped to participant’s who are adherent to taking the study drug.

If a participant wants to change their retention status, it is the site’s responsibility to negotiate the level of retention with the participant. Site staff should update the retention status and/or study drug status as soon as possible after they become aware of a change. See **Appendix 10.1** and **Appendix 11.1d** and **11.1e** for instructions to update the study drug and retention statuses on the study website.

11.5.2.1 STUDY DRUG STATUS

During annual follow-up phone visits, the Call Center will ask the participant if they are regularly taking their study drug. If during a follow-up phone call a participant reports taking their study drug for less than 4 days in a typical week, the site will receive a “Participant has been Non-Adherent” alert on the study website. See **Appendix 11.1d** for instructions to resolve the alert.

Site staff are also responsible for updating the study drug status if they learn a participant has stopped taking their study drug. There may be times when site staff become aware that a participant now has a medication that interacts with the study drug and has stopped taking it or the participant may ask to be withdrawn from the study. It is important to clarify if the participant simply wants to stop taking the study drug or if they want to be entirely removed from the study. Site staff should encourage continued participation even if the study drug is temporarily or permanently stopped—every participant’s data are extremely valuable.

If a participant enters a nursing home, they may be able to continue taking study drug. The decision to continue to the study drug is up to the discretion of the site PI, in conjunction with the participant and their provider. The nursing home’s policies may impact the participant’s ability to continue the study drug. If the nursing home prohibits administration of the study drug, the participant’s study drug status can be updated in the study website to either temporarily or permanently inactive. Communication with a participant’s providers in these facilities is encouraged. Templates for site staff to use when communicating with a participant’s providers are available for modification on the study website under Quick Links > Documents > Recruitment Resources > Clinician Engagement Materials > Clinician Tools.

If a participant moves from an independent living environment to a nursing home or other facility where the study drug can no longer be taken independently or administered, the PI/study clinician should discontinue the study drug. The site coordinator should change the study drug status to “Inactive - Permanent,” review the retention status, and update the participant’s contact information form, especially the new phone number. The Call Center will continue to call the participant or the trusted contact annually or as needed.

11.5.2.2 RETENTION STATUS

If a participant has concerns about staying in the study, site staff should ask the participant questions to identify the specific reason(s) for the participant's concerns. Site staff should listen to the participant's responses and encourage them to stay in the study if possible. Every effort should be made to ensure that the participant's questions and concerns are addressed by the study team, including the PI and/or their designee. Discussions the study team has with the participant, as well as any materials provided to the participant to address their concerns, should be documented in the comments tab of the participant's record on the study website.

If a participant wants to withdraw, ask them whether a modified follow-up plan is acceptable. A participant may not want to have annual calls but may still allow a final phone call or being followed through review of their EHR or annual phone calls to trusted contacts. More information about modified follow up is available on the study website under Quick Links > Documents > Retention and Adherence Resources > Guide to Participant-Led Retention.

The following retention status options should be discussed with each participant to minimize withdrawals from the study:

- Active, complete all procedures
- Modified Follow-up (check all that apply)
 - Do not call for annual phone visits
 - Do not call for final phone visit
 - Do not call trusted contacts (only if specified by participant)
 - Do not review electronic health records (only if specified by participant)
- Full Withdrawal Requested
 - Withdrawn by Participant
 - Withdrawn by PI

If the participant agrees to a modified follow-up plan and indicates they do not want to receive annual phone visits, the Call Center will reach out to the participant's trusted contact during the annual call visit window to complete the Dementia Questionnaire. If the participant does not want the Call Center to talk to the trusted contact, select "Do not call trusted contacts" from the options under Modified Follow-Up and add a note in the Comments tab.

If needed, the participant's retention status can be updated on the Participant Status page of the study website. Any time a retention status is updated, the date of the status change and the effective date must be entered. Site staff updating a participant's retention status are also required to select which site PI approved the change. See **Appendix 11.1e** for instructions to update a participant's retention status.

11.6 WITHDRAWAL FROM THE STUDY

Participant withdrawals after randomization can compromise the scientific integrity of the study as an excessive withdrawal rate can make it difficult to interpret the study results; therefore, unnecessary withdrawal of participants should be avoided. To prevent withdrawals, it is important that, if a potential participant expresses any concerns about being in the study or being assigned to a specific treatment arm, the study coordinator or PI have a discussion with the participant prior to enrollment or randomization about participants' responsibility to follow the study protocol.

If a participant stops taking the study drug, this impacts only their study drug status, not their retention status. If a participant no longer wants to speak with the Call Center annually but they are willing to allow the study to continue collecting their health information via EHR and claims data and/or allow the Call Center to speak with their trusted contact annually, this is considered a modified follow up.

NOTE: A participant who is non-adherent to the study drug is not a withdrawal. Withdrawal status is focused on retention and whether the participant agrees to additional data collection.

If a participant refuses to adhere to their assigned treatment arm or answer Call Center calls, that does not necessarily mean they are withdrawing from the study. There is flexibility in how follow-up information can be collected and most participants will still allow some type of follow up. Participants who agree to modified follow up remain in the study under the appropriate retention status.

If a participant fully withdraws their consent (no further contact and no provision to continue to collect health information electronically), the retention status of “Full Withdrawal Requested” must be entered into the Participant Status page of the study website. Non-VA sites should follow their local policies for withdrawal of consent as documented in their Informed Consent Form.

See **Appendix 11.1e** for instructions to update a participant’s retention status or request a full withdrawal.

11.7 WITHDRAWAL OF HIPAA AUTHORIZATION (VA SITES)

A participant may revoke their HIPAA authorization to use their protected health information (PHI) at any time. A verbal request alone from the participant to the study staff does not revoke a HIPAA authorization; the revocation must be in writing. If a participant requests full withdrawal from the study, the site PI should provide a revocation form to the participant or request the participant’s revocation in writing. Any form of written notification from the participant is acceptable to document revocation. VA sites may use VAF 10-10116: Revocation of Authorization for Use & Release of Individually Identifiable Health Information for Veterans Health Administration (VHA) Research. The revocation is effective as soon as it is received in writing by the site PI.

A revocation of the HIPAA authorization is not the same as withdrawing from the study. As permitted by HIPAA, VA sites may continue to use and disclose protected health information (PHI) that was obtained before the individual revoked their authorization to the extent necessary to preserve the integrity of the study.

11.8 WITHDRAWAL OF CONSENT FOR LONG-TERM STORAGE OF SPECIMENS

Participants who gave consent to provide their blood and DNA samples for long-term storage have the right to withdraw their specimens at any time for any reason. If a participant wants to withdraw their consent to use their samples for genetic testing, site staff must update the response on the participant’s layered informed consent on the study website. If a participant withdraws consent for the storage of their specimens, the DCC will notify the biorepository every time samples are pulled for additional analysis or ancillary studies and the samples for these participants will not be used. The samples will be destroyed at the end of the study.

11.9 PARTICIPANT DEATH

Throughout the study, the site, Call Center, or DCC staff may learn a participant has passed away. If the Call Center or the DCC is the first to learn about a participant's death, the site will receive an alert on the study website home page. If the Call Center is notified of a death outside of the annual follow-up visit window, the Call Center will inform the site via an Unexpected Participant Communication alert (see **Appendix 11.1h**).

See **Appendix 11.1g** for instructions to document a participant's death on the study website and resolve a "Call Status is Participant Deceased" alert.

NOTE: A participant who passes away is not considered withdrawn from the study and the retention status should be left as it was at the time of their death. After the Death Form is completed by the site PI or their designee, site staff do not need to update the study drug status since it will be automatically changed by the system to "Inactive – Permanent" and no future study drug shipments will be sent.

The site PI or their designee is responsible for reviewing the medical records, contacting the participant's clinician or trusted contact, or using other methods to complete the Death Form.

All deaths are classified as cardiovascular, non-cardiovascular, or undetermined. In addition, all deaths are further sub-classified based on the specific cardiovascular or non-cardiovascular categories defined below. The cause of death is determined by the principal condition that resulted in the death, not the immediate mode of death. For example, if a participant hospitalized with a hip fracture dies of a pulmonary embolism (PE) due to immobility, it would be classified as trauma death. Site PIs will use available information, along with clinical expertise, in their assessment of the cause of death.

To determine the cause of death, the site PI or their designee may use the National Death Index, primary diagnosis codes for the hospitalization during which the participant died, and the Death page on the study website. The intent of the Death page is to provide limited but sufficient information to determine if the death was from a cardiovascular or non-cardiovascular cause, underlying, contributing, or proximate cause. The site PI or their designee can add their best assessment of the cause of death and provide a narrative to describe the situation. In addition, for participants who die at home, provide information about the circumstances (e.g., witnessed, situation of discovery, symptoms prior to death, etc.).

11.9.1 CARDIOVASCULAR DEATH

Cardiovascular deaths include:

- a) Sudden Cardiac Death – death that occurs unexpectedly and not following an acute myocardial infarction (MI), and includes the following deaths:
 - i. Death witnessed and occurring without new or worsening symptoms
 - ii. Death witnessed within 60 minutes of the onset of new or worsening cardiac symptoms, unless the symptoms suggest acute myocardial infarction (MI)
 - iii. Death witnessed and attributed to an identified arrhythmia (e.g., captured on an electrocardiographic [ECG] recording, witnessed on a monitor, or unwitnessed but found on implantable cardioverter defibrillator [ICD] review)

- iv. Death after unsuccessful resuscitation from cardiac arrest (e.g., ICD unresponsive sudden cardiac death, pulseless electrical activity arrest)
- v. Death after successful resuscitation from cardiac arrest and without identification of a specific cardiac or non-cardiac etiology
- vi. Unwitnessed death in a participant seen alive and clinically stable ≤ 24 hours prior to being found dead without any evidence supporting a specific non-cardiovascular cause of death

NOTE: If a participant was known to be alive ≤ 24 hours of being found dead or circumstances suggest sudden death, sudden cardiac death should be recorded unless additional information suggests an alternative specific cause of death.

Typical scenarios include:

- A participant was well the previous day but found dead in bed the next day.
- A participant was found dead at home on the couch with the TV on.

NOTE: For participants who were not observed alive within 24 hours of death, “Undetermined” cause of death should be recorded (see below).

- b) Fatal Acute Myocardial Infarction – death by any CV mechanism (e.g., arrhythmia, sudden death, heart failure, stroke, pulmonary embolus, peripheral arterial disease) ≤ 30 days after a MI related to the immediate consequences of the MI, such as progressive heart failure or recalcitrant arrhythmia. There may be assessable mechanisms of CV death during this time period, but for simplicity, if the CV death occurs ≤ 30 days of the MI, it will be considered a death due to MI.

If death occurs before biochemical confirmation of myocardial necrosis can be obtained, adjudication should be based on the clinical presentation and ECG evidence. Sudden cardiac death, if accompanied by symptoms suggestive of myocardial ischemia, new clinically significant ischemic ECG changes, or evidence of new thrombus by coronary angiography and/or at autopsy should be considered death resulting from an acute myocardial infarction (MI), even if death occurs before blood samples or 12-lead electrocardiogram (ECG) could be obtained, or at a time before the appearance of cardiac biomarkers in the blood. Death resulting from a procedure to treat an MI (e.g., PCI/CABG) or to treat a complication resulting from an MI, should also be considered death due to acute MI.

- c) Fatal Heart Failure – death associated with clinically worsening symptoms and/or signs of heart failure (HF), regardless of HF etiology. Deaths due to HF can have various etiologies, including single or recurrent myocardial infarctions, ischemic or non-ischemic cardiomyopathy, hypertension, or valvular disease. Deaths that occur during a hospitalization for HF will generally be attributed to heart failure, even if there is another immediate mode of death (e.g., ventricular fibrillation). Deaths that occur in hospice or other similar palliative care settings for HF will generally be attributed to HF.

- d) Fatal Stroke – death after a stroke that is either a direct consequence of the stroke or a complication of the stroke. Acute stroke should be verified to the extent possible. Deaths that occur within 30 days of a stroke without another primary cause will be considered death due to stroke.
- e) Other Cardiovascular Causes – death due to a CV death not included in the above categories, but with a specific, known cause (e.g. pulmonary embolism or peripheral arterial disease). This includes death caused by the immediate complication(s) of a CV procedure (excluding a procedure to treat an MI). This includes death related to hemorrhage, such as a non-stroke intracranial hemorrhage (e.g., subdural hematoma), non-procedural or non-traumatic vascular rupture (e.g., aortic aneurysm), or hemorrhage causing cardiac tamponade.

11.9.2 NON-CARDIOVASCULAR DEATH

Non-cardiovascular deaths include:

- a) Malignancy – death attributable to leukemia, lymphoma, or other malignancy
- b) Infection (including sepsis) – death attributable to an infectious disease
- c) COVID-19 – death attributable to COVID-19 infection (presumed or confirmed) or sequelae
- d) Pulmonary – death attributable to disease of the lungs, excluding malignancy
- e) Renal failure – death attributable to renal failure (e.g., participants on dialysis where a decision has been made to not continue dialysis)
- f) Bleeding (including intracranial) – death attributable to bleeding that is not considered CV hemorrhage or stroke
- g) Gastrointestinal (including hepatobiliary and pancreatic) – death attributable to disease of the liver, pancreas, gallbladder, esophagus, stomach, or intestines, excluding malignancy
- h) Trauma – death attributable to trauma, including falls leading to death or homicide
- i) Suicide – death attributable to suicide
- j) Dementia/Neurological – death attributable to progressive dementia or other disease of the nervous system, excluding malignancy
- k) Other non-CV – death attributable to a cause other than those listed above

11.9.3 UNDETERMINED DEATH

Undetermined causes of death are instances in which no information is available to determine cause of death and thus cause of death cannot be attributed to one of the above categories of CV or non-CV death. For participants who were not observed alive within 24 hours before being found dead and with no additional information about a specific cause of death, the cause of death will be recorded as undetermined (e.g., the participant was found dead in bed but was not observed alive for over 24 hours prior to death).

NOTE: If the participant was seen alive $\leq$ 24 hours before being found dead, record sudden cardiac death in the Death Form.

11.10 TRANSFERRING A PARTICIPANT BETWEEN STUDY SITES

During the study, participants may leave the area where they enrolled and relocate to another area where there is a study site. To support participant retention and ensure continued follow up through phone calls and EHR reviews, participants who relocate should be encouraged to continue their

participation at their new location or through the central tele-site. Site staff at both the original site (the “transferring” site) and the new site (the “accepting” site) will need to complete the steps below to transfer a participant.

NOTE: Site staff are discouraged from transferring participants who are temporarily visiting vacation homes during the study.

Site staff at the original (transferring) site must first complete these steps:

1. Determine which study sites are closest to the participant’s new location or if the participant should be transferred to the tele-site.
2. Contact the PIs and study coordinators of the applicable sites to ask if they would like to accept the transfer participant.
3. After feedback from the applicable sites, provide the choices of new possible sites to the participant, including the tele-site, and ask them to choose the site to which they would like to transfer.
 - a. The participant and the PI of the transferring site must agree on the transfer.
4. Notify the CCC and DCC of the need for a transfer by submitting a RAP (Report a Problem). The Submit a RAP form is available on the study website under Quick Links.
5. Confirm the site to which the participant will be transferred and the planned date of transfer.
6. Confirm the participant's transfer details with the new site, including the planned date of transfer.
7. Complete all relevant study forms and data edits.
8. Notify the DCC by submitting a RAP that the data entry for the participant is complete and all outstanding data edits, including alerts and queries, have been addressed.
 - a. The DCC then verifies that the data edits are complete in the database at the DCC.
9. Provide the new site with the participant’s name, participant ID number (PID), new contact information, and other pertinent medical information.
10. With the participant’s approval, arrange a tentative in-person or telehealth appointment with the new site.
11. Give the new site’s contact information to the participant.
12. Send paper copies of the participant’s records to the new site via a method that allows tracking.
 - a. The participant’s records will include all PREVENTABLE forms for all study visits the participant has completed to date, a referral letter summarizing the participant’s relevant medical and study history, and a copy of the signed Informed Consent Form.
13. The transferring site retains responsibility for participant follow up until the participant is consented at the new site and the transfer process is completed by the DCC.

Then, site staff at the new (accepting) site will:

1. Confirm with the transferring site that they received paper copies of the participant’s records.
2. Contact the participant to confirm the tentatively scheduled in-person or telehealth appointment to obtain consent.
3. Upon seeing the participant for an in-person or telehealth visit, the new site must re-consent the participant before initiating any PREVENTABLE activities. The new site must re-consent the participant under the existing PID that the new site received from the transferring site.
4. The new site provides the DCC with the re-consent date by submitting a RAP. **This is very important; the new site will not have data entry access for the participant until the site notifies the DCC of the re-consent date.**

5. The DCC will transfer the participant to the new site. After the transfer is complete:
 - a. The transferring site will no longer be able to view the participant's data on the study website.
 - b. The participant will not appear on the transferring site's reports.
 - c. The new site will have data entry access for the transferred participant. Once data entry access has been granted for the accepting site, staff should review the layered consent questions entered at randomization and edit if needed, based on the newly signed consent. ~~as indicated.~~ The original date of consent should stay the same on the participant's consent page in the study website.
 - d. The randomizing site will maintain credit for the randomization.
6. Once the participant is re-consented at the new site and the DCC completes the transfer, the new site becomes responsible for the participant and all related data. The new site will receive site payments for future completed prescription renewals.

If the participant does not consent to be seen at the new site, the transfer will be canceled and the original site will retain responsibility for following the participant. In this event, any paper copies of PREVENTABLE forms sent to the new site will need to be destroyed or returned to the original site.

APPENDIX 11.1 RETENTION AND FOLLOW-UP FEATURES OF THE STUDY WEBSITE

There are multiple features within the PREVENTABLE website that help ensure accurate data collection and study follow up. These features include central comments, alerts, study drug and retention statuses, the 3-month lipid sample sub-study list, and documentation of participant death.

11.1a CENTRAL COMMENTS SYSTEM

The central comments system – the Comments tab on the participant dashboard – allows both sites and the Call Center to document and view information about a participant. Comments default to the most recent at the top but each column can be sorted in ascending or descending order.

Information
Consent
SSN & MBI
Visit List
Enrollment
Comments

Search:

Date	Category	Comment
3/4/21	Call Center - Trusted Contact Attempt	Trusted Contact phone number was busy both times I tried to call
3/4/21	Call Center - Participant Attempt	Participant was in the middle of running errands and asked to be rescheduled to do their call another day.
3/4/21	Clinic - General	The participant's husband just passed away and she isn't sure if she wants to continue with the study. She will call the clinic back in a couple of weeks and let us know if she wants to change her retention level.
1/25/21	Call Center - Participant Attempt	

To add a new comment, use the Enter New Comment box at the bottom of the tab and click “Save.”

NOTE: Once a comment is saved, it cannot be edited or deleted by the user. If a comment was entered in error, submit a RAP to have the DCC manually remove it.

Comments

8/26/20	Participant Attempt	testing CC00 - SCB
8/26/20	Call Center - Trusted Contact Attempt	testing TC attempt 1 - SCB

Enter New Comment:

Save

11.1b OVERVIEW OF ALERTS FOR PARTICIPANT ISSUES

There are multiple post-randomization alerts in the “Participant Issues” category. These alerts are located in 2 places on the study website.

From the study website home page, click the My Site Alerts tab to see the Participant Issues alerts column.

Recruitment Summary	Fall / Winter 2023 Tiers	Announcements	My Site Alerts	Favorites
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SITE	PARTICIPANT ISSUES	ICON LAB	DRUG ORDER SYSTEM	DATA QUERIES
1101 - Northwestern	0	0	0	0
1102 - Rush	6	0	2	0

Or, from the menu at the top of the study website, click on Data > Data Entry to view alerts on the dashboard of the specific site’s summary page.

DATA ENTRY	PID Lookup	Site List	SITE: 1999 - Non-VA Test	Site Summary	Initialize Participant(s)
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ALERTS	FILTERS	PARTICIPANT LIST
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PARTICIPANT ISSUES
2 CAC PREVENTABLE Safety Scan Result - Negative (Normal)
2 CL12 Visit Past Due
1 Call status is Back to Clinic - 12 Month
1 CAC PREVENTABLE Safety Scan Result - Positive (Abnormal)
1 CL24 Visit Past Due

ICON LAB	0
DRUG ORDER SYSTEM	3
DATA QUERIES	178

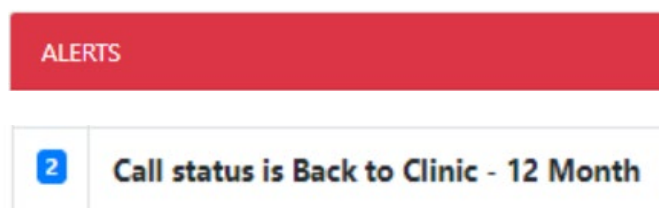
11.1c ANNUAL CALL ALERTS

Alerts designated with 12 Month, 24 Month, 36 Month, etc. are related to the Call Center’s annual cognitive call with participants. Some of these alerts are similar to the baseline call alerts described in **Chapter 9**, however the actions needed to resolve the annual call alerts vary slightly from the baseline call alerts. Alerts related to the annual cognitive call are listed below:

Back to Clinic Alert

Each year, the Call Center will contact the participant to schedule their annual call during their designated annual visit window. If the Call Center is unable to reach the participant after 3 or more attempts, they may make several attempts to reach a trusted contact to confirm the participant’s contact information. If the Call Center is unsuccessful at reaching the participant and the trusted contacts after multiple attempts over the course of at least a week, the Call Center will send an alert “Call status is Back to Clinic – # Month” to the site. Site staff can view and respond to the alert on the Site Summary page under the Alerts header.

Site staff will then try to contact the participant to remind them of the annual call, schedule the annual call (if the participant is willing), or get updated contact information. The participant may be screening calls from the Call Center, but they may answer a call from the site because they recognize the number. Site staff should encourage the participant to add the Call Center number to the contacts in their phone.



To resolve the “Call status is Back to Clinic – # Month,” follow the Action Needed instructions in the blue box at the top of the Alerts page.

Call status is Back to Clinic - 12 Month

ACTION NEEDED:
The PREVENTABLE Call Center has been unsuccessful in completing the participant's 12 Month call. Refer to the Comments tab on the participant's dashboard for any Call Center comments, then reach out to the participant and confirm if they are either refusing to participate in the study or to reschedule their call center appointment.
Update the participant's status by clicking on their name in the list below, then submitting a change from either the blue Respond or Reschedule option in the Call Center Visits section.

PID	Name	Date of Enrollment	Date Randomized	Date of Current Status	Comments
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Max Calls Made Alert

A “**Max Calls Made - # Month**” alert occurs after the annual call has been sent “Back to Clinic” once, resolved by the clinic, and the Call Center has tried at least 3 additional times (at least 6 attempts total) to reach the participant but has been unsuccessful.

Call status is Max Calls Made - 24 Month

ACTION NEEDED:

The PREVENTABLE Call Center has attempted at least six distinct times to call the participant(s) identified below and have been unsuccessful. To resolve the alert, please update the participant's status by submitting a change under either the Respond or Reschedule option in the Call Center Visits section under the participant's Visit List tab.

PID	Name	Date of Enrollment	Date Randomized	Date of Current Status	Comments
		03/30/2021	04/07/2021	03/23/2023	1 - Comments

NOTE: If a call is sent “back to clinic” for any reason and the site staff does not resolve the alert so the Call Center can complete the annual call before the visit window closes, then the call will be **automatically** marked as a “Missed Visit.” Then, the Call center will attempt to reach to the participant’s trusted contact to complete the Dementia Questionnaire (DQ) in place of the participant’s annual call.

If a visit with an alert moves into the “Missed Visit” status and the Respond or Reschedule buttons are no longer available, submit a “Report a Problem” (RAP) on the study website to resolve the alert.

Visit	Call Status	Date		Action
CC00 - Baseline Call Center	Completed	04/06/2021		N/A
TC00 - Baseline Call Center Trusted Contact Battery				
CC12 - Annual Call Center Follow-up	Unable to reach participant or unable to update contact info	03/28/2022		Missed Visit

Occasionally, an annual call will have a “Back to Clinic” alert because the Call Center reached the participant and they **refused** to complete the cognitive call. The participant may want to skip the call this year and do it next year, or they may mistakenly believe they are withdrawn from the study because they stopped taking the study drug.

It is **very important** that site staff review the comments from the Call Center and then contact the participant to encourage them to complete the cognitive call that year, if at all possible. If a participant refuses to complete the call, the Call Center may attempt to reach the participant’s trusted contact to complete the DQ instead, as long as the participant’s retention status allows for the study to speak to the participant’s trusted contact(s).

Resolving Back to Clinic and Max Calls Made Alerts

Back to Clinic and Max Calls Made alerts are both resolved the same way.

1. Click on the name of the alert on the Alerts tab of the Site Summary page.
2. From the list of participants with the alert, click on the participant to navigate to their Visit List tab.
3. Click on the black comment bubble icon to review the comments from the Call Center. A pop-up window will display the order of actions and comments on that visit in order of newest to oldest.

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Visit Comments

Call Status	Date		Action
Completed	11/17/2020		N/A
Back to Clinic	10/14/2021		RESPOND / RESCHEDULE CALL



Action Taken	Status	Source	Comment
2024-01-05 15:20:32.107	Completed	CALLCENTER	PPT had no issues completing CC24 call. 1/5/24-TC
2023-12-15 09:02:21.433	Rescheduled	CALLCENTER	PPT needed to resched due to Holidays. 12/15/23-TC
2023-12-12 10:22:06.29	Scheduled	CLINIC	Call scheduled for: 12/15/2023 09:00 AM ET 12/12 TALKED WITH PARTICIPANT THIS FRIDAY WILL WORK FOR CALL DK
2023-12-11 19:26:15.977	Updating Info - Annual Call	CALLCENTER	LM for TC1 for CC24 - DJ 12/11/23
2023-12-06 15:29:22.747	Updating Info - Annual Call	CALLCENTER	LM on Pri, no Alt trying to schedule CC24 12/06 - HL. Try TC1 next.
2023-12-01 16:09:26.233	Left Message	CALLCENTER	LM for ppt, trying to sch. CC24 12/01/23 BT
2023-11-28 17:31:50.537	Left Message	CALLCENTER	LM for ppt, trying to sch. CC24 11/28/23 BT

- After reviewing the comments, contact the participant to determine the best course of action.
- Submit an answer using **either** the “Respond” or “Reschedule Call” blue button (do not use both).
- Once a respond or reschedule call option is submitted, the Back to Clinic alert will resolve at the top of the following hour.

Visit	Call Status	Date		Action
CC00 - Baseline Call Center	Completed	11/17/2020		N/A
TC00 - Baseline Call Center Trusted Contact Battery				
CC12 - Annual Call Center Follow-up	Back to Clinic	10/14/2021		RESPOND / RESCHEDULE CALL
TC12 - 12 Month Call Center Trusted Contact Battery				
KZ12 - 12 Month Call Center Katz Follow-up				



If the call is rescheduled, only the “Reschedule Call” button should be used. The “Respond” option is used only to communicate with the Call Center **without rescheduling the call**.

NOTE: Any option submitted with the “Response” button will automatically cancel any active, scheduled appointment.

Details about the response and reschedule call options are provided below.

RESPOND TO THE PREVENTABLE CALL CENTER ABOUT PARTICIPANT

Any option submitted below will automatically cancel any active scheduled appointments. Before submitting any response to the Call Center prior to the participant's scheduled call, please use the "Reschedule" button.

Response to Call Center

Contact Info Updated

Declined Annual Call

Updated or Confirmed Participant Retention Status

Unable to reach participant or unable to update contact info

unable to be updated

SEND

“Contact Info Updated” should be selected if the site staff reviewed and updated the contact information on the Contact Information Form. Please note in the comments section of the response the information that was updated. If this is selected, the Call Center will reattempt to contact the participant for this annual visit.

“Declined Annual Call” should be selected if the participant does not want to complete the annual call for this year but agrees to be contacted for next year’s annual call. If this is selected, the Call Center will not reattempt to contact the participant for this annual visit.

“Updated or Confirmed Participant Retention Status” should be selected if the site staff contacted the participant and the participant either fully withdraws from the study or agrees to a modified follow up. If this is selected, the Call Center will not reattempt to contact the participant.

“Unable to reach participant or unable to update contact info” should be selected if the participant’s contact information appears to be correct, but the site is unable to contact the participant. If this is selected, the Call Center will not reattempt to contact the participant for this annual visit.

It is very important to include notes about the selected response in the comment box. After providing details about the response in the comments, click “Send” to save the response.

Site staff should use the blue “Reschedule” button to reschedule the call if they are able to confirm with the participant a new date and time they are available to complete the call. If the site and participant choose to reschedule the call but the participant has an alert for “Max Calls Made,” the Call Center will attempt only **one more time** to call the participant at the scheduled time. If the call is unsuccessful, the Call Center will not attempt to contact the participant again.

Contact Info Incorrect/Missing Alerts

If an automatic email and study website alert notifies the site and/or the site notes that the participant's current call status is "Contact Info Incorrect/Missing," site staff should review the participant's EHR and/or attempt to contact the participant to update the contact information.

4

Call status is Contact Information Incorrect

Select either "Reschedule Call" if the participant confirms a date and time they are available for the annual call or select "Respond" if the call cannot be rescheduled. If the call cannot be rescheduled, choose the appropriate drop-down option and include notes about the selected response in the comment box.

CC12 - Annual Call Center Follow-up	Contact Info Incorrect/Missing	02/14/2022		RESPOND / RESCHEDULE CALL
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Review Trusted Contacts Alert

The "Review Trusted Contacts" alert could appear for any of these types of calls to trusted contacts:

- Trusted Contact (TC): a participant triggers the Extended Cognitive Assessment Battery (signaling possible cognitive impairment) and the Call Center needs additional information from the trusted contact.
- Dementia Questionnaire (DQ): a participant was unable to complete the annual call and a DQ call is attempted in place of the participant call.
- DETHCC: a participant passed away and the Call Center needs to collect final data (in the form of a DQ) from a trusted contact.

If the Call Center is unable to reach the trusted contact(s) after 3 or more attempts, the site will receive an automatic email and an alert on the study website notifying them that "Call status is Review Trusted Contacts."

3

Call status is Review Trusted Contacts

Site staff will need to review the notes from the Call Center by clicking on the comment bubbles icon and take appropriate action. Site staff may need to contact the participant to confirm the contact information is correct for the trusted contact(s) and ask the participant to tell their trusted contact to expect a call from the PREVENTABLE Call Center. Alternatively, site staff may ask the participant to provide a different trusted contact's name and phone number. Once the site staff have attempted to update the trusted contact information, the alert can be resolved by navigating to the participant's Visit List and selecting the blue "Respond" button next to the corresponding trusted contact call.

CALL CENTER VISITS

Visit	Call Status	Date		Action
CC00 - Baseline Call Center	Completed	01/29/2021		N/A
TC00 - Baseline Call Center Trusted Contact Battery	Review Trusted Contacts	02/09/2021	 	RESPOND

Then select either “Trusted Contact Info Updated” or “Unable to Update Trusted Contact Info” from the drop-down menu. In the comments box, make a note about the contact information was updated, if any. Once a response to the Call Center is submitted, the alert will resolve at the top of the hour.

If site staff select “Trusted Contact Info Updated,” the Call Center will try 3 more times to reach the trusted contacts to complete the phone assessment.

11.1d STUDY DRUG STATUS ALERTS AND UPDATES

If site staff become aware that a participant now has a medication that interacts with the study drug and has stopped taking it or the participant tells site staff or the Call Center that they are no longer taking the study drug, it is the site staff’s responsibility to update the participant’s study drug status.

Study Drug Status Has Been Temporarily Inactive For More Than # Days

The study drug statuses "Inactive-Temporary" and "Active-Modified" are meant to be used for a short period of time. The alert “Study Drug Status Has Been Temporarily Inactive For More Than # Days” indicates a participant’s study drug status has been either “Inactive-Temporary” or “Active-Modified” for more than a specific number of days. The alert will occur at 90, 180, and 365 days, and 2 and 3 years if the study drug status has not changed from “Inactive-Temporary” or “Active-Modified” to another status for that amount of time.

If this alert appears for a participant, talk to the site PI and work with the participant to either restart the study drug or move up to the “Active-Full” dose. If the participant agrees, change the study drug status on the participant's Information tab to either "Active-Full" or "Active-Modified," as appropriate. If the participant refuses to restart the study drug or move up to an "Active-Full" dose, determine with the participant and PI if the status should remain as is for a short period of time or if it is appropriate to change the study drug status to "Inactive-Permanent.”

ALERTS

FILTERS

PARTICIPANT LIST

PARTICIPANT ISSUES

4 Study Drug Status Has Been Temporarily Inactive For More Than 90 Days

Once all these steps are completed, enter the relevant details in the comment box and then click the green "Resolve" button to clear the alert.

Study Drug Status Has Been Temporarily Inactive For More Than 180 Days

ACTION NEEDED:

- The study drug status "Inactive-Temporary" is meant to be used for a short time.
- Talk to your site PI and work with the participant in an attempt to have them either re-start study drug or move up to the active - full dose. If the participant agrees, change the study drug status on the participant's "Information" tab to either "Active-Full" or "Active-Modified", as appropriate.
- If the participant refuses to either restart study drug or move up to an "Active - Full" dose, determine with the participant and PI what the appropriate study drug status should be - if the status should remain as-is for another short period of time, or if it's appropriate to change the study drug status to "Inactive-Permanent".
- Enter in the comment box which action(s) you took and then click the green "Resolve" button to clear out the alert.

PID	Name	Status	Status Date	Comments	Resolve
900005017	Sue Walker	Inactive-temporary	2022-08-08	<button>Enter Comment</button>	<button>RESOLVE</button>

Participant Has Been Non-Adherent

If a participant has an "Active-Full" or "Active-Modified" study drug status at the time of the annual call, the Call Center will ask the participant about study drug adherence: "Think about the past month. Over the past month, in a 7-day week, on average how many days would you estimate that you take the study tablets?"

If the participant reports taking the study drug for 4 or fewer days a week, a "Participant has been Non-Adherent" alert will be sent to the site.

Click the alert on the site summary page to view a list of participants who are non-adherent. Follow the Action Needed instructions in the blue box at the top of the Alerts page to resolve the issue.

ALERTS

1

Participant has been Non-Adherent - 12 Month

Click on a participant's name to go to the participant's dashboard and review the Comments tab to check if the Call Center provided any additional information.

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Participant has been Non-Adherent

ACTION NEEDED:

On their annual call, the participant(s) below reported that they are taking the study drug 4 or less days per week. Please review any comments from the Call Center in the Comments tab on the participant dashboard and then contact the participant to discuss their study drug adherence.

If necessary, update the study drug status on the participant information page.

Once you have resolved the issue, mark the alert as "resolved" and include a comment on the action taken (i.e. spoke with participant and they agreed to take Study Drug 7 days per week, changed participant's study drug status to "inactive - temporary" due to ____, etc.)

If the alert is incorrectly resolved, please submit a RAP to have it reset. Do not submit an alert as resolved until you have taken some action.

PID	Name	Date of Adherence Question Form	Visit	Days per week they take drug	MARK RESOLVED
900002182	Vickie Eldrich	07/24/2021	CC12 - Annual Call Center Follow-up	4	RESOLVE

Site staff should contact non-adherent participants to review the study procedures and discuss the importance of remaining adherent to the study drug. If a participant is non-adherent, site staff are responsible for updating the study drug status on the participant's Information tab.

NOTE: It is critical to keep each participant's study drug status up to date to ensure the study drug is shipped only to participants who are adherent to the study drug protocol.

The following **Study Drug** and **Retention Status** Histories information is used in statistical analysis. If there is an error in either of these tables, please submit a detailed RAP so that it can be corrected.

STUDY DRUG STATUS

ENTER

STATUS:	REASON:	DATE ENTERED:	EFFECTIVE DATE:	INACTIVE-PERMANENT APPROVAL:
Active-full		08-19-2020	08-19-2020	--

If a participant reports taking their study drug for 4 days or less per week or they report not they do not take the full dose each day during the week, change the study drug status to "Active-Modified" and enter in the Effective Date field the date the participant started the change in adherence. If the participant reports they have not been taking the study drug and no longer intend to take it, change the study drug status to "Inactive-Permanent." If the participant reports they are only temporarily stopping the study drug, change to the study drug status to "Inactive-Temporary."

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ENTER NEW STUDY DRUG STATUS

Study Drug Status:

Active-modified

Reason:

--

Effective Date: *New Effective Date cannot be prior to current Effective Date*

Conversation Details:

Select which investigator signed off and agrees with this decision:

--

ENTER

When a participant's study drug status is changed to either "Active-Modified" or "Inactive-Temporary," site staff should review the status every 90 days and update it as needed. Note that "Active-Modified" and "Inactive-Temporary" are meant to be used for a short period of time and should not be used as permanent study drug statuses for the duration of the study.

After a new study drug status is entered and submitted, resolve the study drug adherence alert by returning to the alert's participant list and clicking the green "Resolve" button. Do not click "Resolve" until the study drug status has been changed and/or the site PI/clinician or other site staff have talked to the participant about the importance of adherence to the study drug. If site staff are aware of a participant's modified dose and the participant's study drug status is already "Active-Modified," there is no need to enter in again that the participant is on a modified dose and site staff can "Resolve" the alert.

Study Drug Not Received

This alert will occur if a participant reports to the Call Center on their annual call that they did not receive their most recent shipment of the study drug.

*"I see that we shipped your last shipment of study medication on **Sep 19, 2023 2:13:32 PM**. Did you receive the study medication?"*

No

ALERTS

2

Study drug not received - 12 Month

To resolve this alert, follow the Action Needed instructions in the blue box at the top of the Alerts page. If needed, see **Appendix 10.1** for instructions to place a new study drug order for VA and non-VA participants.

Site staff should contact the participant and either update the participant's mailing address in the Contact Information Form and place a new study drug order or change the participant's study drug status. After taking the appropriate action, clear the alert by clicking the green "Resolve" button.

Study drug not received

ACTION NEEDED:

On their annual call, the participant(s) below reported that they did not receive their latest shipment of study drug. Please review any comments from the Call Center in the Comments tab on the participant dashboard and then contact the participant to discuss the failed delivery of the most recent study drug shipment. Once you have resolved the issue, mark the alert as "resolved" and include a comment on the action taken (i.e. corrected the address in contact info form, reordered study drug, changed participant's study drug status etc.) If the alert is incorrectly resolved, please submit a RAP to have it reset. Do not submit an alert as resolved until you have taken some action.

PID	Name	Date of Adherence Question Form	Visit	MARK RESOLVED
900002182	Vickie Eldrich	07/24/2021	CC12 - Annual Call Center Follow-up	RESOLVE 

Study Drug Status "Inactive-Permanent" Was Not Approved

To resolve a "Study Drug Status 'Inactive-Permanent' was not Approved" alert, click on the alert to view a list of participant(s) the alert pertains to and review the approval status under the Study Drug Status table on the participant's Information tab.

Study Drug Status 'Inactive-Permanent' Was Not Approved

ACTION NEEDED:

A Study Drug Status of 'Inactive-Permanent' was requested, but was not approved for each participant listed. To resolve this alert: The Study Drug Status needs to be updated as discussed with your Site Monitor.

Review the notes from your site monitor in the Comments tab of the participant's dashboard to learn why the request was denied and discuss with your site monitor the appropriate study drug status change. Once a new study drug status has been entered and saved, the alert will resolve. If a study drug status change was entered in error, please submit a RAP under Quick Links on the study website.

11.1e RETENTION STATUS ALERTS AND UPDATES

NOTE: It is critical to keep the retention status up to the date to ensure that phone follow-up is only completed on participants who are still retained in the study and have agreed to complete the annual follow-up calls.

The retention status can be updated on the Information tab in the participant's study record in the study website. Any time the retention status is updated, the date of the status change is entered.

The following retention status options should be discussed with each participant to minimize withdrawals from the study:

- Active, complete all procedures
- Modified Follow-up (check all that apply)
 - Do not call for annual phone visits
 - Do not call for final phone visit
 - Do not call trusted contacts (only if specified by participant)
 - Do not review electronic health records (only if specified by participant)
- Full Withdrawal Requested
 - Withdrawn by Participant
 - Withdrawn by PI

To change a participant’s retention status, click the blue “Enter” button next to Retention Status and choose one of these options from the drop-down menu:

- Active, Complete All Procedures
- Modified Follow-up
- Full Withdrawal Request

In the Effective Date field, site staff should enter the date they became aware of the change.

If “Modified Follow-Up” is selected, check the boxes for aspects of the study the participant does **not** want to participate in. Site staff cannot check more than 3 boxes. If all 4 boxes are checked, the retention status should be changed to “Full Withdrawal Requested.”

ENTER NEW RETENTION STATUS

Retention Status:
Modified Follow-up

☐ Do not call for annual phone visits

☐ Do not call for final phone visit

☐ Do not call trusted contacts (only if specified by participant)

☐ Do not review electronic health records (only if specified by participant)

Effective Date: *New Effective Date cannot be prior to current Effective Date*

Conversation Details:

Select which investigator signed off and agrees with this decision:

ENTER

If “Full Withdrawal Requested” is selected, choose the withdrawal type from the drop-down menu and check the attestation box. Enter the effective date and include the details of the retention conversation

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the site staff or investigator had with the participant. Finally, select the investigator who agrees with the decision to submit a full withdrawal request.

ENTER NEW RETENTION STATUS

Retention Status:

Full Withdrawal Requested
▼

Withdrawal Type:

Withdrawn by participant
▼

☐ I attest that the participant has fully withdrawn their consent and is NOT open to ANY modified follow-up which includes annual calls, final visit, communication with trusted contact and EHR review

Effective Date: ***New Effective Date cannot be prior to current Effective Date***

Conversation Details:

Select which investigator signed off and agrees with this decision:

--
▼

ENTER

Site staff should not consider the participant officially withdrawn until the withdrawal request is approved by the site management team. Site staff can check the status of a request in the Withdrawal Approval section. The status turns solid green if the withdrawal request is approved or solid red if it is not approved.

RETENTION STATUS			
STATUS:	DATE ENTERED:	EFFECTIVE DATE:	WITHDRAWAL APPROVAL:
Full Withdrawal Requested	06-01-2021 15:18:40	05-18-2021	APPROVED NOT APPROVED
	Withdrawn by participant		

If the retention status is changed to “Full Withdrawal Requested,” the study drug status is automatically updated to “Inactive-Permanent” with a reason of “Full Withdrawal.”

Full Withdrawal Not Approved Alert

To resolve a “Full Withdrawal not approved” alert. Click on the alert to view a list of participant(s) the alert pertains to and review the approval status under the Retention Status table on the participant’s Information tab.

1 Full Withdrawal not approved

Full Withdrawal not approved

ACTION NEEDED:
A Full Withdrawal was requested and then not approved for each participant listed. The Retention Status needs to be updated as discussed with your Site Monitor.

PID	Name
900000996	Sheryl Pena

Review the notes from your site monitor in the Comments tab of the participant’s dashboard about why the request was denied and then discuss with your site monitor the appropriate retention status. Once a retention status has been entered and saved, the alert will resolve.

Date ▾	Category ▾	Commentor ▾	Comment ▾
7/2/21	Clinic - General	Emily Rives	Full Withdrawal request was denied because this participant had indicated in the Call Center notes that they just didn't want to take study drug anymore but still wanted to do annual follow-up calls.

11.1f 3-MONTH LIPID SAMPLE VISIT ALERTS

Some participants will be randomly selected to have a 3-month lab visit for the lipid sample sub-study. To check if an enrolled participant is in the lipid sub-study group, navigate to the participant’s Information tab and check the Lipid Panel Sub-study section for a status of “yes” or “no.”

Information

Consent

SSN

Visit List

Enrollment

Contact Information Form - Last updated:

CURRENT ELIGIBILITY STATUS:

Enrollment:	ELIGIBLE	Date of Enrollment:	08-08-2019
Randomization:	ELIGIBLE	Date of Randomization:	08-10-2019

LIPID SAMPLE SUBSTUDY: YES



If a participant has a Lipid Sample Sub-study status of “yes” and the answer is “yes” to the question “Were any samples obtained?” on their Baseline Blood Collection Form, the data entry button for the 3-month visit will appear on the Visit List tab. Site staff have the option to enter the date and time of the scheduled 3-month lab visit.

The screenshot shows the PREVENTABLE system interface. On the left is a sidebar with buttons: Information, Consent, SSN, Visit List (highlighted in blue), and Enrollment. To the right of the sidebar is a 'CONT - Contact Information' button. Below this is a 'CLINIC VISITS' table. The table has columns: Visit (click for forms), Date Scheduled, Time Scheduled, and an empty column. There are two rows in the table: 'CL00 - Baseline Clinic' and 'CL03 - 3 Month Clinic Visit'. Each row has input fields for Date Scheduled and Time Scheduled, and a green 'Submit' button. A red arrow points to the 'CL03 - 3 Month Clinic Visit' button.

Visit (click for forms)	Date Scheduled	Time Scheduled	
CL00 - Baseline Clinic			Submit
CL03 - 3 Month Clinic Visit			Submit

If a participant was randomly selected for the 3-month lab visit but a baseline blood sample was not collected, the participant should NOT do the 3-month lab visit and the data entry button for the 3-month visit will not appear on the participant’s Visit List tab. Use the Lipid Sample participant list filter on the Site Summary page to view a list of participants who have a baseline blood sample and who are assigned to the lipid sample sub-study group and thus need to complete the 3-month lab visit.

PARTICIPANT LIST FILTERS			
General		Enrollment Eligibility	
16	All Participants	12	INCOMPLETE
4	Enrolled	0	INELIGIBLE
4	Randomized	4	ELIGIBLE
2	Lipid Sample	Randomization Eligibility	
0	Snowbirds	12	INCOMPLETE
Retention Status		0	INELIGIBLE

3-Month Visit Due for Lipid Sample Blood Collection Alert

The “3 Month Visit due for lipid sample blood collection” alert will appear on the study website when a participant enters the 3-month visit window (60 days post randomization). This alert will occur only for participants who were randomized to the 3-month lipid sample sub-study group **and** who had a baseline blood sample collected and entered in their Baseline Lab Blood Collection Form.

1

3 Month Visit due for lipid sample blood collection

ACTION NEEDED:

The 3 Month visit is due for the participants in the Lipid Sample where samples were collected at the baseline visit. In order to resolve these alerts either the 3 Month Blood Collection form or a Reason Not Collected must be entered.

PID	Name	Date Randomized	3 Month Target Date
800282760	Scooby Doo	08/19/2020	11/17/2020

To resolve this alert, select the participant from the alert list and either enter in the Lab Blood Collection 3 Month Form or enter in a “Reason Not Collected.” One of these options must be submitted to resolve the alert.

VISIT: CL03 - 3 Month Clinic Visit

PRINT	FORM	DATE ENTERED	DATE UPDATED	REASON NOT COLLECTED
	Lab Blood Collection 3 Month	--	--	 ENTER

3 Month Visit PAST Due for Lipid Sample Blood Collection Alert

3 Month Visit PAST Due for Lipid Sample Blood Collection

ACTION NEEDED:

The 3 Month visit window has closed for these participants in the Lipid Sample. The window to collect the 3 month lipid sample is 60-180 days from the date of randomization. In order to resolve this alert, complete at least one of the following:

- Enter data into the CL03 Blood Collection Form
- Enter a reason NOT COLLECTED for CL03 Blood Collection Form

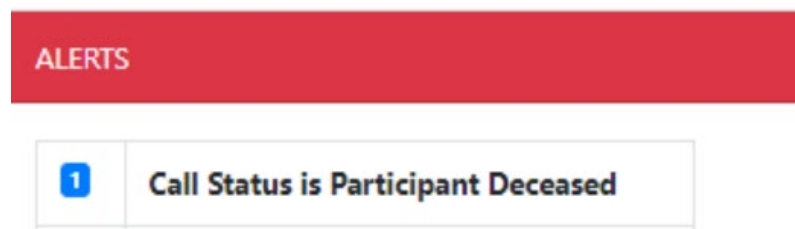
The “3 Month Visit PAST Due for Lipid Sample Blood Collection” alert will appear for participants who are in the lipid sub-study group, had baseline blood collected, have passed the 180-day mark (the end of the 3-month visit window) since randomization AND either no data has been entered in their Lab Blood Collection 3 Month Form OR a “Reason Not Collected” has not been entered.

Resolve the alert by either entering the blood collection data into the Lab Blood Collection 3-Month Form or enter a “Reason Not Collected.”

11.1g CALL STATUS IS PARTICIPANT DECEASED ALERT

Throughout the study, site staff, the Call Center, or another study partner may learn that a participant has died. If the Call Center is the first to learn about a participant's death during an annual follow-up call, the site will receive a Participant Deceased alert on the study website home page. If the Call Center or another study partner, e.g., the VA Cooperative Studies Program (CSP) pharmacy, tele-site, etc. learns of a participant's death outside of an annual study call, the site will receive an Unexpected Participant Communication alert (see **Appendix 11.1h**) notifying them of the participant's status change.

If the Call Center learns a participant is deceased during attempts to complete the annual phone call, the below participant alert will display on the study website dashboard.

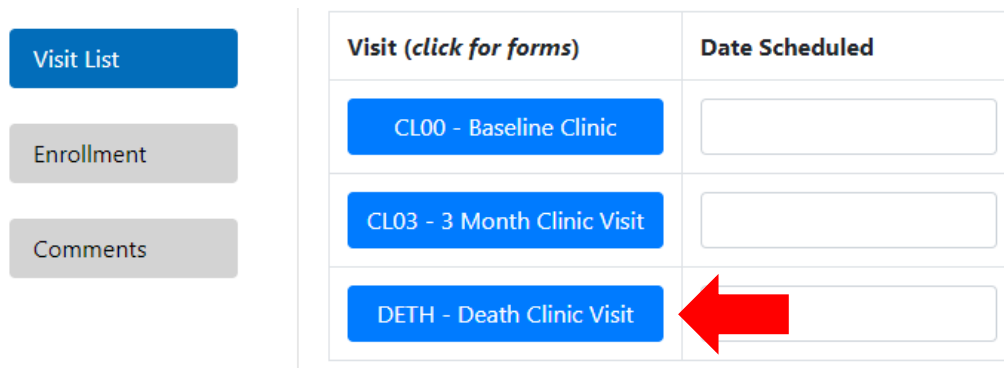


To resolve this alert, follow the Action Needed directions above the alert list.

ACTION NEEDED:
The PREVENTABLE Call Center has been notified that the below participants are deceased.
Please complete and submit a death form for each participant listed below.
Please refer to the comments section on the participant's dashboard for additional details.

PID	Name	Date of Enrollment	Date Randomized	Date of Current Status
800011309	Hermoine Granger	09/16/2020	N/A	06/14/2021

Navigate to the participant's record on the study website. On the participant's dashboard, click the Visit List tab, select "DETH – Death Clinic Visit" then "Death Page" and complete the form.



VISIT: DETH - Death Clinic Visit

PRINT	FORM	DATE ENTERED
☐	Death Page	

VA Sites Only: VA CIRB Form 124 Box Not Checked Alert

This alert occurs if a VA site enters a Death Form for a participant and does NOT check and submit the box next to VA CIRB Form 124 Complete.

VA site staff need to complete both the Death Clinic Visit Form and the VA CIRB Form 124 per VA regulations. Once the site has completed and submitted VA CIRB Form 124 (in the VA system, that form is not completed in the PREVENTABLE database), check the “VA CIRB Form 124 Complete” box and click the green “Submit” button. This will help VA Site Monitors confirm the forms have been submitted to the appropriate parties in case of a VA participant’s death.

CLINIC VISITS				
Visit (click for forms)	Date Scheduled	Time Scheduled	Begin Window	T
CL00 - Baseline Clinic				
DETH - Death Form	☒ VA CIRB Form 124 Complete		Submit	

11.1h UNEXPECTED PARTICIPANT COMMUNICATION ALERT

The “Unexpected Participant Communication” alert will occur if one of the central study units (e.g., Coordinating Centers, the Call Center, VA CSP pharmacy) receives a communication from a participant outside of a study visit that may require action from the clinic. For example:

- The pharmacy (VA CSP) receives a call from a participant that they need to update their mailing address for study drug shipments
- The Call Center receives a call (not during a study visit call) from a trusted contact that a participant passed away.
- The Coordinating Center receives a call from a participant that they were diagnosed with cancer and their oncologist would like to take them off of the study medication.
- The tele-site receives a call from a participant that they have been unable to reach anyone at their site and they would like to discuss their retention status.

The alert will appear under the Participant Issues category.

ALERTS FILTERS PARTICIPANT LIST

PARTICIPANT ISSUES	
16	Unexpected Participant Communication

As with all other alerts, review the Action Needed instructions to determine next steps and then review the Summary of the Call column next to the participant's name in the list below the Action Needed box. Click on the participant's name to view the entire comment.

Unexpected Participant Communication

ACTION NEEDED:

1. One of the central units (e.g., Coordinating Centers, Call Center, VACSP) has received communication from a participant outside of a study visit that may require action on your part.
2. Click on the participant from the list below to review the comment entered by the central unit, under 'comments'.
3. If appropriate, please contact the participant to determine if action is needed. Enter a comment if appropriate. Once the appropriate action has been taken, click 'Resolve' to resolve the alert.
4. If no action is needed, click 'Resolve' to resolve the alert.

PID	Name	Summary of the Call (or Other Communication) Noted	Date	Comments	Resolve Alert
900022108	Will Wheaton	test	10/03/2023	Enter Comment	RESOLVE
900000139	Wesley Roberson	test comment	10/06/2023	Enter Comment	RESOLVE

After contacting the participant and/or taking the necessary action to address the issue noted in the Summary of the Call notes, click "Enter Comment," describe the action(s) taken, and click the green "Resolve" button to resolve the alert.

11.1i PARTICIPANT RESPONSE TO AUTOMATIC TEXT ALERTS

Non-VA participants have the option to opt in to receive study text messages. If a participant opts in to receive automatic text messages, check the "text message" box in question 9 on the Contact Information Form. Participants who opt in will receive a text reminder at several points during the study:

- A retention text at 3 months post randomization that says, "Thank you for taking part in PREVENTABLE, you are a true research hero! Don't forget to take your study drug!"
- An annual text at 6 weeks before their randomization anniversary date that says, "Hello, the PREVENTABLE Call Center will call you in the next 2 weeks to schedule your annual phone visit. The phone number that will display on Caller ID will be either 336-716-9997 or 336-721-7722."
- A text 3 days and 1 day before their scheduled annual Call Center appointment that says, "PREVENTABLE Call Center appt on MM/DD/YYYY at XX:XX (AM/PM) ET/CT/PT (time zone). To reschedule call 833-517-0995."

Participants may respond to the automated texts with questions or important information. Because the texts are automatic and not monitored, there are 4 text response alerts to notify sites to review the participant's latest response to the automated texts and determine if they need to contact the participant or take any other necessary action. The 4 types of text alerts – STOP, Study Drug, Reschedule, Other – and steps to resolve the alerts are described below.

Text Response – STOP

Text Response - STOP

ACTION NEEDED:

1. Please note that because we are unable to predict every response that may be received, we believe this message was related to stopping text messages, however, you should review the text messages before proceeding.
2. Click on the participant from the list below to see the text message received back from the participant, under 'comments'.
3. If the response was STOP, the participant has likely indicated that they no longer want to receive text messages.
 - a. Update the contact information form (Question 5a or 6a) and Question 9 to indicate that texting is not allowed
4. If you are unable to determine if the participant has selected to opt out of text messaging, contact the participant to determine appropriate actions.
5. Once the appropriate action has been taken, click 'Resolve' to resolve the alert.

PID	Name	Message	Message Date	Comments	Resolve
300001100	Kathleen Brice	STOP	08/16/2023	Enter Comment	RESOLVE

The “Text Response – STOP” alert occurs when a participant wants to opt out of text messages and they respond “Stop” to any of the automated texts. If this alert appears, review the text from the participant by going to the Comments tab on the participant's dashboard. The text response from the participant will appear as a comment that does not have a label (is blank) in the Category, Response Choice, and Commenter columns, but has a message in the Comment column.

Date	Category	Response Choice	Commentor	Comment
10/15/2023				Stop

To resolve the alert, if the participant indicated in the comment that they no longer want to receive study texts, go to the participant's Contact Information Form, uncheck the “text message” box in question 9, and submit the form to save the changes. Navigate back to the alert, click the blue Enter Comment button to note the steps taken to address the alert, and then click the “Resolve” button.

Text Response – Study Drug

Text Response - Study Drug

ACTION NEEDED:

1. Please note that because we are unable to predict every response that may be received, we believe this message was related to study drug, however, you should review the text messages before proceeding.
2. Click on the participant from the list below to see the text message received back from the participant, under 'comments'.
3. If the response was related to study drug, contact the participant to determine the appropriate action. Some possible action needed may include:
 - a. Reorder a new prescription (may need to update address on Contact Information Form for snowbirds)
 - b. If Participant wishes to stop study drug, please consult the PI and discuss with the participant the possibility of modified dose and/or temporarily stopping study drug and update study drug status as appropriate
4. Once the appropriate action has been taken, click 'Resolve' to resolve the alert.

PID	Name	Message	Message Date	Comments	Resolve
900001100	Kathleen Brice	MEDICATION	08/08/2023	<button>Enter Comment</button>	<button>RESOLVE</button>
09/21/2022				Are more meds on the way? I have enough for one more week's supply	

The “Text Response – Study Drug” alert occurs when a participant responds to the automated study text with anything related to study drug, medication, refill, ordering, shipping issues, etc. If this alert appears, review the text from the participant by going to the Comments tab on the participant’s dashboard. The text response from the participant will appear as a comment that does not have a label (is blank) in the Category, Response Choice, and Commenter columns, but has a message in the Comment column. Determine if the participant needs to be contacted to discuss the text message and then determine any additional actions that need to be taken (reordering study drug, changing the participant’s mailing address in the contact info form, etc.). Once all of the above steps are taken, click the green Resolve button to resolve the alert.

Text Response – Reschedule

Text Response - Reschedule

ACTION NEEDED:

1. Please note that because we are unable to predict every response that may be received, we believe this message was related to rescheduling a future visit, however, you should review the text messages before proceeding.
2. Click on the participant from the list below to see the text message received back from the participant, under 'comments'.
3. If the response was about rescheduling, the participant likely received a reminder about an upcoming visit and has asked for it to be rescheduled.
4. Please contact the participant to find a time that works to reschedule the visit and document the new visit date by taking one of the following actions:
 - a. For call center visits, go to the visit list for this participant, and select 'Reschedule' next to the appropriate visit
 - b. For clinic visits, go to the visit list for this participant and update the date scheduled
5. Once the appropriate action has been taken, click 'Resolve' to resolve the alert.

PID	Name	Message	Message Date	Comments	Resolve
900001100	Kathleen Brice	I NEED TO RESCHEDULE MY APPOINTMENT.	08/08/2023	<button>Enter Comment</button>	<button>RESOLVE</button>
08/01/2023				I made an appointment for the 15th at 11am. I have to change that appointment. Is there a number I can call to reschedule?	

The “Text Response – Reschedule” alert occurs when a participant responds to the automated study text with anything related to canceling, rescheduling or scheduling a call center or clinic appointment. If this alert appears, review the text from the participant by going to the Comments tab on the participant’s dashboard. The text response from the participant will appear as a comment that does not have a label (is blank) in the Category, Response Choice, and Commenter columns, but has a message in the Comment column. Determine if the participant needs to be contacted to discuss the text message and then determine any additional actions that need to be taken (rescheduling the participant’s call center or clinic appointment, canceling the appointment, changing the retention status, etc.). Once all of the above steps are taken, click the green Resolve button to resolve the alert.

Text Response – Other

Text Response - Other

ACTION NEEDED:

1. We have received a text message back, but are unable to determine if action is needed.
2. Click on the participant from the list below to see the text message received back from the participant, under 'comments'.
3. If appropriate, please contact the participant to determine if action is needed. Once the appropriate action has been taken, click 'Resolve' to resolve the alert.
4. If no action is needed (e.g., participant has just texted back 'Thank you.'), click 'Resolve' to resolve the alert.

05/16/2022

My preventables ID card. Please replace it.

The “Text Response – Other” alert occurs when a participant responds to the automated study text with anything that does not relate to the other text alert categories above. If this alert appears, review the text from the participant by going to the Comments tab on the participant’s dashboard. The text response from the participant will appear as a comment that does not have a label (is blank) in the Category, Response Choice, and Commenter columns, but has a message in the Comment column. Determine if the participant needs to be contacted to discuss the text message and then determine any additional actions that need to be. Once all of the above steps are taken, click the green Resolve button to resolve the alert.

11.1j ADJUDICATION CLASSIFICATION – COGNITIVE OUTCOME ALERT

If a participant receives an adjudication classification that they have met the study cognitive outcome the site will receive a Participant Issues alert.

ALERTS

FILTERS

PARTICIPANT LIST

PARTICIPANT ISSUES

2

Adjudication Classification – Cognitive Outcome

NOTE: It is VERY important that the appropriate steps be completed to resolve “Adjudication Classification – Cognitive Outcome” alerts.

Read the Action Needed directions in the blue box at the top of the Alerts page. Inform the site PI of the adjudication classification for the participant. The PI should be the one to contact the participant to review the information in the Cognitive Testing Results letter. Additionally, the PI should contact the participant’s primary care provider to inform them of the classification and send a copy of the Cognitive

Testing Results letter **ONLY if the participant agreed in their Informed Consent Form to share cognitive changes with their doctor.**

Adjudication Classification - Cognitive Outcome

ACTION NEEDED:

The below participant(s) has reached the study cognitive outcome. Follow-up action is required. To resolve the alert, please complete ALL of the following steps:

1. The site PI/clinician should contact the participant to review the information provided in the **Cognitive Testing Results Letter** (located in Documents > Study Documents and Resources > Cognitive Adjudication).
2. Following the conversation, the **Cognitive Testing Results Letter** should be mailed to the participant.
3. Check the participant's Consent tab to see if they answered 'Yes' to the question "Allowing Report of Cognitive (Memory /Thinking) Changes to Your Doctor?" If 'Yes', mail or email the **Cognitive Testing Results to PCP Letter** (located in Documents > Study Documents and Resources > Cognitive Adjudication).
4. After notifying the participant and PCP, click the "Enter Comment" button to summarize the steps completed and then click the green 'resolve' button to close out the alert.

NOTES:

**For additional information and guidance on this process, see the "Guidance Doc for Site PIs and Staff if Participant reaches the cognitive outcome" under Documents > Study Documents and Resources > Cognitive Adjudication

1. Reaching the study cognitive outcome may prompt a clinical evaluation as desired by the participant, caregivers, and clinicians.
2. This alert should not affect the continuation of study drug administration. The site PI/clinician should use his/her clinical judgment regarding safe continuation of study drug in those who have reached the cognitive outcome.
3. Participants who have reached the cognitive outcome will no longer receive the annual calls from the Call Center. Trusted contacts will be called annually to assess for physical disability and, if appropriate, study drug adherence. If the participant reaches the persistent disability study outcome, the Call Center will not make any further calls to the Trusted Contact.

PID	Name	Date	Comments	Click to Resolve
		09/10/2024	Enter Comment	RESOLVE
		09/13/2024	Enter Comment	RESOLVE

Once all the above steps have been completed, enter a comment to document the appropriate steps were completed and click the green "Resolve" button.

NOTE: Participants who have reached the study cognitive outcome will remain in the study and continue to be followed for other outcomes, e.g., CVD, mortality, persistent disability. If they have not yet met the study outcome for persistent disability, the trusted contact will continue to be called annually until that outcome is met. It is up to the participant and the PI's discretion if the participant should continue taking the study drug after the classification of probable dementia.

11.1k ANNUAL CLINIC VISITS FOR ANCILLARY STUDIES ALERTS

Participants who are enrolled in certain ancillary studies may have annual clinic visits. The following alerts help site staff make sure annual visits are completed within the designated visit windows.

Ancillary Year # (1, 2, 3 etc.) Visit Window Approaching Alert

The "Ancillary Year # Visit Window Approaching" alert indicates that a participant is approaching their window for the annual visit. The alert will occur 10 weeks before the participant's randomization anniversary. The window to complete the visit is 12 weeks long (+/- 6 weeks from the randomization anniversary date). The alert occurs 10 weeks before to the randomization anniversary date to allow the site 4 weeks to contact the participant and schedule the annual visit within the designated window.

To view the visit window dates, navigate to the participant's Visit List page and find the relevant visit, e.g., CL12 – Ancillary Clinic Visit Year 1. The visit window dates are displayed next to the visit button.

To resolve the alert, follow the Action Needed instructions in the blue box at the top of the Alerts page.

Ancillary Year 1 Clinic Visit Window Approaching

ACTION NEEDED:

The Ancillary Year 1 Clinic Visit Window for the below list of participants is approaching. The list of participants below are enrolled in one or more ancillary studies that require an annual clinic visit. On the participant's visit list, the visit code "CL12 - Ancillary Year 1 Clinic Visit" refers to ANY ancillary study forms that should be completed for that participant during the Year 1 visit window. The visit window starts 6 weeks prior to the randomization anniversary date and closes 6 weeks post randomization anniversary date.

In order to resolve this alert, you must complete one of the following actions:

1. Enter in a date scheduled for the CL12 - Ancillary Year 1 Clinic Visit
2. Enter and save data into the CL12 - Ancillary Year 1 Clinic Visit form(s)
3. Enter in a "Reason Not Collected" for the CL12 - Ancillary Year 1 Clinic Visit

Resolve the alert by taking one of these 3 actions:

- Enter a date scheduled on the participant's Visit List page for the relevant visit
- Enter and save data in the relevant visit's form(s)
- Enter and save a "Reason Not Collected" next to the relevant visit's form(s)

SSN & MBI		CLINIC VISITS						
Visit List		Visit (click for forms)	Date Scheduled	Time Scheduled	Begin Window	Target Date	End Window	
Enrollment		CL00 - Baseline Clinic						Submit
Comments		CL12 - Ancillary Year 1 Clinic Visit	01/09/2024		10/04/202	11/15/202	12/27/202	Submit
Payments								

VISIT: CL12 - Ancillary Year 1 Clinic Visit

PRINT	FORM	DATE ENTERED	DATE UPDATED	REASON NOT COLLECTED
☐	Short Physical Performance Battery	--	--	ENTER

OR

Ancillary Year # (1, 2, 3 etc.) Visit Window Past Due Alert

The "Ancillary Year # Visit Window Past Due" alert indicates a participant's annual visit window has passed, which is +6 weeks from the participant's randomization anniversary date. To view the visit window dates, navigate to the participant's Visit List page and find the relevant visit, e.g., CL12 - Ancillary Clinic Visit Year 1. The visit window dates are displayed next to the visit button.

To resolve the alert, follow the Action Needed instructions in the blue box at the top of the Alerts page.

Ancillary Year 1 Clinic Visit Window Past Due

ACTION NEEDED:

Ancillary Year 1 Clinic Visit Window for the below list of participants is past due. The list of participants below are enrolled in one or more ancillary studies that require an annual clinic visit. On the participant's visit list, the visit code "CL12 - Ancillary Year 1 Clinic Visit" refers to ANY ancillary study forms that should be completed for that participant during the Year 1 visit window. The visit window starts 6 weeks prior to the randomization anniversary date and closes 6 weeks post randomization anniversary date.

In order to resolve this alert, you must complete one of the following actions:

1. Enter and save data into the CL12 - Ancillary Year 1 Clinic Visit form(s)
2. Enter in a "Reason Not Collected" for the Ancillary Year 1 Clinic Visit

To resolve the alert, either enter a "Reason Not Collected" or enter and save data in the relevant visit form(s) if data was collected. The alert will automatically resolve after one of these 2 actions is completed.

VISIT: CL12 - Ancillary Year 1 Clinic Visit				
PRINT	FORM	DATE ENTERED	DATE UPDATED	REASON NOT COLLECTED
☐	Short Physical Performance Battery	--	--	ENTER
PRINT				

SP# Call Status: Refused Alert

Participants enrolled in the Short Physical Performance Battery (SPPB) ancillary study have an additional part of the annual call to complete each year. The SPPB ancillary data collection call is designated with "SP" as the visit code. Occasionally, the Call Center may reach out to a participant for an annual SPPB ancillary study call and the participant may **refuse** to complete the call. The participant may state they want to skip the call this year and do it next year or they may request to withdraw from the ancillary study. If a participant requests to withdraw, it is **very important** to review the comments from the Call Center and then reach out to the participant to attempt to negotiate their retention in the SPPB ancillary study.

To resolve the alert, review the comments from the Call Center under the participant's Comments tab or click the comment bubble icon next to the relevant SP visit in the Call Center Visits table on the Visit List tab. Contact the participant to discuss their retention in the study. If necessary, change the SPPB ancillary study retention status on the SPPB Retention tab on the participant's dashboard. If the participant has fully withdrawn from the main PREVENTABLE study, the SPPB ancillary study status will automatically change to inactive and the SPPD study status does not need to be resubmitted.

Enter comments about the conversation with the participant and their decision of whether or not to continue in the SPPB ancillary study. If the participant changes their mind and would like to do the SP call, click the blue "Reschedule" button to reschedule the SP call. Once all relevant comments are entered, click the green "Resolve" button to resolve the alert.

SP36 Call Status: Refused

ACTION NEEDED:

The PREVENTABLE Call Center has attempted to call the participant(s) below for the annual SPPB Ancillary call and the participant refused to complete the call.

To resolve this alert: Please review the notes from the Call Center in the Comments tab of the participant's dashboard and follow-up with the participant to determine if their retention status in the SPPB Ancillary study needs to be changed to 'inactive.'

1. If the participant wants to withdraw from the SPPB Ancillary study, go to the SPPB retention status tab on the participant's page and change the retention status to 'inactive.'
2. If the participant also refused the main trial annual call for the same year and is showing under the corresponding 'Back to Clinic' alert, you may have already reached out to them to discuss their retention in the main trial. If so, you can update the SPPB ancillary retention status as appropriate.

Alerts will resolve at the top of the hour if the appropriate action(s) have been taken.

SP Call Outcome – Deceased Alert

If the Call Center attempts to complete the annual SPPB ancillary call (“SP” visit code) and is notified that the participant has passed away, the site will receive a “SP Call Outcome – Deceased” alert.

SP Call Outcome - Deceased

ACTION NEEDED:

The PREVENTABLE Call Center has been notified that the below participants are deceased.

To resolve the alert(s), please complete and submit a death form for each participant listed below. Please refer to the comments section on the participant's dashboard for additional details.

PID	Name	Date	Comments
900002425	Papa Johns	08/23/2023	<button>Enter Comment</button>

To resolve this alert, follow the Action Needed directions at the top of the Alerts page. To resolve the alert, complete the Death Clinic Visit Form. To access the form, click the Visit List tab on the participant's dashboard, select “DETH – Death Clinic Visit” then “Death Page,” and complete the form.

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CHAPTER 12. DATA MANAGEMENT AND SECURITY

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

12.1 OVERVIEW

PREVENTABLE participant data is entered into the data entry system in the password-protected area of the study website. Study staff can use the website to:

- Enter and review clinical data for participants at their site
- Review Call Center data for participants at their site
- View live reports of participant demographics, recruitment goals, screening yield, and more
- Receive and resolve participant issue alerts
- Access training materials and study documents

The PREVENTABLE database and data entry system on the study website is maintained by the Data Coordinating Center (DCC). Data security in the web-based data system uses 128-bit encryption and Secure Socket Layer (SSL). See **Section 12.5** for details about the database and data security.

12.2 FEATURES OF THE STUDY WEBSITE

12.2.1 WEBSITE ACCESS

All data on the study website are password-protected with individual passwords for each site staff member who needs access to participant data. Study website access is carefully controlled and is limited according to each person’s study site and role on the study.

Contact your site’s clinical research associate (CRA) to request access for a new user. The CRA or the site principal investigator (PI) will use the “Report a Problem (RAP)” feature on the study website to notify the DCC of the new user and the level of access requested.

Once web access is granted, the new user will receive an automatic email that includes a temporary password and instructions to set a password. A four-digit staff ID is automatically assigned to each new user.

If a staff member leaves the study, submit a RAP to request the DCC deactivate the user’s account. Additionally, notify the site CRA of resignation as the person’s access to other systems may also need to be updated.

12.2.2 STUDY ROLES AND WEBSITE ACCESS

Roles are defined by the tasks a study team member carries out for PREVENTABLE at their study site, the coordinating center, as a member of a study committee, etc. Individuals may have more than one role within the study and on the study website. The purpose of roles on the study website is to limit an access to protected health information (PHI), tracking systems, and other study data that are relevant to a person’s role on the study. Sites are responsible for ensuring their staff have completed and documented study-specific training for their responsibilities on the study before requesting access.

Principal Investigator

The Principal Investigator is responsible for oversight of the study conduct at their site as specified by the protocol, associated institutional requirements, good clinical practice (GCP)/regulatory requirements, and study-specific procedures and performance expectations. Only one person at each site can be identified as their site's Principal Investigator on the study website.

Principal Investigator Designees (Sub-Investigators)

Principal Investigator Designees assist the PI with study conduct and monitoring at their site.

Primary Coordinator

The Primary Coordinator is the lead person at a site who manages and monitors the broader aspects of the study. Only one person at each site can be identified as their site's Primary Coordinator on the study website.

Clinic Coordinators

Clinic Coordinators work with the Primary Coordinator to perform study-related activities.

Medication Prescriber

Medication Prescribers confirm a participant is eligible to receive the study drug at enrollment and annually for study drug renewals. **The medication prescriber's name will appear as the prescriber on the study drug bottle.** Only one person at each site can be designated on the website as the Medications Prescriber, although there may be more than one Medication Prescriber on the Delegation of Authority Log.

Lab

These are individuals who enter and manage data for processing, aliquoting, and shipping lab samples. There can be multiple individuals identified in the lab role associated with a site. However, at least one lab person at each site should be designated in the lab role on the study website for the DCC or Clinical Coordinating Center (CCC) to contact for lab-related needs.

Other

These are individuals, such as research managers or regulatory monitors, who have other roles requiring access to the data on the study website.

12.3 DATA ENTRY GUIDELINES

Each PREVENTABLE study site must maintain appropriate medical and research records for this study in compliance with ICH E6 as well as regulatory and institutional requirements for the protection of confidentiality of participants. As part of participating in an NIH-sponsored study, clinical records for the purposes of quality assurance reviews, audits, and evaluation of study safety, progress, and data validity will be made available as required. Data collection is the responsibility of the site staff under the supervision of the study site's principal investigator (PI). The PI is responsible for ensuring the accuracy, completeness, legibility, and timeliness of the data reported.

Data will be collected at all participant encounters, either by entering data directly in electronic Case Report Forms (eCRF) or by writing on printed CRFs that are later data entered into the eCRF. Data entry

screens on the study website mirror the printed CRFs for ease and accuracy of data entry. Clinical sites will capture progress notes as needed and maintain these notes in the participant's study record.

To make it easier for site staff to enter data on the study website, the forms needed for each study visit are preloaded on the study website, and each form prints with the participant ID and a unique barcode. The barcode contains information on the participant ID, visit, and form. Scanning the barcode on a form will take the study staff directly to the form in study website to enter data for the participant.

PREVENTABLE -

Participant ID: 100002161	Visit: CL00
 B100000037	Date of Visit: (MM-DD-YYYY) ____ - ____ - 20 ____
	Administered By: ____

NOTE: Data entry should be completed within 7 days of the visit for all study forms, except for the TICS-M. The TICS-M Form must be entered by Call Center staff immediately after the data is collected to determine if they need to administer the Extended Cognitive Battery to the participant.

12.3.1 SITE ID

Sites are assigned a number by the Clinical Coordinating Center. The site ID is a 4-digit number. The first two digits correspond to the “network” (e.g., VA, non-VA) and the last two digits are unique to the institution. The number is used to identify each site on all study site-related materials and reports.

12.3.2 PARTICIPANT ID (PID)

Participant IDs (PID) are nine-digit numbers ranging from 100000118 – 999999869. The last two digits are check digits to help confirm the barcode number. Participant IDs are generated automatically by the DCC and pre-printed on the Eligibility and Baseline Visit form packets.

NOTE: It is incredibly important to print the forms directly from the study website and do not make copies.

When a form is scanned from the Eligibility and In-Person Baseline Visit packet, the PID is associated with the participant. The PID will follow the participant through eligibility, randomization, and follow-up visits and will be pre-printed on all their study forms. The PIDs do not contain any encoded information about the study site and will not change throughout the course of the study. Participant names are pre-printed on the eCRFs to make it easier to organize and track forms from visit to visit.

NOTE: If data has been collected on a participant and a PID has been associated with them, the PID should **never** be reused for another participant.

If a participant is not screened or enrolled, but has been associated with the PID, the site can mark the participant as “Discontinue Participant” (see **Chapter 4: Screening and Enrollment**). The “Discontinue Participant” option should only be selected if the participant chooses not to proceed with enrolling in

the study after they have consented. If a participant is found to be ineligible between when they consent and when they are enrolled by the site, the appropriate reason for ineligibility should be documented in the Eligibility Form and the site should NOT select “Discontinue Participant.”

NOTE: If a participant is rescreened at any time, the site must keep the same PID and update the in-person Baseline Visit forms at each rescreening visit, including the date of the visit on all forms. To update the forms, scan the barcode on the forms or click on the participant in the site list on the study website to open the Participant Page > click on the Visit List tab > click on the appropriate visit (e.g., CLOO – Baseline). If a participant is mistakenly rescreened using two separate PID numbers, submit a RAP with the ID that needs to be deleted and a brief reason for the deletion request.

12.3.3 DATA VALIDATION, ERROR MESSAGES, AND WARNINGS

The study website is programmed to automatically check key variables during data entry to identify data that are inconsistent, incomplete, or inaccurate. When an entry check is triggered, data may be flagged for review or prevented from being saved in the study database. See **Chapter 13: Quality Control and Monitoring** for more information about the data query system.

12.4 TRACKING SYSTEMS

The study website offers multiple tracking systems to help sites manage study procedures. Access to the tracking systems depends on a person’s role on the study and their designated role on the study website (see **Section 12.2**).

Below are general descriptions of the tracking systems for PREVENTABLE. All tracking systems within the study website feed reports and tracking tables for overall study and site-specific monitoring. Additional systems may be added if needed.

The Dashboard is the home page after logging into the study website. It is the location of all general study announcements, the calendar and directory, committee lists, documents, and reports. This is a repository of documentation for the study (e.g., protocol, manual of operations, training links) in addition to the central location for communication of study information. Documents are uploaded in this system and the directory and committee lists are updated as staff changes are made.

The Data Entry System is used to enter all eCRF information. This system includes a filter page where site staff can easily see the number of participants in a certain category. Users can use the Participant List to see a list of all participants. The Participant Page within the data entry system includes information about the participant’s eligibility status and consent information. For randomized participants, the Participant Page includes information about their retention and study drug status, visit list, payment status, and ancillary study participation.

The Study Drug Tool is used to manage participants’ study drug orders. Site staff will be notified via an alert on the study website when a participant’s study drug prescription needs attention.

The Site Management System is used to track site-specific metrics and provide site-specific details. The Protocol Deviation Tracking System is part of this system. The Site Management system can also be used to create site-specific mailing lists.

The Protocol Deviation Tracking System is used to track and monitor reportable protocol deviations. Sites can enter a new Protocol Deviation Form, view their reported protocol deviations, and provide documentation and further information as needed. Coordinating centers use this system to view and respond to protocol deviations.

The Alerts System is part of the website dashboard for study sites and is available to site monitors under Data > Monitor Page. The dashboard for study sites summarizes all open data queries, participant issue alerts, lab alerts, and study drug order alerts. Through the Monitor Page of the Alerts System, CRAs can approve full withdrawal requests and permanent study drug withdrawal requests.

The Lab Tracking System is used to track and monitor all sample shipments. Study sites use this system to scan aliquots and create shipping manifests to ship PREVENTABLE lab samples to the Mayo Clinic Biorepository and ICON central lab.

The Payment Tracking System is used by payment admins who are responsible for reviewing the status of all participant payments. In this system, payment admins can review reissue requests and send alerts back to the clinic if a check is returned as undeliverable.

The Publications & Presentations (P&P) Tracking System is used to submit and track all abstracts, presentations, and publications. In the P&P system, study team members can upload a proposal for the P&P Committee to review and vote on. Once approved, proposals are tracked until publication.

The Ancillary Study Tracking System is used to submit and track all ancillary studies. In this system, study team members can upload an ancillary proposal for the Ancillary Studies Committee to review and vote on. Once approved, ancillary study proposals will be tracked until a funding decision is made.

In addition to the tracking systems described above, clinical laboratories, the Pharmacy Coordinating Center, the Data Coordinating Center, and other central units have appropriate access to download or transfer study data through the PREVENTABLE website.

12.5 SECURITY AND DATA PROTECTION

The confidentiality of all participant information, including but not limited to any genetic analysis, is protected at the clinical sites and the DCC. Paper records and computer files must be appropriately safeguarded from unauthorized access. Data security is paramount in the digital age.

The DCC follows data security protection procedures that meet or exceed security recommendations specified by the National Institute of Standards and Technology (NIST). All communication between study staff and the PREVENTABLE study website is encrypted to prevent unauthorized viewing of study information. All servers storing study data are housed at a state-of-the-art data center a few miles from the DCC's campus, with strong physical security controls to prevent unauthorized access to the servers and storage.

The DCC also employs a 2-part disaster backup program. Data are backed up nightly to institutional backup resources and an internal departmental backup system, which acts as a secondary recovery point. If the primary backup system is unavailable to restore critical data, data can be restored from the secondary point. The secondary backup is kept in a location separate from the data center in a locked fireproof and waterproof storage cabinet. Study sites are responsible for having local procedures in place for backup and recovery of data following a disaster.

12.5.1 DATA SECURITY AT STUDY SITES

Electronic and paper records of study data must be handled with care and stored securely at each study site. Paper and electronic records for study participants must be stored at study sites in accordance with the study protocol and local policies.

Only PREVENTABLE study staff who have completed the appropriate PREVENTABLE trainings will have access to study CRFs.

Access to the data in any local PREVENTABLE database will be controlled by a system of user identification names and passwords to ensure only authorized staff can enter.

12.5.2 DATA SECURITY AT THE DATA COORDINATING CENTER (DCC)

All data collected, including necessary medical records, are identified by PID number and stored at the DCC via the secure study website. Access to the study website and participant data is managed by the DCC and CCC teams. Level of access for each individual is specified by the DCC. All passwords stored within the study website are encrypted using SSL encryption.

Confidentiality of information within the DCC is protected through a variety of procedures and facilities:

- The confidential nature of the data collected, processed, and stored at the DCC is explained to all new staff.
- Access to DCC office space where data is stored is controlled through a single door, which is locked and only accessible by key or security badge.
- All participant data sent to the DCC is encrypted.
- All participant data stored on the Wake Forest School of Medicine's servers are also encrypted.
- All databases are password-protected.
- All printouts and reports containing personally identifiable information (PII) are produced on printers within the DCC's secure office space. Participant ID numbers, rather than names, are used to label participants on all reports.
- PREVENTABLE is covered under a Certificate of Confidentiality, which prevents researchers from being forced to disclose identifying information by certain legal proceedings.

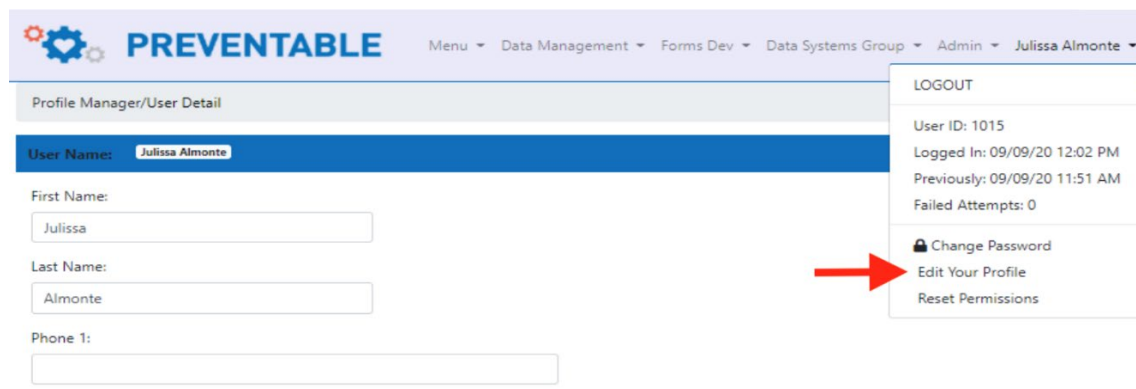
The Social Security Number (SSN) and Medicare Beneficiary Identifier (MBI) (if available) will be collected for each participant. If a potential participant is unwilling to provide their SSN, enrollment will be declined and the reason documented as "Unwilling to provide SSN." Highly sensitive information, such as SSNs and MBIs, is securely stored separately from other clinical data. Access to these highly sensitive data are limited to a few people at the DCC and only the PID connects these data.

APPENDIX 12.1 DATA MANAGEMENT AND SECURITY

12.1a STUDY WEBSITE FEATURES

WEBSITE ACCESS

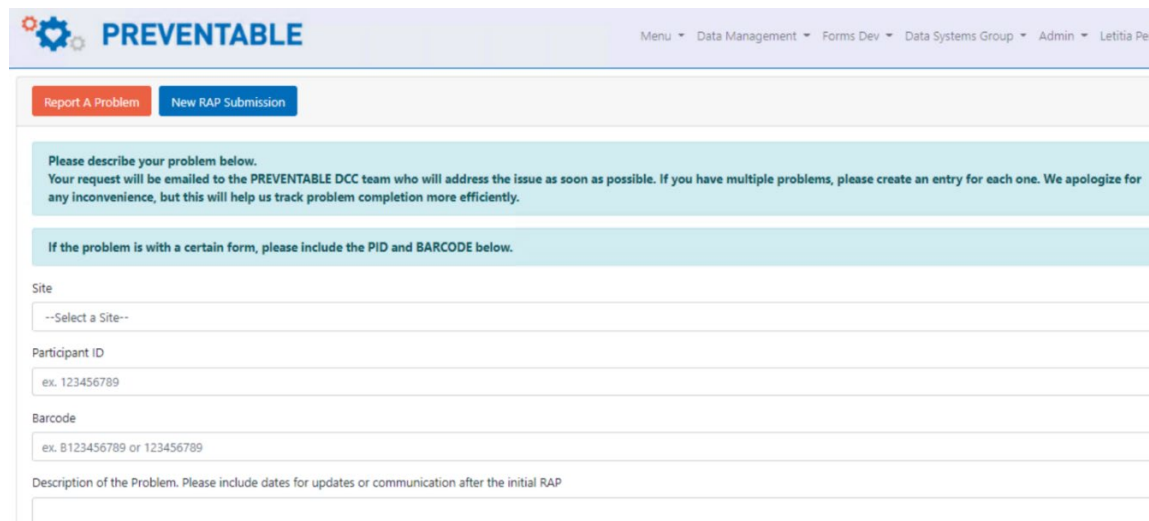
When the DCC grants someone access to the study website, the new user will receive an automatic email that includes a temporary password and instructions to set a password. New users should review their website profile by clicking their name in the upper right-hand corner of the main page > Edit Your Profile. If needed, use this page to update contact information, e.g., name, address, email, fax and phone numbers, and specialty, if applicable.



The screenshot displays the 'Profile Manager/User Detail' page for a user named 'Julissa Almonte'. The page includes input fields for 'First Name' (filled with 'Julissa'), 'Last Name' (filled with 'Almonte'), and 'Phone 1'. In the top right corner, a user menu is open, showing the user's name 'Julissa Almonte' and a dropdown menu with options: 'LOGOUT', 'User ID: 1015', 'Logged In: 09/09/20 12:02 PM', 'Previously: 09/09/20 11:51 AM', 'Failed Attempts: 0', 'Change Password', 'Edit Your Profile', and 'Reset Permissions'. A red arrow points to the 'Edit Your Profile' option.

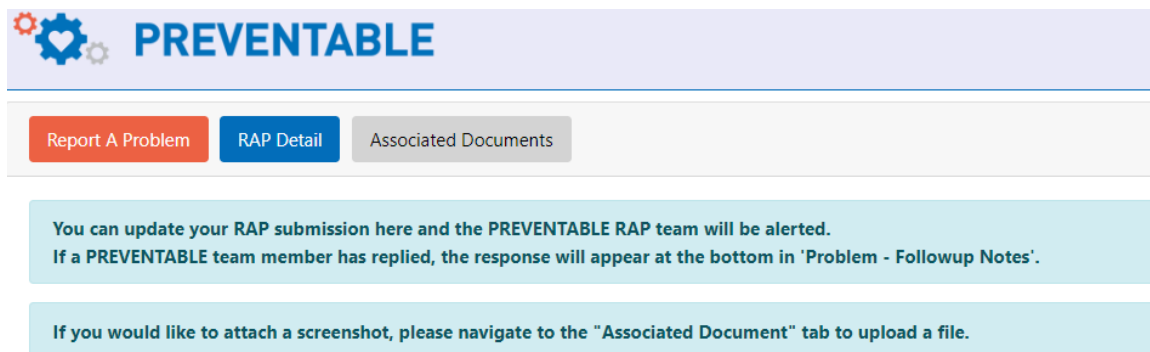
REPORT A PROBLEM (RAP) SYSTEM

For website support, use the Report a Problem (RAP) feature to contact the DCC. Click “Submit a RAP” under Quick Links on the home page or click Menu > Submit a RAP. Enter the Participant ID, barcode ID, form, and visit information if applicable. The RAP form includes space for a complete description of the problem and the option to copy others on the RAP. When the RAP is submitted, the DCC will send it to the appropriate study team member for assistance. Within 24 business hours, the DCC will respond with a resolution or request for more information.



The screenshot shows the 'Report A Problem' form on the PREVENTABLE website. The form has a title bar with 'Report A Problem' and 'New RAP Submission' buttons. Below the title bar, there is a text area for 'Please describe your problem below.' followed by instructions. Then, there is a section for 'If the problem is with a certain form, please include the PID and BARCODE below.' with fields for 'Site', 'Participant ID', and 'Barcode'. Finally, there is a text area for 'Description of the Problem. Please include dates for updates or communication after the initial RAP'.

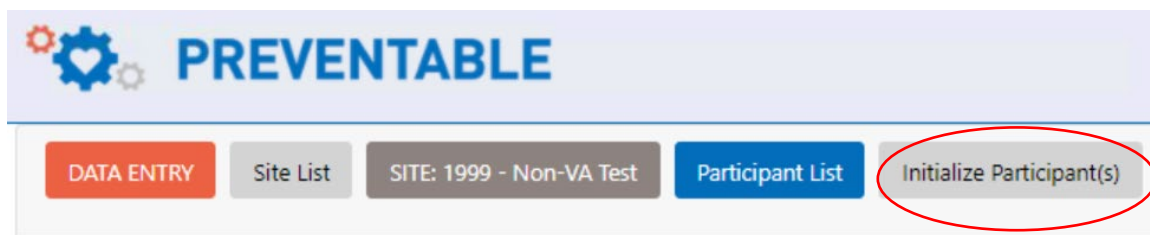
After a RAP is submitted, a tab for “Associated Documents” will appear at the top of the screen. Click on the “Associated Documents” button to add a document or screenshot to the RAP if needed.



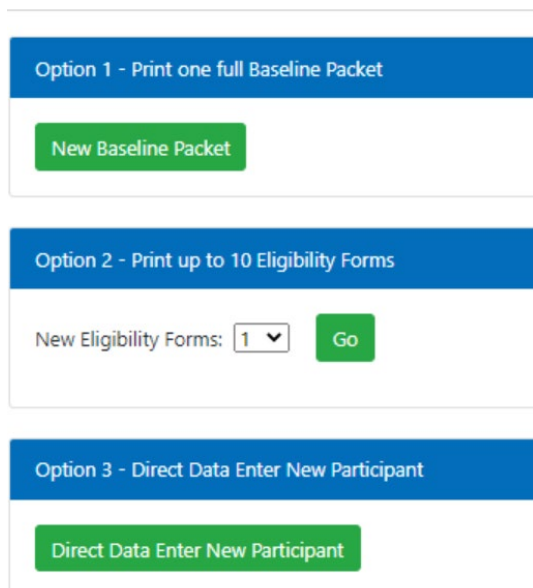
12.1b DATA ENTRY GUIDELINES

PARTICIPANT ID (PID)

To initialize PREVENTABLE IDs and print Eligibility and Baseline Visit forms, in the main menu click Data > Data Entry > select your site > click the “Initialize Participant(s)” button.



Clicking on “Initialize Participant(s)” will open a page with 3 options: Print one full Baseline Packet, Print up to 10 Eligibility Forms, or Direct Data Enter New Participant.



ELECTRONIC CASE REPORT FORMS (ECRF) NAVIGATION

To see the forms associated with each visit, in the main menu click Data > Data Entry > select your site > Click on the Participant List tab > type part of the participant’s name or their PID in the search bar.

DATA ENTRY Site List SITE: 1999 - Non-VA Test Participant List ENTER BARCODE

SITE: 1999 - Non-VA Test New Baseline Packet New Eligibility Forms: 1 Go Direct Data Enter New Participant

Search

PID	First Name	Last Name	Lipid Study	Enrollment Status	Date Enrolled	Eligibility Status	Date Randomized
-----	------------	-----------	-------------	-------------------	---------------	--------------------	-----------------

In the Visit List tab, click on the appropriate visit (e.g., CLOO – Baseline).

DATA ENTRY PID Lookup Site List SITE: 1999 - Non-VA Test Site Summary PID: 101142463 NAME: Derek Shepherd ENTER BARCODE

CONT - Contact Information

Information
Consent
SSN & MBI
Visit List
Enrollment
Comments
Payments
CAC Ancillary
CAC Consent

Visit (click for forms)	Date Scheduled	Time Scheduled	Begin Window	Target Date	End Window	Submit
CLOO - Baseline Clinic						Submit
CL03 - 3 Month Clinic Visit			03/28/2024	04/27/2024	07/26/2024	Submit
DETH - Death Form						

The page will display the list of all applicable forms to be completed at the visit. Review the applicable MOP chapter for instructions to conduct each visit and administer the forms. The “Date Entered” and “Date Updated” are automatically entered for each form.

DATA ENTRY Site List SITE: 2999 - VA Test Participant List PID: 100002161 NAME: John Smith ENTER BARCODE

Information
Consent
SSN
Visit List
Visit Status
Enrollment

VISIT: CL00 - Baseline Clinic

PRINT	FORM	DATE ENTERED	DATE UPDATED	REASON NOT COLLECTED	Q by Q
☐	Eligibility	--	--	ENTER	
☐	Demographics	--	--	ENTER	
☐	Medical History	--	--	ENTER	
☐	Lab Blood Collection	--	--	ENTER	
☐	Short Physical Performance Battery	--	--	ENTER	

PRINT

If a form is not completed, click the red “Enter” box in the “Reason Not Collected” column and select from the drop-down menu the reason the form was not collected.

PREVENTABLE Manual of Procedures

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VISIT:	CL03 - 3 Month Clinic Visit
FORM:	Lab Blood Collection 3 Month

Reason Lab Blood Collection 3 Month Form Not Collected:

Please choose an option

Please choose an option

Refused

Administration Error

Unable to understand instructions

Telehealth

Certified staff not available

Other

Site staff should not initiate a form and save it with no data. Once it is documented that a form was not collected, the site will be blocked from entering the eCRF. If a form is mistakenly marked as not collected and is later collected, submit a RAP to reopen the form.

DATA VALIDATION MESSAGES AND WARNINGS

Green messages indicate the participant is eligible according to the criteria shown in green. If a yellow warning message appears, the data are saved but an error needs to be addressed. Clicking on an error/warning message will take the user directly to the field where the error needs to be corrected. If a form is submitted and a red error message appears, none of the data will be saved until the errors, highlighted in red text, are corrected.

 **PREVENTABLE**

FORM DATA WAS SAVED - [Click here to Continue](#)

ELIGIBILITY STATUS - click row to jump to question.

ELIGIBLE - First Name:
ELIGIBLE - Last Name:
ELIGIBLE - Address 1:
ELIGIBLE - City:
ELIGIBLE - State:
ELIGIBLE - Zip Code:
ELIGIBLE - Is the Mailing Address validated
INCOMPLETE - 3. Is your physical address different from your mailing address?
INCOMPLETE - 5. What is your preferred telephone number?
INCOMPLETE - First Name:
INCOMPLETE - Last Name:
INCOMPLETE - 14. What is their preferred telephone number?

WARNINGS - click row to jump to question.

MISSING - 7. Is your preferred language English or Spanish?
MISSING - 9. Preferred Contact method expected
MISSING - What is the best time for us to call you expected
MISSING - 24. Do you currently live in the same household as other PREVENTABLE participant?

FORM DATA WAS NOT SAVED

ERRORS - click row to jump to question.

MISSING - Administered By

Short Physical Performance Battery - Version 1

Participant ID:	100000118
Barcode ID:	B100003965
Visit Code:	CL00
Date of Visit (mm/dd/yyyy)	02/09/2025
Administered By:	

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CHAPTER 13. QUALITY CONTROL AND MONITORING

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

13.1 OVERVIEW

Protocol training and adherence is a major focus of the investigator and site training and any necessary retraining. Identifying and correcting non-adherence is best accomplished in a stepped approach. Each site is responsible for protocol adherence. Data quality issues and significant non-adherence concerns will be escalated to Hub leadership and/or the Executive Committee.

13.2 PROTOCOL ADHERENCE

To promote adherence, sites will be provided with these resources:

- Virtual, in-person, and recorded training sessions available on the study website
- Documents and resources (e.g., eligibility worksheet, lab flow diagram) to ensure all protocol activities are completed for each participant enrolled
- An assigned Clinical Research Associate (CRA)/site manager, Hub Coordinator, and Principal Investigator (PI) to answer questions about the protocol or participant issues
- Contact information for key operational areas (e.g., pharmacy, laboratory, e-consent, ancillary studies)
- Alerts on the study website to notify staff to take action when needed to ensure compliance with key metrics and protocol activities
- DCC reports on eligibility, data quality, and queries on study data. This includes the Action Card highlighting the expected performance level (EPL) for key metrics and protocol activities.

13.3 REPORTS

All participant and site-specific reports and queries are available on the study website (see **Appendix 13.1**). Additional reports are generated for the subcommittees, Clinical Coordinating Center (CCC), and study sites. Reports include information on recruitment, retention, study drug status, data completeness, and data management.

To run one of a real-time report from live study data, navigate to the Reports page and click on the link to the report. The report will be processed by SAS and appear in the user’s web browser. Study sites have access to live data reports of their site’s progress toward their goals and projections of activity needed to meet their goals.

13.3.1 SITE-SPECIFIC REPORTS

To identify potential issues as soon as possible, site-specific reports of live data on the study website can be accessed by study sites, Hub leadership teams, the CCC, and subcommittees. These reports are utilized to identify and notify study sites if they are not meeting study standards. Data monitoring and cleaning occur continuously throughout the study.

To assist study sites and the CCC with quality control, the Data Coordinating Center (DCC) developed:

- A data entry system that incorporates real-time data quality assurance features, such as range and logic checks
- Data query reports
- A dashboard for study sites and CRAs to quickly see study-related alerts
- An Action Card describing clinic performance (see **Section 13.3.2**)
- Other site-specific reports (e.g., baseline demographics including breakdown of minority recruitment, participant listings to facilitate retention activities and monitor participants' study drug status, common data model participant listing to determine study outcomes, PHQ-8 alerts to ensure participant safety)

13.3.2 ACTION CARD

The Action Card on the study website is a summary of key metrics to monitor the overall success of PREVENTABLE and individual site performance. Metrics include but are not limited to recruitment, visit completeness, retention, study drug adherence, data entry, and data completeness. An expected performance level (EPL) has been set by study leadership and is listed in the header of the Action Card report—a different expectation for each metric (e.g., An Enrolled:Randomized ratio of at least 95%; Two Trusted Contacts listed for at least 75% of participants; etc.).

The Action Card typically displays live data. Twice a year, on March 1 and September 1, the Action Card data is frozen and sites are asked to respond by the end of the month for any metrics that are below the EPL. Responses should focus on new or continued strategies to be implemented over the coming week or months and not on providing context for why a certain metric is below the EPL. Sites should email responses to their CRAs/site managers and the appropriate PREVENTABLE mailbox:

- Non-VA sites: PREVENTABLE@dm.duke.edu
- VA sites: VAPREVENTABLEnetwork@va.gov

Responses are reviewed by the Measurements, Procedures, and Quality Control (MPQC) Committee, as well as Hub leadership and the Coordinating Center. Responses help identify common themes across sites and ensure proper support and training is provided to each site.

13.4 PROTOCOL DEVIATIONS

A protocol deviation is any noncompliance with the study protocol by a participant, a PI, or study staff.

Protocol deviations that impact the rights, safety, or welfare of a study participant, a participant's willingness to continue participation, the integrity of the research data, and/or are out of compliance with regulatory requirements (e.g., VHA handbooks, federal regulations, etc.) must be reported within 5 working days of becoming aware of the deviation. All reportable protocol deviations should be submitted via the study website (see **Appendix 13.1**). Protocol deviations related to the main PREVENTABLE study, as well as any ancillary studies, are also reported via the study website.

Examples of reportable protocol deviations may include any situations on the list below. If you are unsure whether a protocol deviation is reportable, please contact your CRA/site manager.

Inclusion/Exclusion Criteria

- Randomization of an ineligible participant
- Participant met an exclusion criterion, but was enrolled

Informed Consent Procedures

- Failure to obtain informed consent
- The consent form used was not the current version
- The consent form does not include all required signatures

Other

- PHI was entered in the study database prior to consent
- Breach of confidentiality
- Sharing of database passwords
- MD license expired
- Site staff who isn't listed on the Site Delegation Log performs a study procedure
- Using participant materials that are not IRB approved

Site investigators at VA sites are responsible for notifying the VA Central IRB (CIRB) of any unexpected problems (UPs), deaths, or protocol deviations for the duration of the study. All UPs, deaths, and protocol deviations should be reported to the VA CIRB in accordance with [VHA Directive 1058](#) regulations and the VA CIRB Table of Reporting Requirements, available under Important Links on the [VA CIRB website](#). Sites should contact the Boston Coordinating Center at VAPREVENTABLEnetwork@va.gov to receive instructions on CIRB-required reporting. Sites should use the following CIRB event reporting forms, available in the IRBNet Forms Library:

- **VA Central IRB Form 124 – Reportable Events**
 - Reporting of Research Death that is both unanticipated and related or possibly related and
 - Unanticipated Problem Involving Risks to Subjects or Others (UPIRTSO) that is both unanticipated and related or possibly related
- **VA Central IRB Form 115d – Protocol Deviation Log**
 - Protocol deviations, violations, and/or noncompliance

13.5 SITE MONITORING

The monitoring strategy for this study employs remote, risk-based monitoring except in cases of serious site issues. Site issues are addressed with the CRA/site monitor as they arise, and periodic teleconferences are held to discuss strategies to mitigate site issues. In addition, the CRAs/site monitors have regular contact with sites to ensure the study is conducted in compliance with the protocol and applicable regulatory requirements. Site issues may also be discussed with the Hub PI or Hub Coordinator, as appropriate.

If serious issues are detected during remote data review, a for-cause onsite or remote site visit may be conducted. Serious issues can include:

- Persistent lack of PI oversight
- Numerous consent process issues
- Enrolling ineligible participants

These data sources may be reviewed during a site visit:

- Case Report Forms
- Medical records
- Paper records and source worksheets
- Informed Consent Forms (ICFs)

During a site visit, site PIs must ensure the CRA has access to all study documents, including ICFs and source documents. Once the onsite or remote visit is complete, a report is given to the site to document any issues uncovered during the visit and provide action items to correct them. A timeline will be agreed upon and included in the report to ensure that issues are resolved and follow-up is completed. A visit follow-up letter will be given to the site PI and study coordinator. Site staff are expected to review the findings and resolve action items by the specified target dates.

13.5.1 ALERTS ON THE STUDY WEBSITE

The study website uses an alert system to direct staff to actions needed to ensure compliance with key metrics and protocol activities. The alert system is part of the study dashboard displayed after logging into the PREVENTABLE website. Alerts are also available for site monitors under Data > Monitor Page. The dashboard and Site Summary page provide a count of all participant issue alerts, lab alerts, and study drug alerts.

Site alerts should be addressed in a timely manner to ensure protocol compliance and participant safety. Sites can resolve alerts by following the steps outlined under “Action Needed” in the blue box at the top of alerts. Specific alerts and actions required are described throughout this MOP in relevant chapters. Some alerts are resolved immediately upon study staff completing the appropriate action(s) and others are resolved overnight after study staff the necessary action(s) to address the alert.

13.6 DATA QUERY SYSTEM

Queries about electronic Case Report Forms (eCRFs) and data entry are generated by programmed warnings established by the CCC and DCC (see **Chapter 12: Data Management and Security**). Sites are responsible for responding to the queries and verifying or updating the data entered to ensure accuracy (see **Appendix 13.1**). Data queries are updated daily.

Study staff can access site-specific data queries on the study website dashboard. The data queries can be filtered by participant, form, or user. These options allow site staff and monitors to check for patterns across forms or users to identify possible training areas.

After selecting participant, form, or user, the study website will display a list of site-specific queries. The query report table includes the form, visit ID, category, description of the query, values, and the user who entered the data. There is also a button to verify the data after review.

- The form and visit ID provide information about the location of the data.
- The 4 query categories indicate the reason for the query:
 - RANGE – the value falls outside the expected range for the question
 - SKIP – the data entered does not follow the established skip pattern
 - MISSING – there is information missing from the data entry
 - NUMERIC – the data entered does not comply with the specified format

- The description and value provide the item number, question, and the data entered, if provided
- The “Entered By” column lists the user provided in response to the question of “Administrated by” on the form.

The data should be reviewed and corrected in relation to the category of the query (e.g., NUMERIC – update the barcode to match lab-specific formatting). See **Appendix 13.1b: Data Query System** for the steps to resolve queries.

13.7 SITE CLOSE-OUT VISITS

Site closeout visits will be performed via phone at study completion or site closure. The overall goals of the site close-out visit are described in **Chapter 17: Study Completion and Close-Out**.

APPENDIX 13.1 QUALITY CONTROL AND MONITORING

13.1a REPORTS

To access the reports, login to the study website and click “Reports” under Quick Links on the left side of the home page.

The report folders displayed depend on the user’s role on the study and level of access in the study website. Click on a folder to review reports related to the selected topic.

FAVORITE	NAME - <i>click report name to run the report</i>
★	RECRUITMENT
♡	RETENTION
♡	MONITORS
♡	STUDY DRUG
♡	CALL CENTER
♡	SITE SPECIFIC REPORTS

Search:

FAVORITE	NAME - <i>click report name to run the report</i>
♡	PHQ-8 Alerts
♡	Retention and Study Drug Status -- site listing
♡	Common Data Model Participant Listing

13.1b DATA QUERY SYSTEM

To access the Data Query System on the study website dashboard, click the My Site Alerts tab.

Recruitment Summary	Fall / Winter 2023 Tiers	Spring/Summer 2024 Tiers	Announcements	My Site Alerts	Favorites
Tumbler Challenge	Web Change Logs				

SITE	PARTICIPANT ISSUES	ICON LAB	DRUG ORDER SYSTEM	DATA QUERIES
1705 - Wake Forest - Cardiology	106	1	7	500
1706 - Duke	3	1	0	15
1990 - Non-VA Turned On	0	0	0	5
1991 - Non-VA Turned Off	1	0	1	3

Data queries can be filtered by participant, form, or user. Click on a row to view the form, visit, category, description, value, and the user who entered the data.

DATA QUERIES	Site List	SITE: 1999 - Non-VA Test	Participants	Forms	Users
---------------------	-----------	--------------------------	---------------------	-------	-------

Search	
--------	--

PID	First Name	Last Name	Query Count
100000118	John	Smith	5
100000541	Minerva	McGonagall	19
100000703	Adam	Test	1
100000894	Eva	Iuna	14

To open the form where the data query needs to be addressed, click anywhere in the row for that query.

DATA QUERIES	Site List	SITE: 1999 - Non-VA Test	Participants	←	PID: 100000118	Forms	Users
Search							
STUDY	FORM	VISIT	CATEGORY	DESCRIPTION	VALUE	ENTERED BY	
Clinic	Contact Information	CONT	MISSING	MISSING - 24. Do you currently live in the same household as other PREVENTABLE participant?			Verify
Clinic	Lab Blood Collection	CL00	MISSING	MISSING - 1. Were any samples obtained expected.			Verify

Queries will appear as warnings the top of the page. If the data is updated and the form is resaved, the query will be resolved. if the data is already accurate and does not need to be updated, return to the Data Query System and verify it is accurate by clicking the blue “Return to Data Query System” button and then the yellow “Verify” button.

Some queries cannot be verified or resolved unless the data is updated.

FORM DATA WAS SAVED -
[Return to Data Query System](#)

WARNINGS - click row to jump to question.

- MISSING - 1. Cardiovascular - Heart Failure expected
- MISSING - 2. Cardiovascular - Atrial Fibrillation (AFib) expected
- MISSING - 3. Cardiovascular - Valvular Heart Disease expected
- MISSING - 26.b Most recent HbA1c is expected
- MISSING - 28.b Most recent AST expected

After clicking “Verify,” the user will be asked to confirm they are sure. The query will be resolved after the user clicks “Ok.”

preventabletrial.org says

Are You Sure?

OK

Cancel

13.1c SITE MONITOR PAGE

NOTE: Only CRAs, site monitors, and Hub leadership for VA and non-VA sites can access the Site Monitor page. On the study website, click Data > Monitor Page.

The Monitor Page shows alerts by site that require action by a site monitor. Examples of monitor alerts include, but are not limited to, approval for a participant to permanently discontinue the study drug, approval for a participant to fully withdraw from the study, review a reported protocol deviation, and review a new Death Form.

MONITOR PAGE

Monitor Alerts

Site Alerts

Category	Count
A New Death Form Has Been Entered	20
Approval Needed for Full Study Withdrawal	5
A New Protocol Deviation Has Been Entered	17
Approval Needed for Permanently Discontinuing Study Drug	8

Click on a category to view a list of participants who have an alert related to the category, and a summary of the action needed to resolve the alert.

MONITOR PAGE

Monitor Alerts

Site Alerts

Search

Approval Needed for Full Study Withdrawal

The below site(s) have entered requests to fully withdraw a participant from the study. To resolve the alert, take the following steps:

1. Click on the participant from the list below and then navigate to the 'Comments' tab of that participant's page to read any comments the site has entered in about the full withdrawal request.
2. Discuss with the coordinator or site staff (via email, phone or web conference) if the site attempted to negotiate retention (ex: only following the EHR data, just doing a final study phone call instead of yearly calls, etc.).
3. Determine with the site if the full study withdrawal request is the appropriate action for this participant.
4. Navigate to the participant's information tab on their participant page and scroll down to the retention section. Select either the green Approved or red Not Approved next to the full withdrawal requested retention status. The alert will resolve once you have selected one of those options.
5. If the request is 'Not Approved' the site will be alerted to update the retention status to reflect the appropriate retention that was agreed upon between the site, the participant, and the monitor.

Days Open	HUB	Site ID	Site Name	PID	Participant Name
21		1999	Non-VA Test	900007494	Oscar(3) Blair

13.1d PROTOCOL DEVIATION LOG

Reportable protocol deviations are entered in the study website under Data > Site Management > Protocol Deviations. The page will open to a list of all previously reported deviations.

SITE MANAGEMENT:

Site List

SITE: 1990 - Non-VA Turned On

Protocol Deviations

Mailing Lists

Payment Letters

Add New Deviation

	Reference Number	Deviation Date	Study	Type	Deviation Description	Corrective Action Taken	Date reported to sIRB/cIRB	Site Monitor Initials
	1099	04/09/2023	Main Study	Informed Consent Procedures	"I did do something bad"			
🔒	1088	02/01/2023	Main Study	Other	I enrolled someone I shouldn't	I'm very sorry	02/02/2023	LHP

To enter a new protocol deviation, click the blue “Add New Deviation” button. Do not enter special characters into any text field. Characters that are accepted are: A-Z, 0-9, and '!,.(). Complete all fields on the Protocol Deviation Form and click “save” once the entire form is complete.

Date of Deviation:

Study:

Deviation Type:

Deviation Description (include PIDs, if applicable):

Corrective Action Taken:

Corrective Action Comments:

_____ To be completed by Site Monitor _____

The site monitor will be notified when a protocol deviation is entered. The CRA, site manager, or IRB contact will contact the person who submitted the protocol deviation if they have questions or if they need additional information. The site monitor will lock the form after reviewing and reporting the protocol deviation to the sIRB or CIRB. No changes will be able to made to any forms that are locked.

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CHAPTER 14. SAFETY MONITORING AND REPORTING

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

Safety events in PREVENTABLE are collected only from electronic health record (EHR) data queries. The Data and Safety Monitoring Board (DSMB) will monitor participant safety and review the performance of the study. The DSMB will review aggregate safety events, primary and secondary endpoints, reasons for stopping the study drug, hospitalizations, events of special interest, and deaths. The DSMB will review rates in each treatment group (atorvastatin 40 mg vs placebo) to inform their recommendation to stop or continue the study or modify the protocol, considering the nature of the outcomes and lag time for data accrual. Details on DSMB processes can be found in the DSMB Charter.

Please note that deaths, hospitalizations (all), and hospitalizations for myocardial infarction, stroke, heart failure, and coronary revascularization are collected by data curation. These study outcomes are secondary endpoints but not unanticipated events. There is no need to report these directly. However, site staff should complete a Death Form with all available details as appropriate.

14.1 UNANTICIPATED PROBLEM (UP)

The Office for Human Research Protections (OHRP) considers unanticipated problems involving risks to participants or others to include, in general, any incident, experience, or outcome that meets all the following criteria:

- Unexpected in terms of nature, severity, or frequency given (a) the research procedures that are described in the protocol-related documents, such as the Institutional Review Board (IRB)-approved research protocol and informed consent document; and (b) the characteristics of the population being studied;
- Related or possibly related to participation in the research (“possibly related” means there is a reasonable possibility that the incident, experience, or outcome may have been caused by the procedures involved in the research); and
- Suggests that the research places participants or others at a greater risk of harm (including physical, psychological, economic, or social harm) than was previously known or recognized.

14.1.1 REPORTING REQUIREMENTS (NON-VA SITES)

Site investigators at non-VA sites are responsible for reporting unanticipated problems (Ups) for their participants within 5 business days of discovering the event. Sites should contact their Clinical Research Associate (CRA) to receive instructions on UP reporting. Sites should follow guidelines from the sIRB and their local IRB’s reporting instructions.

14.1.2 REPORTING REQUIREMENTS (VA SITES)

Site investigators at VA sites are responsible for notifying the VA Central IRB (CIRB) of any UPs, deaths, or protocol deviations for the duration of the study. All UPs, deaths, and protocol deviations should be reported to the VA CIRB in accordance with [VHA Directive 1058](#) regulations and the VA CIRB Table of Reporting Requirements, available under Important Links on the [VA CIRB website](#). Sites should contact the Boston Coordinating Center at VAPREVENTABLEnetwork@va.gov to receive instructions on CIRB-

required reporting. Sites should use the following CIRB event reporting forms, available in the IRBNet Forms Library:

- **VA Central IRB Form 124 – Reportable Events:**
 - Reporting of Research Death that is both unanticipated and related or possibly related
 - Unanticipated Problem Involving Risks to Subjects or Others (UPIRSTO) that is both unanticipated and related or possibly related
- **VA Central IRB Form 115d – Protocol Deviation Log:** Protocol deviations, violations, and/or noncompliance

14.2 EVENTS OF SPECIAL INTEREST (EOSI)

Events of special interest are those possibly related to use of atorvastatin. Events of special interest will be captured only from EHR data transfers. If a site is notified of an event of special interest, no additional action is required by the site. Collection of EOSI solely from the EHR, as opposed to reporting by the clinic staff, ensures consistent and unbiased collection of these events.

Participants' laboratory data, including glucose, HbA1C (if available), creatine kinase, liver enzymes, specifically aspartate aminotransferase and alanine aminotransferase, will also be collected from EHR data transfers.

14.2.1 NEW DIABETES

Diabetes surveillance will include glucose and HbA1c (if available) or new diagnosis codes for diabetes.

14.2.2 HEPATIC FAILURE

Surveillance for hepatic failure will include hospitalization for hepatic failure or significant elevation of liver enzymes.

14.2.3 MYOSITIS

Myositis surveillance will include laboratory data on creatine kinase and aspartate aminotransferase/alanine aminotransferase or hospital or emergency room visit diagnoses for injurious falls, myositis, or elevated CK in the absence of an MI diagnosis.

14.2.4 NEW CANCER

Cancer surveillance will include incident cancer diagnosis classified by type as incident cancer is a competing event of interest.

14.3 PHQ-8 ALERT

Sites will receive an immediate Participant Issues alert on the study website, "PHQ-8 score greater than 14," if a participant at their site scores above a 14 when the Call Center administers the PHQ at either baseline or annual calls. The alert instructs site staff to confirm the participant has given permission for the site to report any changes in cognitive status to their primary care doctor. Sites should review the PHQ answers prior to contacting the PCP or participant. See **Appendix 14.1** for instructions to address and resolve PHQ-8 alerts.

- If the participant consented to allow reporting to their PCP, the PI should send a High PHQ-8 Score letter to the participant's PCP. A template is provided on the study website under Quick Links > Documents > Enrolled Participants Resources > High PHQ-8 Score PCP Letter Template, available in English and Spanish.
- If the participant did NOT consent to allow reporting to their PCP, the PI should contact the participant to advise on seeking medical follow-up.

Site staff can also view a report of all participants at their site who scored higher than a 14 on the PHQ. The report is available on the study website under Login > Quick Links > Reports > Site Specific Reports > PHQ-8 > Select site and click "Run Report."

The Clinical Coordinating Center (CCC), Hub lead staff, and clinical sites can also access the PHQ-8 report on the study website under Menu > Reports > Site Specific Reports > PHQ-8 Alerts.

APPENDIX 14.1 ALERTS FOR PHQ-8 SCORE GREATER THAN 14

The “PHQ-8 score greater than 14” alert will appear if the Call Center has administered the PHQ-8 depression screening to a participant and that participant has scored a 14 or higher, which could indicate possible depression or depressive symptoms.

2	PHQ-8 score greater than 14 - Baseline
----------	---

NOTE: It is very important that the clinic follow the Action Needed directions at the top of the participant listing for this alert as soon as possible.

PHQ-8 score greater than 14 - Baseline

ACTION NEEDED:
The below participant(s) registered a high score (greater than 14) on the Patient Health Questionnaire 8 (PHQ-8) administered by the Call Center at the Baseline Visit.

Please confirm that the participant has consented to allowing reported changes in their cognitive status to their doctor (found on the participant's dashboard in the "consent" tab) and then have the site PI send the templated letter (found on the study website under Documents - Enrolled Participant Resources - High PHQ Score PCP Letter Template) to the participant's PCP. Enter a comment with the date that the letter was sent.

Once these actions are completed, click the RESOLVE button in the below listing.

PID	Name	Date PHQ Completed	Visit	PHQ Score	PHQ Answers	Comments	Click to Resolve
900000058	Harry Potter	08/07/2020	CC00 - Baseline Call Center	22	View PHQ Info	2 - Comments	RESOLVE

To confirm if a participant has allowed for the clinic to report the PHQ-8 score to their PCP, navigate to the Consent tab in the participant's dashboard on the study website and verify the participant answered “YES” to that question.

Consent

Information

SSN & MBI

Visit List

INFORMED CONSENT
Open REDCap eConsent

Date of Consent:

Allowing Use of Your Genetic Sample for Research?

Allowing Report of Cognitive (Memory /Thinking) Changes to Your Doctor?

08-19-2020

YES

YES

To resolve the alert, the PI or their designee will need to send the High PHQ-8 Score letter to the participant's PCP. A template is provided on the website under Quick Links > Documents > Enrolled Participant Resources > High PHQ-8 Score PCP Letter Template. If the participant did NOT consent to allow reporting to their PCP, the PI should contact the participant to advise on seeking medical follow-up. Once one of these actions have been taken, the site staff can open the alert and click the green “Resolve” button and the alert will disappear. Site staff should **not** click the “Resolve” button until the Action Needed steps in the alert have been completed.

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CHAPTER 15. REAL-WORLD DATA SOURCES FOR STUDY OUTCOMES AND CAUSE OF DEATH

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

15.1 OVERVIEW

PREVENTABLE is a pragmatic clinical trial that uses multiple real-world data (RWD) sources to determine study outcomes and support study operations. RWD data sources include:

- The database on the PREVENTABLE website where data is entered by study teams
- Electronic health record (EHR) data from the National Patient-Centered Clinical Research Network (PCORnet)
- Non-VA/non-PCORnet site EHRs
- Veterans Health Administration (VHA) EHRs
- Medicare
- National Death Index (NDI)
- Other sources which may become available during or after the study

Sites participating in PREVENTABLE include VHA sites using their common EHR; PCORnet sites using a [Common Data Model \(CDM\)](#); and non-VA/non-PCORnet sites that provide EHR data that is transformed into a PCORnet CDM, thereby allowing the data team to run queries and thus are treated as "PCORnet" for the purposes of this document.

15.2 PCORNET DATA

Data collected as part of routine clinical care and recorded in participants' EHR will be queried twice a year to determine outcomes. These data include all encounters, diagnoses, procedures, medication orders, dispensed medications (as available), lab orders and results, and information on hospitalizations and death.

All sites participating in PCORnet have created a PCORnet DataMart (DM) based on their EHR data. PCORnet sites extract, transform, and load their EHR data to this DM in accordance with the structure described by the CDM. The CDM provides standard terminology, value sets, and implementation guidance to allow clinical research to be conducted across all PCORnet sites in a standard and efficient way. If non-VA site staff are unsure if their site is part of PCORnet, they can contact Darcy Louzao (darcy.louzao@duke.edu) at the Clinical Coordinating Center to discuss.

Assessing data quality and identifying opportunities for continued improvement is a key tenet of PCORnet and is accomplished, in part, by data curation. Data curation occurs on a quarterly basis, whereby sites update their DMs and are assessed on a number of data checks. There are some data checks that must be passed and others where exceptions are permitted, with explanation. PCORnet data undergo data curation in January, April, July, and October, with data expected to be complete with no more than a 2-month lag. For example, in a July refresh, data are expected to be complete through at least May. Non-VA sites that are not part of PCORnet are expected to provide local data using similar processes and timelines.

15.2.1 PCORNET DATA ACQUISITION

PREVENTABLE biannual queries are sent after the April and October data curation cycles. A SAS query is sent to all PCORnet site data administrators from the PCORnet Coordinating Center at the Duke Clinical Research Institute (DCRI) via the standard PCORnet data transfer mechanism (PopMedNet). The anticipated request and data availability schedule is shown below.

PCORnet Query Date	Includes Data Through	Expected Availability at DCC
Apr-2021	Jan-2021	Aug-2021
Oct-2021	Jul-2021	Feb-2022
Apr-2022	Jan-2022	Aug-2022
Oct-2022	Jul-2022	Feb-2023
Apr-2023	Jan-2023	Aug-2023
Oct-2023	Jul-2023	Feb-2024
Apr-2024	Jan-2024	Aug-2024
Oct-2024	Jul-2024	Feb-2025
Apr-2025	Jan-2025	Aug-2025
Oct-2025	Jul-2025	Feb-2026
Apr-2026	Jan-2026	Aug-2026
Oct-2026	Jul-2026	Feb-2027
Apr-2027	Jan-2027	Aug-2027
Oct-2027	Jul-2027	Feb-2028

15.2.2 PCORNET DATA FLOW

Query results are submitted to the DCRI, reviewed, and remediated as needed. Data are enhanced (addition of code descriptions, addition of a pre/post randomization indicator, etc.) then transferred to Wake Forest University School of Medicine (WFUSM) via sFTP for post-processing and preparation of the analysis datasets.

15.2.3 SITE RESPONSIBILITIES IN THE ACQUISITION OF PCORNET DATA

Study teams at PCORnet sites participating in PREVENTABLE play a critical role in the acquisition of PCORnet data for the study. To identify PREVENTABLE participants within a site's PCORnet DataMart, a special table, called the PCORNET_TRIAL table, that associates PREVENTABLE participant identifiers with local patient identifiers, must be created and maintained by each site. The general process of creating this table is described below.

15.2.3.1 SITE CROSSWALK

For each participant at their site, the site study coordinator must develop and maintain a study-specific crosswalk between the PREVENTABLE Participant ID (PID) assigned by the Data Coordinating Center (DCC) and a site-specific identifier that uniquely identifies that person (e.g., medical record number). This crosswalk allows the site's PCORnet data administrator to identify the correct person within their PCORnet DataMart, which uses a pseudo-identifier called the PATID for patient-specific identification.

15.2.3.2 STUDY TABLE POPULATION

General information about the expected structure and contents of the PCORNET_TRIAL table can be found in the [PCORnet CDM specifications](#). Each site must populate and maintain this table within their PCORnet DataMart to ensure all appropriate participant data is returned when queried by DCRI. The PCORNET_TRIAL table should include all participants who ever enrolled in the study, including those who have withdrawn from the study.

The information needed to populate the PCORNET_TRIAL table is shown below.

Field	Origin	Description
TRIALID	Assigned by the WFUSM team and registered with the PCORnet Coordinating Center	"PREVENTABLE"
TRIAL_SITEID	Assigned by the WFUSM team	Study-specific site identifier
PARTICIPANT ID	Assigned by the WFUSM team	The unique identifier generated for each participant in the study.
PATID	Assigned by the Study Site's CDM team using a crosswalk file provided by the WFUSM team	The unique participant identifier in the CDM.
TRIAL_ENROLL_DATE	Assigned by the WFUSM team	The date the participant is randomized in PREVENTABLE.

TRIAL_END_DATE	Assigned by the WFUSM team	The date the participant completes the study. Must be internally consistent with other CDM dates for the same participant. Is participant-specific based on informed consent language.
TRIAL_WITHDRAW_DATE	Assigned by the WFUSM team	The date the participant withdraws full consent (i.e., does not allow passive follow-up of data) to take part in the study.

WFUSM provides each site with a structured file that includes all the relevant information for participants at their site, *except for the PATID*. Using the crosswalk table, the site's PCORnet data administrator must complete this table by assigning the appropriate PATID value to each PARTICIPANTID value.

15.2.3.3 DATA CHECKS

Upon receipt of the PREVENTABLE PCORnet query results, the PCORnet Coordinating Center will perform data quality checks. Below are examples of potential data check issues, investigation needed, and study team members involved in resolving the issue.

Issue	Investigation	Study Coordinator	Data Administrator
Number of participants in the query does not match site enrollment information from the PREVENTABLE DCC	Confirm crosswalk and identify if additional participants need to be included.	X	
Data query failed to execute properly	Identify execution issues, remediate, and rerun the query.		X

15.3 VETERANS HEALTH ADMINISTRATION (VHA) DATA

The PREVENTABLE VA Coordinating Center (VA CC) collects data for VA sites. The VA CC will query VA participants' medical records in the Computerized Patient Record System (CPRS) and will focus on data collected as part of routine clinical care including all encounters, diagnoses, procedures, medication orders, dispensed medications (as available), lab orders and results, and information related to hospitalizations and death.

To ensure data quality and validate the outcomes data collected, the VA CC will access randomly selected EHRs of participants through the Compensation and Pension Record Interchange, CAPRI/VistAWeb. Data elements, including specific study outcomes detailed in the protocol, will be examined by the VA CC to determine the accuracy of reports. The VA CC will also work with the DCRI and WFUSM to troubleshoot any potential discrepancies between Medicare data and VA-reported data.

15.3.1 VHA DATA ACQUISITION

The VA CC queries VA participants' EHR data for outcomes 4 times a year. The data collected is reported to WFUSM bi-annually following the same schedule as non-VA Sites provided in **Section 15.2.1**.

15.3.2 VHA DATA FLOW

All data for post-processing and preparation of the analysis datasets is sent from the VA to WFUSM through secure VA-approved encryption methods.

15.3.3 VA SITE RESPONSIBILITIES IN THE ACQUISITION OF VHA DATA

While the VA CC collects data for VA sites, site staff may be asked to further investigate a reported event or provide more information about a participant's outcome (e.g., specific cause of death) in situations where this information cannot be determined from EHR queries.

Although the VA CC receives randomized participant information from the WFUSM, the VA CC maintains the right to request that each VA site provides the following information on enrolled participants: full name, date of birth, full Social Security number, and assigned study identification number (SID). If requested, VA sites will provide data to the VA CC via secure VA-approved encryption methods.

15.4 MEDICARE DATA

In the United States, the Medicare program, administered by the Centers for Medicare & Medicaid Services (CMS), covers the health insurance needs of nearly all Americans 65 years and older.

CMS provides multiple files of data from the Medicare program to researchers:

- **Master Beneficiary Summary File (MBSF):** Medicare program eligibility and enrollment, as well as date of death, where applicable
- **Claims files:** healthcare visits and services reimbursed through the Medicare fee-for-service program (Parts A & B)
- **Encounter files:** healthcare visits and services provided through the Medicare managed care program (Part C)
- **Prescription drug event files:** prescription medications dispensed through a Medicare drug plan (Part D)
- **Medicare Provider Analysis and Review (MEDPAR) files:** hospital inpatient services

In September 2024, about half of Medicare beneficiaries were enrolled in the fee-for-service plan while the other half were enrolled in a managed care plan. Around 80% of Medicare beneficiaries were enrolled in a Part D plan.

All beneficiaries have information in the MBSF, but will only have claims or encounter data in the files corresponding to their program enrollment. This is important because while CMS has been making the fee-for-service program claims data available for decades, the managed care encounter data has only been made available recently and is not yet timely.

Relevant to the PREVENTABLE study, information about mortality is available for all study participants enrolled in any Medicare program. The Social Security Administration is the primary source of death dates in the Medicare data, although CMS does initiate a confirmation process for deaths reported in

claims. Confidence in mortality based on Medicare data is very high, as all death dates in these data are validated. Information about hospitalization events is available for all study participants in the Medicare Provider Analysis and Review (MEDPAR) file, which contains records for 100% of Medicare beneficiaries who use hospital inpatient services.

CMS provides comprehensive annual files for each of the file types listed above. Annual Medicare files are considered 100% complete. The MBSF, fee-for-service claims files, and prescription drug event files are typically available for research 12–14 months after the end of the calendar year. The encounter files for managed care are typically available 3 years after the end of the calendar year.

CMS also provides more timely but incomplete quarterly files for the MBSF and claims files. The quarterly claims files are considered approximately 93% complete and are typically available for research 6 months after the end of the quarter. Part D files are not available as quarterly files.

15.4.1 MEDICARE DATA ACQUISITION

Sites are not involved in the collection of Medicare data. This process is managed by the Data Coordinating Centers at DCRI and WFUSM. Also, because the PREVENTABLE informed consent form for the main study includes working covering the acquisition and use of study participants' Centers for Medicare & Medicaid Services (CMS) data, no additional consent is required to collect CMS data.

Medicare data for PREVENABLE participants will be obtained, free of charge, through the national Institute of Aging (NIA) LINKAGE Program (LINKAGE). This program a) links PREVENTABLE study data with data from CMS, and b) provides a cloud-based environment for data analysis. Use of this program requires a CMS Data Use Agreement (CDA). A DUA for PREVENTABLE was approved by CMS in Q4/2024.

The WFUSM DCC submitted an initial finder file to LINKAGE for crosswalk generation and data extraction in Q1/2025. As participants continue to be recruited into PREVENTABLE, the DCC will submit cumulative finder files to LINKAGE annually. Finder file include personally identifiable information (PII) required for linkage, including name, date of birth, sex, address, Social Security number (SSN), and Medicare Beneficiary Identifier (MBI).

Data requested for extraction from LINKAGE after each finder file submission will include the most updated version of the following files:

File name	File Description
Master Beneficiary Summary File (MBSF): Base – Segment (A/B/C/D)	The MBSF-Base Segment file contains beneficiary demographic, eligibility, and enrollment information for Medicare Parts A, B, C, and D
Carrier Claims	The Medicare carrier claims file contains health services information (e.g., dates of service, diagnoses codes, and procedure codes) from professional non-institutional providers (e.g., individual/group practitioners, non-hospital labs, or physicians) for Medicare beneficiaries.

Durable Medical Equipment Claims	The Medicare durable medical equipment claims file contains health services information on items (e.g., prosthetic device, ventilator, and wheelchair) that are medically necessary and doctor-prescribed to Medicare beneficiaries. DME suppliers submit these claims.
Home Health Claims	The Medicare home health claims file contains health services information (e.g., diagnoses codes, discharge status, and service costs) from agencies that provide home healthcare services (e.g., skilled care, home health aide, and physical therapy) to Medicare beneficiaries.
Hospice Claims	The Medicare hospice claims file contains health services information (e.g., procedure codes, provider characteristics, and service costs) from healthcare providers or facilities that offer care to terminally ill Medicare beneficiaries.
Inpatient Claims	The Medicare inpatient claims file contains health services information (e.g., diagnose codes, drug codes, and procedure codes) from overnight inpatient hospital stays for Medicare beneficiaries.
Outpatient Claims	The Medicare outpatient claims file contains health services information (e.g., beneficiary demographics, procedure codes, and processing codes) from institutional outpatient providers (e.g., outpatient rehabilitation facilities and renal dialysis facilities) for Medicare beneficiaries.
Skilled Nursing Facility Claims	The Medicare skilled nursing facility claims file contains health services information (e.g., diagnoses codes, facility type, and processing codes) from facilities that provide skilled nursing and therapy care to Medicare beneficiaries.
Medicare Provider Analysis & Review (MedPAR)	The MedPAR file contains health service information (e.g., eligibility status, processing codes, and service costs) from two Part A settings—Inpatient (IP) and Skilled Nursing Facility (SNF)—organized into stays, where 'stay' refers to the time between a beneficiary's admission to an IP or SNF facility and the beneficiary's discharge from the facility.
Carrier Encounters	The Medicare carrier encounter file contains health services information (e.g., dates of service, diagnoses codes, and procedure codes) from professional non-institutional providers (e.g., individual/group practitioners, non-hospital labs, or physicians) for Medicare beneficiaries enrolled in Medicare Advantage plans.

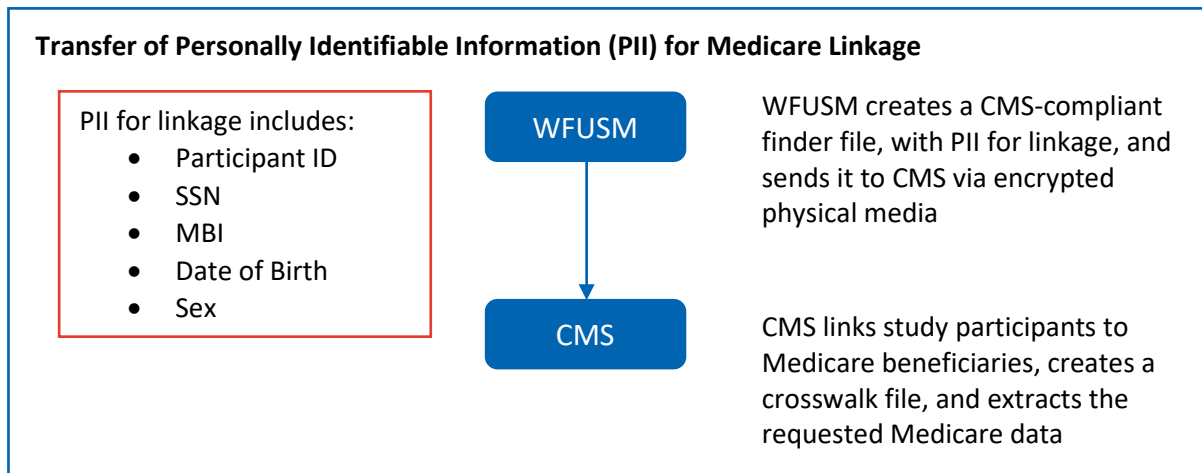
Durable Medical Equipment Encounters	The Medicare durable medical equipment encounter file contains health services information on items (e.g., prosthetic device, ventilator, and wheelchair) that are medically necessary and doctor-prescribed to Medicare beneficiaries enrolled in Medicare Advantage plans. DME suppliers submit these claims.
Home Health Encounters	The Medicare home health encounter file contains health services information (e.g., diagnoses codes, discharge status, and service costs) from agencies that provide home healthcare services (e.g., skilled care, home health aide, and physical therapy) to Medicare beneficiaries enrolled in Medicare Advantage plans.
Inpatient Encounters	The Medicare inpatient encounter file contains health services information (e.g., diagnose codes, drug codes, and procedure codes) from overnight inpatient hospital stays for Medicare beneficiaries enrolled in Medicare Advantage plans.
Outpatient Encounters	The Medicare outpatient encounter file contains health services information (e.g., beneficiary demographics, procedure codes, and processing codes) from institutional outpatient providers (e.g., outpatient rehabilitation facilities and renal dialysis facilities) for Medicare beneficiaries enrolled in Medicare Advantage plans.
Skilled Nursing Facility Encounters	The Medicare skilled nursing facility encounter file contains health services information (e.g., diagnoses codes, facility type, and processing codes) from facilities that provide skilled nursing and therapy care to Medicare beneficiaries enrolled in Medicare Advantage plans.
Part D Prescription Drug Events	The Medicare Part D prescription drug event file contains information about outpatient prescription drug fills for Medicare beneficiaries with Part D coverage.

15.4.2 MEDICARE DATA FLOW

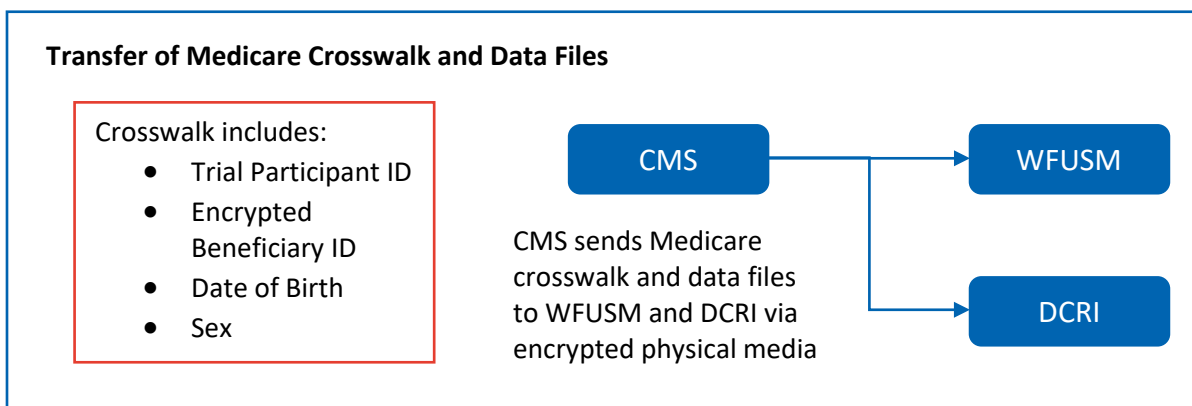
The process of developing and sending a finder file with PREVENTABLE participant identifiers to the Medicare data distributor (CMS) is described below.

1. The DCC at WFUSM sends a finder file to LINKAGE via secure data transfer.
2. LINKAGE uses the finder file to identify study participants and create a crosswalk from the study identifiers to study-specific Medicare identifiers.
3. LINKAGE extracts the requested Medicare data for the linked participants.

4. LINKAGE makes the Medicare crosswalk and extracted data files available to WFUSM and DCRI personnel within the LINKAGE data enclave, a secure cloud-based environment.



After the data is accessible, DCRI provides guidance and programming support to WFUSM regarding the use of the data.



15.5 NATIONAL DEATH INDEX (NDI) DATA

The National Death Index is a centralized database of death records in state vital statistics offices, administered by the National Center for Health Statistics within the Centers for Disease Control and Prevention (CDC). It is considered a definitive source of mortality in the United States.

PREVENTABLE acquires data from NDI Plus, which includes date of death and cause of death. Similar to the Medicare data release schedule, the CDC offers an early release of NDI data that is over 90% complete. Early release files are typically available within 3 months of the end of the calendar year. For example, the 2025 NDI early release file should be available in March 2026. The NDI final release file that is considered 100% complete is typically available approximately 15 months after the end of the calendar year. For example, the 2025 NDI final file should be available in March 2027.

15.5.1 NDI DATA ACQUISITION

Sites are not involved in the collection of NDI data. This process is managed by the Data Coordinating Centers at DCRI and WFUSM.

NDI data is acquired according to the application processes established by the CDC. The DCRI takes primary responsibility for this application process.

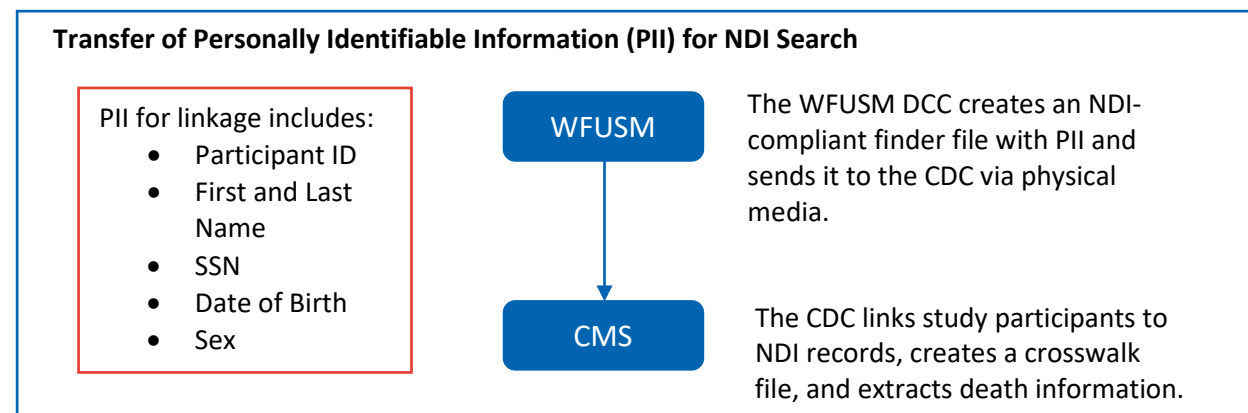
When the request is approved, the WFUSM DCC submits a finder file to the CDC to search the NDI for PREVENTABLE participants. The finder file includes the personally identifiable information (PII) required for linkage—Social Security number (SSN), first name, last name, date of birth, and sex. See the [NDI User Guide](#) for details about the structure of the finder file. The DCC sends one finder file per year for all enrolled participants.

At each linkage, the DCC requests a search of the most recent data that became available since the prior search. The contents and timing of these searches are shown in the table below.

Search #	Data Through	Request Initiated	Finder File Sent	Data Available at DCC
1	Dec-2021	Feb-2022	May-2022	Aug-2022
2	Dec-2022	Feb-2023	May-2023	Aug-2023
3	Dec-2023	Feb-2024	May-2024	Aug-2024
4	Dec-2024	Feb-2025	May-2025	Aug-2025
5	Dec-2025	Feb-2026	May-2026	Aug-2026

15.5.2 NDI DATA FLOW

The process of developing and sending a finder file with PREVENTABLE participant identifiers to the CDC is described below.



The CDC returns NDI search results to WFUSM DCC, and WFUSM sends a copy of the NDI data to DCRI. After the data is received, DCRI provides guidance and programming support to WFUSM regarding the use of the data.

15.6 REAL-WORLD DATA FOR STUDY OUTCOMES

15.6.1 PRIMARY ENDPOINT

Real-world data (RWD) helps determine survival, a component of the primary outcome of survival free of new dementia or disability. EHR data have some information about mortality, often limited to deaths that occurred within a covered healthcare setting, Medicare has complete mortality data for all beneficiaries, and the National Death Index contains comprehensive data reported by state vital statistics systems.

RWD does not directly inform the determination of new dementia or persistent disability, but it is used as part of the overall adjudication process and may trigger a follow-up call if the data indicates specific diagnoses related to dementia or disability. Additionally, these data may be used by the Primary Endpoint Adjudication Committee to understand other healthcare utilization associated with potential endpoints.

The *International Classification of Diseases, Tenth Revision, Clinical Modification* (ICD-10-CM) diagnosis codes are used to identify potential dementia or disability.

15.6.2 SECONDARY ENDPOINTS

RWD is the primary source of information for the secondary cardiovascular (CV) endpoint, which is a composite of cardiovascular death, hospitalization for myocardial infarction (MI), hospitalization for unstable angina (UA), hospitalization for heart failure (HF), hospitalization for stroke, or a coronary revascularization procedure. All components of this CV endpoint, except hospitalization for heart failure, are also part of the exploratory CV endpoint. RWD is also the primary source for cancer-related death and heart failure subtype, where applicable. RWD does not directly inform the determination of mild cognitive impairment (MCI) or probable dementia, the other secondary endpoint.

15.6.2.1 CAUSE OF DEATH DETERMINATION

Death status is updated via the Death Report Form on the study website, Medicare beneficiary status change, death recorded in the EHR, or National Death Index record. If the DCC first learns about the death of participant via Medicare, EHR, or NDI data, the site will receive an alert on the study website to complete the Death Report Form.

Cardiovascular (CV) cause of death is primarily determined using information from the NDI and from the Death Report Form and is supplemented by hospitalization records from claims and EHR data within 90 days of the death date in cases of disagreement. Non-CV cause of death is further divided into non-CV (malignancy) cause of death and non-CV (other) cause of death subgroups.

The process for determining the cause of death for study participants involves 2 steps. First, information from each data source is classified appropriately:

- In the NDI data, cause of death is classified as CV if the underlying cause of death code has the first three positions between I00 and I99, and as cancer-related if the underlying cause of death code has the first three positions between C00 and D49.

- On the Death Report Form, site staff will classify the cause of death as CV, non-CV (malignancy), or non-CV (other). Site staff will also complete the death narrative on the Death Report Form to provide additional details if available.
- In the claims and EHR data, hospitalization records are classified as CV-related if they have a principal diagnosis with the first three positions between I00 and I99, and are classified as cancer-related if they have a principal diagnosis with the first three positions between C00 and D49.

Second, information from these different sources are reconciled as follows to classify the cause of death as CV, non-CV (malignancy), or non-CV (other):

- If only one source of cause of death is available, the death is classified based on that source.
- If there is agreement between the cause of death in the NDI and on the Death Report Form, the death is classified as determined.
- If there is a discrepancy between the cause of death in the NDI and on the Death Report Form, the final cause of death is determined by a panel of 3 physicians. The panel will review all available information including the NDI data, Death Report Form (including death narrative), and hospitalization records within 90 days of the date of death, and decide on the final classification of cause of death. No additional source documents from the sites are required.

15.6.2.2 HOSPITALIZATION-BASED EVENTS

EHR data and Medicare data each contribute to the hospitalization-based components of the CV endpoint—hospitalization for myocardial infarction (MI), hospitalization for unstable angina (UA), hospitalization for heart failure (HF), and hospitalization for stroke. These endpoints are identified by looking for specific diagnosis codes in the principal diagnosis position on a record or claim associated with an inpatient or hospital encounter. Observation stays that do not result in admission to a hospital are not considered inpatient encounters.

Codes for MI, HF, and stroke events align with the CMS Hospital Compare definitions used to identify cohorts for their mortality measure. See the [CMS Mortality Measures Methodology web page](#) for complete specifications associated with this program. The DCC will monitor this web page throughout study follow-up for changes to these code lists.

Of note, the CMS MI code list excludes Type 2 myocardial infarctions, and the CMS stroke code list includes only ischemic strokes.

15.6.2.3 CORONARY REVASCULARIZATION

EHR data and Medicare data each contribute to the coronary revascularization component of the CV endpoint. These endpoints are identified by looking for specific procedure codes in any position on any record or claim associated with a healthcare encounter in any setting. These codes cover the following procedures and related procedures: coronary artery bypass graft (CABG), coronary angioplasty, coronary atherectomy, and coronary stenting.

15.6.2.4 HEART FAILURE SUBTYPE

For study participants with heart failure, any information from the available RWD sources may be used to classify heart failure into subtypes. This information may include ejection fraction results and

diagnosis codes indicating HF subtypes. The HF subtypes, defined by ejection fraction, are provided in the table below:

HF Subtype	Ejection Fraction Range
Heart failure with reduced ejection fraction (HFrEF)	<40%
Heart failure with mid-range ejection fraction (HFmrEF)	40–50%
Heart failure with preserved ejection fraction (HFpEF)	>50%

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CHAPTER 16. ADJUDICATION OF DEMENTIA, MCI, AND DISABILITY

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

16.1 ADJUDICATION OVERVIEW

A primary goal of PREVENTABLE is to ascertain incident cases of all-cause dementia and determine whether the incidence is related to the treatment assignment. Since the PREVENTABLE intervention could impact the incidence of sub-clinical or pre-dementia syndromes, Mild Cognitive Impairment (MCI) will also be assessed. Participants not classified as probable dementia or MCI will be considered to have no cognitive impairment.

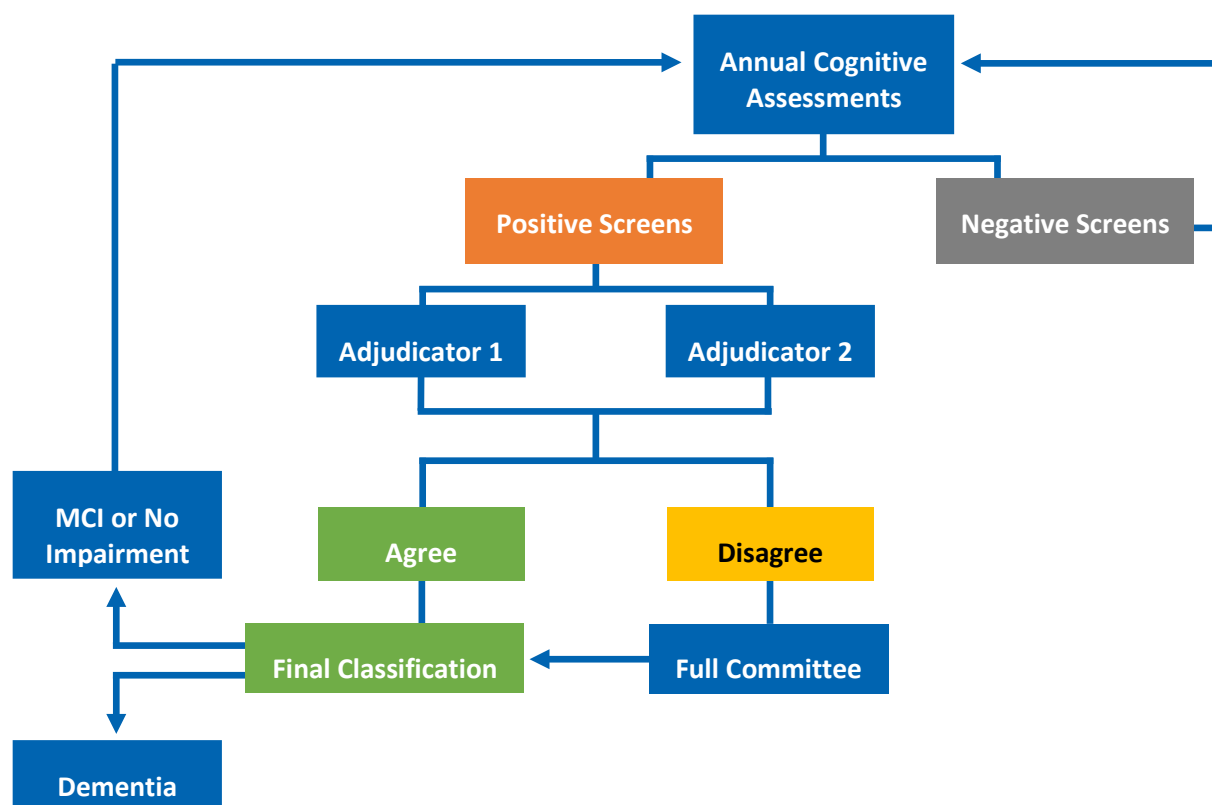
Adjudication is based on information collected in PREVENTABLE including cognitive assessments via phone, the Functional Assessment Questionnaire (FAQ) completed by a Trusted Contact, as well as demographic information, medical history, medications other than study drug, depressive symptoms, and reports of serious adverse events including the date, type of event, and diagnoses. To evaluate the significance of measured cognitive deficits and changes over time, adjudicators will consult appropriate norms for each cognitive test. An individual participant’s change in test performance over time will also be considered in adjudication.

16.2 ADJUDICATION PROCEDURES

Classification of Probable Dementia, MCI, or No Impairment in PREVENTABLE is made by an Adjudication Panel of experts in the diagnoses of dementia and MCI. Adjudicators will remain masked to study drug assignment, study incidence rates, and other information that might bias their evaluations. The steps in the adjudication process are:

1. The initial pool of potential cases will include all participants where the modified Telephone Interview for Cognitive Status (TICS_m) test scores triggered the administration of the extended cognitive battery and the FAQ from a Trusted Contact.
2. Additional cases are added if a participant triggers the completion of a Dementia Questionnaire (DQ) by a Trusted Contact either in cases of participant death or if a participant is unable to complete an annual call.
3. Two adjudicators are randomly assigned to independently review each case.
 - a) Both adjudicators will independently login to the PREVENTABLE website, review all available data, and enter their final classification (Probable Dementia, MCI, or No Impairment). For MCI classifications, the adjudicator will note an MCI sub-type of amnesic-single domain, amnesic-multi-domain, non-amnesic single domain, non-amnesic-multi-domain or cannot classify.
 - b) If both adjudicators independently agree, their classification will be recorded as the final adjudication classification for the participant.
 - c) In instances where adjudicators disagree, the case will be reviewed during a regularly scheduled consensus meeting. The case will be presented to the Adjudication Panel by the 2 primary reviewers and then discussed by the entire panel until consensus (or a majority) is reached. The panel’s decision will be recorded as the final adjudication classification.

Below is a flowchart of the adjudication process.



16.2.1 CRITERIA FOR PROBABLE DEMENTIA

To identify cases of dementia, the Adjudication Panel will use these criteria recommended by the National Institute on Aging and the Alzheimer's Association:

- Significant decline in 2 cognitive domains; and
- Significant impairment in social or occupational functioning due to cognitive problems; and
- The observed/measured cognitive impairment represents a decline from previous level of functioning (or significantly below normative levels); and
- Cognitive and functional impairments are not explained by delirium, other medical conditions, or major psychiatric disorders.

NOTE: Dementia subtype is not classified in this study.

16.2.2 CRITERIA FOR MILD COGNITIVE IMPAIRMENT (MCI)

In PREVENTABLE, MCI is a secondary outcome. This outcome is important because it represents a transition between no cognitive impairment and probable dementia and is a key risk factor for dementia. Specific subtypes of MCI are highly predictive of subsequent incident dementia. Thus, identifying MCI will provide valuable information about whether sub-clinical cognitive impairment is related to the PREVENTABLE intervention. To identify cases of MCI, the Adjudication Panel will use these criteria recommended by the National Institute on Aging and Alzheimer's Association:

- Significant decline in 1 or more cognitive domain; and
- Cognitive deficits do not cause significant impairment in social or occupational functioning; and

- The observed/measured cognitive impairment represents a decline from previous level of functioning (or significantly below normative levels); and
- The criteria for dementia have not been met currently or previously.

NOTE: MCI is sub-classified into 1 of 5 categories using criteria adapted from Winblad, et al.: amnesic-single domain, amnesic-multi-domain, non-amnesic single domain, non-amnesic-multi-domain and cannot classify.

16.3 POST-ADJUDICATION FOLLOW-UP AND ASSESSMENT

All participants adjudicated as MCI or no impairment will continue to receive regularly scheduled cognitive assessments and will undergo subsequent adjudication when indicated.

If a participant is adjudicated as reaching the study cognitive outcome they will no longer receive annual calls; however, assessment for persistent disability and study drug adherence, if applicable, will continue through annual calls with the participant's Trusted Contact. Calls to the participant's Trusted Contact will continue until the participant reaches the persistent ADL disability outcome or study closeout.

An alert for "Adjudication Classification – Cognitive Outcome" will be triggered in the study website by a probable dementia adjudication classification. See **Appendix 11.1** for details on the alert and how to resolve it.

The site principal investigator or clinician should use their clinical judgment about the safety of continuing the of study drug. The continuation of the study drug is a priority unless the investigator and clinician determine it is inappropriate. Primary cause of death and secondary outcomes may still be influenced by continuation of the study drug. Regardless of a cognitive outcome or continuation of the study drug, all participants will be followed for cardiovascular disease (CVD) endpoints and mortality through the end of study.

For additional guidance on follow-up for participants who have reached the study cognitive outcome, see the **Guidance Document for Site PIs and Staff if Participant Reaches the Study Cognitive Outcome** on the study website under Login > Quick Links > Documents > Study Documents and Resources > Cognitive Adjudication. Frequently Asked Questions (FAQs) about diagnosis of dementia and cognitive outcomes are available under Quick Links > Documents > Study Documents and Resources > Quick References and FAQs.

16.4 DETERMINATION OF DISABILITY

All participants are given the Katz ADL assessment at enrollment. A participant cannot enroll if they have a significant disability other than incontinence in the Katz ADL.

Participants are also given the Katz ADL assessment at each annual follow-up. If 1 or more disability is reported, the Call Center will contact the participant 3–6 months after the disability was reported to confirm the disability. If the disability persists, then the participant meets the criterion for the new persistent ADL disability outcome. The disability outcome criterion can also be met if the Trusted Contact reports a disability and it is confirmed 3–6 months later at the follow-up call.

NOTE: Participants will be given the Katz ADLs assessment until they are determined to have a persistent disability. If the participant meets the ADL disability outcome criterion, they will continue to receive annual calls and will only be administered the TICS_m and, if triggered, the subsequent Extended Cognitive Battery.

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CHAPTER 17. STUDY COMPLETION AND CLOSEOUT

Upon completion of enrollment and follow-up of all study participants, the Clinical Coordinating Center (CCC) and Data Coordinating Center (DCC) will prepare to lock the database and close out the study. Study files, including site and project history files, will be internally audited and confirmed to ensure completeness. Detailed instructions about the closeout process will be provided as the study nears completion.

The study Multiple Principal Investigators (MPIs), the National Institute on Aging (NIA), the National Institutes of Health (NIH), and the Data and Safety Monitoring Board (DSMB) reserve the right to end the study in its entirety or at specific sites at any time for these reasons:

- Study protocol is too burdensome for participants
- Recruitment of eligible participants is too low
- Unacceptably low investigator or participant adherence
- Critical change in study staff or resources at sites
- No significant result will be obtained as anticipated

17.1 PARTICIPANT NOTIFICATION

A detailed study closeout plan will be developed to support closeout activities and ensure complete and accurate data collection. The study closeout plan will include a process to notify participants that the study has finished collecting data, thank them for their participation, and provide information about their treatment assignment.

17.2 STUDY DRUG PROCEDURES

Participants will have a supply of the study drug during follow-up through the determination of study outcomes during follow-up. The last supply of the study drug mailed to each participant will be timed with the end of study and their final follow-up call. Study staff will provide each participant taking the study drug with instructions to destroy leftover study drug pills.

17.3 INVESTIGATOR CLOSEOUT PROCEDURES

At the conclusion of the study or at the close of a study site, the site investigator will be asked to provide assurances and approve the final participant records.

17.4 RECORD RETENTION

Research records must be retained for a minimum of six (6) years after the study's closure with the IRB. This means all relevant data and documentation related to human subjects research should be kept accessible during this time in the event of a regulatory inspection. All records shall be accessible for inspection and copying by authorized representatives of the regulatory authorities at reasonable times and in a reasonable manner [45CFR §46.115(b)]. Source documents, such as participants' medical records, also will be retained for at least six years after completion of the study.

17.5 CLINICALTRIALS.GOV STUDY RECORD

A description of PREVENTABLE is available on <https://clinicaltrials.gov/study/NCT04262206>, as required by US law, and is updated as necessary. The registration number for PREVENTABLE is: NCT04262206. This website can be searched at any time and does not include information that can identify participants. The study record will be updated to include a summary of the study results once the study is completed.

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CHAPTER 18. ANCILLARY STUDIES

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

18.1 ANCILLARY STUDIES COMMITTEE

The Ancillary Studies Committee (ASC) is led by 2 co-chairs and is supported by a Data Coordinating Center (DCC) facilitator. The ASC includes at least 1 representative from the DCC, the Clinical Coordinating Center (CCC), and the National Institute on Aging (NIA) Program Office.

The purpose of the ASC is to support the development of ancillary studies that enhance the scientific output of the PREVENTABLE study. The primary responsibilities of this committee are to:

- Protect the integrity of PREVENTABLE by overseeing the review and approval process of submissions for ancillary studies and providing recommendations to the SC
- Identify opportunities, synergies, and collaborations for ancillary studies
- Provide guidance to other investigators regarding study designs that best integrate with the main protocol
- Monitor participant burden and safety

18.2 ANCILLARY STUDY PROPOSAL SUBMISSION

Principal investigators (PIs) who want to propose an ancillary study should:

1. Review the Ancillary Studies Policy and Procedures document on the study website under Login > Quick Links > Documents > Committees > Ancillary Studies > Reference Documents.
2. Check the list of submitted proposals on the study website to ensure a similar study is not already under review or approved. If there is a similar study, please contact the PI to ask about potential collaborations. The list of proposals is available on the study website under Menu > Ancillary Studies.
3. If there is not a similar study under review or approved, complete the Ancillary Study Proposal Form and write a brief 2–3-page synopsis of the aims, background, and methods. The proposal form is available on the study website under Menu > Ancillary Studies > Download Proposal Form (top right blue button) and in the Documents > Committees > Ancillary Studies > Reference Documents folder.
4. Submit the study proposal and supporting documents to the ASC at least 2 weeks prior to the next scheduled meeting. There are 2 options to submit a proposal:
 - a) Complete the online form on the study website under Menu > Ancillary Studies > Submit New Proposal (top right green button)
 - b) Email Karina Kapitanovsky (kkapitan@wakehealth.edu)

18.3 ANCILLARY STUDY PROPOSAL REVIEWS

The study review process typically takes 3–4 weeks after a proposal is submitted. The review process is not complete until the ASC discusses the proposal in a committee meeting. Ad hoc ASC reviews by email may occur if needed.

NOTE: Study proposals should be submitted **at least 2 months prior** to any related grant submission deadlines to allow adequate time for study-required reviews.

After a PI submits an ancillary study proposal to the ASC:

1. The ASC co-chairs review the proposal to determine if study-related issues need to be addressed.
2. After the initial review, 2 expert reviewers on the ASC are assigned to review the proposal and present their review at an ASC meeting. There is a simultaneous operational review by lead operation team members and a PREVENTER.
3. At the ASC meeting, the committee members will discuss the study proposal for scientific merit and relevance to the PREVENTABLE study. The operational and participant feedback is assessed separately.
4. The ASC will decide to approve the proposal, request revisions, or reject the proposal. Two votes are cast, one for scientific approval and one for operational approval.
 - a) If an approved study involves additional site burden or any contact with participants, the PI will give a 10-minute presentation (approximately 5 slides) at the next Hub monthly meeting. After the meeting, each Hub Investigator will vote on supporting the study.
 - b) If an approved study requires new data or biospecimens to be collected or additional participant study activities, the proposal must also be approved by the Data and Safety Monitoring Board (DSMB).
5. Upon final approval, PIs should either submit their proposal for funding to the NIH or another funding source or begin their study within 2 years of receiving final approval.

NOTE: The ASC co-chairs will send letters at each stage of the review process to notify the PI of ASC, Hub, and DSMB decisions on their proposal, i.e., approved, revise and resubmit, or rejected.

Frequently Asked Questions (FAQs) about ancillary studies are available on the study website under Quick Links > Documents > Study Documents and Resources > Quick References and FAQs.

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CHAPTER 19. REMOTE STUDY PROCEDURES AND COVID-19 CONSIDERATIONS

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

19.1 SAFETY OVERVIEW

During the COVID-19 pandemic, patient care and clinical research practices changed dramatically and the PREVENTABLE study adapted to ensure the safety of potential participants and study staff. Telehealth (video and phone visits) for outpatient care skyrocketed due to the pandemic and is now an accepted substitute for traditional, in-person visits in many health systems. Although in-person operations have resumed, older adults may still be cautious of contact with people outside their home. This may extend to concerns about in-person research visits and a preference for telehealth visits.

PREVENTABLE is a pragmatic study that is remarkably well suited for telehealth.

- The study’s telesite can remotely screen and enroll participants.
- eConsent offers participants the option to consent with little or no face-to-face contact.
- The study drug is shipped directly to participants’ homes.
- There is very limited to no in-person follow-up. Follow-up of cognitive function and disability is done via annual phone calls from the Call Center and data collection from participants’ electronic health records (EHR).
- Retention and adherence activities, including prescription renewals, can be done remotely.

19.2 REMOTE ENROLLMENT

The original protocol was designed for the baseline visit to be conducted in person. However, a remote-ready protocol was operationalized during the COVID-19 pandemic.

Baseline Visit	At the Site	Remote Enabled
Screening and Eligibility	Yes, website, self-reported via potential participant interview and clinical EHR	Yes, website, clinical EHR, self-reported via participant interview
Consent	Available as a printed ICF or eConsent on a tablet	eConsent can be completed online or a printed ICF can be mailed to the participant and reviewed via phone
Baseline History and Trusted Contacts	Yes, website, self-reported via potential participant interview and clinical EHR	Yes, website, clinical EHR records, self-reported via participant interview
Call Center Visits	Scheduled via the study website and by phone	Scheduled via the study website and by phone

Short Physical Performance Battery (SPPB)	Yes	Skip SPPB or add to a clinic visit that occurs for another reason prior to randomization
Biorepository	Yes	Collect samples at a 'lab and leave' visit or at a clinic visit that occurs for another reason prior to randomization
Distribution of the Study Drug	Mailed directly to participants from the Pharmacy Coordinating Center (PCC)	Mailed directly to participants from the PCC

Follow Up	At the Site	Remote Enabled
Assessment of Primary and Secondary Study Outcomes	Yes, Medicare and clinical EHR	Yes, Medicare and clinical EHR
Assessment of Cognitive and Disability Outcomes	Yes, Call Center	Yes, Call Center
Collection of 3-Month Lipid Sample (Sub-study group only)	Yes	'Lab and leave' visit
Distribution of the Study Drug	Mailed directly to participants from the PCC	Mailed directly to participants from the PCC

19.3 COLLECTION OF BLOOD SAMPLES

The collection of blood samples at baseline is important but is not required for participation in the study. If a participant does not want to visit a site for a blood draw, other options to contribute to the biorepository should be discussed with the participant. For example, if a participant has an in-person visit scheduled with another provider, it may be possible for the study staff to arrange a brief visit to collect blood samples or perform the Short Physical Performance Battery (SPPB) prior to randomization.

19.4 OTHER COVID CONSIDERATIONS

COVID-19 illness among participants and the potential for COVID-19 deaths over time is anticipated. PREVENTABLE is collecting COVID-19 data from the EHR. The Death Page on the study website includes "COVID-19 (presumed or confirmed)" among the options. Biorepository samples from these participants may be tested for prior COVID-19 illness or exposure.

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CHAPTER 20. STUDY LEADERSHIP AND INTERNAL COMMUNICATIONS

NOTE: Throughout the MOP, unless otherwise noted, the “PREVENTABLE website” or the “study website” refers to the password-protected area of the [PREVENTABLE website](#) where study staff can enter information in the study database and access study documents and training materials.

20.1 INTRODUCTION

Close collaboration and timely, accurate communication among all project teams is key to the successful completion of study activities. The purpose of this chapter is to define:

- The project team structure
- Key project team roles and responsibilities
- The communication framework for the project team, including the pathway for escalating issues within and among organizations
- The content, distribution, and frequency of information shared among project teams to ensure successful completion of the study.

The frequently asked questions (FAQs) and meeting documentation described in this chapter ensure procedures are followed and information is shared consistently throughout the study. Recurring and ad hoc meetings provide a forum to discuss study operations and identify any deviations from standard study processes. Ongoing review of study documentation (e.g., EDC data, pharmacy, or Call Center issues) also helps identify deviations from established processes.

20.2 STUDY LEADERSHIP

The PREVENTABLE study leadership team includes the Executive Committee (EC) members, Steering Committee (SC) members, and other partners who are responsible for:

- Ensuring study team leaders are adequately performing their tasks
- Responding to all communications relevant to their area(s) of responsibility
- Addressing issues in a timely manner

The study also has several other committees and working groups that collaborate with study leadership, partners, and site staff to conduct the study. See **Chapter 1** for more details on PREVENTABLE study leadership and committees.

20.2.1 EXECUTIVE COMMITTEE (EC)

The grant awardees worked closely with the National Institute on Aging (NIA) to identify individuals to lead the study via a Steering Committee (SC) and an Executive Committee (EC). Contact information for EC members is available on the study website under Login> Quick Links > Committees > Executive > Members.

The EC is a subset of the larger SC and includes:

- An NIA Program Officer

- Principal Investigators (PIs) from the Clinical Coordinating Center (CCC) at Duke Clinical Research Institute (DCRI)
- PIs from the Data Coordinating Center (DCC) and Geriatric Outcomes Assessment Center at Wake Forest University School of Medicine (WFUSM)
- Key operational contacts from WFUSM and Duke

Close partnership between these groups is essential to the success of the study. The EC meets quarterly in person, alternating between Wake Forest and Duke, and twice a month virtually to discuss key study issues. Key faculty members may be included in EC meetings to report on the status of the study or to bring issues to the EC for discussion.

20.2.2 STEERING COMMITTEE (SC)

The Steering Committee (SC) provides overall leadership for the study and includes the EC, core team leaders, a participant advocate, Hub Network leaders, and the VA CC PI. The SC is co-chaired by a geriatrician and a cardiologist. Contact information for SC members is available on the study website under Quick Links > Committees > Steering > Members.

The SC developed the study design, prepared the final protocol, reviews the study forms and manual of operations, and oversees the overall conduct of the study. The SC also oversees data collection procedures to identify and correct issues and adopts changes in the study protocol as needed.

SC members who are Hub Network leaders and site network representatives are responsible for:

- Escalating operational issues from their sites to study leadership
- Identifying barriers to recruitment among their sites
- Ensuring their sites perform their contracted responsibilities

Site network representatives may also be involved in decisions about site withdrawal and replacement.

The SC meets quarterly, either in person or virtually, to share information and make study decisions based on consensus. Voting SC members are representatives from the site networks, Hub Network leaders, and PIs from the CCC and DCC.

The study subcommittees and working groups report to the SC.

20.2.3 FACULTY LEADERS AND KEY OPERATIONAL CONTACTS

Faculty leaders direct the day-to-day operations of the study within their core areas and are responsible for deliverables from their core teams. The core teams include:

- The Data Coordinating Center/Statistics, led by Walter Ambrosius
- Recruitment, Retention and Engagement, led by Bryan Batch
- Biorepository, led by Kristin Newby and Mine Cicek
- Clinical Sites, led by Karen Alexander and Jacob Joseph

The faculty leaders identify key contacts to provide operational expertise for PREVENTABLE. These team members are involved in specific study functions and may attend key meetings. Additional study staff

may report to the key contacts to support the completion of tasks and deliverables. Depending on their expertise, key contacts and supporting staff may be more involved during certain stages of the study.

The faculty leaders and key operational contacts provide regular status reports to the EC and SC.

20.2.4 MEASUREMENT PROCEDURES AND QUALITY CONTROL (MPQC) SUBCOMMITTEE

The Measurements, Procedures, and Quality Control (MPQC) subcommittee is responsible for developing and implementing quality assurance and control mechanisms for all aspects of the study conduct. The subcommittee works closely with the EC and SC to identify issues that could affect trial integrity and may require retraining and/or modifications to the protocol or Manual of Procedures (MOP).

The MPQC monitors a variety of data to identify study-wide and site-specific issues including data on:

1. Retention and adherence
2. Data validation
3. Site responsiveness to study alerts
4. Visit completion (Call Center)
5. Completion rates, quality, and results of 3-month lab assessments
6. Timeliness of lab shipments

For site-specific issues, the MPQC will discuss the issue with the appropriate site monitors to determine appropriate actions, which may include follow-up phone calls or site visits. The MPQC works with the site monitors and may review a portion of the site visit reports.

The MPQC subcommittee also generates and monitors the PREVENTABLE Action Card, a summary of the top 10–15 metrics most relevant during the current phase of the study. Metrics included in the Action Card will change over time as the study shifts from recruitment to follow up to closeout.

On a rotating quarterly basis, staff from the Call Center, ICON lab, and the Mayo Biorepository attend the MPQC meetings. The group may meet more frequently if needed to address study-wide concerns.

20.3 DATA AND SAFETY MONITORING BOARD (DSMB)

The Data Safety Monitoring Board (DSMB) includes 7 members with relevant clinical and study experience. Members who are employed by institutions participating in the study in any way or those with ongoing projects testing similar concepts are considered to have a conflict of interest.

DSMB Member	Affiliation	Email
David Reuben, MD Chair	University of California Los Angeles (UCLA)	dreuben@mednet.ucla.edu
Hermes J. Florez, MD, MPH, PhD	Medical University of South Carolina	florez@musc.edu
Philip Greenland, MD	Northwestern University	p-greenland@northwestern.edu
Scott Solomon, MD	Harvard Medical School	ssolomon@bwh.harvard.edu

John C. S. Breitner, MD, MPH	Douglas Mental Health University Institute and McGill University	john.breitner@mcgill.ca
Peter Peduzzi, PhD	Yale University	peter.peduzzi@yale.edu
Thomas G. Trivison, PhD	Hebrew SeniorLife	tgt@hsl.harvard.edu

20.4 COORDINATING CENTERS AND PROJECT COLLABORATORS

PREVENTABLE is a collaborative study managed by the Data Coordinating Center (DCC) and Geriatric Outcomes Assessment Center at Wake Forest University School of Medicine (WFUSM) and the Clinical Coordinating Center (CCC) at Duke Clinical Research Institute (DCRI).

Other collaborators include:

- Vanderbilt University Medical Center (VUMC)
- The National Patient-Centered Clinical Research Network (PCORnet)
- Veterans Affairs Coordinating Center (VA CC)
- VA Cooperative Studies Program Pharmacy Coordinating Center (VA CSP PCC)
- Mayo Clinic Biorepository
- ICON Laboratories

20.4.1 RESPONSIBILITIES OF THE COORDINATING CENTERS

Clinical Coordinating Center (DCRI)	Data Coordinating Center and Geriatric Outcomes Assessment Center (WFUSM)	VA Coordinating Center (VA Sites Only)
• Overall project leadership • Site selection (non-VA sites) • Site management and monitoring (non-VA sites) • Oversight of VA Coordinating Center sites • Site-specific status reports • sIRB coordination (with non-VA sites) • Recruitment and Retention Core (R&R) co-leadership and support for R&R efforts including participant payments and direct-to participant mailings • Contracts and payments for sub-awardees, sites, and vendors	• Database development and management • Randomization • Safety surveillance per protocol requirements • Data quality control monitoring • Quality control reports • Statistical support and analysis • Geriatric outcomes assessments and adjudication	• Site selection • Site management and monitoring • Regulatory oversight • Recruitment lists • Study queries • Data quality control monitoring • Electronic trial master file (eTMF) • Interface with VA data systems

• Vendor management • Regulatory oversight • Electronic trial master file (eTMF) • EHR data collection (non-VA sites) • Medicare data collection • National Death Index data collection	• Data and Safety Monitoring Board (DSMB) management • Study website development and maintenance • Call Center activities	
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20.4.2 STUDY COLLABORATOR ROLES

Organization	Primary Study Role
National Institute on Aging (NIA)	• Provides funds
National Heart, Lung and Blood Institute (NHLBI)	• Provides funds
Vanderbilt University Medical Center - Trial Innovation Network and Recruitment Innovation Center	• Developed and maintains the eConsent platform • Created recruitment videos
Participant Advisory Group (PREVENTERS)	• Reviewed the protocol and participant materials • Advise on study activities that involve participants
VA Cooperative Studies Program Pharmacy Coordinating Center (VA CSP PCC)	• Manufactures the placebo • Packages and mails the study drug to participants
ICON Laboratories	• Provides supplies for sample collection (except biorepository cryovials) • Analyzes blood samples for lipid panels
Mayo Clinic Biorepository	• Processes and stores biospecimens • Participates in the Biomarker Committee
TransPerfect	• Translates participant-facing documents into Spanish
Checkbook, Inc.	• Sends pre-specified payments directly to participants

20.5 PREVENTABLE OPERATIONS TEAM STRUCTURE AND ESCALATION PATHWAY

The structure of the PREVENTABLE Project Team and Escalation Pathway is provided below and on the study website under Quick Links > Documents > Internal Operations > Coordinating Centers > Resources and Materials. Any issues identified by a specific person or team can be escalated to the team member(s) listed at the level above them. Information about the structure of the Hubs is available in **Chapter 1**.

Each CCC, DCC, and core team is responsible for resolving day-to-day issues within their organizations. Only those issues that cannot be resolved internally are escalated to the next level. **Issues that may have an impact on the study timeline, budget, or regulatory/GCP compliance will always be escalated to the key faculty leaders.**

Study issues should be communicated to the appropriate study team members as follows:

- **Urgent issues:** email all members of the EC
- **Non-urgent issues:** email the CCC PL who will forward the issue to the EC members as needed
- **Data issues:** email the WFUSM DCC team
- **Clinical issues:** email the DCRI CCC team

All study issues should be documented according to each institution's standard operating procedures (SOPs) (e.g., in the CTMS). Significant findings may be included in the next version of the MOP, at the discretion of the organization. If it is unclear if an issue should be included in the Site FAQs, the question should be raised to the next level for a final decision.

20.6 STUDY DIRECTORY

The DCC maintains a directory of study contacts on the study website under Quick Links > Directory. The directory includes a quick reference "Who to Contact" tab as well as tabs for Hub Contacts, Clinical Sites, Administrative Sites, and Site Contacts. Study staff and site management teams should ensure the directory information is kept current and staff who leave the study are removed.

20.7 PROCEDURES FOR STUDY COMMUNICATION AND DOCUMENTATION

Effective communication between and within operational groups and study site teams is key to the success of PREVENTABLE.

20.7.1 GENERAL PROJECT TEAM COMMUNICATIONS

General communication includes any questions, comments, or information that may apply to the entire project team. Examples include non-site-specific questions about the central lab, study visits, protocol, or the database on the study website.

Recommendations for general communication:

- Email questions, comments, or information to the primary operational contact for the related institution identified in the **Project Team Structure and Escalation Pathway**.
- Copy the primary operational contacts from **all institutions** on the email.

- This information can be discussed during regular meetings or more urgently via email or phone if deemed necessary by faculty leadership.

Information that could benefit project teams or study staff outside the discussion should be shared in multiple ways, including:

- Standing operational meetings
- Regular 1:1 calls between Clinical Research Associates (CRAs) and site staff or All-Sites calls
- A monthly site newsletter emailed to all study partners and sites
- A semi-annual participant newsletter included in study drug shipments or mailed from sites directly to participants

20.7.2 ORGANIZATION-SPECIFIC COMMUNICATIONS

Organization-specific communication includes any questions, comments, or information specific to a single organization. Examples include site-specific issues with study startup, study conduct, or vendor performance.

Recommendations for organization-specific communication:

- Email questions, comments, or information to the primary operational contacts in the relevant area(s). Primary contacts are identified with an asterisk in the **Project Team Structure and Escalation Pathway**.
- If a question, comment, or information could have a study-wide impact, copy all the primary operational contacts on the email. The primary contacts can determine if the information could impact overall study operations and should be shared more broadly and/or escalated to the faculty leadership level. This information sharing can take place during regular meetings or via email or phone if the issue is urgent.

20.7.3 SHARING DOCUMENTS

PREVENTABLE documents are shared with project teams and sites via Duke Box and the password-protected area of the study website. Study leadership determines which documents are shared on Box or the website.

Folders on Duke Box allow teams to collaborate on documents and share materials for meetings. WFUSM manages the public-facing and password-protected areas of the study website with content support from the DCRI.

20.7.4 NAMING DOCUMENTS AND VERSION CONTROL

Project plans, manuals, guidelines, and other study-related documents should be consistently labeled with the name, version number, and date. Significant document revisions require a new version number. Minor, administrative revisions may be documented in a note to file.

Draft versions of project documents (not site documents) should be numbered with a fraction (e.g., Version 0.1, Version 0.2) and the first final version of a document should be labeled with a whole number (e.g., Version 1.0, Version 2.0). Subsequent draft versions after a final version is approved should use fractions after the whole number (e.g., Version 1.0 is to be corrected so the team develops Version 1.1, 1.2, 1.3, etc. until a new final version is approved as Version 2.0).

20.7.5 MEETING SCHEDULES, AGENDAS, AND MINUTES

In-person and virtual meetings are held regularly for team members to provide status updates, discuss critical issues, and track study progress and timelines. The team leading the meeting is responsible for sending the meeting invitations, agendas, minutes, and action items to all attendees. Meeting agendas should be sent to all attendees at least 1 business day before the meeting. Meeting minutes and action items should be sent to all attendees for review and comments no later than 5 business days after the meeting. Final minutes will be posted on Duke Box or the study website and saved in the eTMF.

Recurring meeting schedules may be adjusted throughout the study. Ad hoc meetings may be scheduled to discuss specific issues. It is essential that the DCC and CCC share information about ad hoc meetings and escalate issues to study leadership when appropriate. For example, if the CCC schedules an urgent call with a site to discuss good clinical practice (GCP) concerns, the CCC would inform the DCC to determine if there any data issues at the site. If needed, the issue could be escalated to the executive leadership team and/or to the appropriate site organization (i.e., DCRI, VA CC, or the Hubs Network).

Recurring Meeting	Attendees	Frequency	Organizer
Executive Committee	DCC and CCC faculty, project leads (PLs), and NIH program office (PO)	First and third Tuesdays of each month, 8:00 AM ET	CCC
Steering Committee	Hub PLs and core team leads, CCC and DCC operations leadership, NIH scientific officers, and other key representatives	Quarterly, the second Wednesday of the month, with the Hubs 5:00–6:00 PM ET	CCC
Operational Leadership Team	PLs and operational teams from DCC, CCC, and VA CC	Weekly on Wednesdays, 10:00–11:00 AM ET	CCC
Internal Project Teams	Internal staff assigned to the study	Scheduled by each institution	Institution's internal team
Data and Safety Monitoring Board (DSMB)	DSMB members; National Institute on Aging (NIA); National Heart, Lung, and Blood Institute (NHLBI); DCC; and CCC	Per DSMB charter: for protocol reviews and semi-annually during data collection	DCC
Topic-Specific Meetings for Study Operations (e.g., biorepository, pharmacy, etc.)	DCC, CCC, and VA CC PLs and the operational team from the appropriate partner(s)	Approximately monthly, led by the DCC, CCC, and/or VA CC but all 3 are invited to each meeting	VA CSP PCC: DCC VA sites: VA CC Non-VA sites: CCC Biorepository: CCC ICON Lab: CCC

Core Teams and Subcommittee Working Groups	Core leadership and key operational staff	Determined by the working group, led by DCC or CCC but both are invited to each meeting	Recruitment and Retention Core: CCC Geriatric Assessment and Adjudication: DCC Claims-based Assessment: CCC Ancillary Studies: CCC Biorepository Working Group: CCC
Study Training and Updates (including All-Sites Calls)	Study teams at participating sites	As needed	CCC, DCC, or site organization (VA CC or DCRI)
Participant Advisory Group (PAG or PREVENTERS)	Recruitment and Retention (R&R) leadership, PREVENTERS	Quarterly	R&R Core, Renee Leverty is lead
Individual Hub Calls	Hub Investigator and Coordinator, PI and coordinators from each site in the Hub, DCC and CCC others as needed	TBD by each Hub	Hub Coordinators
Hub Investigator / Study Coordinator Calls	Hub Investigators, Hub Coordinators, CCC, DCC, and others as needed	Second Wednesday of each month, 5:00 PM ET	R&R PL at CCC

20.8 FAQs FOR SITES AND FAQs FOR PARTICIPANTS

Frequently asked questions (FAQs) help identify issues that may warrant a change in study operations, including revisions to the protocol or a specific project plan. Any issues that significantly impact study operations are escalated to the key faculty leaders for discussion. The operational team discusses issues during their topic-specific meetings and comes to consensus on appropriate responses.

The operational team maintains an FAQs document for sites and an FAQs document for participants to ensure common questions are answered consistently across sites. Information from the FAQ documents may be shared during monthly All-Sites calls or included in site newsletters to ensure responses are consistent across sites. The final FAQs will be uploaded to the eTMF at the end of the study.

The FAQs for sites and related documents are available on the study website under Quick Links > Documents > Study Documents and Resources > Quick References and FAQs. The **ICON Laboratory iSite User Guide** includes ICON-specific FAQs. The user guide is available on the study website under Quick Links > Documents > Vendors > ICON Lab.

The participant FAQs document is available on the [public-facing study website](#). The participant FAQs document addresses common questions about statins and participating in the study. Sites may include the FAQs in initial mailings to potential participants to address common questions in advance of screening or to answer participants' questions during visits.

CHAPTER 21. GLOSSARY OF ACRONYMS

Acronym	Definition
ADL	Activities of Daily Living
ADPAC	Automated Data Processing Applications Coordinator (VA sites)
AE	Adverse Event
ASC	Ancillary Studies Committee
CAQ	Cohort Assessment Query
CCC	Clinical Coordinating Center
CDM	Common Data Model
CFR	Code of Federal Regulations
CIRB	Central Institutional Review Board
CMS	Centers for Medicare and Medicaid Services
CPRS	Computerized Patient Record System (VA sites)
CRA	Clinical Research Associate
DCC	Data Coordinating Center
DCRI	Duke Clinical Research Institute
DSMB	Data and Safety Monitoring Board
eCRF	Electronic Case Report Form
EDC	Electronic Data Capture
EDTA	Edetic Acid or Etheylenediaminetetraacetic Acid
EHR	Electronic Health Record
EOSI	Event(s) of Special Interest
EPL	Expected Performance Level
eTMF	Electronic Trial Master File
FDA	Food and Drug Administration
GCP	Good Clinical Practice
HRP	Human Research Protections
ICF	Informed Consent Form
ICH	International Council on Harmonisation
LSI	Local Site Investigator (VA sites)
MBI	Medicare Beneficiary Identifier
MoCA	Montreal Cognitive Assessment tool
MOP	Manual of Procedures
MPIs	Multiple Principal Investigators
MPQC	Measurement Procedures and Quality Control (Subcommittee)

NDI	National Death Index
NHLBI	National Heart, Lung, and Blood Institute
NIA	National Institute on Aging
NIH	National Institutes of Health
OHRP	Office for Human Research Protections
PAG	Participant Advisory Group (PREVENTERS)
PCC	Pharmacy Coordinating Center
PHI	Protected Health Information
PII	Personally Identifiable Information
PHQ-8	Patient Health Questionnaire-8
PI	Principal Investigator
PID	Participant Identifier
PL	Project Leader
RAP	Report-a-Problem (to the Data Coordinating Center)
RWD	Real-World Data
SAE	Serious Adverse Event
SAMS-CI	Statin-Associated Muscle Symptoms Clinical Index
SC	Study Coordinator
SC	Steering Committee
sIRB	Single Institutional (or Independent) Review Board
SOP	Standard Operating Procedure
SPPB	Short Physical Performance Battery
SSN	Social Security Number
UP	Unanticipated Problem(s)
VA	Veterans Affairs
VA CMOP	Consolidated Mail Outpatient Pharmacy
VA CSP	Veterans Affairs Cooperative Studies Program
VHA	Veterans Health Administration
WFUSM	Wake Forest University School of Medicine

CHAPTER 22. REVISION HISTORY

The PREVENTABLE Manual of Procedures (MOP) will be revised as needed, including when changes to operational practices or changes in the scope of work occur. The Clinical Coordinating Center is responsible for distributing updated versions and addendums.

The MOP is maintained on the password-protected area of the [PREVENTABLE website](#) under Login > Quick Links > Documents > Study Documents and Resources > MOP so sites always have access to the current version.

Version 1	Chapter	Date of Revision	Summary of Revision
1.0	All	07.13.2020	Initial Final Version
1.1	3	09.03.2020	Added link for Storefront
	4: 4.9.1	09.03.2020	Added section to provide additional information about the 2 components of the CAQ that sites will receive
	5: 5.4.1	09.03.2020	Clarification that Katz Index of Independence in ADL tool is embedded in Eligibility Form
	5: 5.5 and Appendix 1	09.03.2020	Additional information about and clarification of SPPB process
	6	09.03.2020	Updates throughout chapter to provide additional detail for sample processing
	18	09.03.2020	Updated ASC process, form, and FAQ
	19	09.03.2020	Wordsmithing of “telehealth” and “virtual”
	20	09.03.2020	Helpline number added
	21	09.03.2020	Updated details on process
	23	09.03.2020	Updated Revision History

Version 2	Chapter	Date of Revision	Summary of Revision
2.0	2	09.25.2020	Minor text clarifications
	4	09.25.2020	Minor text clarifications and screen shots added
	4: 4.3	09.25.2020	Clarification of statin use inclusion criteria

	5	09.25.2020	Minor text clarifications; section 5.3 Physical Measurements removed
	6	09.25.2020	Text clarifications; page numbers added
	7	09.25.2020	Details added for scheduling calls with Call Center, CC scripts and procedures; screen shots added
	9	09.25.2020	Minor text clarifications and screen shots added
	10	09.25.2020	Additional details added; Appendix 2 added
	10: 10.3	09.25.2020	Section header changed to better reflect section content
	11	09.25.2020	Minor text clarifications, page #s and screen shots added
	12	09.25.2020	Details added for website navigation; screen shots added; deleted section on website features not yet available
	18	09.25.2020	Updated FAQ #4 now that enrollment has begun
	21, Section 21.12	09.25.2020	Text added to clarify role of site representatives to Steering Committee
2.1	23	11.10.2020	Added text to describe MOP update process
	4, Section 4.1	11.11.2020	Added text for consent required prior to entering PHI in database
	4, Section 4.3	11.11.2020	Clarified process if potential participant plans to live outside the US during study

Version 3	Chapter	Date of Revision	Summary of Revision
3.0	7, Section 7.2	11.17.2020	Added text to describe distribution of participant handouts for virtual visits
	7, Section 7.5.3	11.17.2020	Added text describing the PHQ-8 Alert process
	4, Section 4.1.2	12.04.2020	Updated number of days required for form updates from 30 to 45 for re-screening

	6, Section 6.3.1	12.04.2020	Updated number 14 to include instructions regarding scanning cryovials into the Blood Collection Form
	6, Section 6.3.1	12.04.2020	Updated number 15 to indicate shipments to ICON will be completed approximately every 3 months
	6, Section 6.3.2.3	12.04.2020	Updated number 2 with instructions for navigating to the manifest for Mayo
	6, Section 6.3.2.4	12.04.2020	Updated instructions for generating the manifest for ICON shipments
	6, Section 6.2.1.3	12.04.2020	Updated instructions for collecting buffy coat
3.1	6, Section 6.6	12.21.2020	Added section 6.6
	6, Section 6.2.1.3	12.21.2020	Added an image of the Biorepository cryovial tube and ICON Cryo Box
	6, Section 6.1.4	12.21.2020	Added Barcode Scanner Instructions
	6, Section 6.1.4	1.22.2021	Added additional Barcode Scanner Instructions
	6, Section 3.2.4	1.22.2021	Updated ICON shipping instructions

Version 4	Chapter	Date of Revision	Summary of Revision
4.0	1, Section 1.4.8	2.17.2021	Added section and charge for MPQC
	3 TOC	2.17.2021	Added R&R Playbook
	3, Section 3.3	2.17.2021	Updated that Spanish Version of forms are also available
	3, Section 3.3.1	2.17.2021	Added Enrolled Participant Handout description Added Call Center Brochure description
	3, Section 3.5	2.17.2021	Added Recruitment and Retention Playbook description
	4, Section 4.5.1	2.17.2021	Added instructions about creating a note to file if the PDF has an undecipherable signature

	4, Section 4.5.6	2.17.2021	Updated to indicate consent must be entered before any additional data entry is available
	4, Section 4.7	2.17.2021	Updated to indicate consent must be entered before any additional data entry is available
	4, Section 4.7.2	2.17.2021	Added information about swapping addresses on the Contact Information Form if participant has two mailing addresses (i.e., snowbirds)
	5, Appendix 1	2.17.2021	Removed “less than” to just 10 seconds for the semi-tandem stand
	7	2.17.2021	Minor grammatical and formatting edits throughout chapter
	7, Section 7.2.1	2.17.2021	Updated instructions for site rescheduling participant baseline call, including new screen shot
	7, Section 7.2.2	2.17.2021	Updated instructions for baseline call if clinic does not schedule the call at time of clinic visit
	7, Section 7.2.3	2.17.2021	Updated instructions for clinics if they receive an email alert that either the Call Center has been unable to contact the participant or participant has refused to complete baseline phone call
	7, Section 7.5.3	2.17.2021	Updated instructions for how clinic staff are alerted of elevated PHQ-8 score
	7, Section 7.6	2.17.2021	Updated procedure and information on action needed for clinics if Call Center is unable to get in contact with all Trusted Contact(s) identified on contact information form, including new instructions and screen shots
	9, Section 9.2	2.17.2021	Added statement that the baseline call must be completed within 45 days of the baseline call. If completed post 45 days, the site will need to review Eligibility Form and update date of visit prior to participant being randomized.
	9, Section 9.2	2.17.2021	Added statement that the mailing address on the contact information must be validated
	9, Section 9.3	2.17.2021	Added statement that the automatic randomization process requires the Consent

			Form to be uploaded (for non-VA sites only) and the Eligibility Form to be completed no more than 45 days before the date of the baseline call, otherwise the randomization process will not be completed.
	9, Section 9.5	2.17.2021	Added instructions if participant is enrolled but not randomized and becomes ineligible prior to randomization
	10 TOC	2.17.2021	Added Contact Information 10.2.1., Study Drug Disposal 10.5.2., and Appendix 3 Study Drug Handout
	10, Section 10.2.1	2.17.2021	Added Contact Information section describing toggle button feature and snowbird indicator addition. Added information for non-VA sites regarding the VA CSP using the information from the main database to verify consent.
	10, Section 10.2.1	2.17.2021	Updated initial shipment details
	10, Section 10.2.2	2.17.2021	Added Study Drug Handout Information, picture of the study drug bottle label, and easy open cap and pillbox information
	10, Section 10.5.2	2.17.2021	Changed length of time for documenting study drug discontinuation from 14 days to 30 days
	10, Section 10.5.2	2.17.2021	Added statement that non-adherence to the study drug does not mean that the participant is not retained. A participant can be full retained without being adherent to study drug.
	10, Section 10.5.3.	2.17.2021	Added Study Drug Disposal
	10, Appendix 3	2.17.2021	Added photo of Study Drug Handout
	11, Section 11.3.2	2.17.2021	Updated directions for documenting missed visit
	11 Section 11.4.3	2.17.2021	Removed Retention Status screen shot, will be replaced with updated screen shot in later version of MOP

	11, Section 11.4.3	2.17.2021	Added statement that non-adherence to the study drug does not mean that participant is not retained. A participant can be full retained without being adherent to the study drug.
	11, Section 6	2.17.2021	Updated retention status choices and directions to site
	11, Section 11.6.3	2.17.2021	Updated instructions for Clinic completing Death Form
	12, Section 12.2.4	2.17.2021	Added screenshot for RAP and updated instructions
	14, TOC	2.17.2021	Added 14.3 Safety Alerts, 14.3.1 PHQ-8 Alert, moved Data and Safety Monitoring board to 14.4
	14, Section 14.3.1	2.17.2021	Added information regarding PHQ-8 Alerts
	16	2.17.2021	Minor grammatical changes
	21, TOC	2.17.2021	Added section 21.13 MPQC
	21, Section 21.13	2.17.2021	Added MPQC Charge detailed information
	11, Section 11.6	2.23.2021	Updated partial withdrawal of consent
	13, Section 7	2.25.2021	Added protocol deviation log
	4, Section 5.2	3.3.2021	Added VA remote consenting procedures
	10, Section 6.2	3.29.2021	Updated unintentional unblinding of participant
4.0	10, TOC	4.16.2021	Added Sections 10.6, 10.6.1, 10.6.2, 10.6.3, 10.6.4, 10.6.5
	10, Section 10.6	4.16.2021	Added description of the Study Drug Order Action Needed alert system Added description of the Smart Action Options
	10, Section 10.6.1	4.16.2021	Added screen capture of Action Needed Alert
	10, Section 10.6.1	4.16.2021	Added the instructions and smart action options regarding the Pending Action Needed Alert for Non-VA sites only

	10, Section 10.6.2	4.16.2021	Added the instructions and smart action options regarding the Outstanding Action Needed Alert
	10, Section 10.6.3	4.16.2021	Added the instructions and smart action options regarding the Cancelled Action Needed Alert
	10, Section 10.6.4	4.16.2021	Added the instructions and smart action options regarding the Shipment Action Needed Alert
	10, Section 10.6.5	4.16.2021	Added the instructions and smart action options regarding the Expected Actions Needed Alert for VA sites only
	10, Section 10.6.3	4.22.2021	Added information for VA sites only on resolving a Cancelled Action Needed Alert
4.1	11, Section 11.3.1	4.19.2021	Changed visit window for 3-month lab draw to 60-180 days
	11, Section 11.4	04.19.2021	Added note with directions about how to handle a participant who is enrolled but not randomized and does not want to continue in study
	11, Section 11.4.3	04.19.2021	Added new section on Central Comments System, changed subsequent section numbering as a result.
	14, Section 14.1.2	04.19.2021	Removed references to Unanticipated Problem Reporting System
	4, Section 4.7	05.28.2021	Added paragraph on how sites can delay enrolling a participant up to 30 days from consent to allow for SSPB and lab samples to be collected prior to enrollment
	1	6.01.2021	Removed references to Hawthorne Effect throughout Chapter 1
	11	06.01.2021	Removed references to Hawthorne Effect throughout Chapter 11
	11, Section 11.4.4	06.01.2021	Added screen shots and directions for updating retention status
	11, Section 11.6.3	06.01.2021	Updated directions for how Call Center notifies site of participant death

	19	06.01.2021	Removed references to Hawthorne Effect throughout Chapter 19
	21	06.01.2021	Removed references to Hawthorne Effect throughout Chapter 21
	11, Section 11.6.3	06.21.2021	Updated details for completing Death Form
	4, Section 4.5.1	09.20.2021	Added paragraph on how to make requests if eConsent system assistance is needed
	6	09.20.2021	Updated main contact for Mayo Clinic Biorepository
	6	10.11.2021	Updated main contact for ICON Laboratory Services; Added Figure 4 – Biorepository Rack

Version 5	Chapter	Date of Revision	Summary of Revision
5.0	1, Section 1.4.2	4.18.2022	Added details about Hub structure in Steering Committee section
	1, Section 1.4.9	4.18.2022	Removed reference to Recruitment & Retention Subcommittee that was sunset
	1, Section 1.4.10.1	4.18.2022	Added details about Hub structure
	2, Section 2.6	4.18.2022	Minor grammatical edits and added information about payments and invoicing for postage
	3, Section 3.1	4.18.2022	Removed Dan Munoz from R&R Core; included information about Hub interactions
	3, Section 3.2	4.18.2022	Added information about multi-modal touch
	3, Section 3.3	4.18.2022	Table: Added suggestion to include FAQ and brochure in initial outreach letters; added details on Clinician Referral Tools, greeting cards and participant newsletters
	3, Section 3.4	4.18.2022	Added Recruitment Materials section

	3, Sections 3.3.2, 3.4, 3.5	4.18.2022	Minor grammatical edits
	4, Section 4.1	4.18.2022	Added “or by asking the participant the prescreening questions and reviewing their current medications”
	4, Section 4.1.1	4.18.2022	Expanded list of ways potential participants can be contacted
	4, Section 4.2	4.18.2022	Added primary language of English or Spanish to inclusion criteria
	4, Section 4.3	4.18.2022	Clarified dementia exclusion criteria -- <i>This could be indicated by taking any of the following prescription medications (donepezil, galantamine, rivastigmine, memantine), however individuals without dementia may also be taking these medications and a prescription for any of these medications is not an exclusion. A diagnosis or clinical evidence of dementia should be confirmed by review of the medical records.)</i>
	4, Section 4.3	4.18.2022	Added “unable to talk” to exclusion criteria
	4, Section 4.3	4.18.2022	Clarified active liver disease exclusion criteria is based on clinician determination
	4, Section 4.3	4.18.2022	Updated procedures if participant moves out of country, phone follow-up will be discontinued, but we will attempt to complete the Dementia Questionnaire with the participant if possible
	4, Section 4.4	4.18.2022	Removed Eligibility Worksheet and added it as Appendix 4.2
	4, Section 4.4	4.18.2022	Clarified that sites can confirm participant eligibility based on participant self-report or review of the medical record
	4, Section 4.4	4.18.2022	Clarified that if a participant refuses the baseline phone call, the Call Center will document this, the participant will be marked as ineligible for randomization, and the clinic will receive an alert to respond after they have followed up with the participant

	4, Section 4.4	4.18.2022	Added sentence “Research records of participants who screen failure should be maintained in the investigator site file and per local policies.”
	4, Section 4.4	4.18.2022	Removed sentence, “Therefore, a site may opt to phone screen potentially eligible participants.”
	4, Section 4.5	4.18.2022	Clarified that local site processes should be followed if the informed consent process is conducted via mail
	4, Section 4.5.1	4.18.2022	Added that if the participant signs with a name other than their full legal name, a Note to File should be documented
	4, Section 4.5.1	4.18.2022	Added NOTE: Local electronic consent platforms may be used in lieu of the Vanderbilt REDCap system as long as those systems conform to standard processes to ensure the participants are consented properly. In addition, a copy of the signed consent form must be uploaded into the PREVENTABLE website to allow dispensation of study drug.
	4, Section 4.5.2	4.18.2022	Updated section to include instructions for consent options for VA: ORD-approved DocuSign, RMS Encryption, and MyHealtheVet
	4, Section 4.5.4	4.18.2022	Renamed section title from “Paper and VA consent” to “Paper consent”
	4, Section 4.7	4.18.2022	Clarified that SPPB form only appears if there is a certified staff member associated with the site
	4, Section 4.7	4.18.2022	Clarified that sites can delay “enrollment” of an eligible participant by waiting to click the “Enroll Participant” button
	4, Section 4.7	4.18.2022	Added sentence, “Even if a participant has only completed the informed consent and no longer proceeds with any additional forms, the PID cannot be used for any other participants.”
	4, Section 4.7.1	4.18.2022	Clarified that the Eligibility Form should be entered into the database for all participants whom site staff contacts and consents

	4, Section 4.7.1	4.18.2022	Added sentence, “No PHI can be entered into the system until the consent is signed and documented in the system.”
	4, Section 4.7.1	4.18.2022	Clarified dementia exclusion criteria
	4, Section 4.7.3	4.18.2022	Added section on what to do if participant declines to proceed to enrollment
	4, Section 4.8.2	4.18.2022	Clarified that participant who is tentatively considered eligible and consented should have the Eligibility Form data entered in the website
	4, Section 4.8.2	4.18.2022	Removed the Screening Process Flow Chart
	4, Section 4.8.2	4.18.2022	Moved the Eligibility Worksheet to the appendix
	4, Section 4.8.4	4.18.2022	Added section on Prescreening Log entries
	4	4.18.2022	Removed screenshots and website data entry and added Appendix 4.1
	5, Section 5.2.3	4.18.2022	Added detail regarding Trusted Contact
	5, Section 5.3.1	4.18.2022	Clarification on Katz eligibility
	5, Section 5.3.2	4.18.2022	Clarification on SPPB appearing in database
	5, Section 5.4	4.18.2022	Added detail regarding SharePoint site Added clarifying detail regarding Recertification
	5, Section 5.5	4.18.2022	Updated information regarding participant handouts to include information about Trusted Contacts and Call Center
	5, Section 5.5.2	4.18.2022	Minor grammatical edits
	5, Section 5.5.3	4.18.2022	Minor grammatical edits
	5, Section 5.6	4.18.2022	Minor grammatical edits
	5, Appendix 2	4.18.2022	Added Recertification Checklist
	6	4.18.2022	Updated table of contents
	6	4.18.2022	Removed all web and database related information and entered into appendix, referenced appendix throughout chapter

	6, Section 6.1.4	4.18.2022	Added Figure 1 – Baseline Kit Contents and Figure 2 – 3-month kit contents
	6, Section 6.2.2	4.18.2022	Added section 6.2.2 Sample destruction
	6, Section 6.3	4.18.2022	Added section 6.3 Lab Supply Ordering and Shipping
	6, Section 6.4	4.18.2022	Added section 6.4 ICON Queries and Alerts with subsections 6.4.1 ICON Lab Tracking Alerts and 6.4.2 ICON Query Process Updated ICON contact number
	6, Section 6.1.3	4.18.2022	Updated ICON iSite information
	6, Section 6.1.4	4.18.2022	Updated ICON and Mayo Biorepository Lab Supplies instructions
	6, Section 6.2.1.2	4.18.2022	Updated Collection Specimen Tube description
	6, Section 6.2.3	4.18.2022	Added 6.2.3 Randomized 3-Month Visit with instructions
	6, Section 6.2	4.18.2022	Updated figure 5 instructions
	6, Section 6.3.2.4	4.18.2022	Updated Shipping Frozen Samples to ICON instructions
	6, Section 6.4.2	4.18.2022	Updated ICONs query process instructions
	7, Section 7.2	4.18.2022	Updated information regarding participant handouts to include information about Trusted Contacts and Call Center. Added information regarding text and email messages.
	7, Section 7.4	4.18.2022	Updated information regarding annual follow-up call
	7, Section 7.5.5	4.18.2022	Updated information regarding annual follow-up call
	7, Section 7.6	4.18.2022	Updated information regarding Trusted Contact follow-up
	9	4.18.2022	Removed references to “in-person” clinic baseline visit

	9, Section 9.2	4.18.2022	Added information about storage of SSN and MBI numbers in the study database
	9, Section 9.3	4.18.2022	Added that an automatic email is sent to clinic staff when a participant is randomized
	9, Section 9.5	4.18.2022	Added “submit a protocol deviation” if enrollment/randomization entered incorrectly
	9	4.18.2022	Removed screenshots and website data entry and added Appendix 9.1
	10	4.18.2022	Updated table of contents
	10	4.18.2022	Removed all web and database related information and entered into appendix, referenced appendix throughout chapter
	10, Section 10.1	4.18.2022	Updated section 10.1 overview
	10, Section 10.1.1	4.18.2022	Updated section 10.1.1 regarding dosing guideline information
	10, Section 10.2.1	4.18.2022	Updated contact information feature details
	10, Section 10.2.2	4.18.2022	Updated initial orders regarding dosage ordering
	10, Section 10.3	4.18.2022	Updated section 10.3 Considerations for Study Drug Order, information regarding the contraindicated medications table, questions for considerations for study drug order/reorder
	10, Section 10.5.1	4.18.2022	Added information regarding Study Drug Status
	10, Section 10.6	4.18.2022	Removed sections 10.6.1-10.6.5
	10, Section 10.7	4.18.2022	Updated wording from unblinding to unmasking
	11	4.18.2022	Changed title of chapter to “Retention and Follow-Up”
	11, Section 11.1	4.18.2022	Added definition of retention as compared to adherence

	11, Section 11.1	4.18.2022	Removed strategies related to study drug adherence, and focused on strategies related to retention
	11, Section 11.2	4.18.2022	Added information about 3-month retention messages and reminders about upcoming annual assessments
	11, Section 11.2	4.18.2022	Added additional strategies for increasing retention among participants
	11, Section 11.2.1	4.18.2022	Added paragraph about centralized reimbursement to participants
	11, Section 11.2.2	4.18.2022	Updated so that newsletters will be sent periodically
	11, Section 11.2.2	4.18.2022	Added sentence about site's responsibility for sharing the newsletter with participants who are no longer receiving the study drug
	11, Section 11.3.1	4.18.2022	Updated window for Trusted Contact call
	11, Section 11.3.1	4.18.2022	Updated window for confirmation of disability and expanded to include Trusted Contact or participant reported a disability
	11, Section 11.3.1	4.18.2022	Updated table to reflect visit window modifications above
	11, Section 11.4	4.18.2022	Added section on 3-month lab visit
	11, Section 11.5	4.18.2022	Added "and their trusted contact"
	11, Section 11.6	4.18.2022	Added additional instructions about PI talking to participants who have concerns about continuing participation in the study
	11, Section 11.6	4.18.2022	Updated retention statuses to reflect those designated in the website
	11, Section 11.6.2	4.18.2022	Added and updated details regarding classification of deaths
	11	4.18.2022	Removed references to home visits

	11	4.18.2022	Removed references to “lost to follow-up,” participants not completing study visits can still be followed through EHR unless fully withdrawn
	11	4.18.2022	Added references to hub leadership throughout the chapter
	11	4.18.2022	Removed screenshots and website data entry and added Appendix 11.1
	12, Section 12.2.1	4.18.2022	Clarified the process for obtaining website access, “The CRA will submit a RAP notifying the DCC of the new user and the level of access requested.”
	12, Section 12.2.2	4.18.2022	Added sentence, “Users will not receive an additional email when access is granted to the training website.”
	12, Section 12.2.3	4.18.2022	Updated study roles to match designation on website: Principal Investigator, Sub-investigator, Clinic Coordinator, Pharmacy, Medications Subscriber, Lab, Other
	12, Section 12.3	4.18.2022	Removed sentence, “Clinical site staff will be expected to review hard copy CRFs for accuracy and completeness and resolve any data issues prior to data entry.”
	12, Section 12.3.2	4.18.2022	Added sentence, “If any data has been collected on a participant and a PID has been associated, the PID should never be reused for another participant.”
	12, Section 12.4	4.18.2022	Added sections on Site Management Systems and Alerts
	12, Section 12.5.1	4.18.2022	Clarified the study guidelines for storage of paper source documents
	12	4.18.2022	Removed screenshots and website data entry and added Appendix 12.1
	13, Section 13.2	4.18.2022	Changed training method in number 1 to “just in time training”
	13, Section 13.2	4.18.2022	Added number 5 for vendor and laboratory contacts

	13, Section 13.2	4.18.2022	Removed descriptions of different views of the data
	13, Section 13.5	4.18.2022	Updated paragraph 3 to indicate monitoring is completed per the CMP
	13, Section 13.5	4.18.2022	Changed wording to indicate that site visits may be onsite or remote
	13	4.18.2022	Added references to hub leadership throughout the chapter
	13	4.18.2022	Removed screenshots and website data entry and added Appendix 13.1
	13	4.18.2022	Added Appendix 13.2, Protocol Deviation Log
	14	4.18.2022	Updated overview to be consistent with the protocol in relation to monitoring conducted by the DSMB
	14, Section 14.1.2	4.18.2022	Split out the requirements for non-VA and VA sites and clarified requirements for each
	14, Section 14.3.1	4.18.2022	Updated description about method of PHQ-8 alert
	14	4.18.2022	Removed screenshots and website data entry and added Appendix 14.1
	15, Section 15.4	4.18.2022	Added information regarding death data from CMS
	15, Section 15.4.1	4.18.2022	Updated linkages to Medicare data table
	16	4.18.2022	Changed title to reflect primary outcome of dementia
	16, Section 16.1	4.18.2022	Removed reference to in-home/face-to-face testing
	16, Section 16.2	4.18.2022	Minor grammatical edits
	16, Section 16.3	4.18.2022	Updated Trusted Contact information to assess for disability following dementia adjudication
	16, Section 16.4	4.18.2022	Updated disability endpoint information

	18, Section 18.2	4.18.2022	Clarified process for SC approval
	18, Section 18.3.2, FAQ #2	4.18.2022	Removed outdated table with dates of calls
	18, Section 18.3.2, FAQ #8	4.18.2022	Added that CCC and DCC staff should be consultant during protocol development
	19	4.18.2022	Removed references to home visits
	19, Section 19.4	4.18.2022	Removed references to COVID ancillary study that was not funded
	20	4.18.2022	Changed term “blind” to “mask” — <i>Ensure we change title of this chapter to “Unmasking” on overall TOC</i>
	21	4.18.2022	General updates to reflect that the website is now active and used for communications
	21, Section 21.4.3	4.18.2022	Added information about the Hub structure
	21, Section 21.5, Figure 1	4.18.2022	Replaced Figure 1 with updated version
	21, Section 21.8, Table 5	4.18.2022	Table updated with Hub leadership information
	21, Section 21.9	4.18.2022	Updated information about communication modes
	21, Section 21.10	4.18.2022	Removed figure
	21, Section 21.16	4.18.2022	Updated FAQ information
5.1	2	11.11.2022	Changed references to “VA Boston CC” to “VA CC” Updated location of materials on website
	3	11.11.2022	Added information about recruiting participants not within the site’s health system, Hubs, new location on website for materials, Participant FAQ, and Zoom Informational Sessions

	4, Section 4.2	11.11.2022	Added clarification on community-dwelling definition
	4, Section 4.2	11.11.2022	Added clarification on Spanish-speaking inclusion criteria
	4, Section 4.3	11.11.2022	Reformatted Exclusion section in order to add FAQs on: enrolling participants who are currently taking gefibrozil or fenofibrate, participants traveling or moving outside of US, memory enhancing medications, frailty and disability, and low cognitive scores on previous cognitive tests
	4, Section 4.3.1	11.11.2022	Added FAQs on exclusion criteria
	4, Section 4.5.1	11.11.2022	Added information on emailing the eConsent to potential participants
	4, Section 4.5.1	11.11.2022	Added information about participants remotely signing the eConsent
	4, Section 4.5.1	11.11.2022	Added clarification about emailing or printing of signed consent
	4, Section 4.5.1	11.11.2022	Added information about second signature line, if required locally
	4, Section 4.5.1	11.11.2022	Changed directions for how to find eConsent training docs in documents area of website
	4, Section 4.7.1	11.11.2022	Added clarifying information on eligibility to participate if there is some mild cognitive impairment or mild memory loss
	4, Section 4.7.2	11.11.2022	Added clarification on SSN necessity and security
	5, Section 5.5.3	11.11.2022	Removed references to Participant FAQ
	5, Section 5.5.1	11.11.2022	Updated information on where to find the enrolled participant handouts and materials in the Documents area of the website
	5, Section 5.2.4	11.11.2022	Added section 5.2.4 on Lab/Biorepository samples and included information from the participant FAQ
	6, Section 6.1.2	11.11.2022	Added ICON Laboratory Manual information

	6, Section 6.1.4	11.11.2022	Updated alerts information of the PREVENTABLE website; Added ICON Laboratory Manual as Appendix 6.2
	6, Section 6.3	11.11.2022	Updated study alias information
	6, Section 6.3.5	11.11.2022	Removed ICON lab manual information and moved to appendix
	6, Section 6.3.6	11.11.2022	Removed ICON lab manual information and moved to appendix
	6, Section 6.4.2	11.11.2022	Updated information regarding ICON query process
	7, Section 1	11.11.2022	Added information about what to do if a participant develops dementia during the study
	7, Section 7.2	11.11.2022	Updated information on where to find enrolled participant handout and materials in the Documents area of the website
	7, Section 7.4	11.11.2022	Added information on length of time for annual call center assessments from participant FAQ; Added new COVID question
	7, Section 7.5.3	11.11.2022	Added details on site notification/alert of high PHQ score and where to locate template letter to notify PCP of high PHQ score
	9, Section 9.3	11.11.2022	Added sentence that upon randomization, the study drug ordering system is updated so the site can monitor when study drug is delivered
	10, Section 10.1	11.11.2022	Added that the PCC can ship only to US territories and why atorvastatin 40mg was selected as the test article; neither drug contains peanut or peanut derivatives and can be considered gluten free
	10, Section 10.1.1	11.11.2022	Added additional information regarding other medication prescribed along with atorvastatin
	10, Section 10.1.2	11.11.2022	Added section on information regarding the placebo and its contents
	10, Section 10.1.3	11.11.2022	Added section on the timing of taking study drug

	10, Section 10.2.2	11.11.2022	Added that the PCC can ship only to US territories
	10, Section 10.3	11.11.2022	Added information regarding there being no interaction between the study drug and acetaminophen and other medications and recommendation to stop study drug for 1-2 weeks while taking Paxlovid for COVID-19
	10, Section 10.4	11.11.2022	Added information regarding CoQ10, grapefruit and statin levels as well as no requirement for annual LFTs
	10, Section 10.5	11.11.2022	Added additional information about dosing due to side effects and discontinuing study drug
	10, Section 10.5.1	11.11.2022	Added information on dosing for participants with difficulty swallowing
	10, Section 10.5.2	11.11.2022	Updated from 90 to 14 on the number of days a participant is inactive on study drug that needs to be reported
	10, Section 10.7.2	11.11.2022	Clarified plan if a participant is unintentionally unmasked and that all participants will be notified of their study arm at the end of the trial
	10, Appendix 10.1, Section 10.1a	11.11.2022	Added that the PCC can ship only to US territories
	10, Appendix 10.1, Section 10.1b	11.11.2022	Added Study Drug Tool update regarding Study Drug Renewal Confirmation
	10, Appendix 10.1, Section 10.1c	11.11.2022	Added Figure 10.1.3. Screen Capture of Participant's Tab in the Study Drug Tool
	11, Section 11.5	11.11.2022	Added clarifying sentence regarding process if participant develops dementia during the study and continuing study drug when a participant moves to a nursing home
	11, Section 11.6	11.11.2022	Added instructions if the participant agrees to a modified follow-up plan, but does not want the Call Center to call their Trusted Contact(s)

	11, Section 11.6.2	11.11.2022	Added sentence clarifying that if a participant dies, this does not impact retention status. Study drug status is automatically updated upon entry of the Death Form.
	12, Section 12.1.3	11.11.2022	Added clarification on the purpose of “roles” within the study website. Added role and definition for “Primary Coordinator” and added clarification on only one Medical Prescriber role per site.
	14	11.11.2022	Added sentence clarifying that deaths and hospitalizations are study outcomes, not unanticipated events
	14, Section 14.3.1	11.11.2022	Added updated information on PHQ-8 alerts and reports and clarified action needed related to notification to participants’ providers or the participants
	14, Appendix 14.1, Section 14.1a	11.11.2022	Update path to get to the High PHQ Score PCP Letter Template
	14, Appendix 14.1, Section 14.1a	11.11.2022	Clarified action needed related to notification to participants, if they did not give permission to contact their PCP
	16, Section 16.2	11.11.2022	Added information about notification of adjudication of dementia and continuing to follow for secondary endpoints
	16, Section 16.3	11.11.2022	Added information about post-dementia adjudication assessment through Year 4/closeout
	16, Section 16.4	11.11.2022	Added information about post-disability determination assessment through Year 4/closeout
	21	11.11.2022	Updated meeting frequencies Changed references to “VA Boston CC” or “Boston VA CC” to “VA CC” Added information about Hub calls

Version 6	Chapter	Date of Revision	Summary of Revision
6.0	All	5.30.2025	Version 6.0 of the MOP includes significant edits to all chapters to align with current study procedures
	7, Section 7.7.7	6.10.2025	Removed the paragraph about procedures for notifying the site PI and primary care provider about a participant's symptoms of depression on the PHQ-8 questionnaire
	4, Section 4.2.1; Appendix 4.2, Section 4.2b	6.25.2025	Changed text specifying two trusted contacts to "a trusted contact" and "at least 1 trusted contact" to align with the protocol